



REDE D'OR

Constellation Challenge 2025

押忍
Oss Capital



Bernardo Godoy | Eduardo Braga

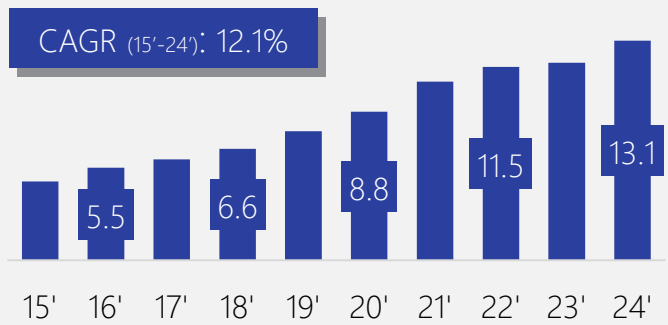


Rede D'Or at a Glance

The golden company of Brazilian healthcare

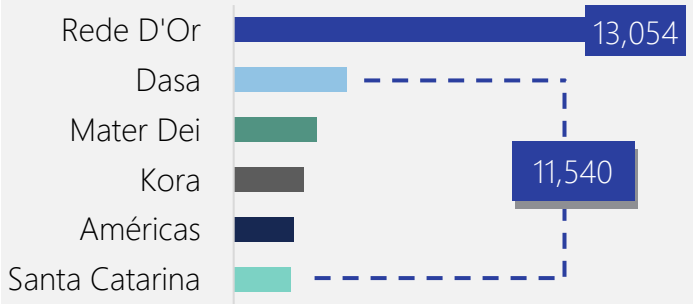
Rede D'Or is the largest player in hospitals...

Total beds evolution [th]



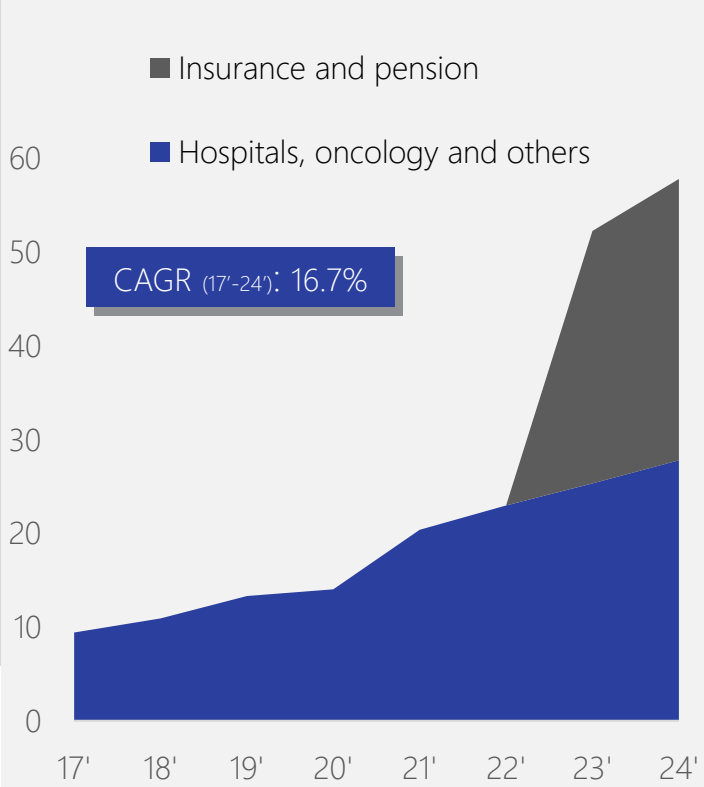
...even larger than the other top five players combined...

Total beds EoP [#]



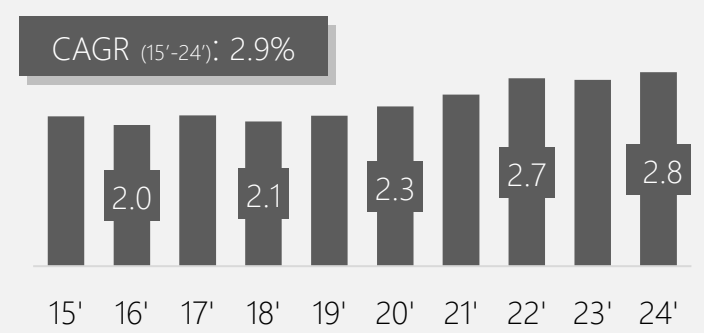
...with strong revenue growth and a new stream accelerating performance...

Net Revenue Evolution [BRL bn]

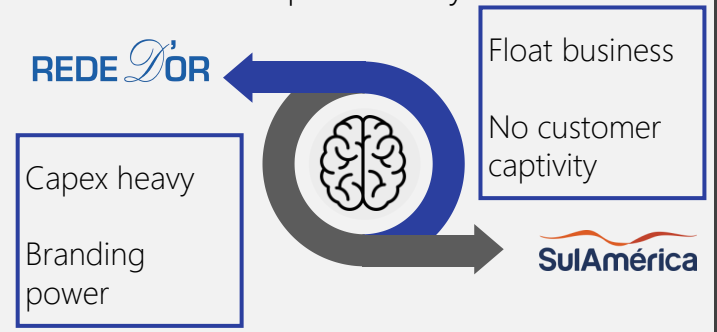


...a key pillar to lead the industry going forward...

Sul América health beneficiaries [mn]

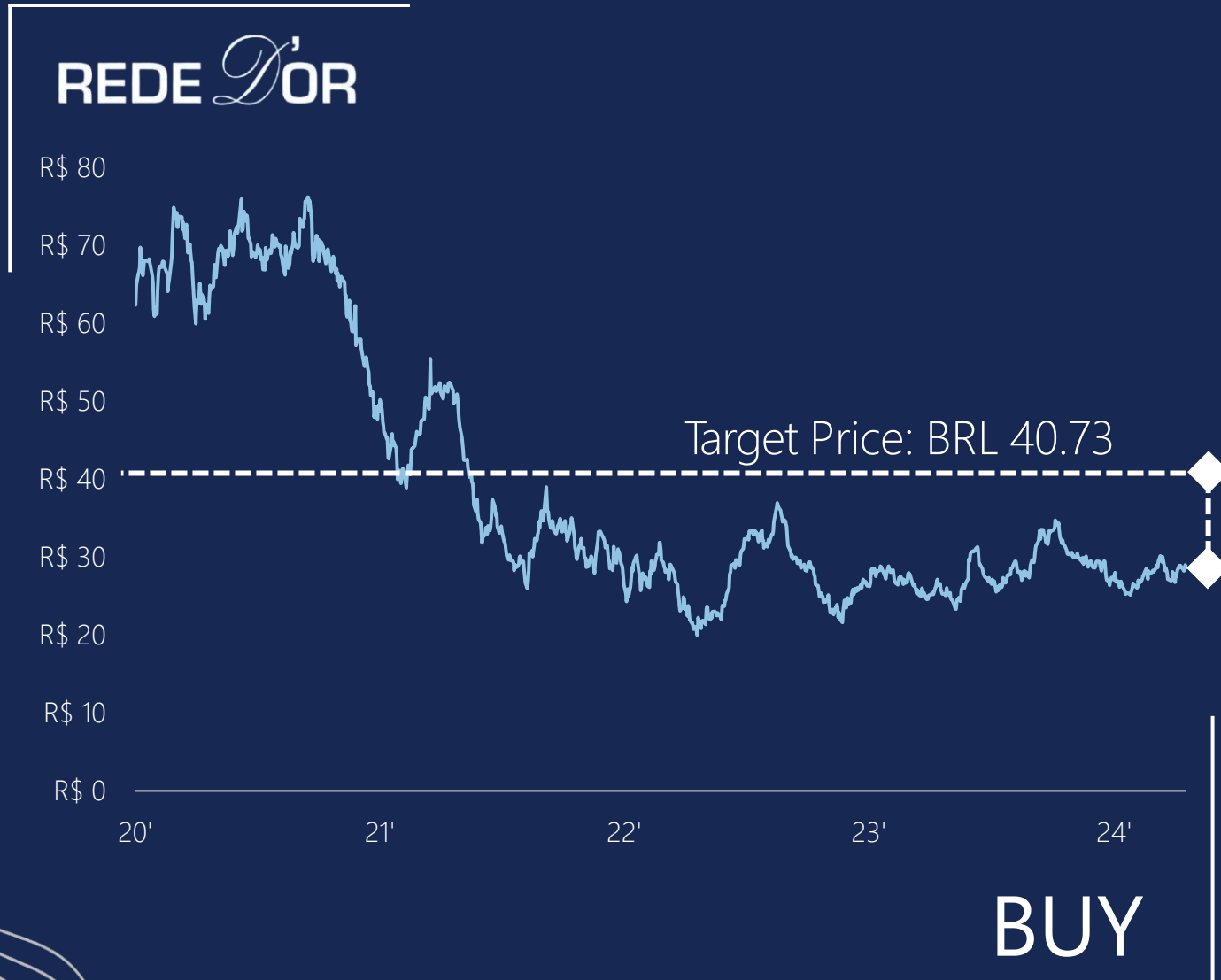


...as the two businesses are extremely complementary



RDOR3 – Would we be partners of Rede D'Or?

Yes, we would! We see Rede D'Or trading at a discount to its intrinsic value



Based on:

DCF: 37.4% Upside

Current Price: BRL 29.65
Target Price: BRL 40.73

IRR (5-Year): 23.4%

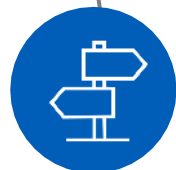
Ke: 15.7%
IRR - Ke: 7.7%



Best-in-class: ***Sucesso made in Brasil***

Creating Competitive Advantages in Brazilian Healthcare

REDE D'OR



Management: ***Outliers***

Seizing Golden Opportunities with Capital Allocation



Expansion Ahead: ***Big Dream***

Derisking the Operation and Gaining Share



Best-in-class: ***Sucesso made in Brasil***

Creating Competitive Advantages in Brazilian Healthcare

REDE D'OR



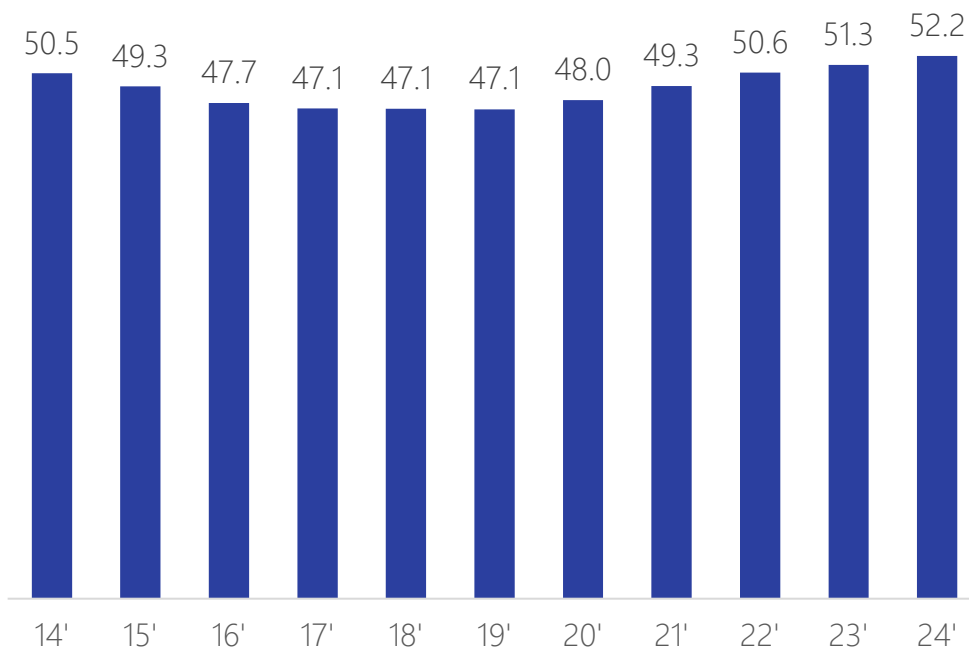
Brazilian healthcare: growing with spending

The industry has a unique mechanism to keep expanding

As the number of beneficiaries has not expanded much...

Number of beneficiaries evolution [mn]

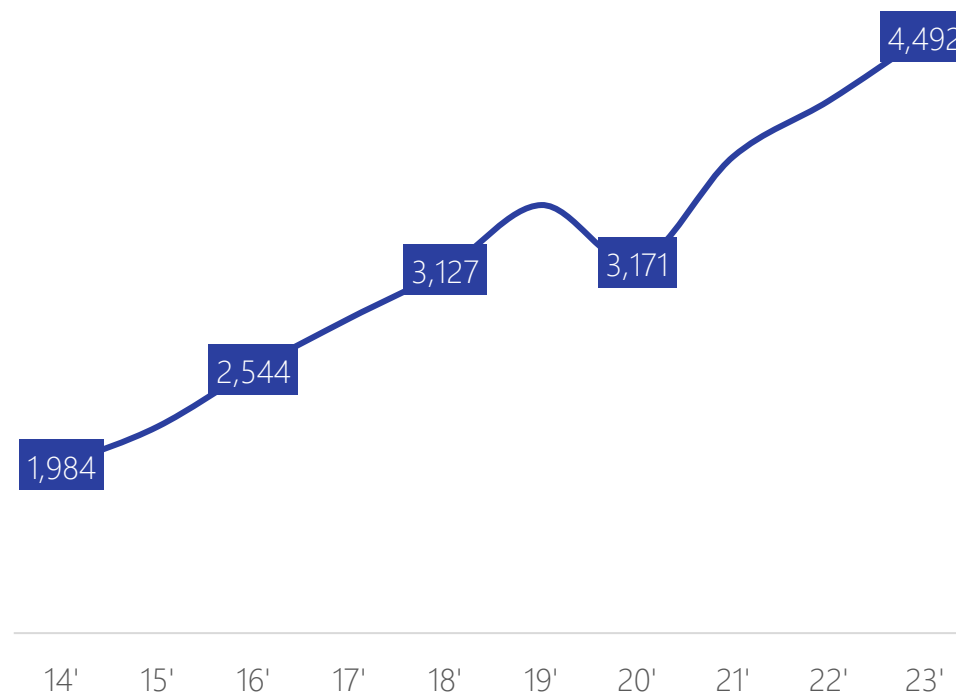
CAGR (14'-24'): 0.3%



...the industry has relied on higher spending per beneficiary

Average health expenditure per beneficiary [BRL]

CAGR (14'-23'): 9.5%

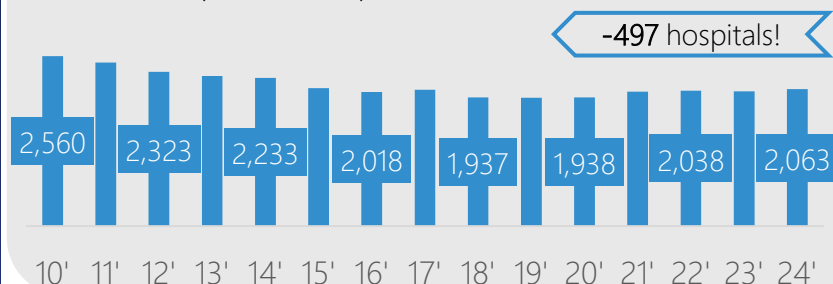




However, it has been a rough path

Brazil's macro scenario has harmed players in the recent past

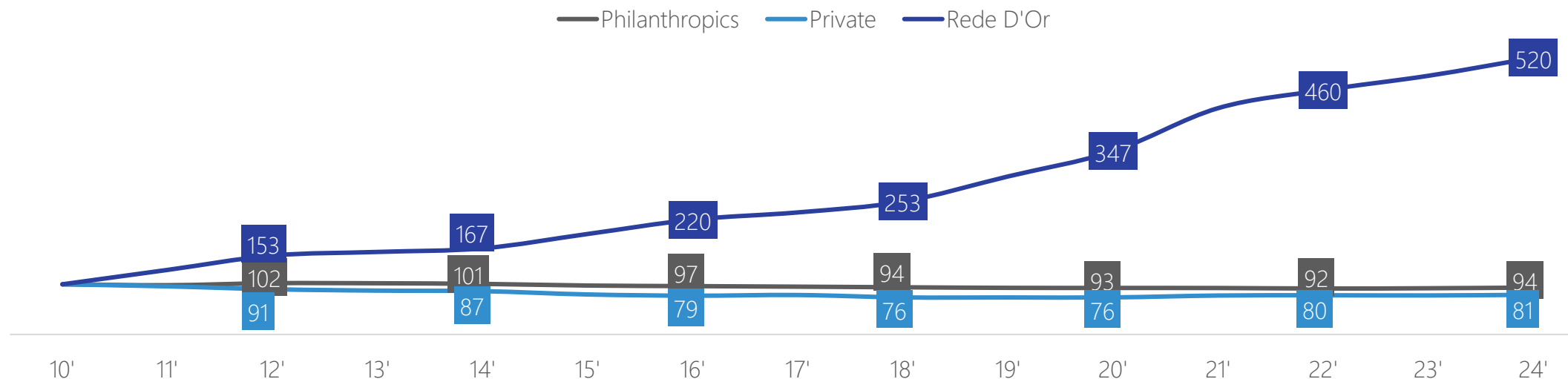
Number of private hospitals evolution [#]



Rede D'Or has constantly outpaced Brazil's turbulence

Still, the company expanded way above the industry for more than 15 years

Rede D'Or hospitals evolution vs. Industry [2010 = 100 base]



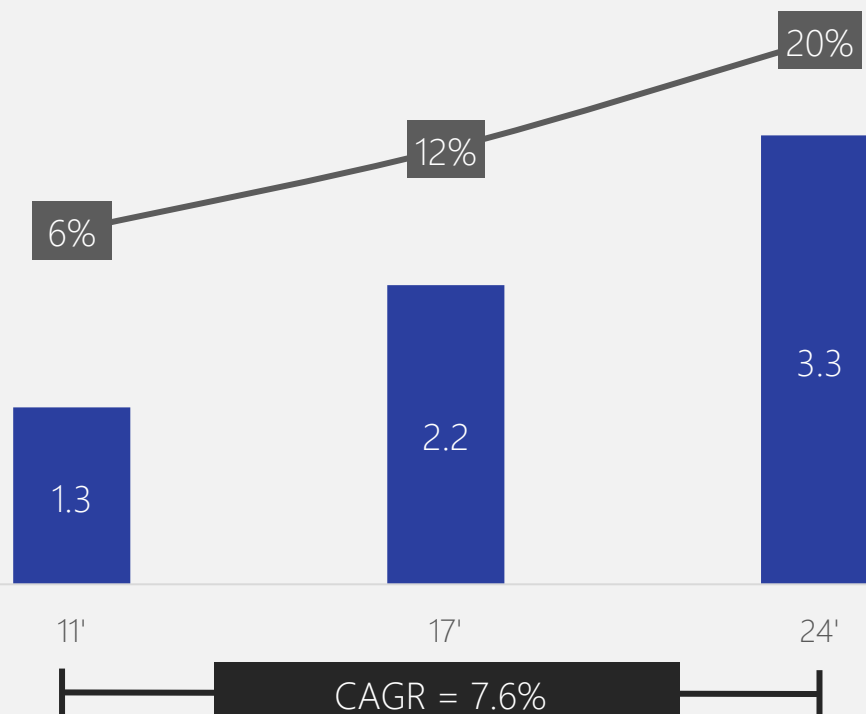


Rede D'Or is in a league of its own

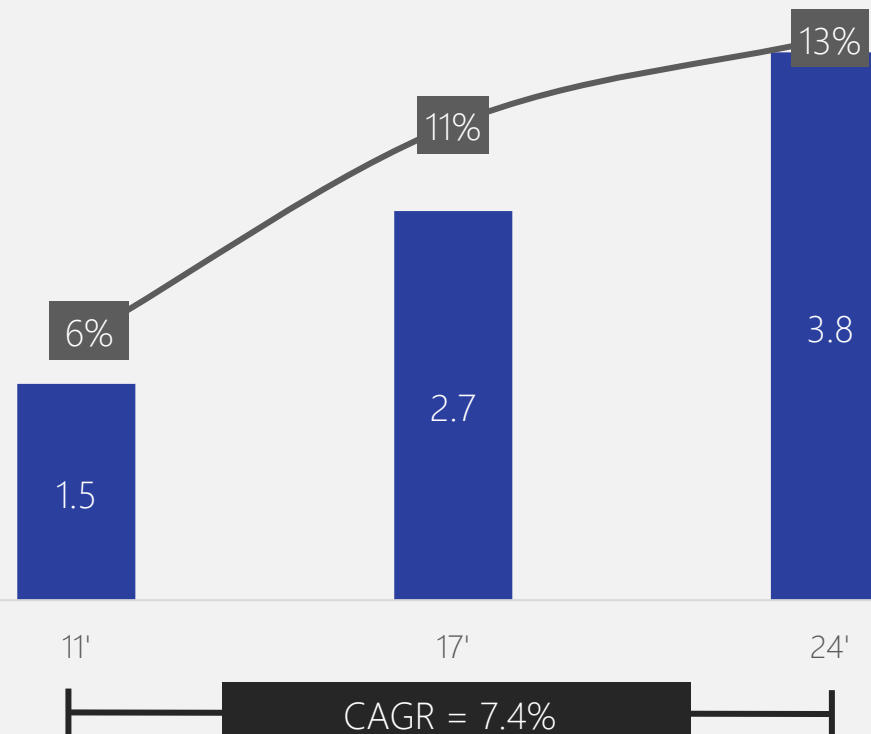
The company has an important footprint in RJ and SP, and it has been gaining share

Operating beds and share of private beds by UF evolution [th ; %]

Rio de Janeiro



São Paulo

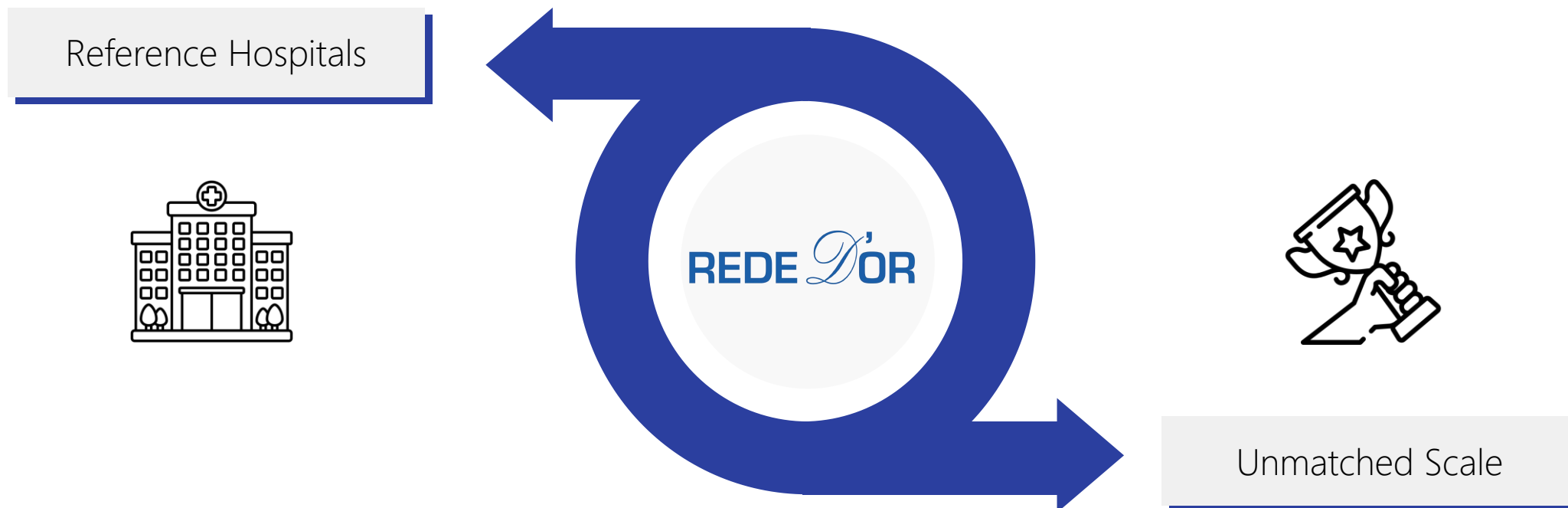




Rede D'Or: the king of Brazilian healthcare

The company has created strong competitive advantages to outpace competitors

How did Rede D'Or stand out?

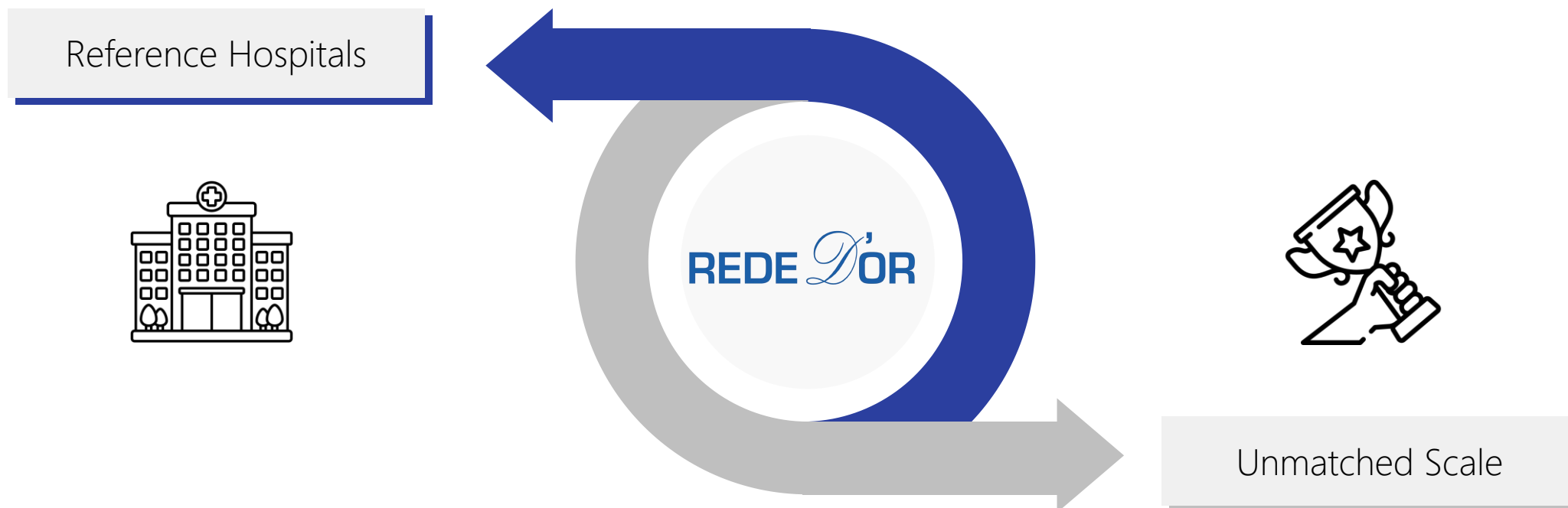




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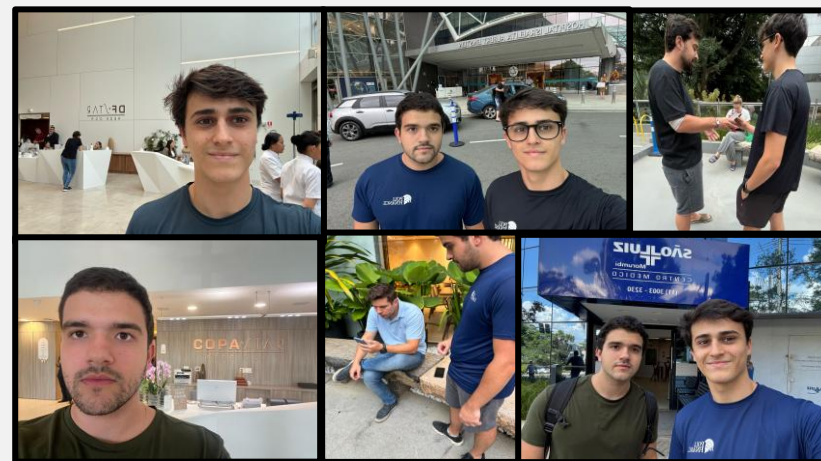
But what makes a good hospital, after all?

Rede D'Or's outperformance happened for a reason

We talked to **clients** from hospitals in **three** states...

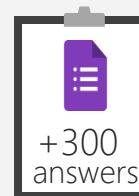
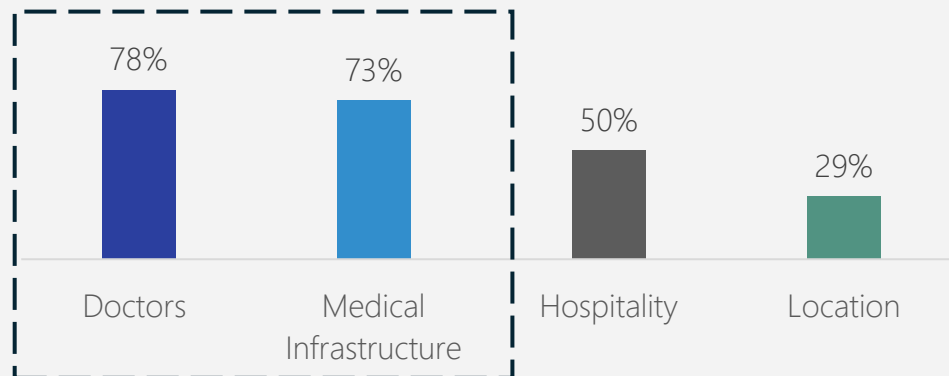
...to understand how demand mostly works and as we went into the field...

...we noticed **the main drivers**



"What are the main reasons that you go to a hospital?"

Field research findings [%]

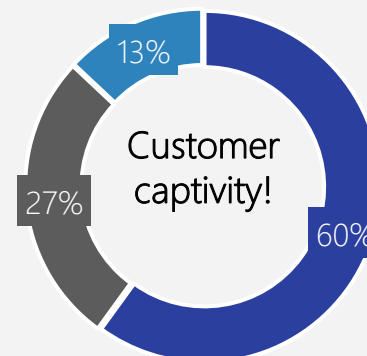


Hospital Albert Einstein
Hospital São Luiz Morumbi
Hospital São Luiz Itaim
Hospital Sírio Libanês
Hospital HCor
Hospital Vila Nova Star
Hospital Copa D'Or
Hospital Copa Star
Hospital DF Star

"What keeps you going to the same hospital?"

Field research findings [%]

- Doctors Indication
- Habit
- HMO Coverage



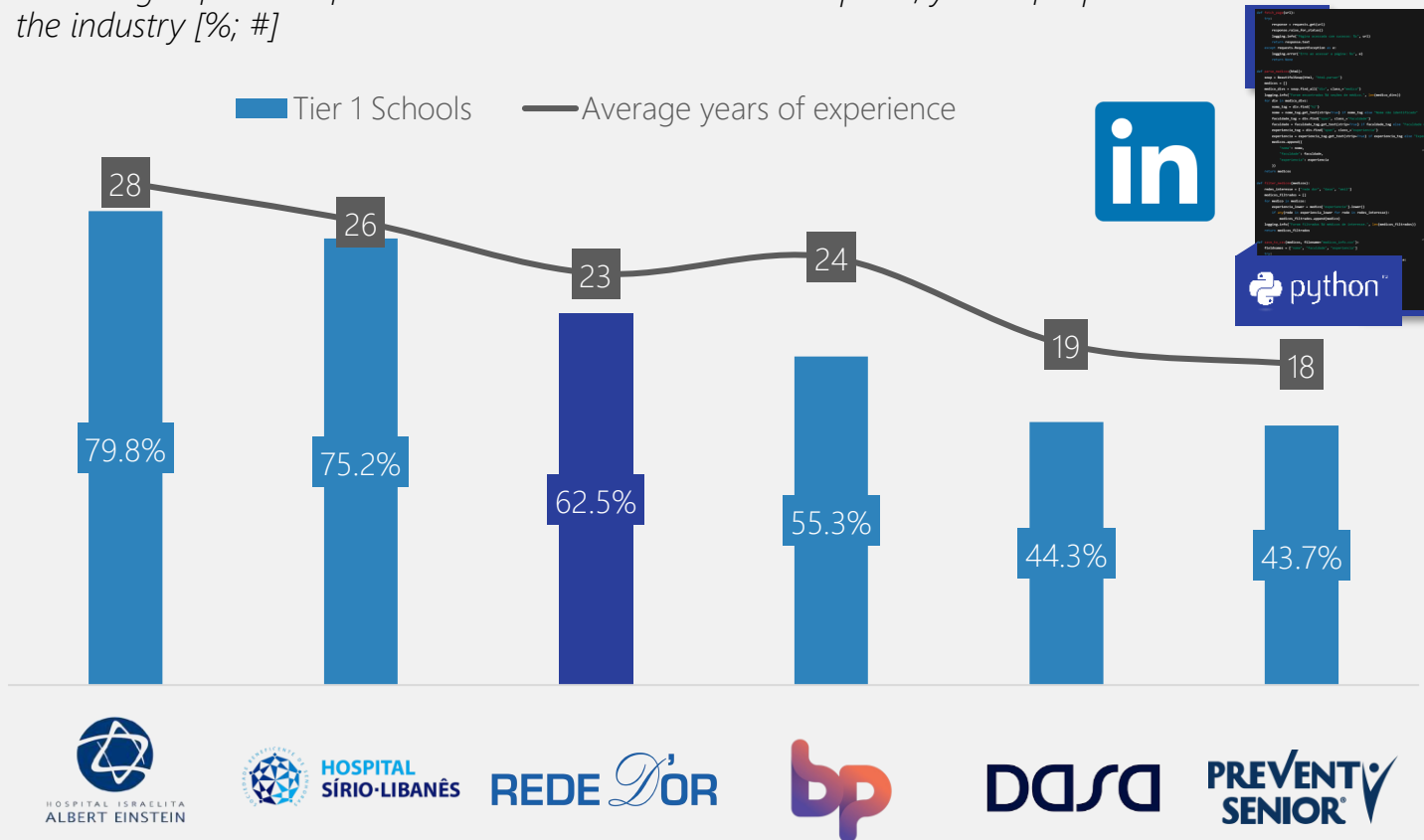


"The client is the doctor!"

Bringing the best team together is a competitive advantage

Rede D'Or has a focus on hiring top doctors

Percentage of doctors from Tier 1 schools in SP and RJ hospitals, years of experience in the industry [%; #]



Talking to the one who knows...



1999



2024



"In the end, **medical staff quality** is a very important thing. For example, 90% of our doctors were from Hospital das Clínicas (USP)"

Flavia Foronda
Pediatrics Coordinator at Vila Nova Star

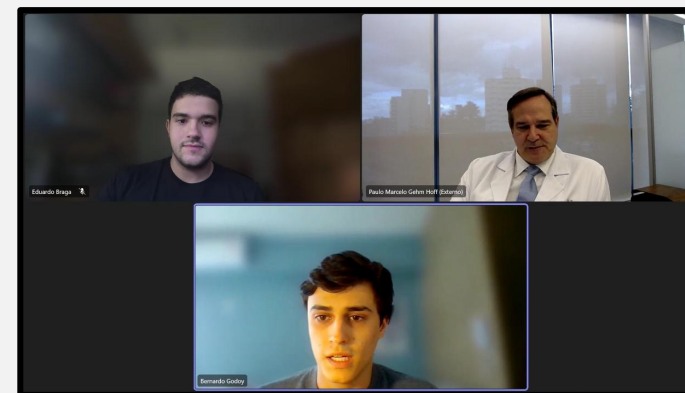
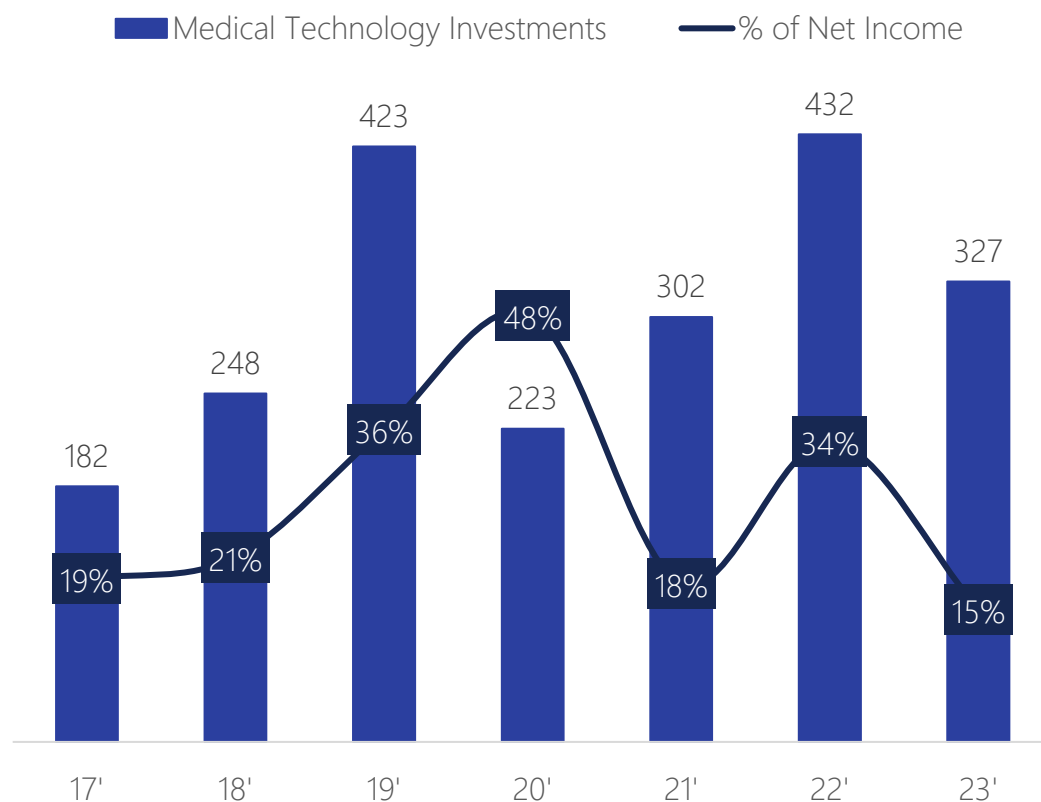


Technology has a paramount role going forward

We sought to understand how these investments are affecting Rede D'Or

- The company has kept a strong pace in investing in technologies....

Med Tech Investments and % of Net Income [BRL mn ; %]



HOSPITAL
SÍRIO-LIBANÊS

REDE D'OR

2005

2017

34 Years at Healthcare sector

"Technology is essential to increase complexity.
And our effort to step up our tech position is stronger than ever, as we have recently acquired many of new devices for our hospitals, such as the nano-knife, the cyber-knife and the gamma knife treatments."

Paulo Hoff – Oncology Chief at Rede D'Or

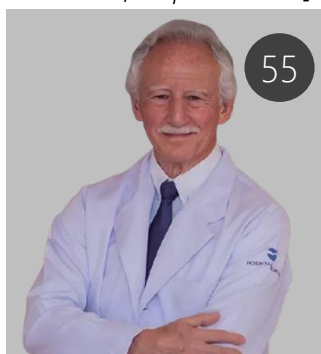


Having a top tier medical team with technology

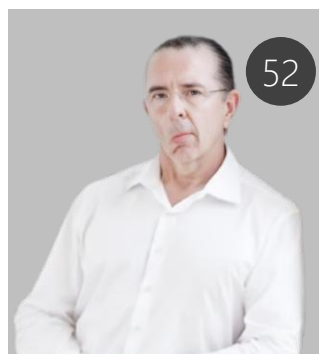
Rede D'Or also has hired some well-known specialists to lead its doctors

- Big shots have mixed effects of **bringing clients** as well as other good doctors to perform **more complex surgeries**

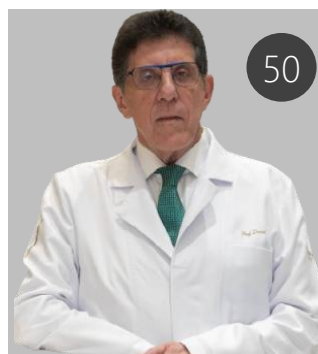
Years of experience [#]



Dr. Srougi
Urologist



Dr. Macedo
Vascular Surgeon



Dr. David Uip
Infectologist



Dr. Paulo Hoff
Oncologist

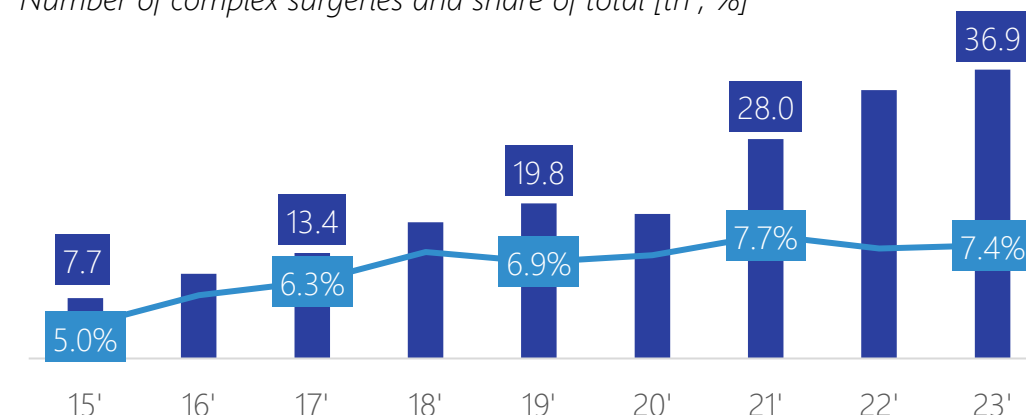


Dr. Ludhmila
Cardiologist

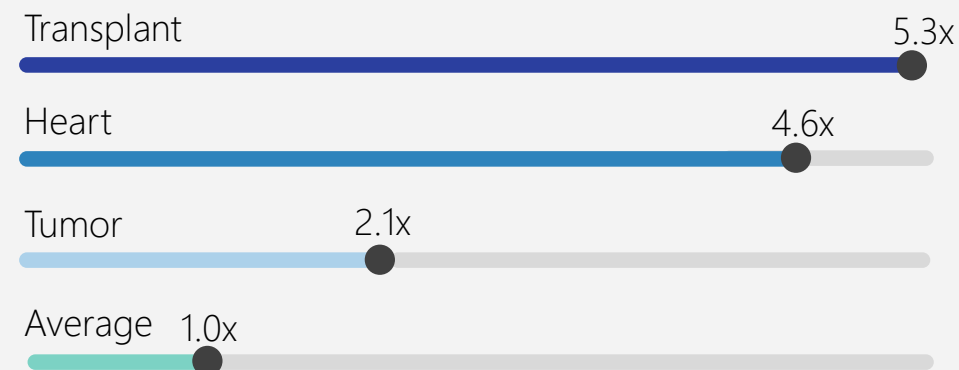


Dr. Machuca
Thoracic Surgeon

Number of complex surgeries and share of total [th ; %]



Average ticket per type of surgery [x]





A powerful brand known for service quality

The most important effect these efforts have is bringing customer captivity

And all this leads to a good patient perception of its services!

Google review [#/5]

Being able to compete in top clusters with the Star brand...

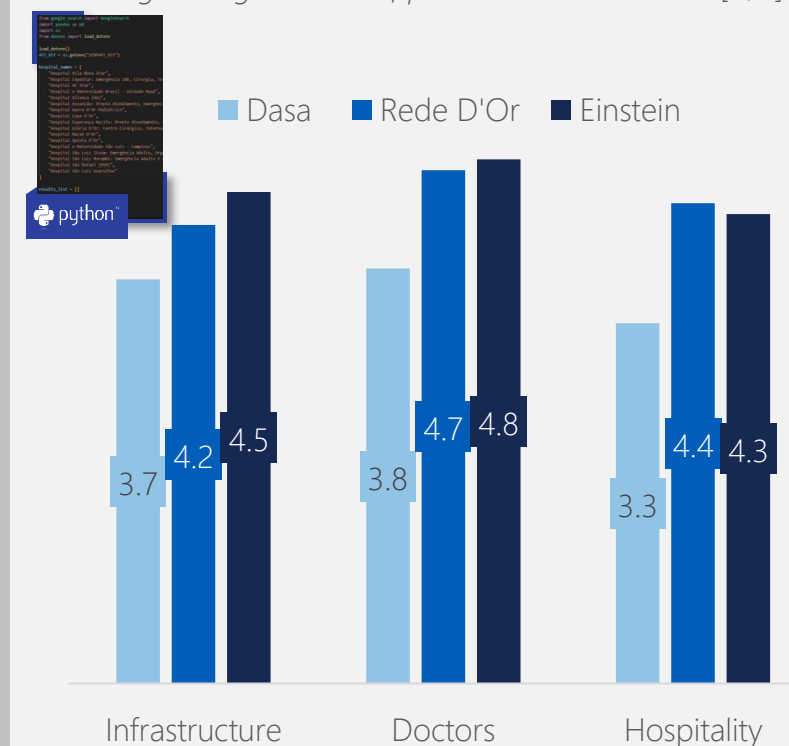


...and on the medium top with D'Or and other strong names



Our **python** analysis of **google reviews** showed us **how Rede D'Or stands out**

Average Google review of post with related words [#/5]

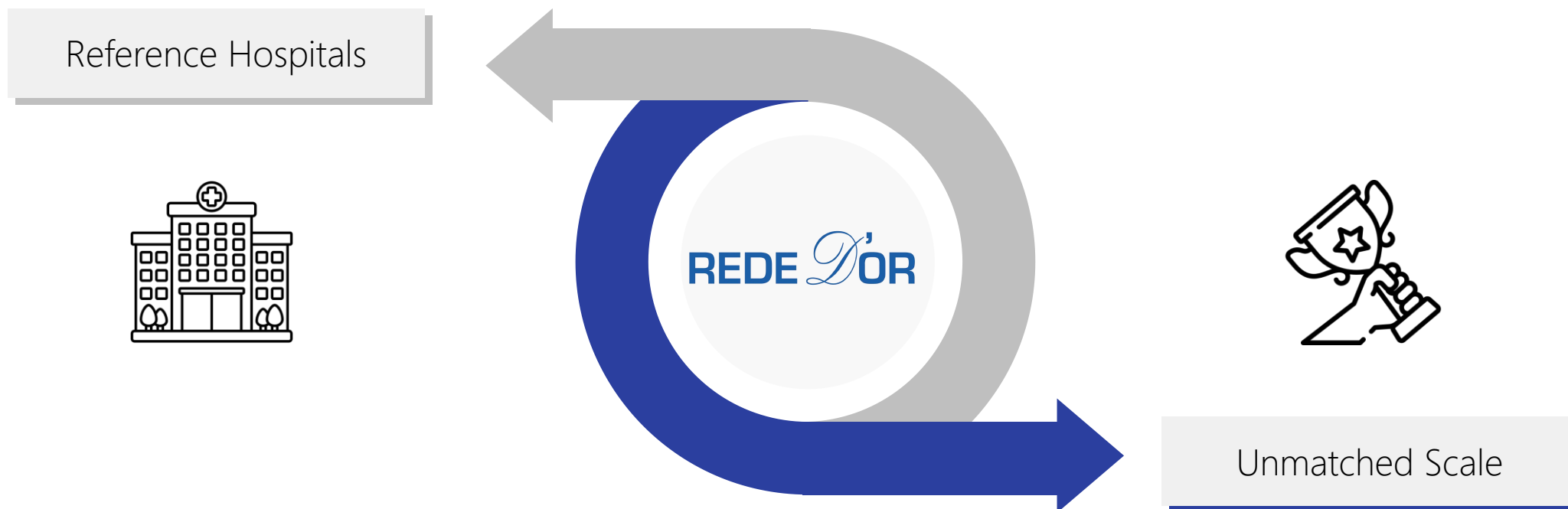




Rede D'Or: the king of Brazilian healthcare

The company has created strong competitive advantages to outpace competitors

How did Rede D'Or stand out?



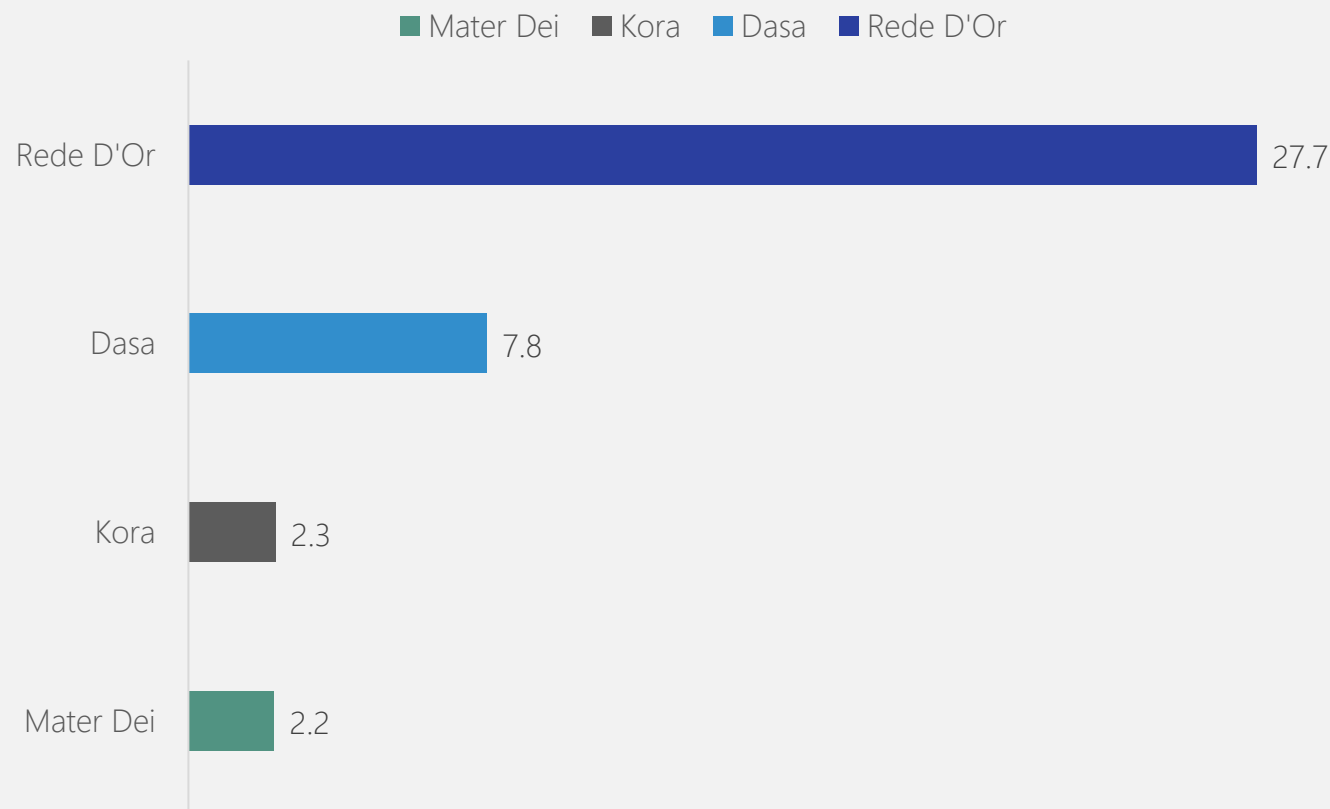


Rede D'Or has scale like no other player

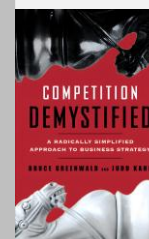
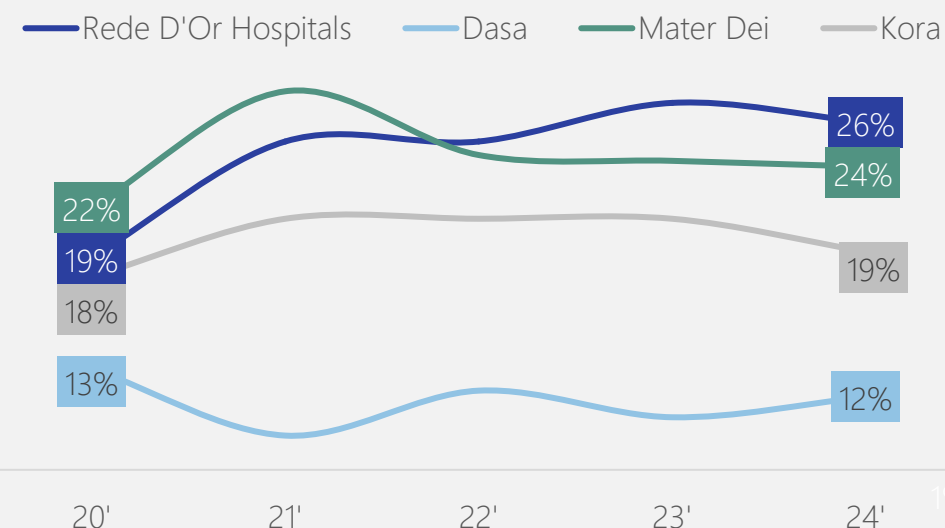
The company is the largest player in Brazilian healthcare and profits from it

Rede D'Or dwarfs its competitors' operations in size, making room for economies of scale

Companies' revenue [BRL bn] – hospitals only



LTM EBITDA margins [%]



"Scale advantages are the most durable competitive advantages because they create cost structures that are difficult for new entrants to replicate."

"Competition Demystified" - Bruce Greenwald



In the end, size matters

Rede D'Or uses its scale to better negotiate with suppliers and dilute costs

UFMG MaterDei
Rede de Saúde

1982

1983

42 Years at Mater Dei



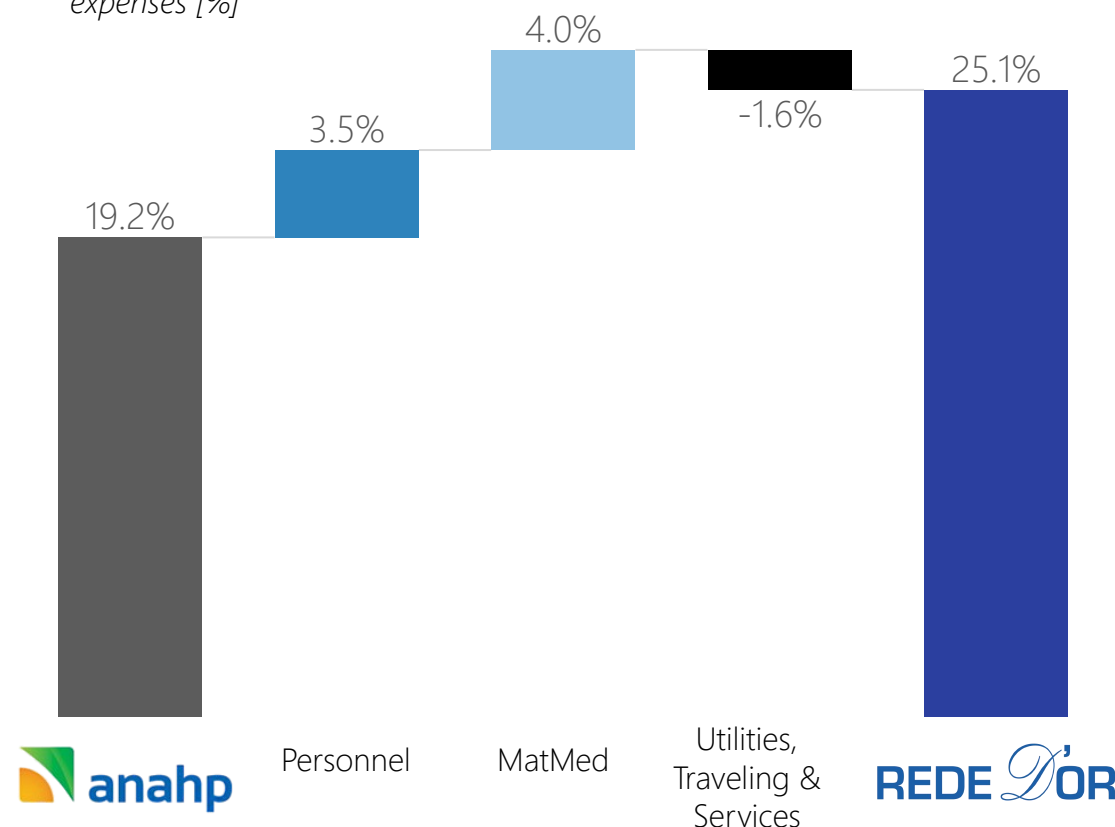
"Having **scale is extremely important** going forward because big players can **negotiate better** with suppliers and, with more beds, they can dilute costs with personnel. It can also use its **regional strength** to create a **better brand** with locals."

Henrique Salvador – Chairman at Mater Dei



Something seen in bigger dilutions in MatMed and personnel expenses...

Avg EBITDA Margin (17'-24') difference composition excluding other expenses [%]





In the end, size matters

Rede D'Or uses its scale to better negotiate with suppliers and dilute costs



Something seen in bigger dilutions in MatMed and personnel expenses...

Avg EBITDA Margin (17'-24') difference composition excluding other expenses [%]

4.0%

25.1%

But Customer Captivity + Scale = What?

because big players can negotiate better with suppliers and, with more beds, they can dilute costs with personnel. It can also use its **regional strength** to create a **better brand** with locals."

Henrique Salvador – Chairman at Mater Dei



Personnel

MatMed

Utilities,
Traveling &
Services

REDE D'OR

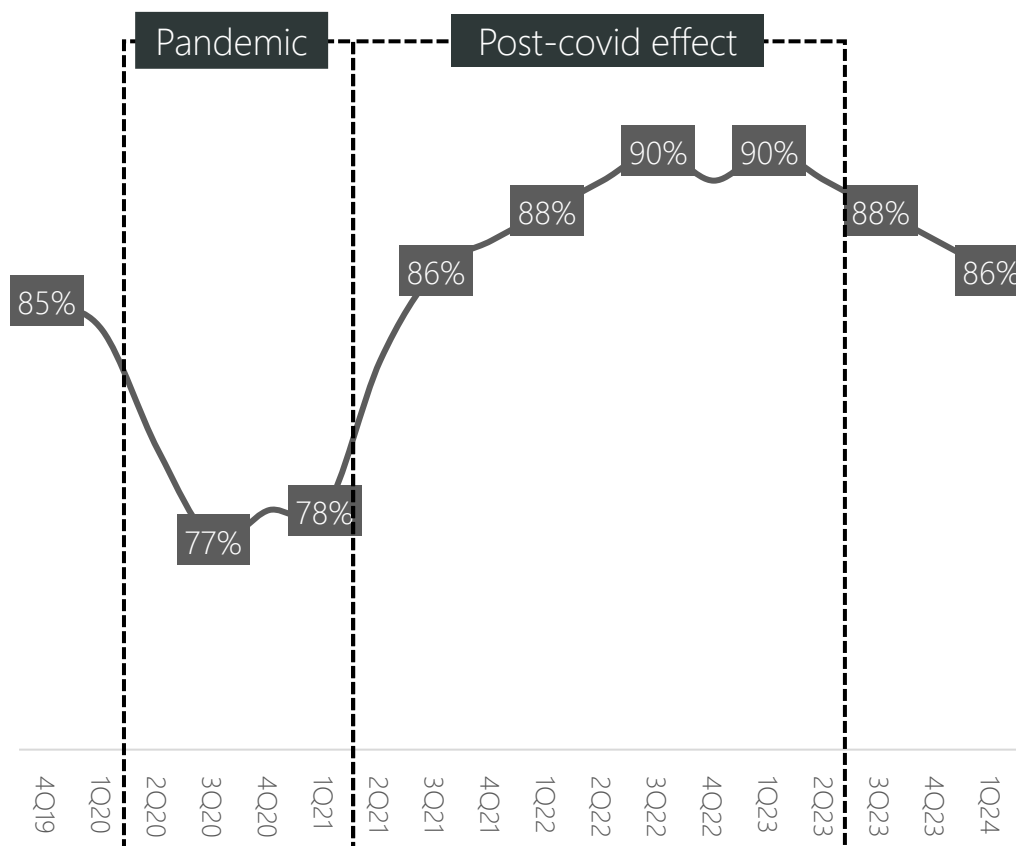


The power imbalance between hospitals and insurers

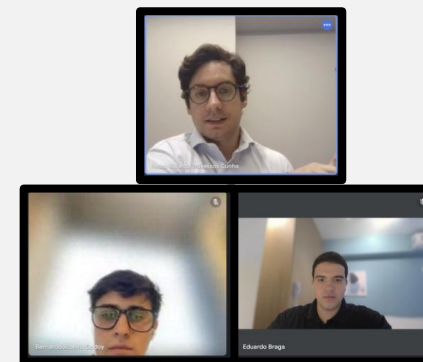
Since insurers are the payers, they are the ones who set the rules

- Insurance had a hard time during Covid and hospitals paid the bill...

Industry Medical Loss Ratio LTM quarterly [%]



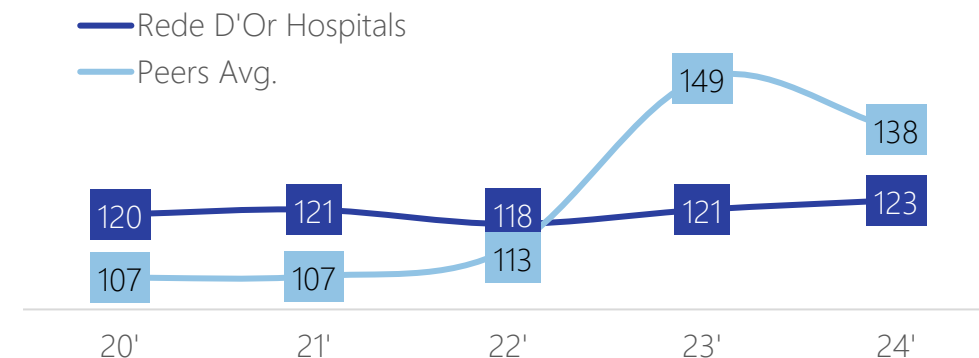
"After the pandemic, **insurers delayed their payments**, and we faced **numerous claim denials** (*glosas*). This situation created **serious liquidity challenges** for players."



Pedro Cunha – Financial Director at Kora Saúde

- ...however, **Rede D'Or was less affected** due to its bargaining power

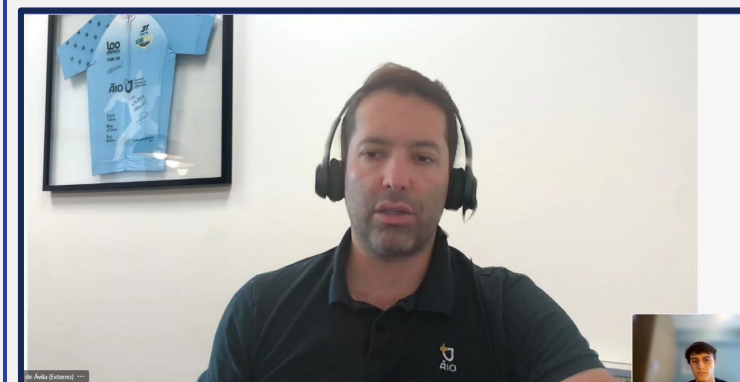
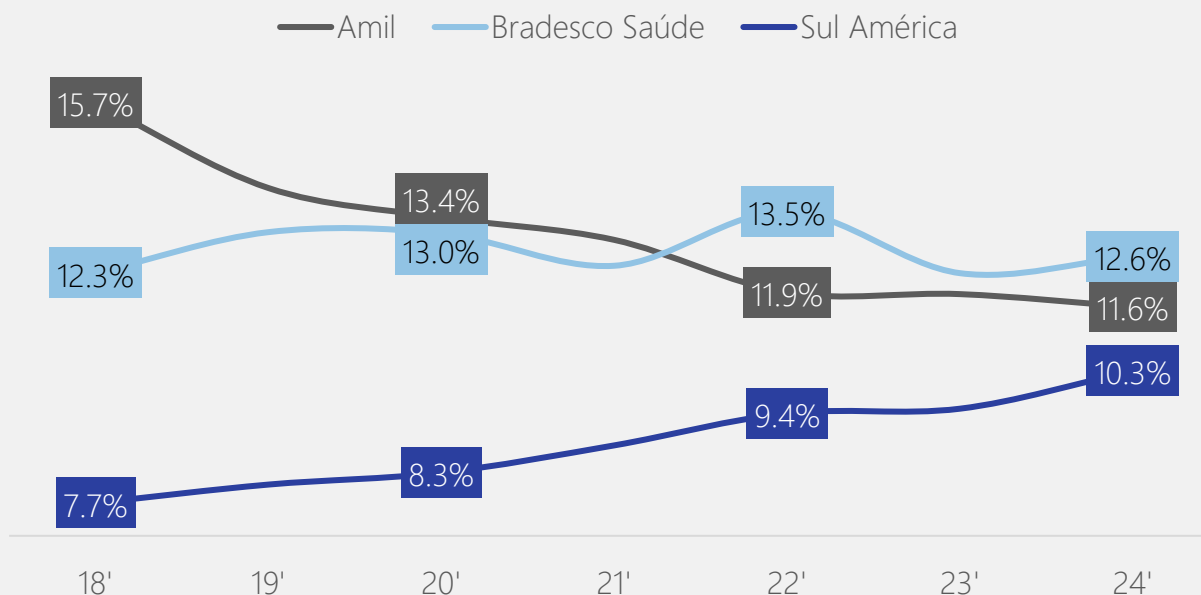
Yearly average of days of receivables [#]



The power imbalance between hospitals and insurers

However, when Amil removed the RDOR network from its plans in 2019, it suffered severe client losses. Since Rede D'Or has the largest network of reference hospitals, beneficiaries want access to it—giving the company a clear market advantage.

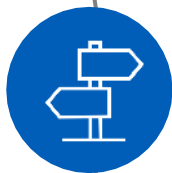
Share of Beneficiaries Evolution per company in Rio de Janeiro city [%]



"Rede D'Or has created a scale and a brand that makes it hard to sell to mid to high-end tickets without it"

Luciano Ávila – Founder at AIO Corretora

REDE D'OR



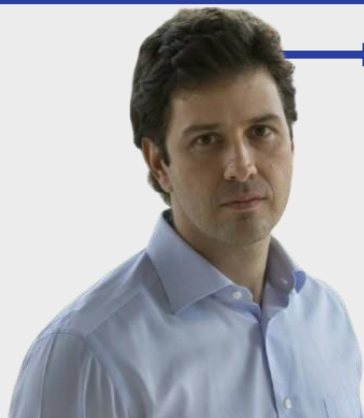
Management: ***Outliers***

Seizing Golden Opportunities with Capital Allocation

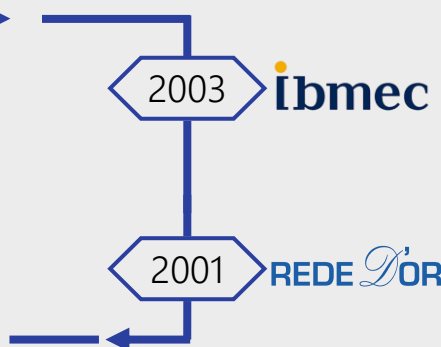


The Moll family guides the company forward

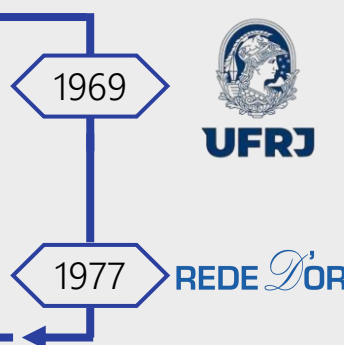
Rede D'Or's management reports to hands-on shareholders



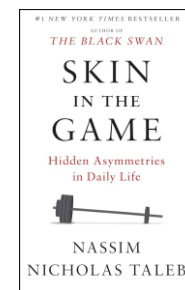
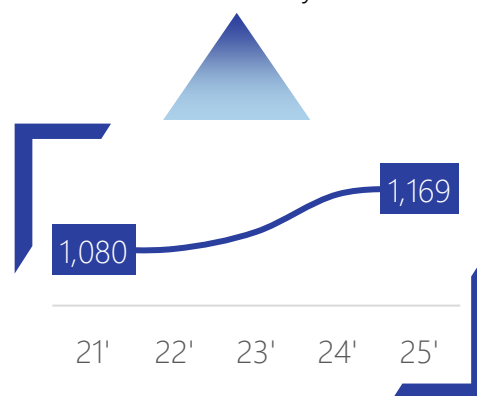
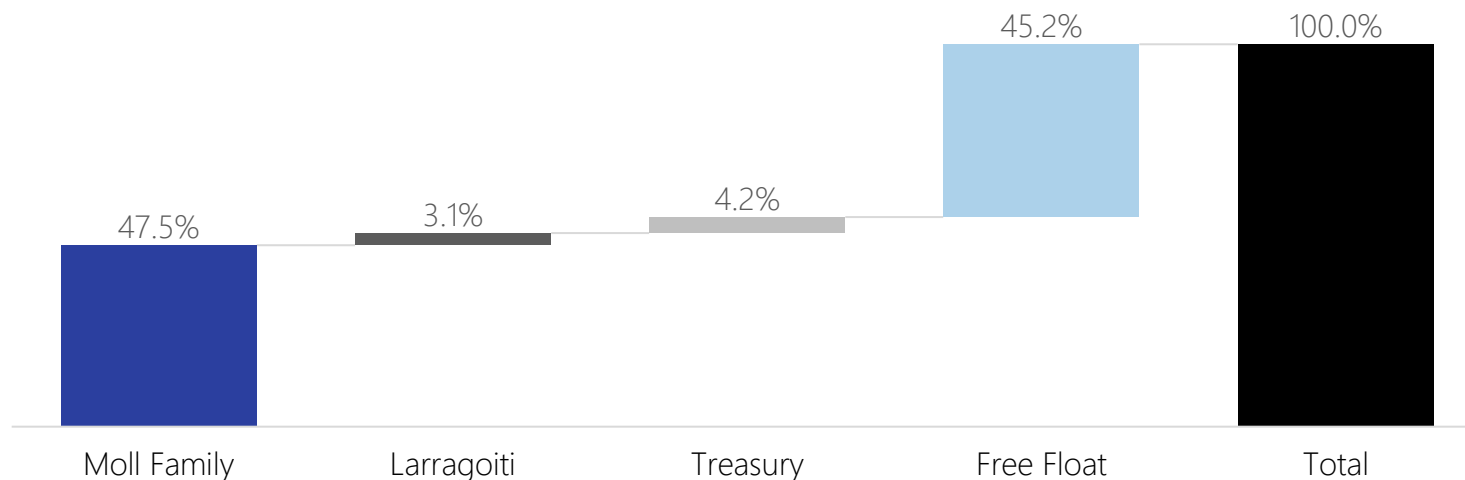
Paulo Moll – CEO of Rede D'Or



Jorge Moll – Chairman at Rede D'Or



 **Skin in the game:** Rede D'Or has a controlling family aligned with shareholders
Shares composition breakdown and Moll Family Shares [% ; mn]



“This idea of skin in the game is woven in history: historically, **all warlords and warmongers were warriors themselves**, and, with a few curious exceptions, **societies were run by risk takers, not risk transferers**”

“Skin in the Game” – Nassim Nicholas Taleb

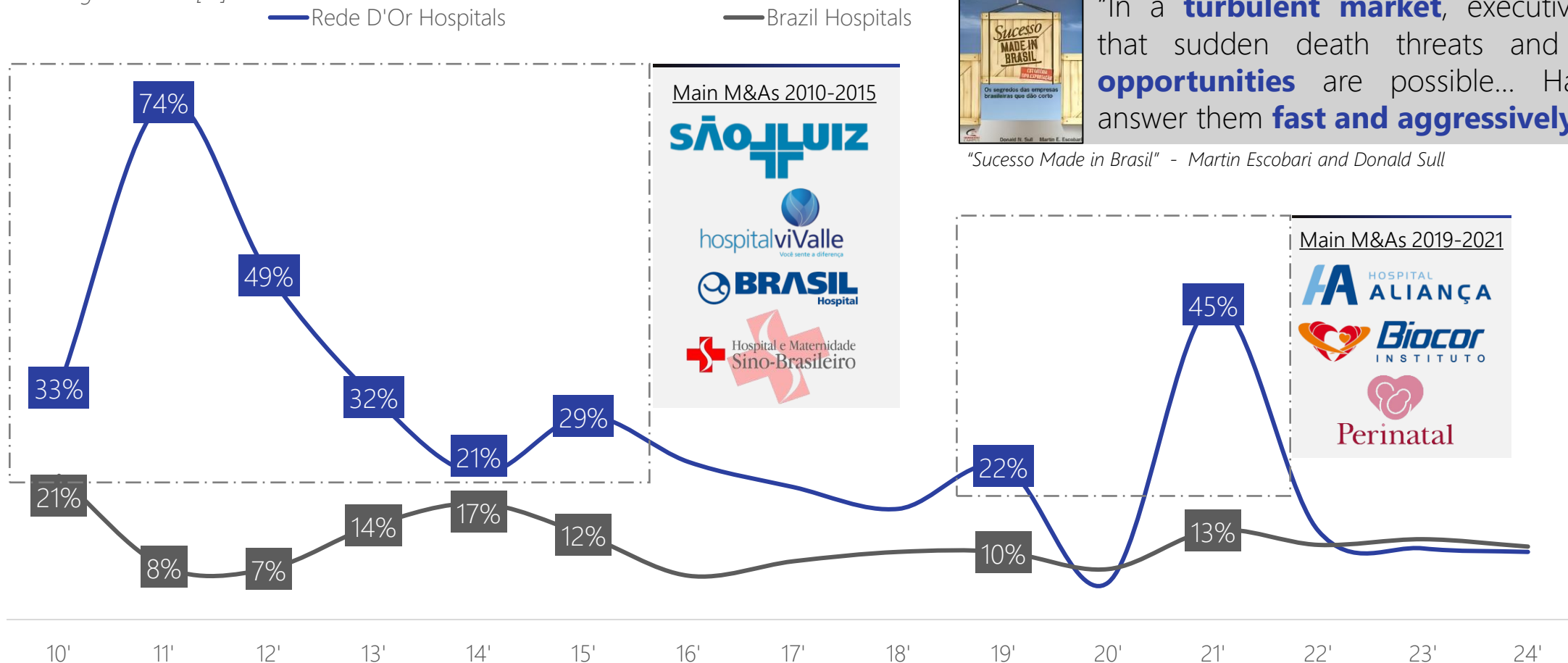


Embracing Golden Opportunities, Rising Fast

For many years, the company grew well above the industry by seizing M&A opportunities

 The effects of the company's strategic acquisitions over the years are evident when observing its topline evolution...

Net Revenue growth YoY [%]



"In a **turbulent market**, executives know that sudden death threats and **golden opportunities** are possible... Having to answer them **fast and aggressively**"

"Sucesso Made in Brasil" - Martin Escobari and Donald Sull

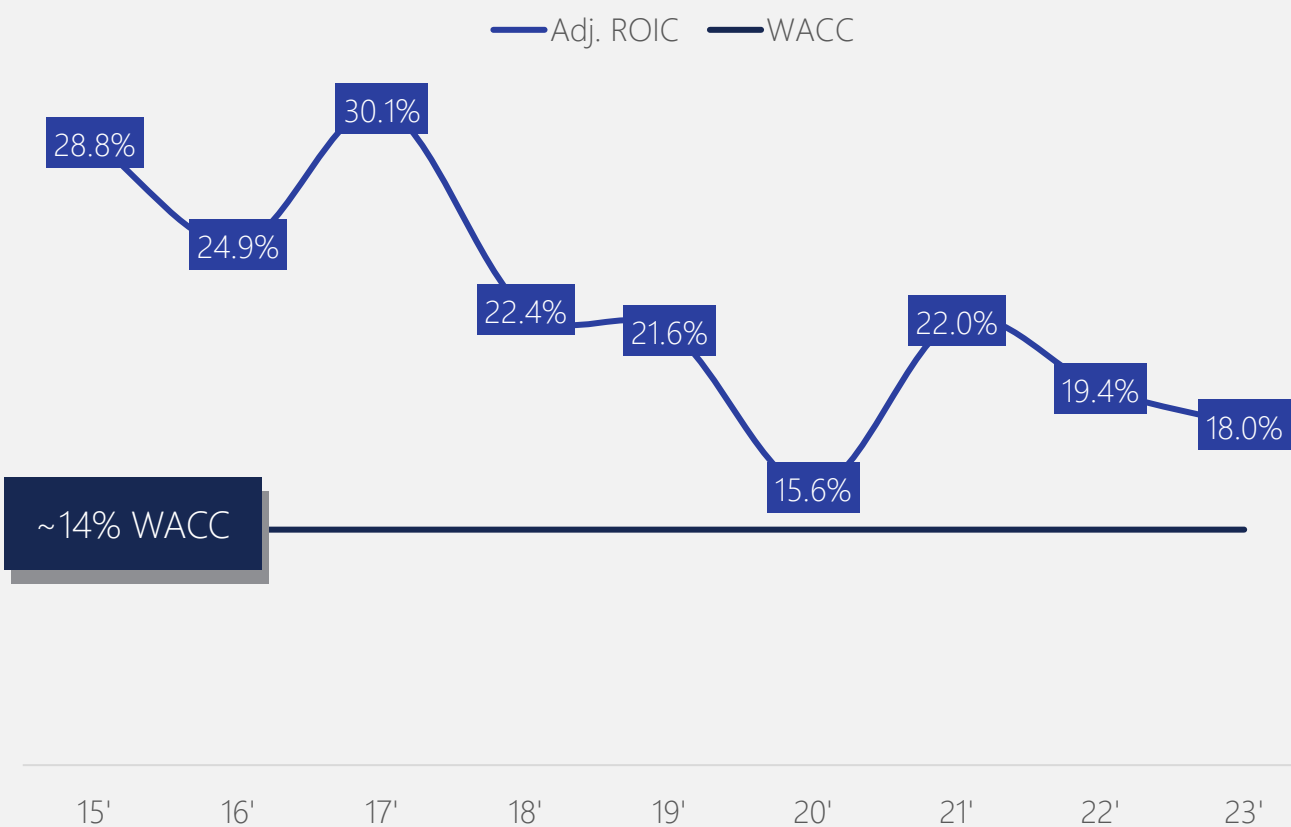


Rede D'Or has mastered allocating capital

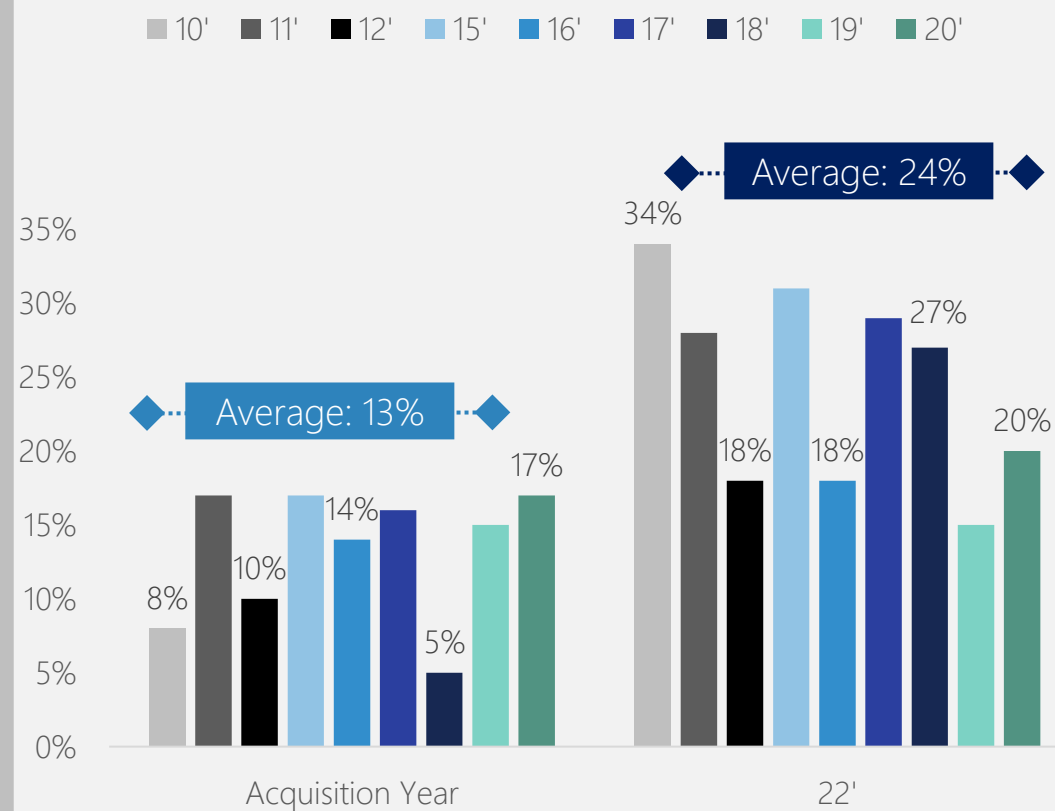
The company has constantly maintained its returns above cost of capital

 Growth alone is not enough to create value for shareholders, but when combined with good capital allocation it is exponential

Adjusted ROIC [%]



EBITDA margin [%]

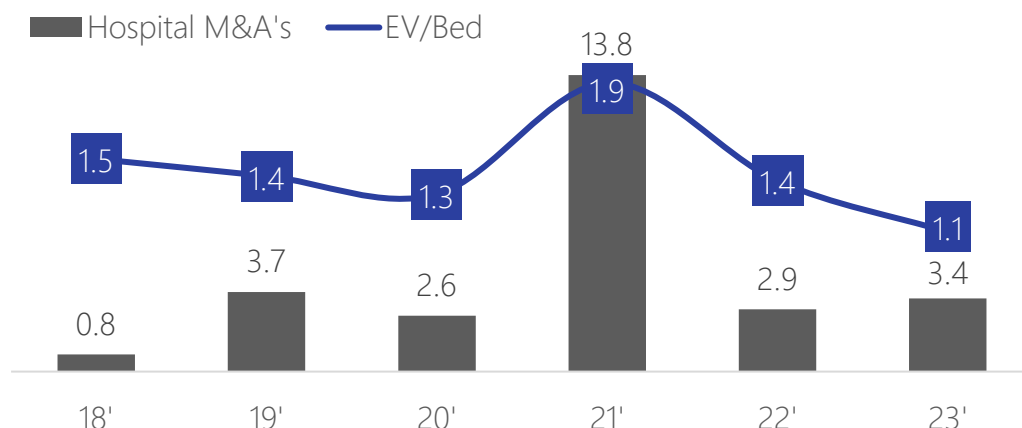


Acquisition fever: the hospital gold rush

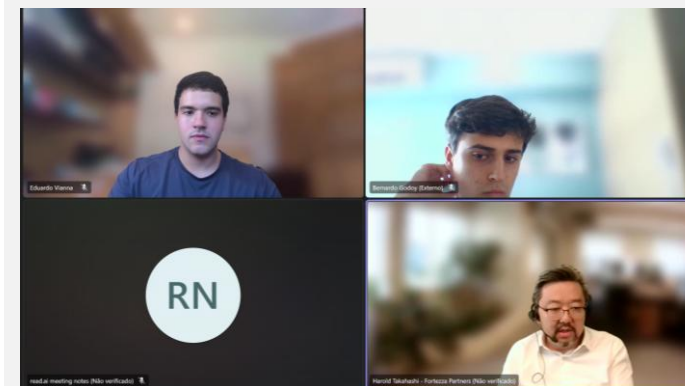
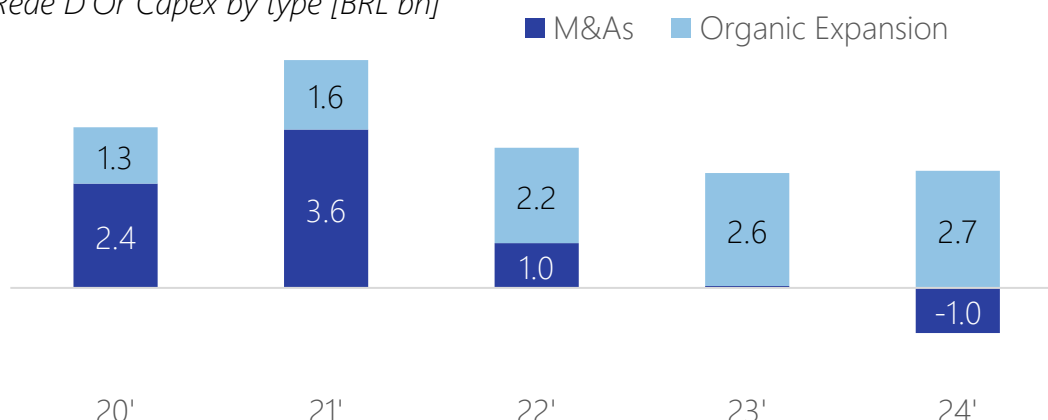
Competition for assets in the sector increased, making them more expensive

There was an M&A boom in the industry, which changed the scenario...

Hospital M&A's Total Value and Avg. EV/Bed [BRL bn ; BRL mn]



Rede D'Or Capex by type [BRL bn]



12 Years at Healthcare sector

"As the pace of M&A activity intensified, especially after 2020, hospital valuations started to become increasingly inflated. Faced with this scenario, Rede D'Or began to view the market as **less attractive for new acquisitions**, opting instead for a more selective approach"

Harold Takahashi – Former Head of M&A at Fleury

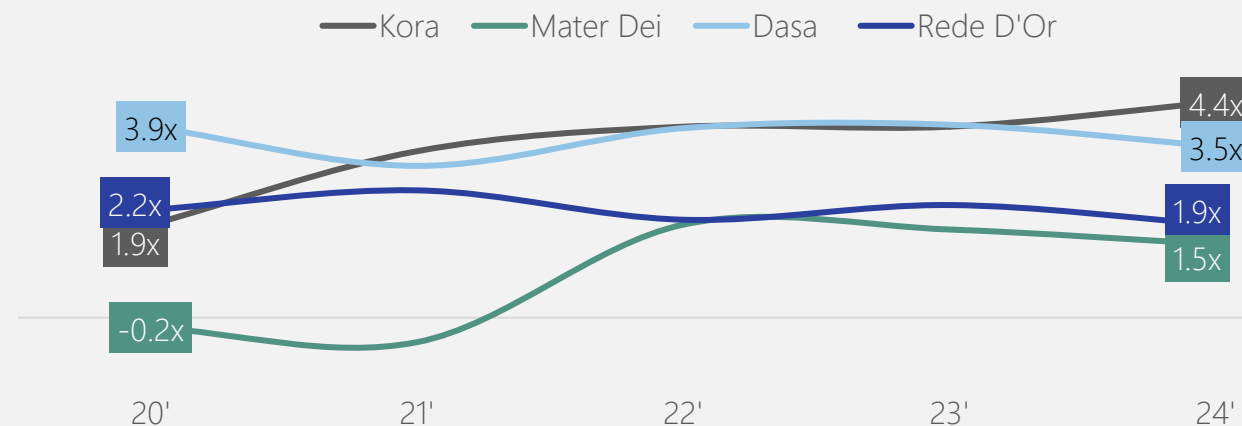


Now: chances to gain share over weak competitors

Rede D'Or will face a weaker competition than in the past after the frenzy

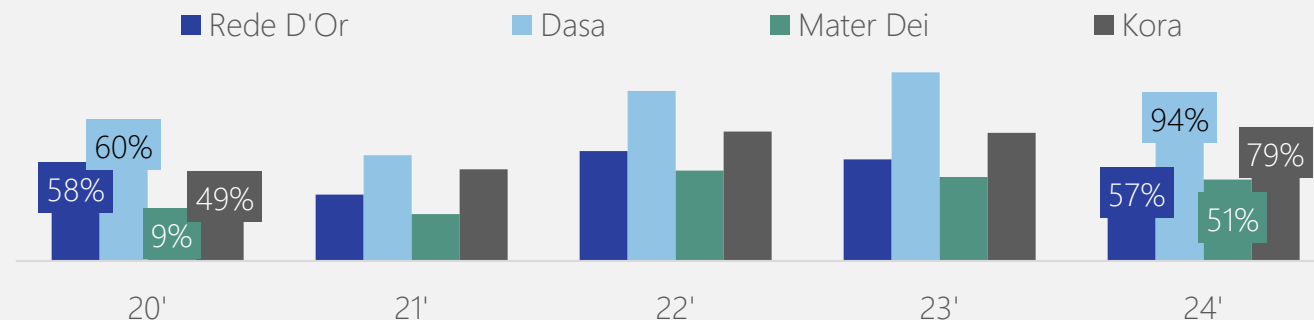
Its main private rivals are now highly leveraged...

Net Debt/LTM EBITDA [x]



...having to spend most of their EBITDA just on interest...

Interest expenses/LTM EBITDA [%]



...and philanthropic ones have showed increased difficulties to continue to operate well, mostly with higher debt

JOTA

20/09/2024

Santas Casas and philanthropic hospitals seek renegotiation of record debt

Valor

30/06/2023

Philanthropic hospitals operate in the red

veja

07/09/2024

Dasa puts assets up for sale and transfers BRL 4 billion in debt to a new business

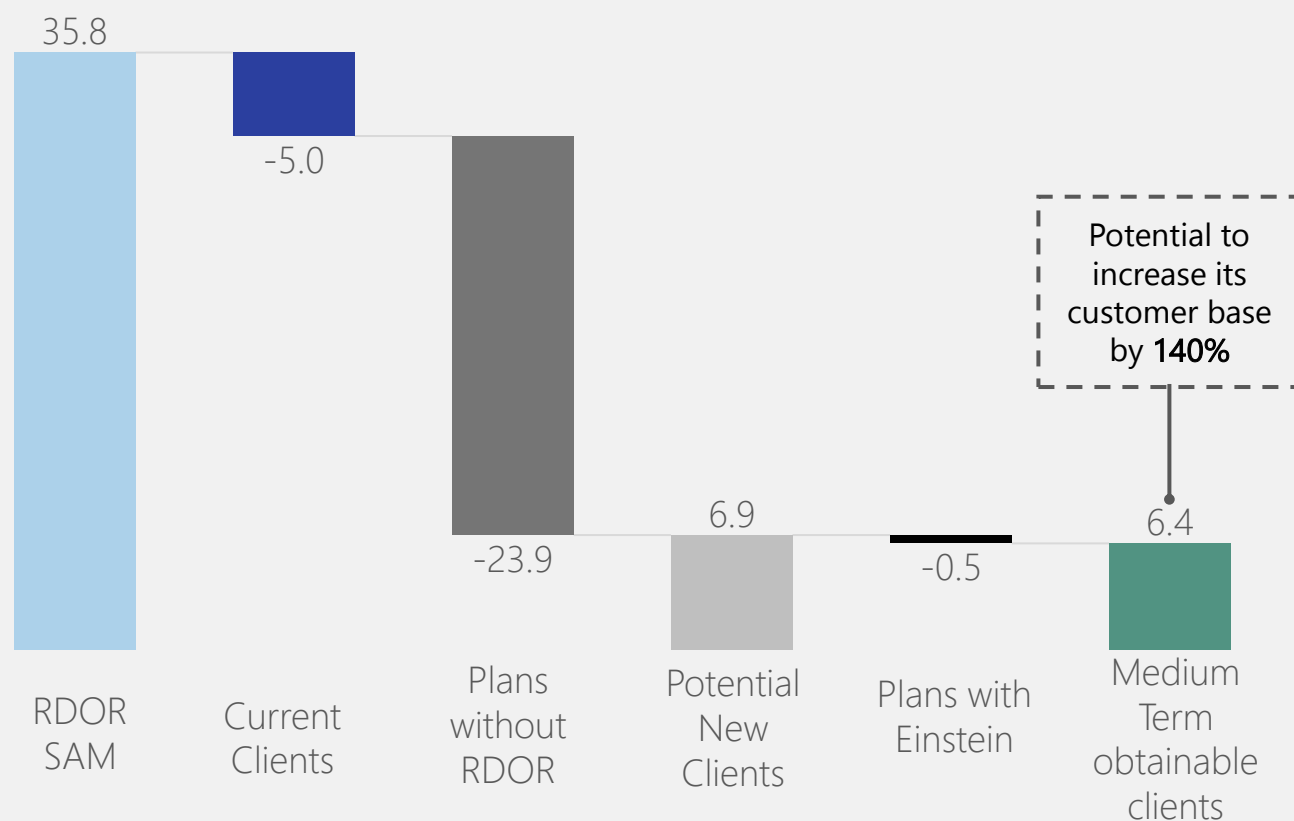


And can Rede D'Or take advantage of it?

We sought to understand how many beneficiaries can attend Rede D'Or facilities

So what is the size of Rede D'Or's market?

Company's Addressable market and potential new patients [mn]



1988



2020

37 years at
Healthcare Sector



“Operating a hospital is a **highly complex activity**, which makes it difficult for most to **remain relevant and profitable** in the long run, **causing many to fall behind**”

Fernando Torelly – CEO at HCor

REDE D'OR



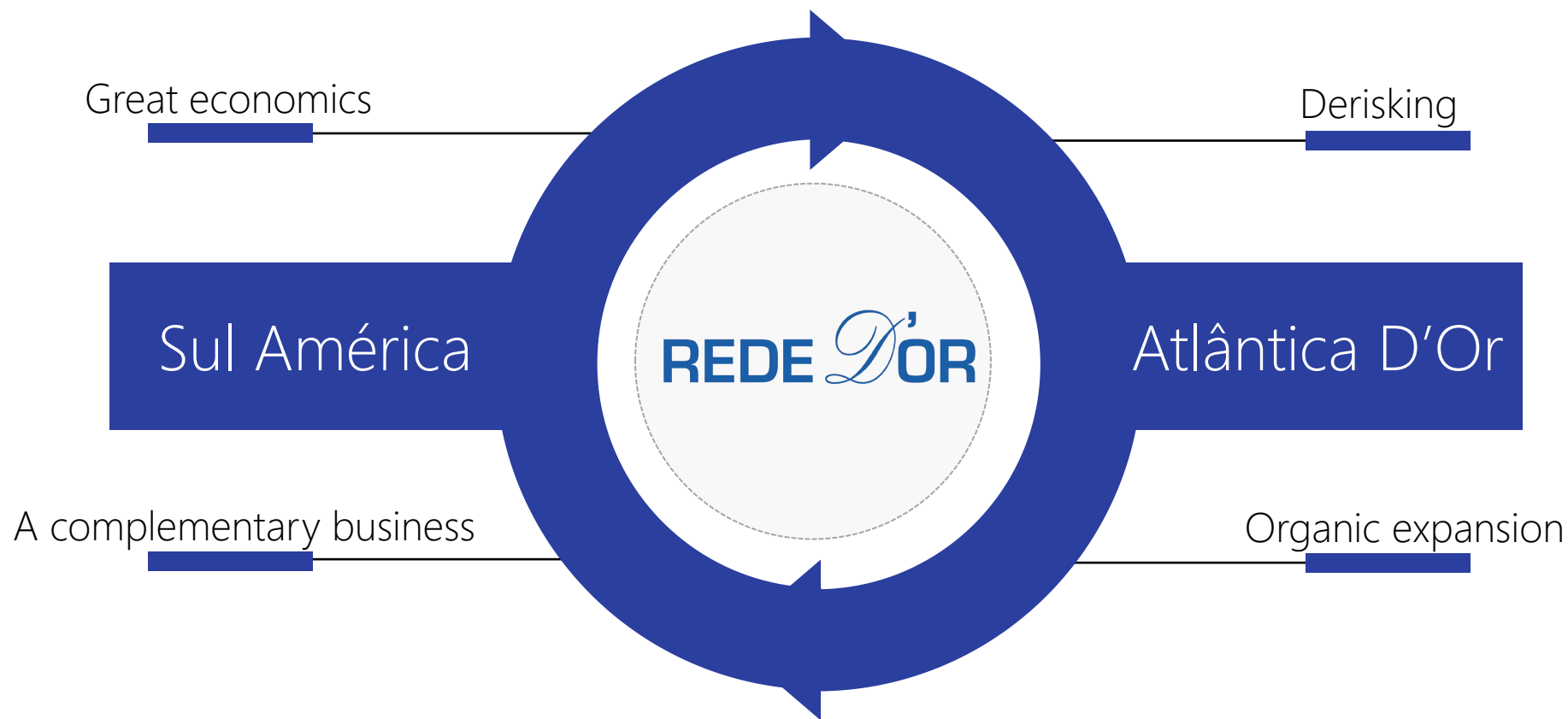
Expansion Ahead: ***Big Dream***

Derisking the Operation and Gaining Share



What is the *Big Dream* for Rede D'Or?

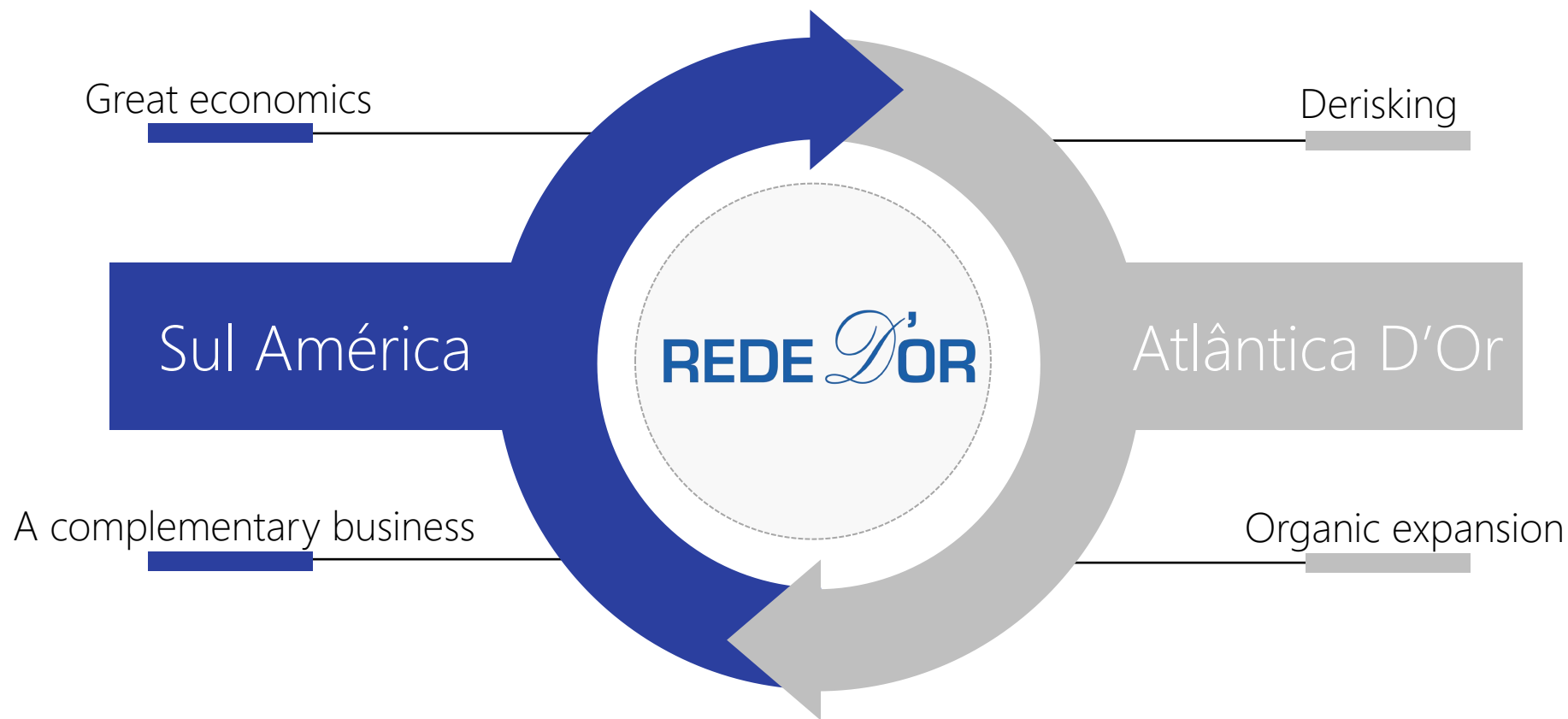
The company has changed the game with its recent deals





What is the *Big Dream* for Rede D'Or?

The company has changed the game with its recent deals





The masterstroke: acquiring Sula in an all-stock deal

The deal that took Rede D'Or to the next level did not cost a dime

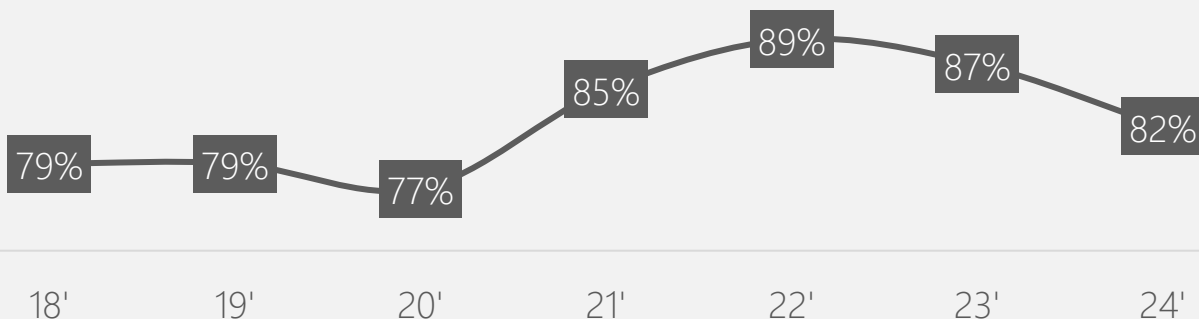
Rede D'Or took advantage of its high valuation at the time...

RDOR3 [BRL]



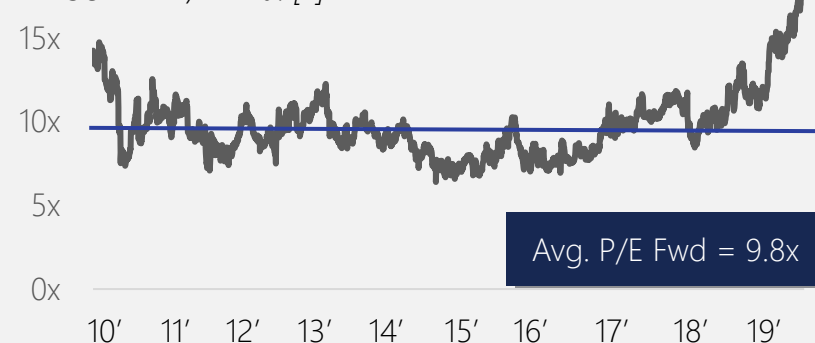
...when Sul América was in trouble with rising MLR!

Sul América's Yearly Medical Loss Ratio exc. Changes in Insurance Technical Provisions [%]



So adjusting for normal times, it was a great deal!

SULA11 P/E Fwd. [x]



$$\begin{array}{lcl} P & = & \text{New Shares} \times \text{Share Price} \\ E & = & \text{2023 Rev.} \times \text{His. Net Margin} \end{array}$$

$$\text{P/E Fwd 23'} = 7.7x$$

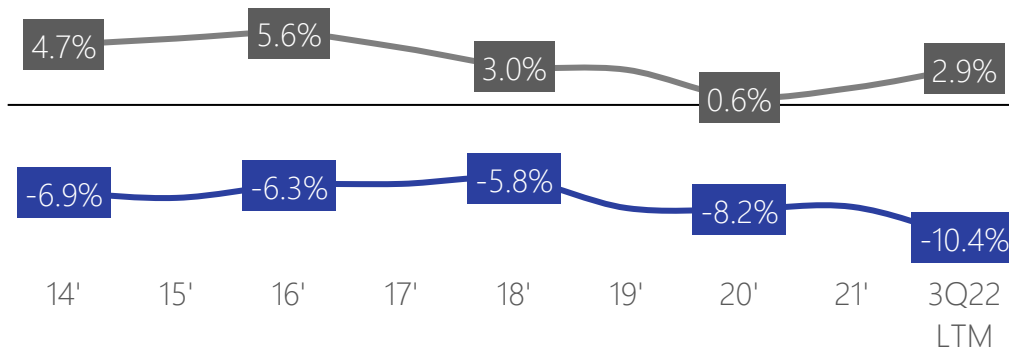


Combining strengths, building financial harmony

The Sul América deal reduces significant risks and challenges for Rede D'Or

Insurance is a float business, hospitals are a leveraged business...

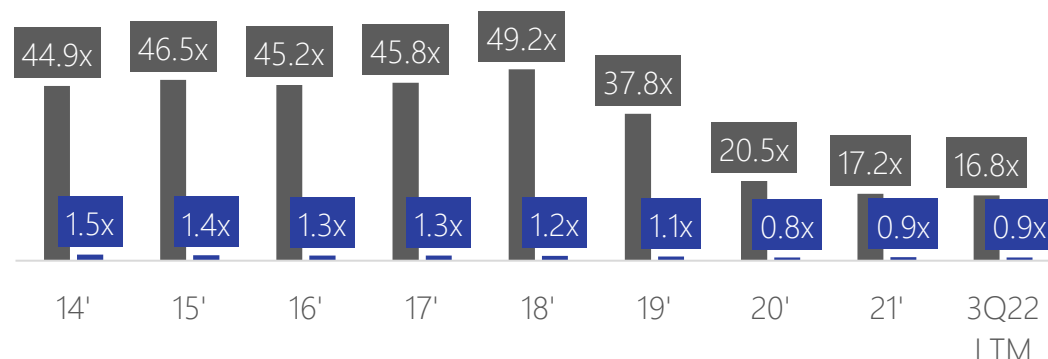
Financial Result / Net Revenue [%] — Sul América — Rede D'Or Hospitals



...one is asset light, the other is capital-intensive...

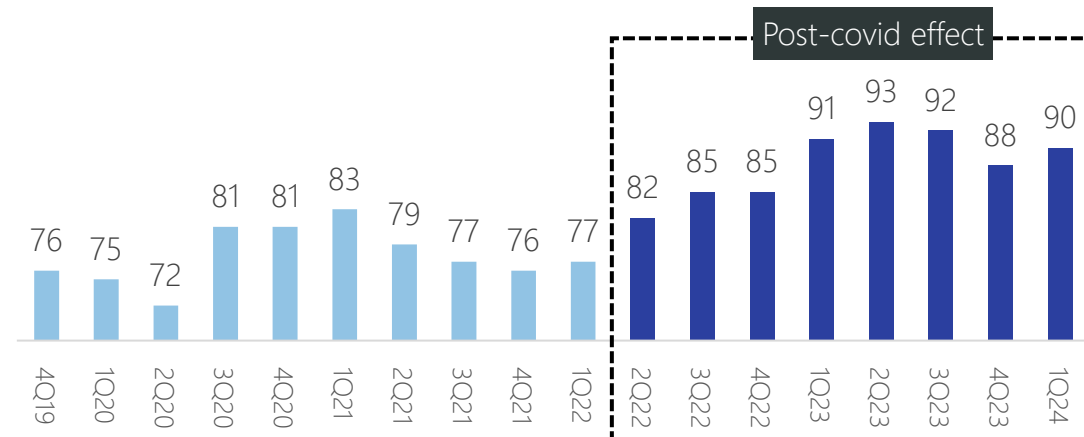
Net Revenue / Fixed Assets [#]

■ Sul América ■ Rede D'Or Hospitals



...and by being the paying source, insurers hold the power

Operators' days payable outstanding LTM quarterly [#]



“By having both an insurer and a hospital, we have a **natural hedge**. We are **less exposed** to potential **payment delays** and **claim denials** from insurers.”



Paulo Moll – CEO at Rede D'Or

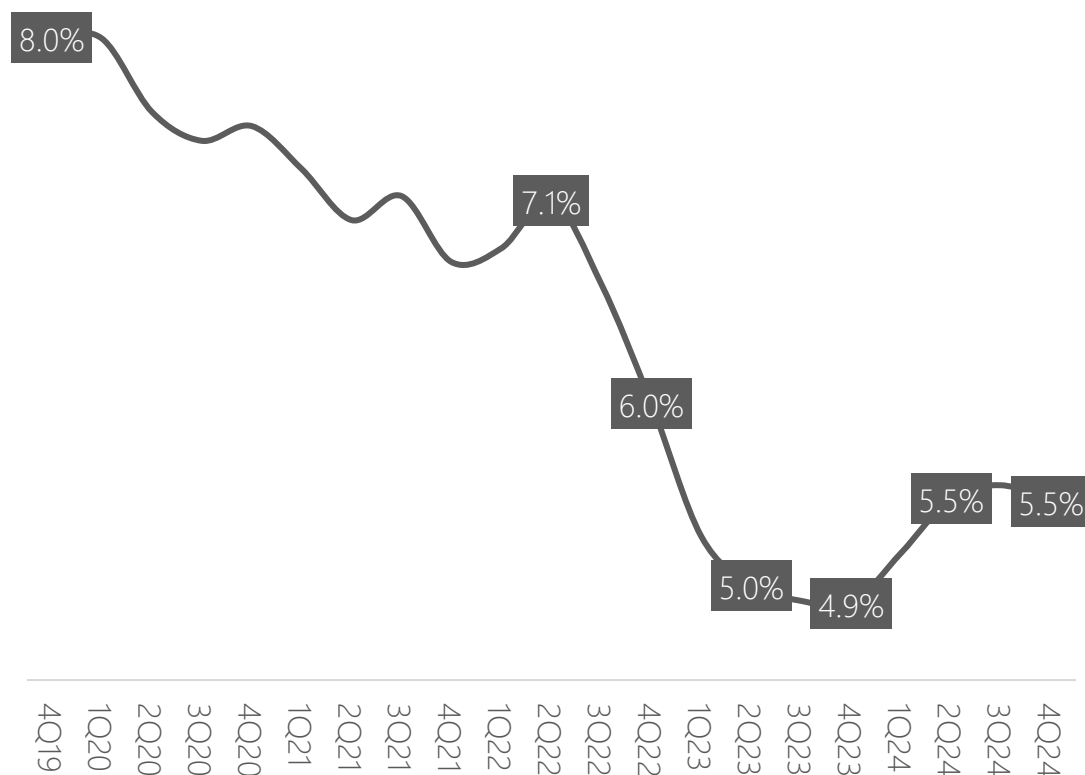


SulA is also improving powered by D'Or

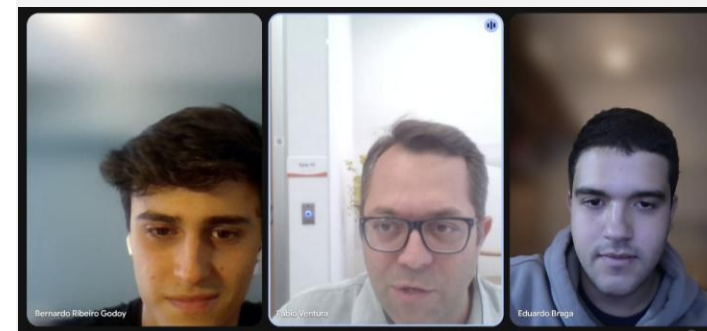
The operator has improved margins and derisked its operation with the deal

The numbers show synergies...

SulA Cash G&A Expenses / Net Revenue LTM Quarterly [%]



...but there is even more to it!



Porto Saúde

SulAmérica

2003

2019

22 Years at insurance sector

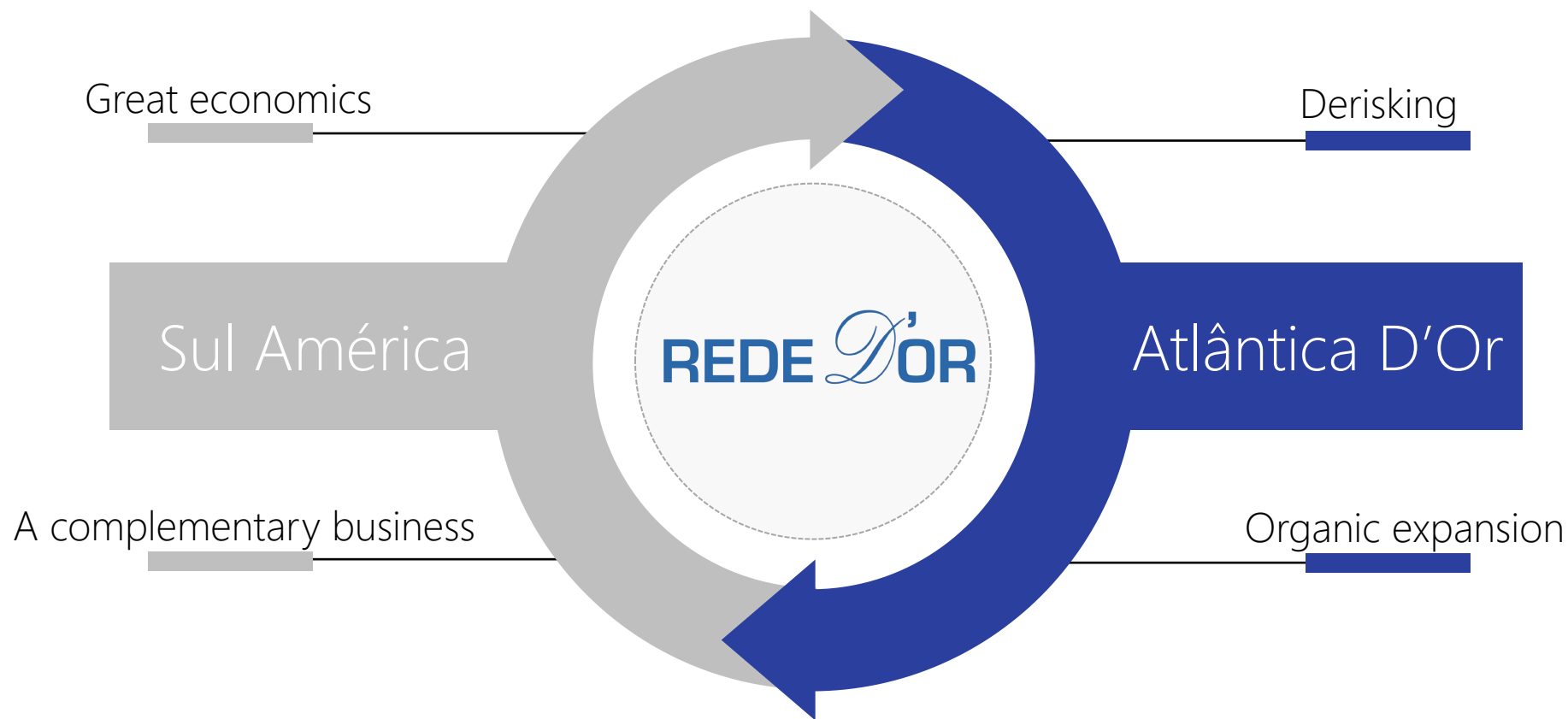
"With the deal with Rede D'Or, **Sul América was able to clean its hospitals base** from bad contracts,. In addition, **the company now has a much better position to negotiate with new hospitals**, as it knows exactly what rivals are paying for them and has the RDOR network accredited."

Fabio Ventura – Commercial Superintendent at Sul América



What is the *Big Dream* for Rede D'Or?

The company has changed the game with its recent deals



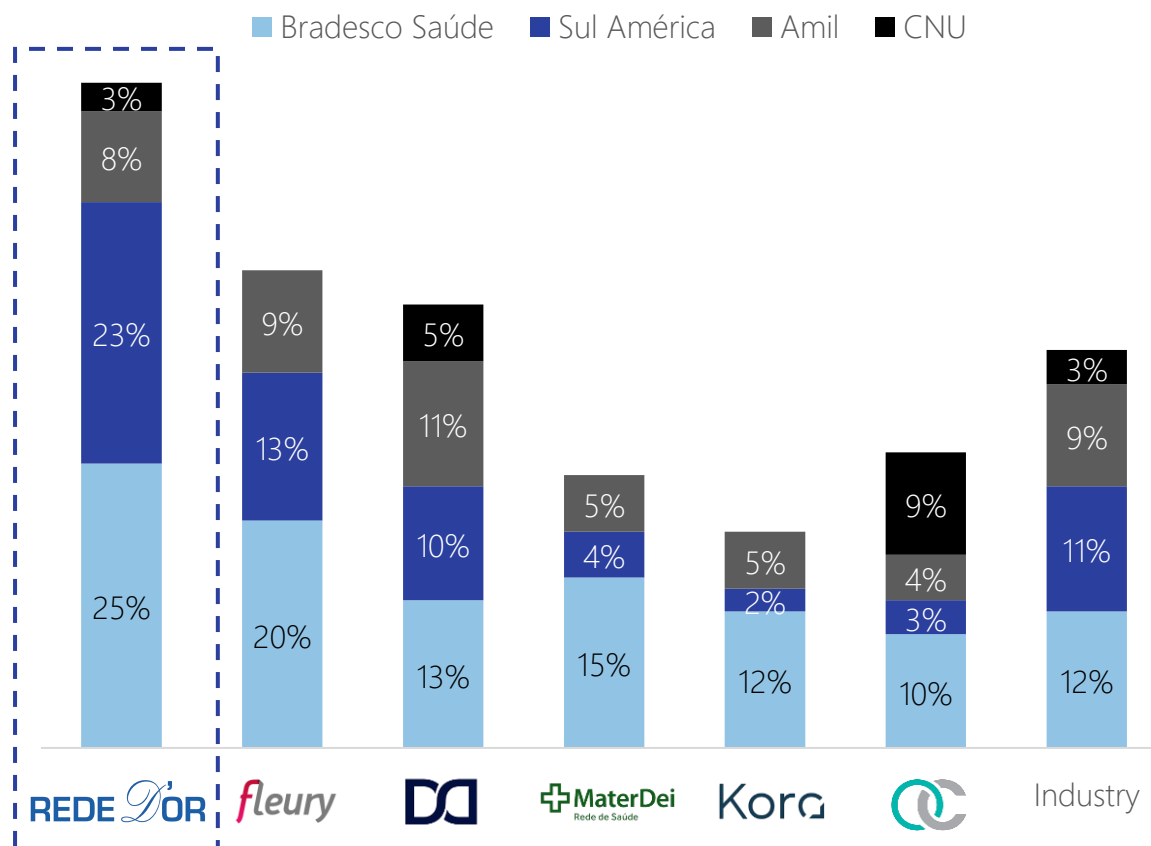


Taking out the trash: A payor detox

The combination of the recent JV and SulA's ownership allows greater selectivity in its payor base

- The company generates nearly **half of its revenue** from "in-house" sources...

Share of revenue per company by insurer [%]



...and has been cutting ties with poor-quality payors.

ESTADÃO

08/11/2023

Rede D'Or discontinues contracts with three health insurers

InfoMoney

08/01/2025

Rede D'Or will no longer treat patients from Unimed Ferj

exame.

30/07/2024

Rede D'Or terminates Amil contracts in three hospitals in Rio

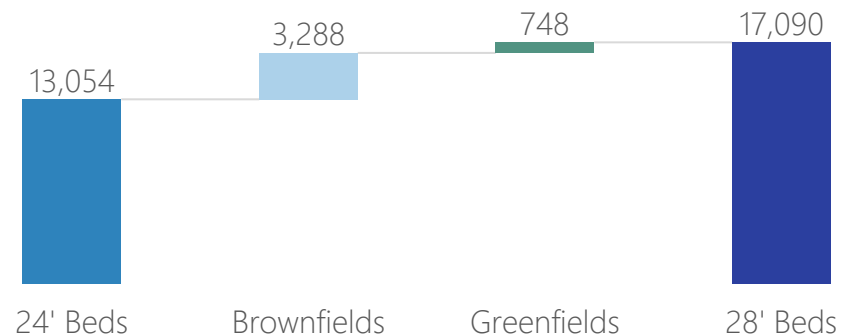


What's next for Rede D'Or?

The company pursues aggressive organic growth, mainly through brownfield projects

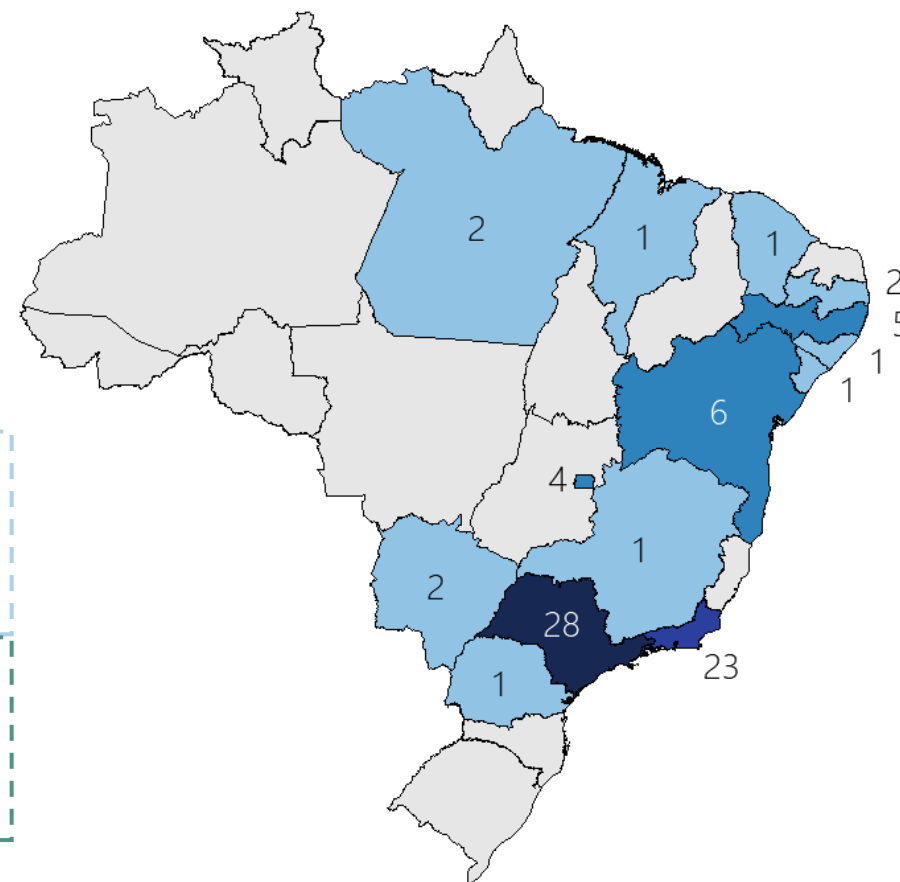
 Brownfields: less risk and better returns...

Company's bed capacity expansion guidance [#]

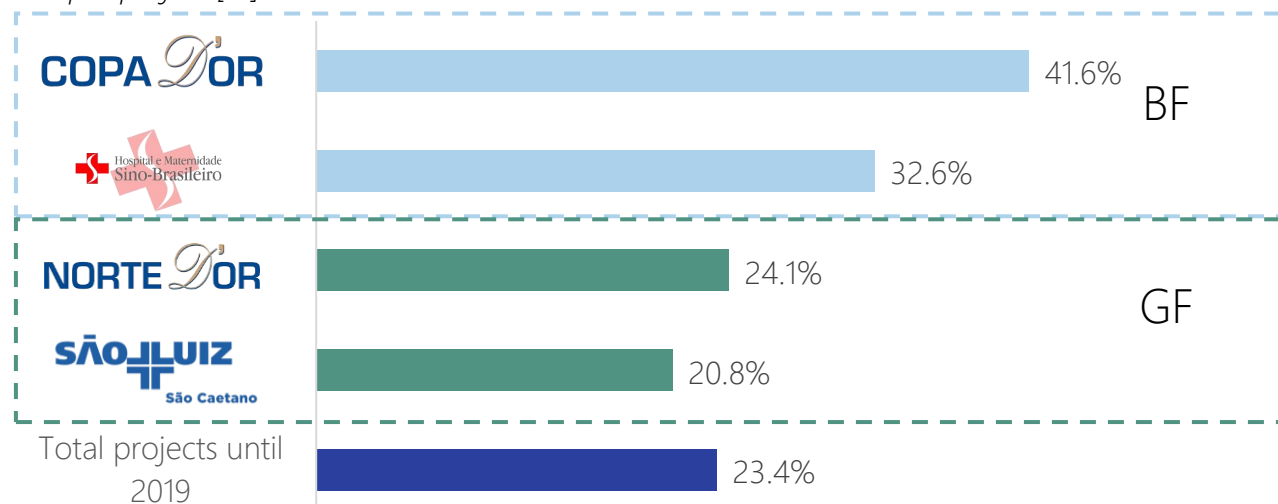


 ...mostly focused on Rio de Janeiro and São Paulo

Rede D'Or's hospitals per state [#]



ROIC per project [%]



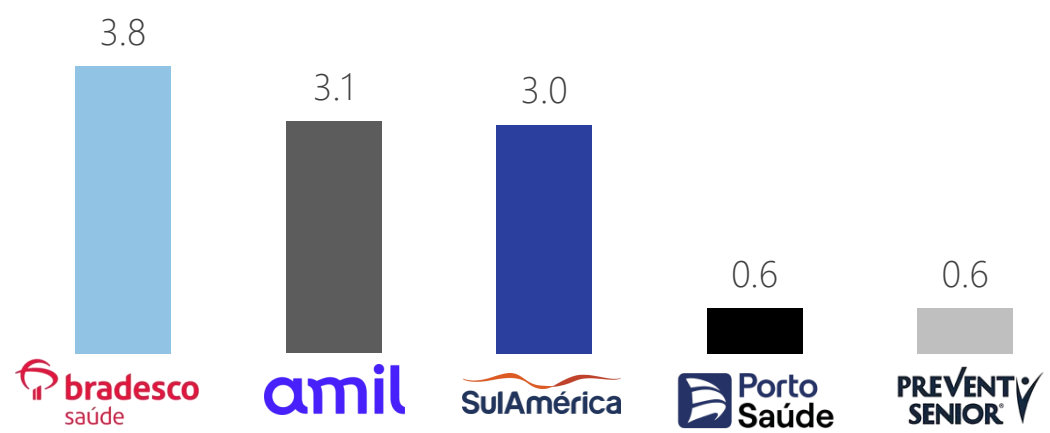


Redrawing market boundaries with insurer deals

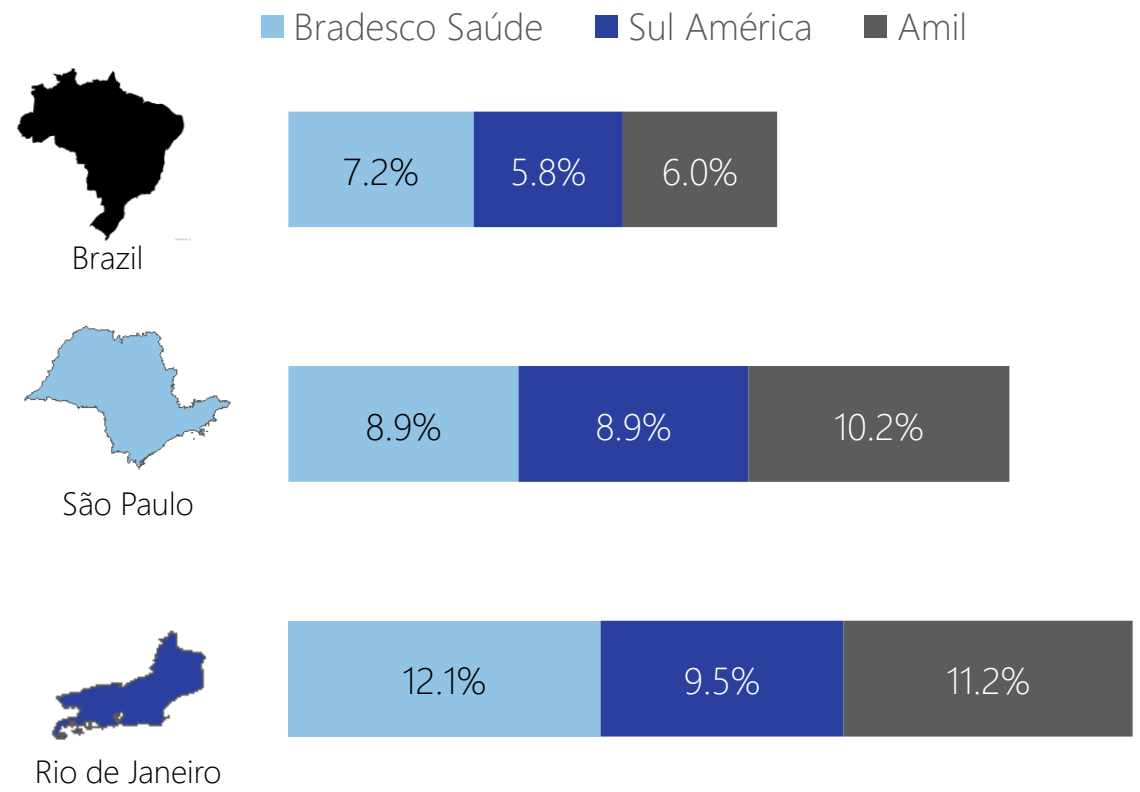
Rede D'Or has developed relationships that ensure its accreditation for a broad base of beneficiaries

Through the acquisition of Sul América and the Atlântica D'Or joint venture, the company can open hospitals with the **guaranteed beneficiary base of two of the largest insurers in its segment**, redefining its addressable market

Medical beneficiaries in jan/25 by company [mn]



Share of medical beneficiaries in jan/25 by region [%]



"Moving into adjacent segments reflects a **fundamental willingness to adapt** in order to escape complacency and the gravitational pull that typically affects market leaders. (...) At times, the market development strategy emerges from a **redefinition of the addressable market scope**"



Dynamo letter 107 "Growth Pathways"

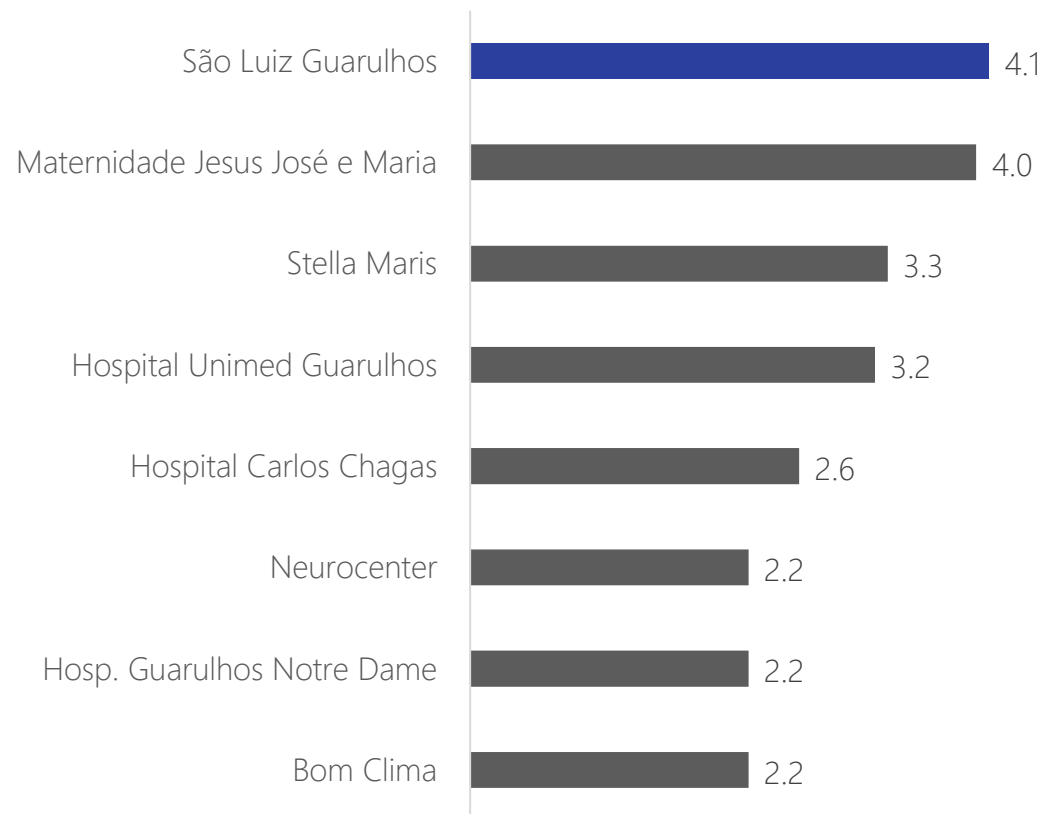


Where can Atlântica D'Or set sail to?

We started by analyzing its recent openings

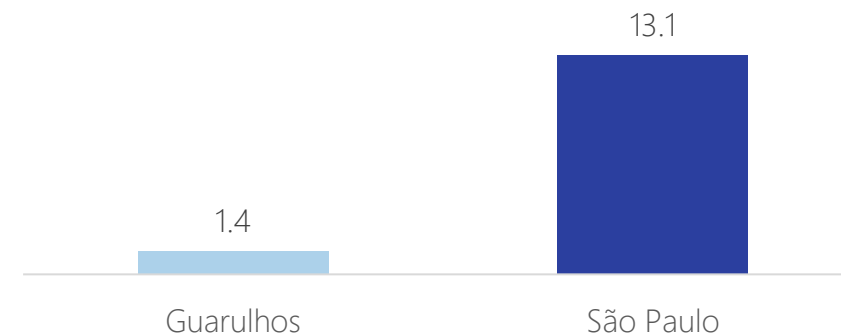
The success of its hospital in **Guarulhos** is evident...

Google Rating [# / 5]



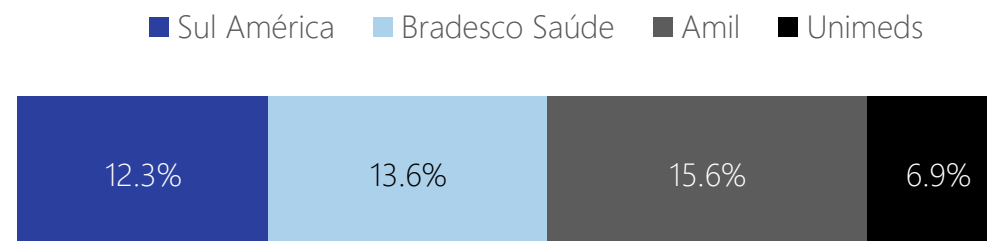
...and the opening was supported by a city with **unmet demand** and a **lack of reference hospitals**...

Private and Philanthropic Beds per 1000 beneficiaries by city in 2023 [#]



...combined with a **strong presence** of **SulA** and **Bradesco** in the region.

Share by operator in the Metropolitan Region in 2023 [%]



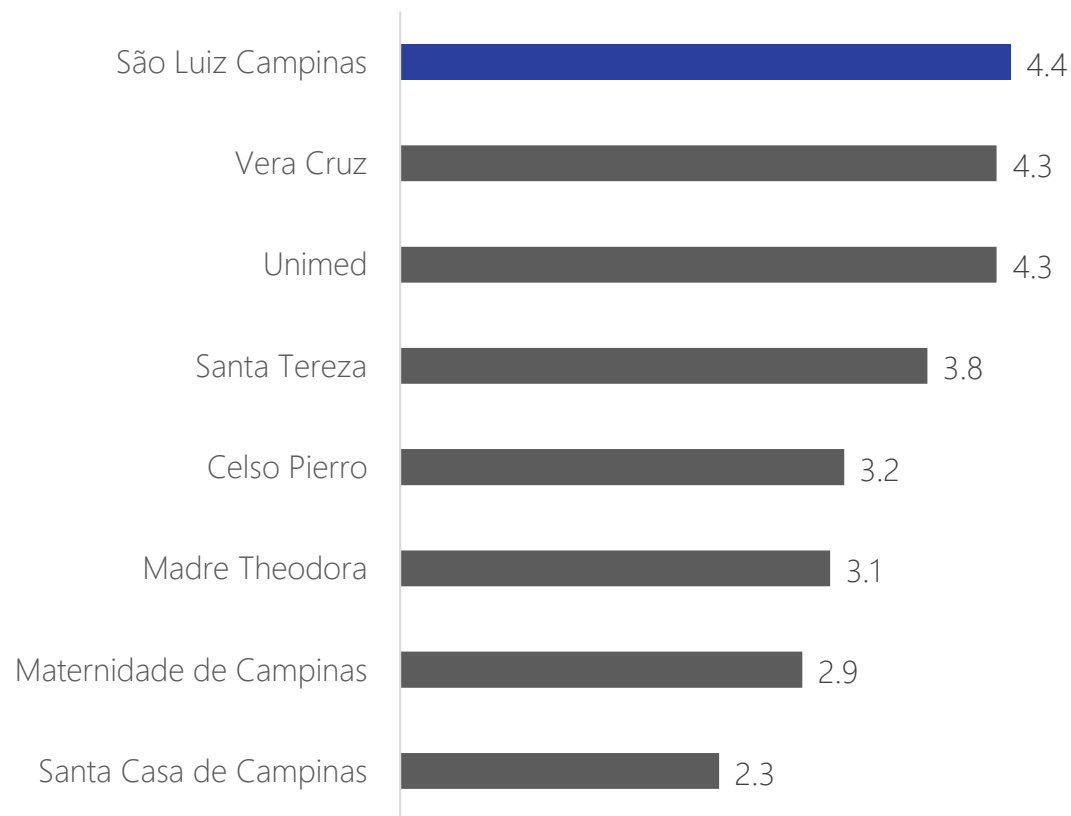


Where can Atlântica D'Or set sail to?

We started by analyzing its recent openings

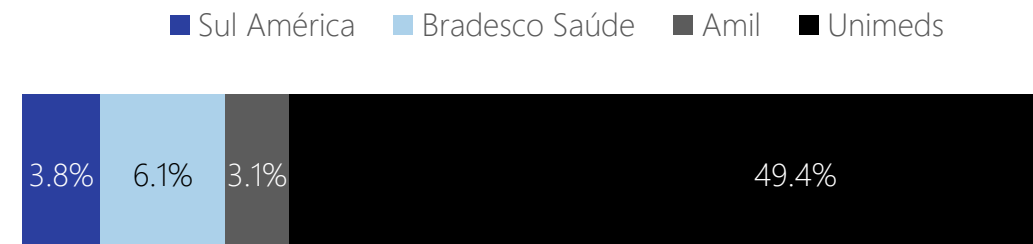
The success of its hospital in **Campinas** is evident...

Google Rating [# / 5]



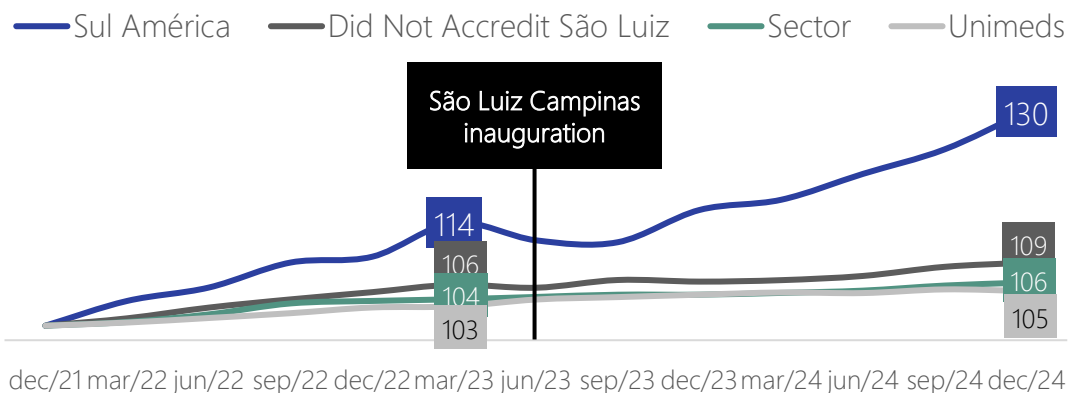
...however, the **presence of insurers was not the same...**

Share by operator in the Metropolitan Region in 2022 [%]



...but it was **shifted** through aggressive **commercial strategies**.

Number of beneficiaries in Campinas per operator [Dec. 2021 = 100 base]

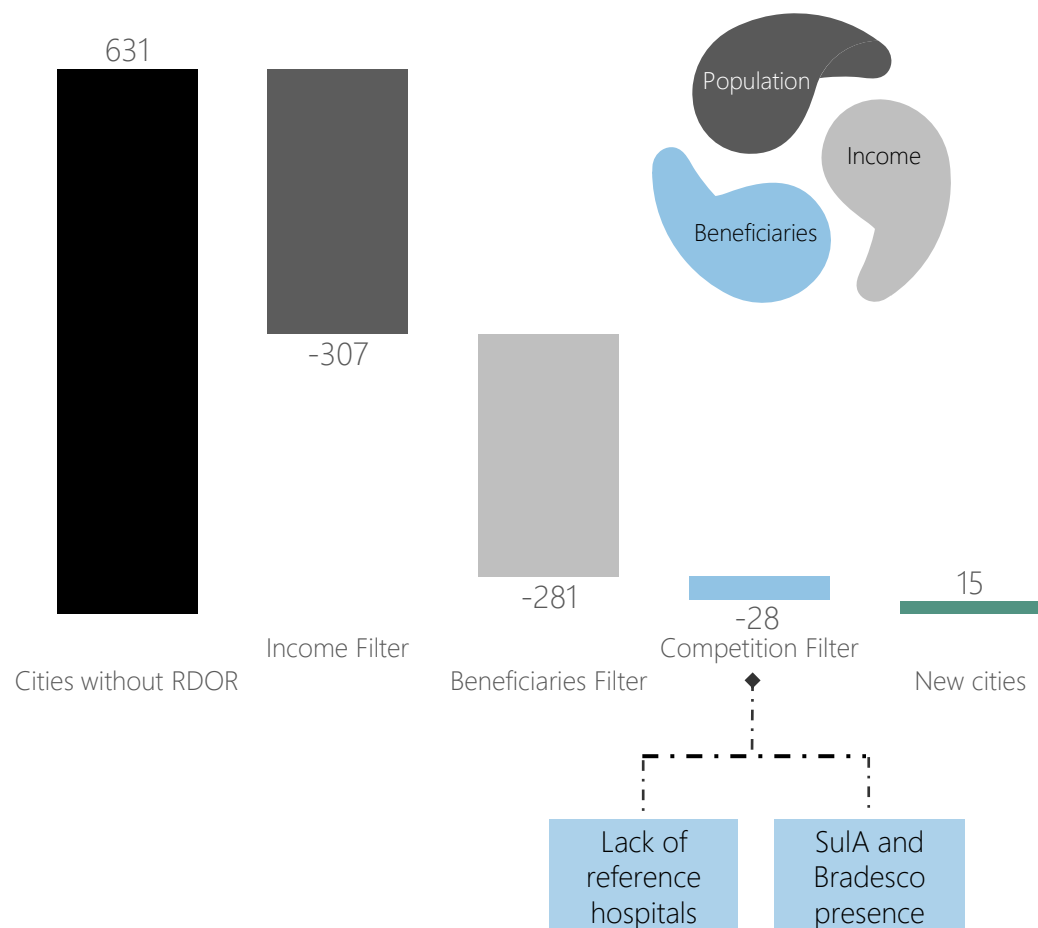




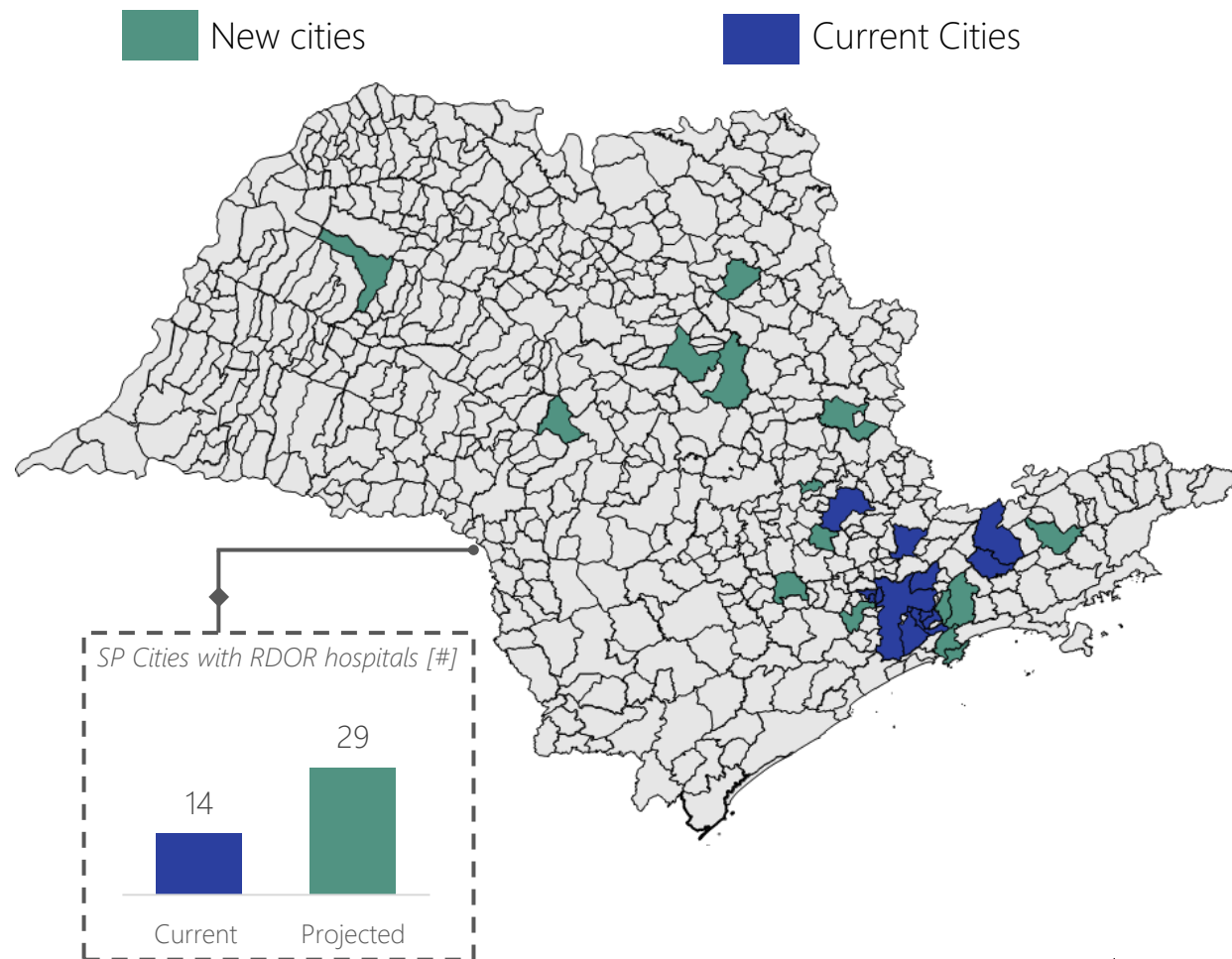
Where can Atlântica D'Or set sail to?

We sought to understand which cities the company could expand into in its key market

With 3 main filters, we found 15 new attractive cities



Expected new cities





Navigating with Atlântica: Is organic expansion worth it?

We sought to understand the returns of the organic expansion and how the JV impacts them

To better understand the JV, we dove into the unit economics of a hospital with 180 beds...

Main Assumptions



Capex per bed: 2.0 BRL mn



5% ticket growth



85% mature occupancy rate



25% mature EBITDA margin



5% perpetuity growth



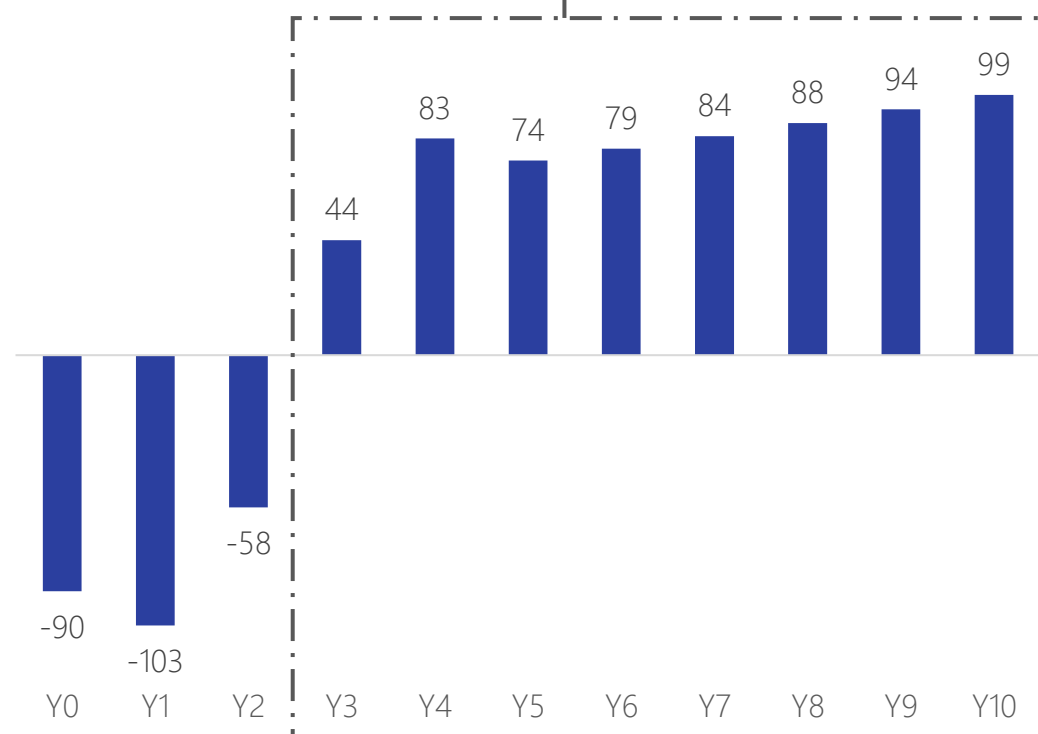
Initial capex: 75% Debt

...and we saw it as a good investment...

FCFE [BRL mn]

IRR = 25.2%

Mature unit





Navigating with Atlântica: better together

Rede D'Or can derisk much of its operation with Bradesco and SulA on its side

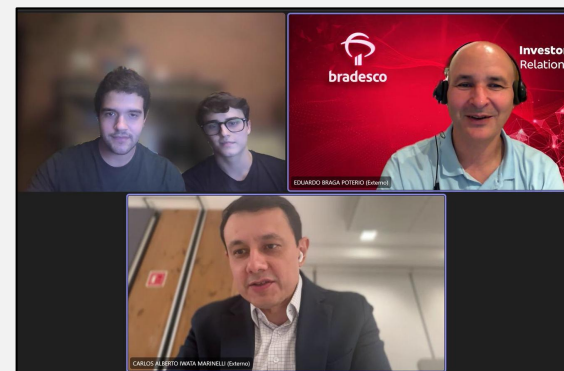


...so we ran sensitivities to see the impact of maturation time and margins on the investment...

IRR Sensitivity Analysis [p.p ; #]

EBITDA margin delta	Years to mature					
	Nominal IRR: 25.2%	0	1	2	3	4
	(-) 4 p.p	26%	22%	21%	17%	15%
	(-) 2 p.p	30%	25%	23%	18%	16%
	0 p.p	34%	28%	25%	19%	17%
	(+) 2 p.p	38%	31%	28%	20%	18%
	(+) 4 p.p	42%	35%	30%	22%	20%

Without scale and accreditation from the operators, there would be value destruction.



Grupo Fleury

bradesco
seguros

2005

2021

20 Years at Healthcare sector

"Atlântica D'Or combines Rede D'Or's **strong hospital brand** with Bradesco's **extensive beneficiary base** to create a powerful healthcare platform, enabling a **profitable and less risky expansion** by leveraging existing demand and brand trust, **especially in regions lacking reference hospitals.**"

Carlos Marinelli – CEO at Bradesco Saúde

Financials & Valuation



We see that Rede D'Or is an outstanding business, but how much is this golden company worth?



REDE *D'OR*



Revenue Build-Up

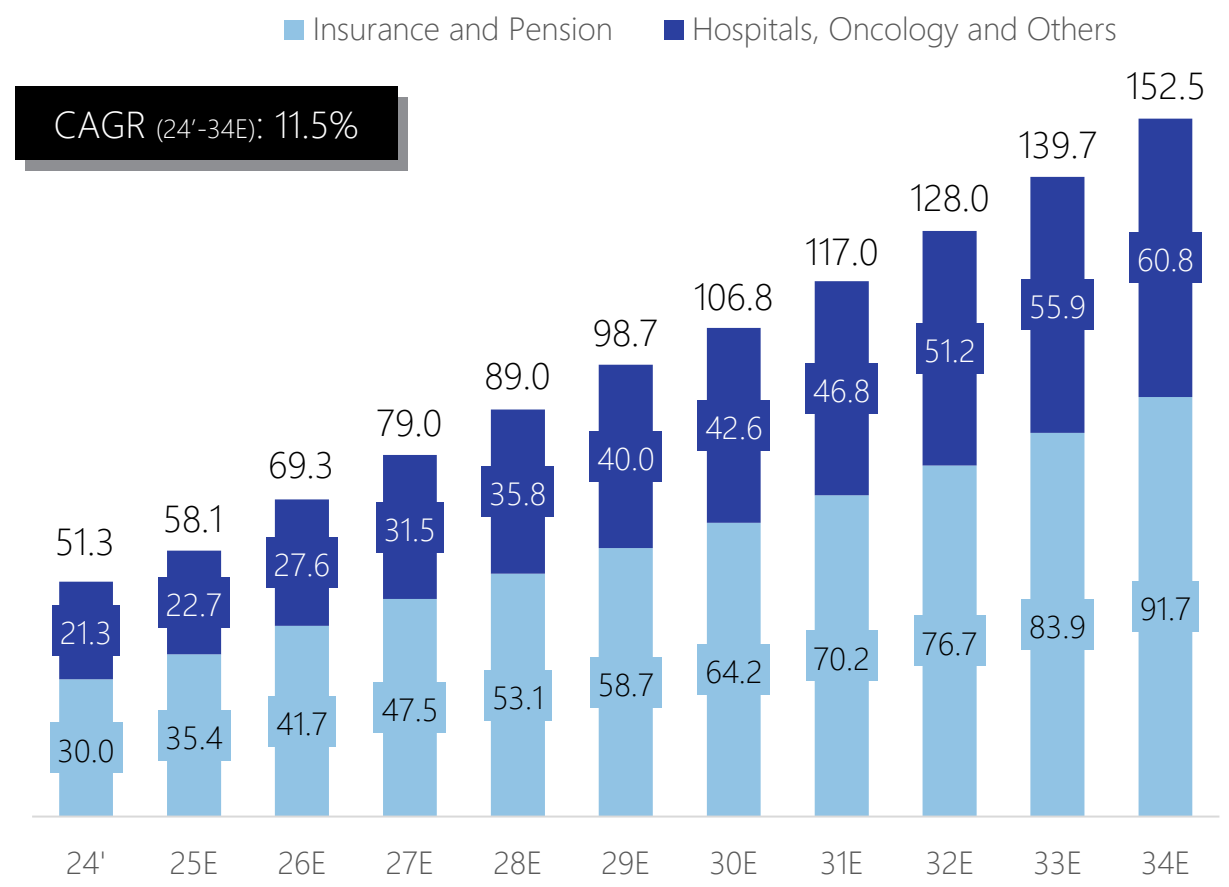
Topline growth will come from both of the company's business segments

- Revenue projections were based on these assumptions:
- With a **11.5% CAGR**, reflecting the its growth potential

Main Revenue Assumptions

-  Beds addition with an 10% haircut to the company's guidance
-  Hospitals ticket growth: IPCA + 1%
-  Occupation rate increasing to 87% by 2034
-  Oncology: 11.5% of hospital services revenues
-  Sula: 3% CAGR in medical beneficiaries
-  Sula Medical ticket growth: IPCA + 4%

Net Revenue [BRL bn]



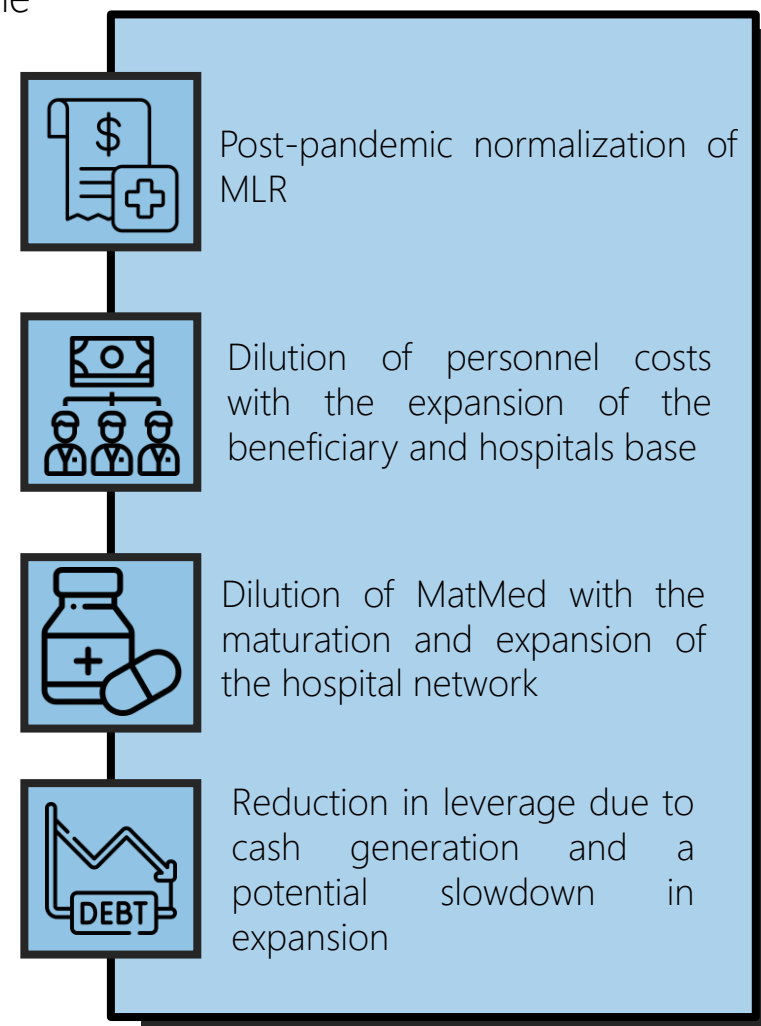
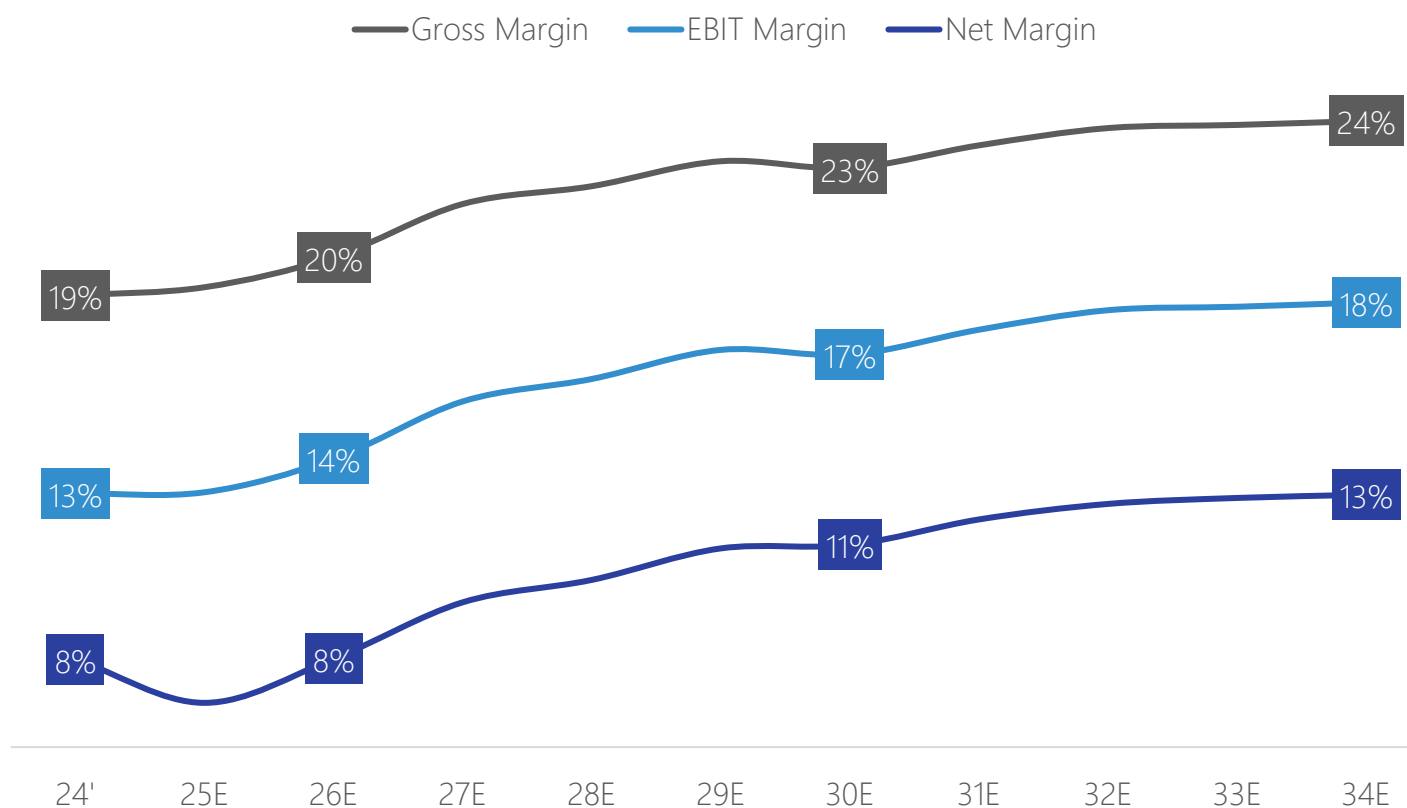


Margins evolution

We do not see huge changes in Rede D'Or's margins for the future

- We see a slight margin improvement due to **economies of scale** and changes in the company's capital structure in the long term.

Margins evolution [%]





DCF: 40.7 Target Price with a 37.4% Upside

RDOR proved to be a great company, but is it trading above its fair value?

 A strong cash generation going forward...

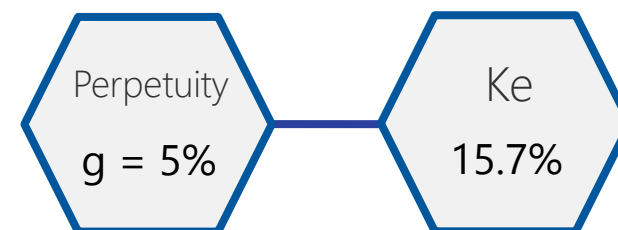
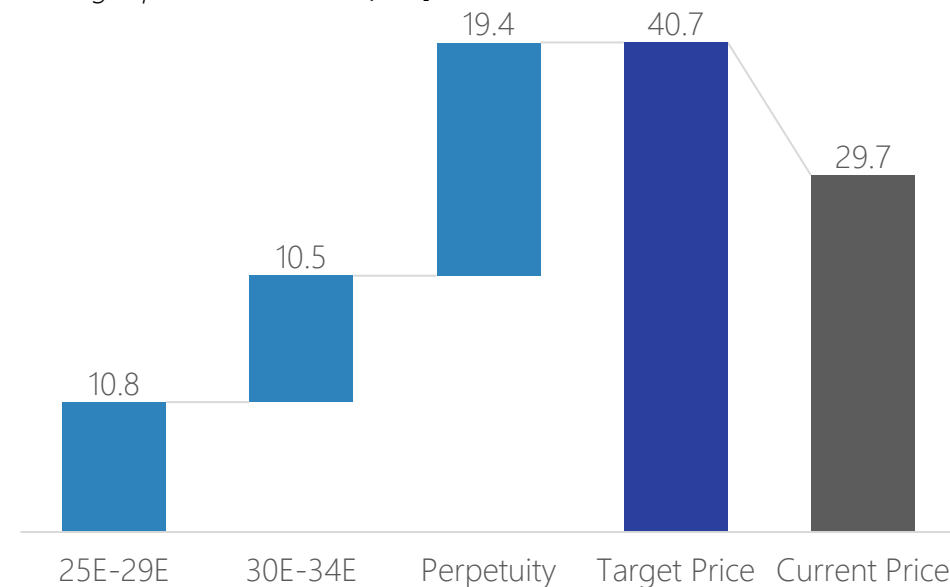
FCFE [BRL bn]

CAGR (25E-34E): 16.8%



 ...but mostly based on its expansion ahead

Target price breakdown {BRL}

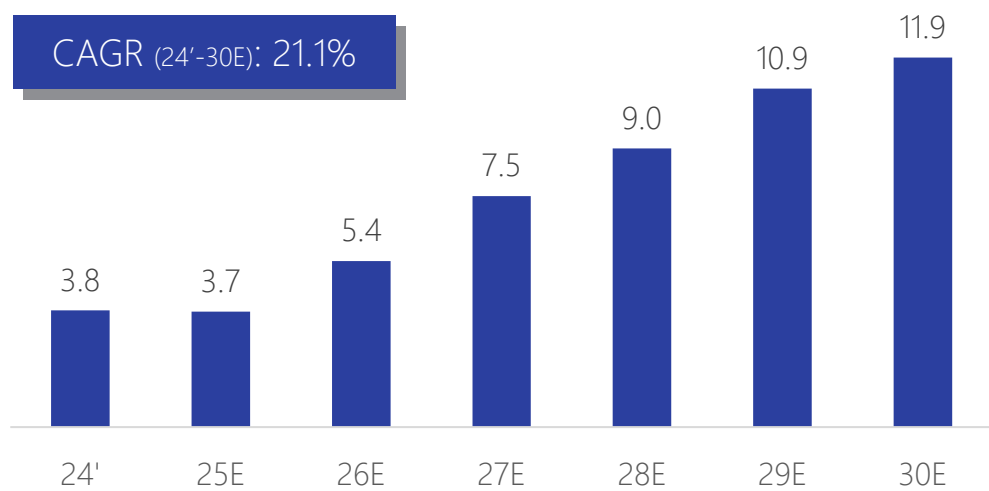


23.4% IRR: Signaling Attractive Returns

We also analyzed and sensitized our IRR analysis with an exit multiple in 30E

5y IRR indicates a good investment...

Adj. Net Income [BRL bn]



Exit P/E Fwd 1y = 14x

IRR = 23.4%

Spread = 7.7%

And our sensitivity analysis reinforces our thesis...

IRR Sensitivity Analysis [% ; x]

5y Exit P/E fwd 1y

Nominal IRR: 23.4%	9.5x	11.0x	12.5x	14.0x	15.5x	17.0x	18.5x
12.1%	6%	9%	11%	14%	16%	18%	19%
15.1%	9%	12%	15%	17%	19%	21%	23%
18.1%	12%	15%	18%	20%	22%	24%	26%
21.1%	15%	18%	21%	23%	26%	28%	30%
24.1%	18%	21%	24%	27%	29%	31%	33%
27.1%	21%	24%	27%	30%	33%	35%	37%
30.1%	24%	28%	31%	33%	36%	38%	41%

IRR – Ke > 4% in 63% of the stressed cases!

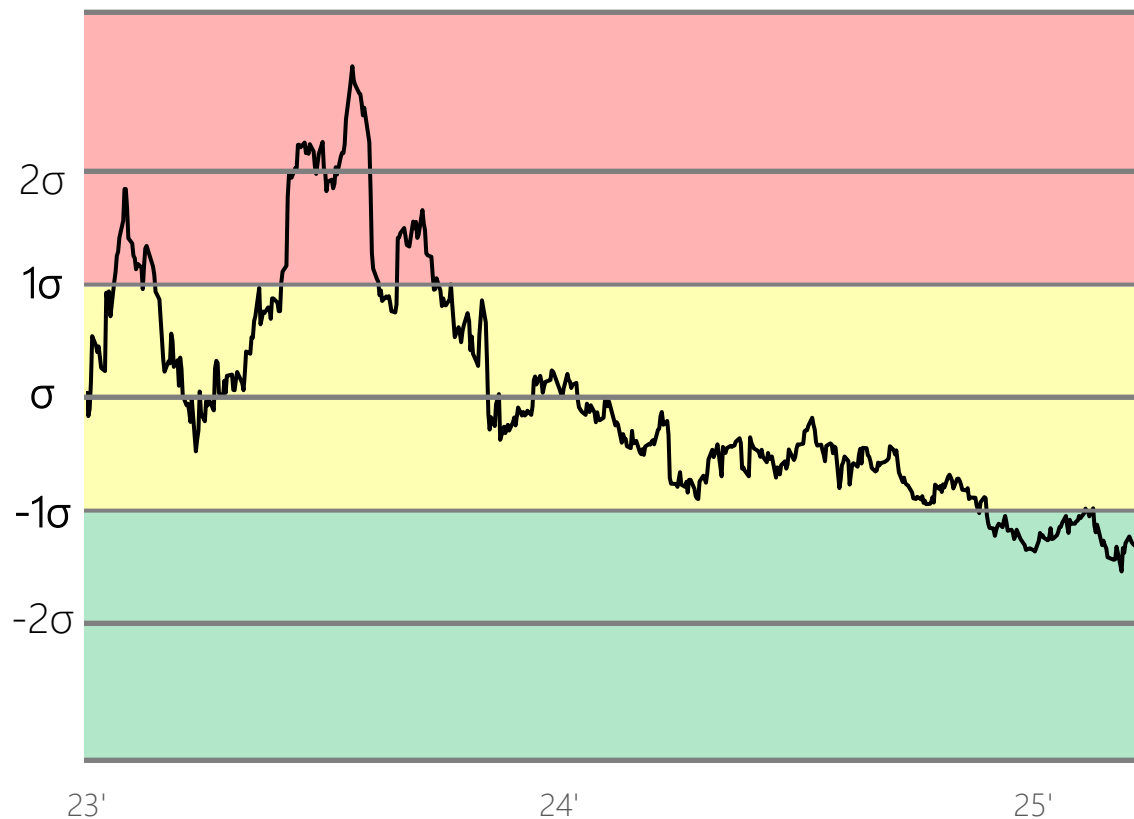


What about multiples?

We believe RDOR is trading at a discount

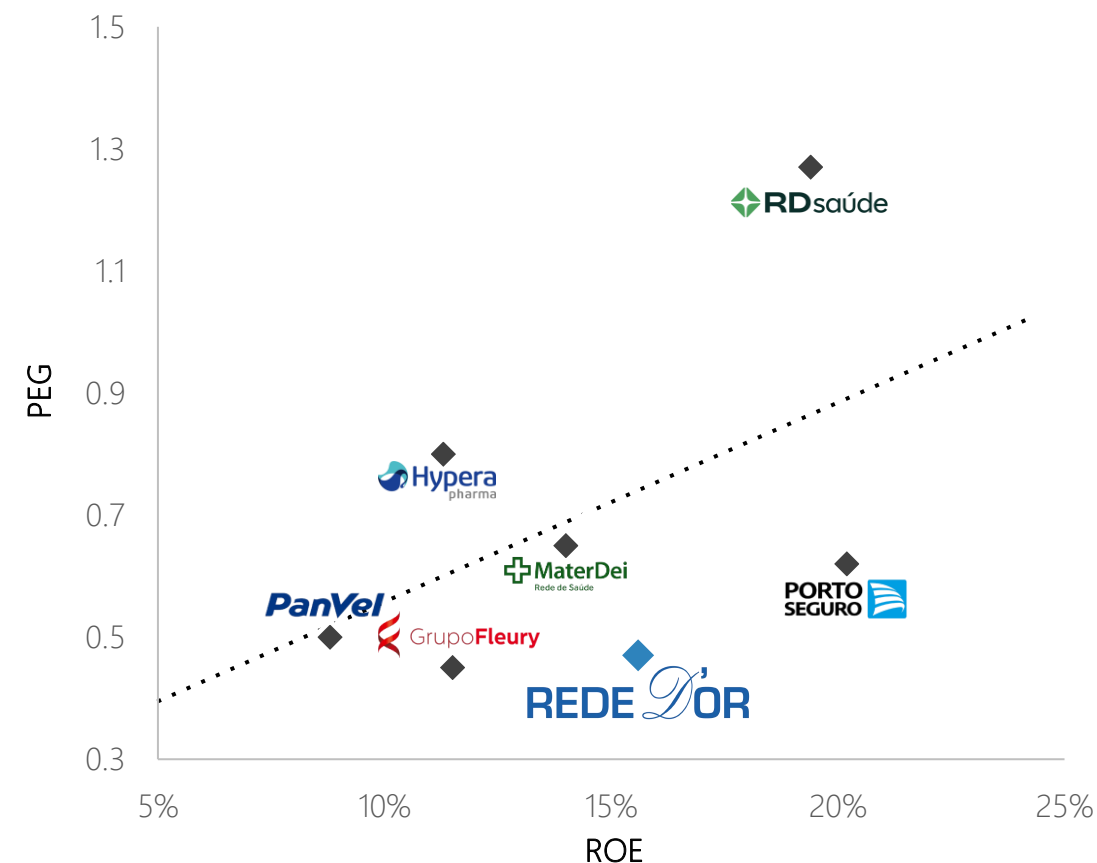
- Rede D'Or's P/E is below its post-covid and Sula acquisition average...

RDOR3's 1Y Fwd P/E evolution [x]



- ...and when looking at companies with similar returns in the industry, it seems even more attractive

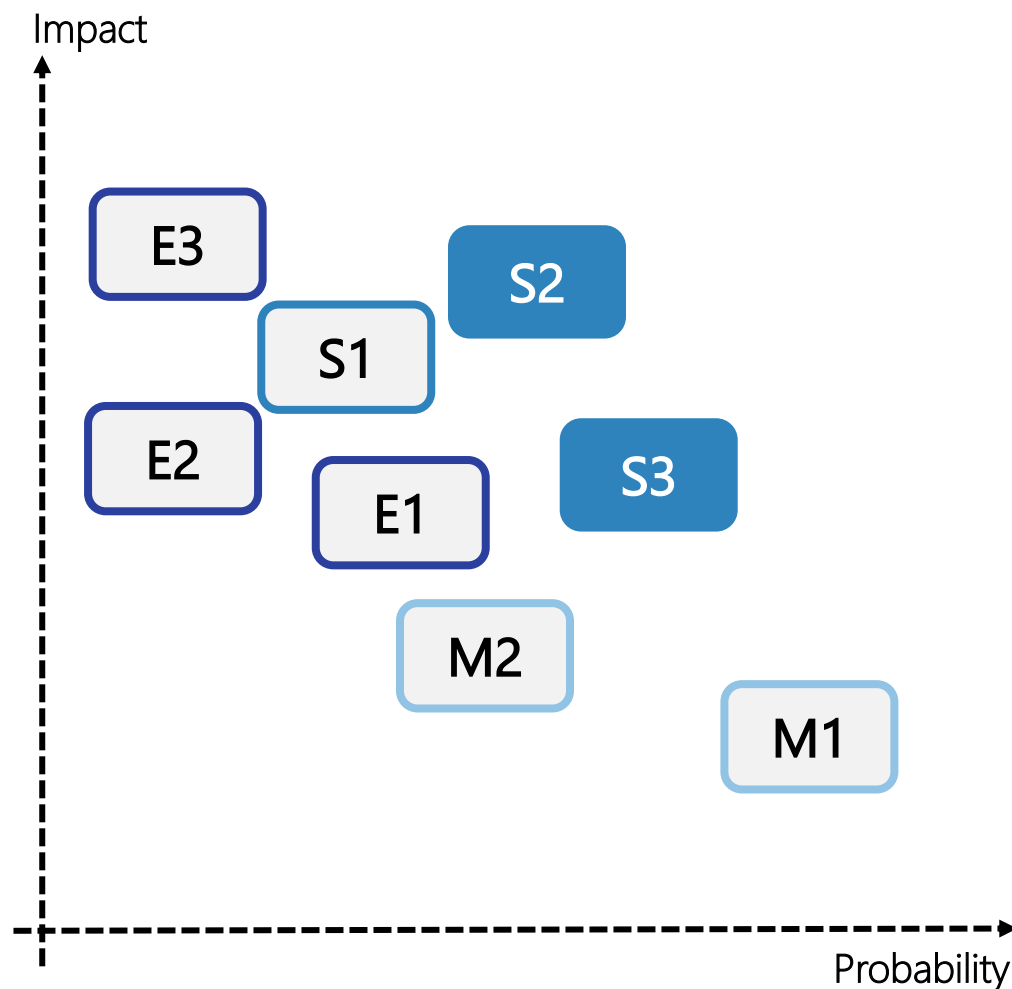
PEG Ratio 1Y Fwd [x] vs ROE 1y Fwd [%]





"The most important thing is...Understanding Risk"

What should investors watch out for?



E1: Higher difficulties with greenfields

E2: New acquisitions for worse assets

E3: Deterioration in the relationship with Bradesco

Enterprise Risks

S1: Changes in regulation for insurers

S2: SP philanthropic hospitals strengthening

S3: Definitive cut in ties with Amil

Industry Risks

M1: Higher unemployment rate

M2: End of JCP tax benefits

Macro Risks

But *why* invest in Rede D'Or *now*?

We see RDOR trading at a discount from its intrinsic value and with lower risk than ever before

◆ *Sucesso Made in Brasil*: Top player in the industry

◆ *Outliers*: Seizing Golden Opportunities ahead

◆ *Big Dream*: Derisked operation with insurer deals

Based on:

DCF: 37.4% Upside

Current Price: BRL 29.65
Target Price: BRL 40.73

IRR (5-Year): 23.4%

Ke: 15.7%
IRR - Ke: 7.7%

We are open for the Q&A!

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Call C – Henrique Salvador, Mater Dei

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Call E – Eduardo Almeida, Amil

Call F – Marcio Mendes, Fleury

Call G – Jaques Rosenzvaig, AHFIP

Call H – Fabio Ventura, Sul América

Call I – Harold Takahashi, Ex-Fleury

Call J – Luciano Ávila, AIO Corretora

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Debt Build-Up

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Multiple premium vs. peers

Private investment and duration

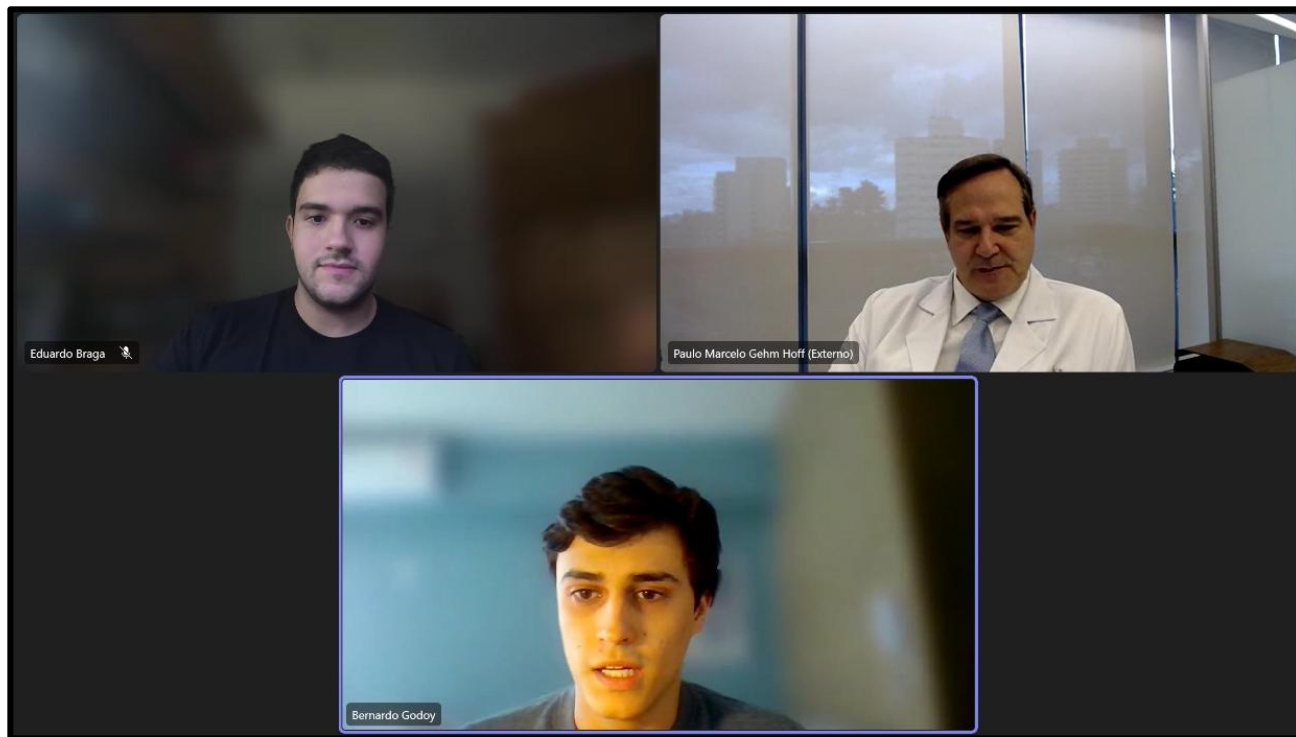
EBITDA and Net Debt

MLR projection



Call A – Dr. Paulo Hoff, Oncology Chief at Rede D'Or

Conversation happened on 07/04



- Rede D'Or has invested in bringing in **top technology to its hospitals**, mostly for the Star brand. This includes the **nano-knife, the cyber-knife and the gamma knife treatments**;

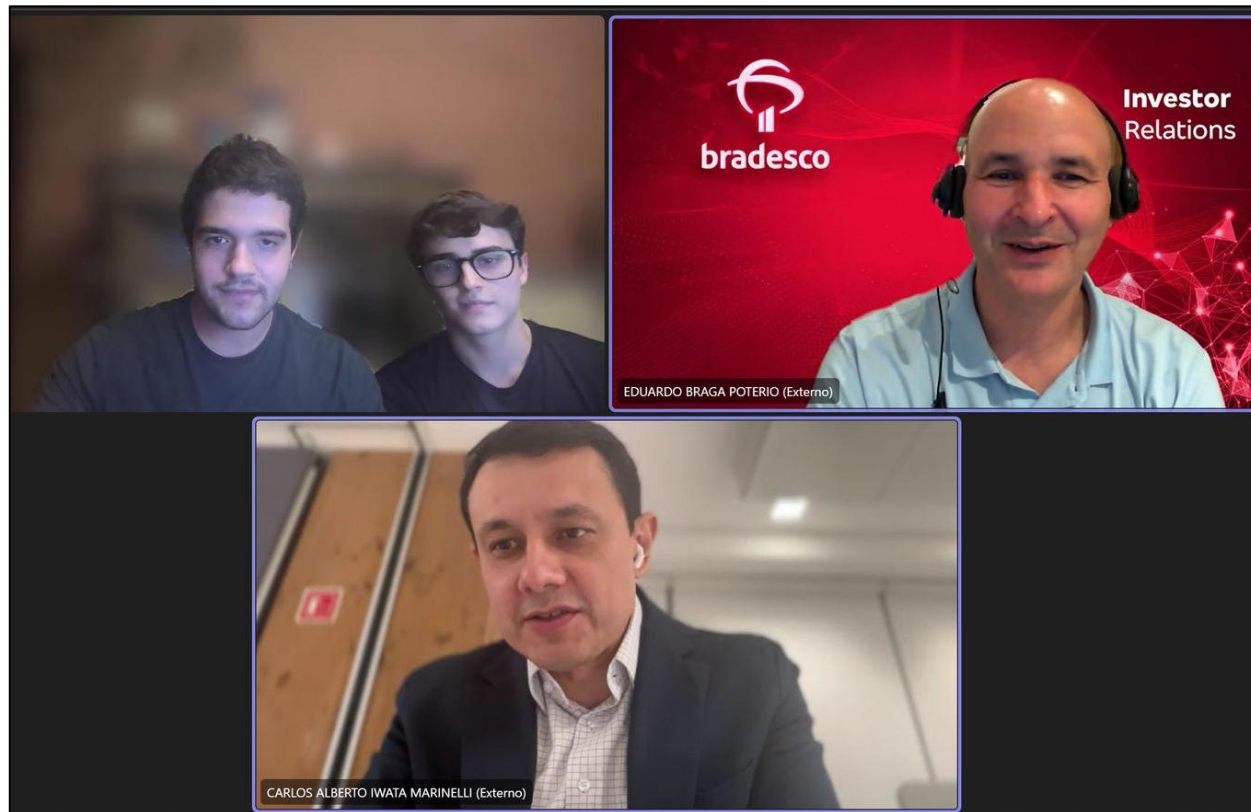
- The **top doctors** the company hires **attract other doctors**, as they want to work with the best in their fields;

- For **transplants**, for example, it is wise to create a **reference hospital** for it, such as the São Luiz Itaim, **where you can have all your best tech**.



Call B – Carlos Marinelli, CEO at Bradesco Saúde

Conversation happened on 28/03



- The balance of power with Rede D'Or is different from other players, as **beneficiaries want to have access to the company's hospitals;**

- Atlântica D'Or openings will focus on locations with **higher income but without a reference hospital yet**, like Campinas and Guarulhos;

- **Atlântica is not a hospital operator**, but it has projects and partnerships with other service providers such as Albert Einstein, Mater Dei, Grupo Santa and Fleury.



Call C – Henrique Salvador, Chairman at Mater Dei

Conversation happened on 10/04



- **Scale will be essential** for players going forward. Yet smaller hospitals with ~50 beds still survive trying to put down fires;
- The industry had an intense **M&A fever**, but it has now changed to other trends. Now, the **assets have worsened** and are unattractive;
- Mater Dei is now trying to **partner up with operators without verticalization** as it tries to compete with players that have brought operators closer to its hospitals.



Call D – Pedro Cunha, Financial Director at Kora

Conversation happened on 20/02

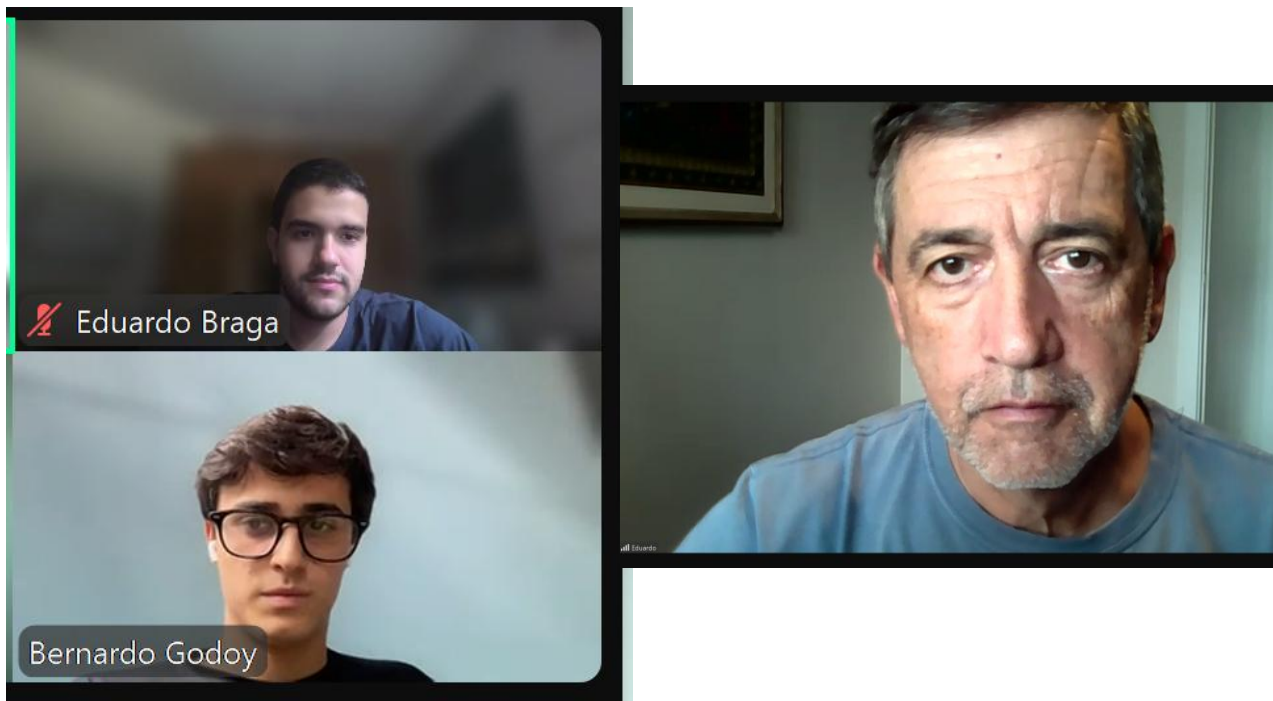


- Kora has had a hard time after it tried to expand with M&As at all costs during the 2020-21 period;
- The company is now with high debt because it took advantage of the low interest rates environment at the time to leverage its balance sheet. This has also put it in a weak position to deal with operators that know it;
- Kora is trying to develop verticalized plans in Espírito Santo, in an effort to cut costs and reduce accreditation risk.



Call E – Eduardo Almeida, Former Director at Amil

Conversation happened on 01/04



- The **industry** has changed a lot, and it seems in a **consolidation path** going forward;
- **Dasa and Amil have joined forces to fight leverage and improve operations.** They may try to go vertical using Amil's plans network and Américas and Dasa's hospitals footprint;
- **Rede D'Or** has a higher percentage of **emergency care beds** in their hospitals than average, to increase average ticket.



Call F – Marcio Mendes, Chairman at Fleury

Conversation happened on 26/02

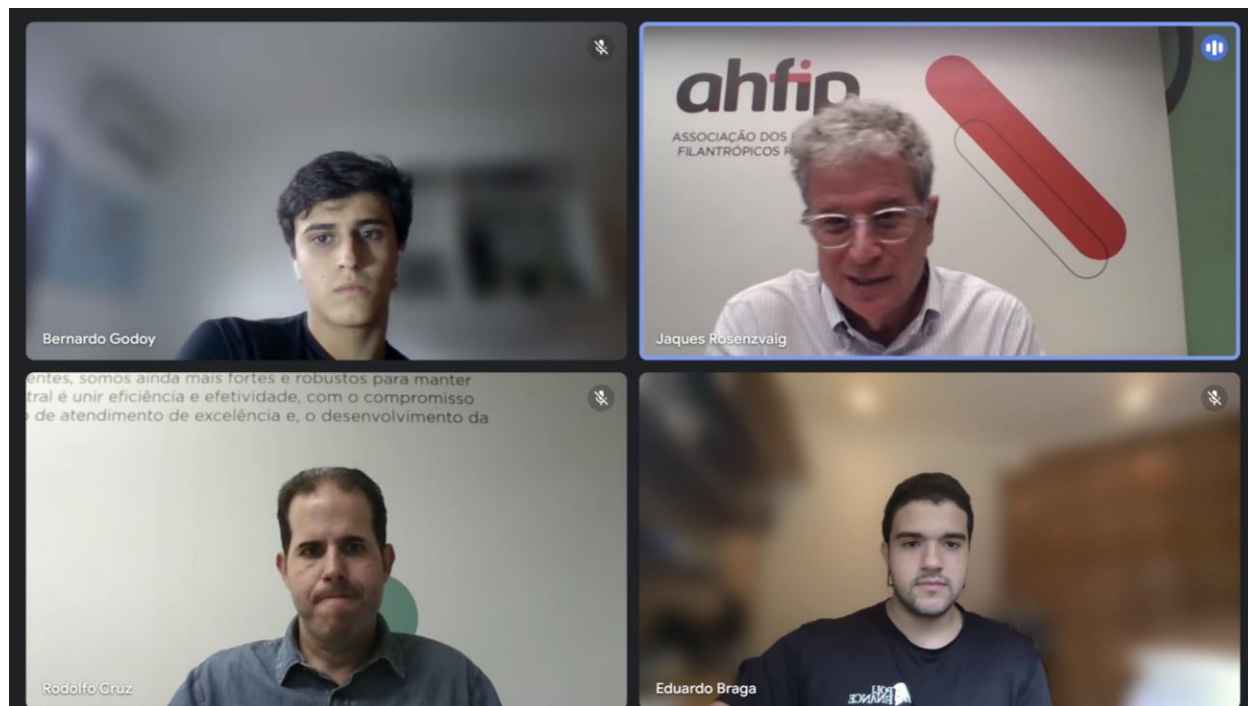


- Service providers are usually in a weaker spot than HMOs, as they can get affected by claim denials (*glosas*) and payment delays from operators;
- Fleury expanded a lot by M&As in the past as they found good assets that were already part of an accredited network;
- In the end, the way to protect yourself from operators is by providing a good service for clients and maintaining a strong brand.



Call G – Jaques Rosenzvaig, Director at Ahfip

Conversation happened on 26/02



- Household **philanthropic names do not face big challenges for survival** in the coming years like smaller ones;

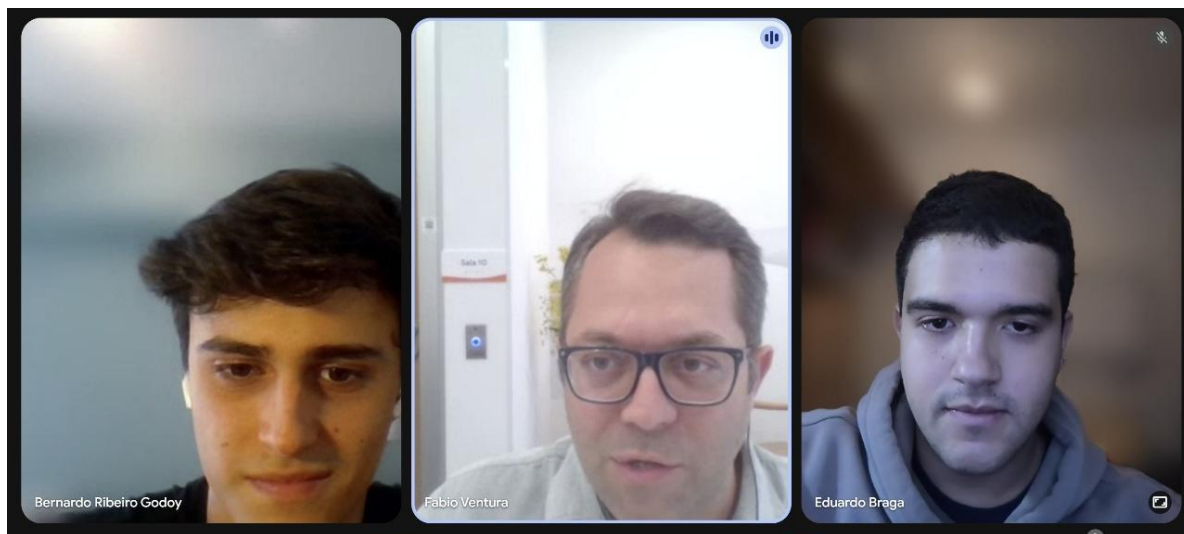
- A beneficiary might prefer going to a philanthropic hospital because at first sight they have no conflict of interest as a private one may have;

- **São Paulo has very big addressable market that is able to support many different models** that suit beneficiaries, something that is not true for other places that have different competitive environments.



Call H – Fabio Ventura, Comercial at Sul América

Conversation happened on 03/04

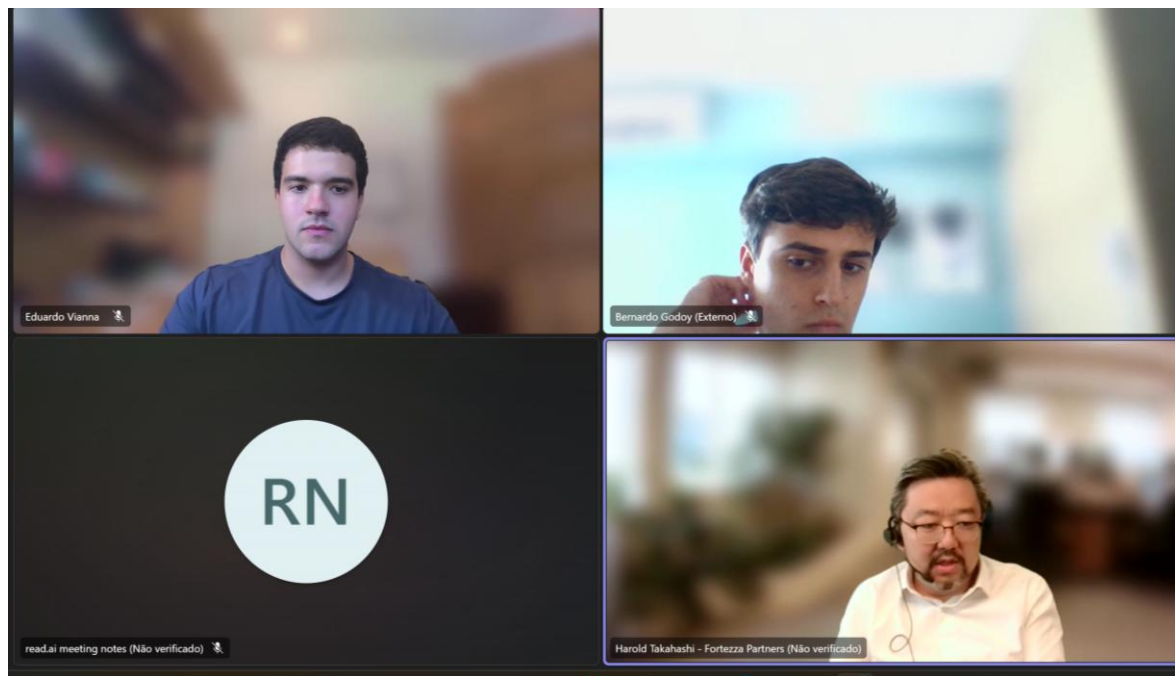


- Sul América is in a **better position to deal with other hospitals**, since it already has RDOR hospitals accredited in its regions. This means that it does not need to accept bad terms or settle for worse hospitals;
- Sula is very important for **greenfields** projects, since it is able to bring in new beneficiaries with promotions and deals;
- Sula still has much **space to grow**, as it can offer **better service** than competitors such as Unimed.



Call I – Harold Takahashi, Former Head of M&A at Fleury

Conversation happened on 13/03

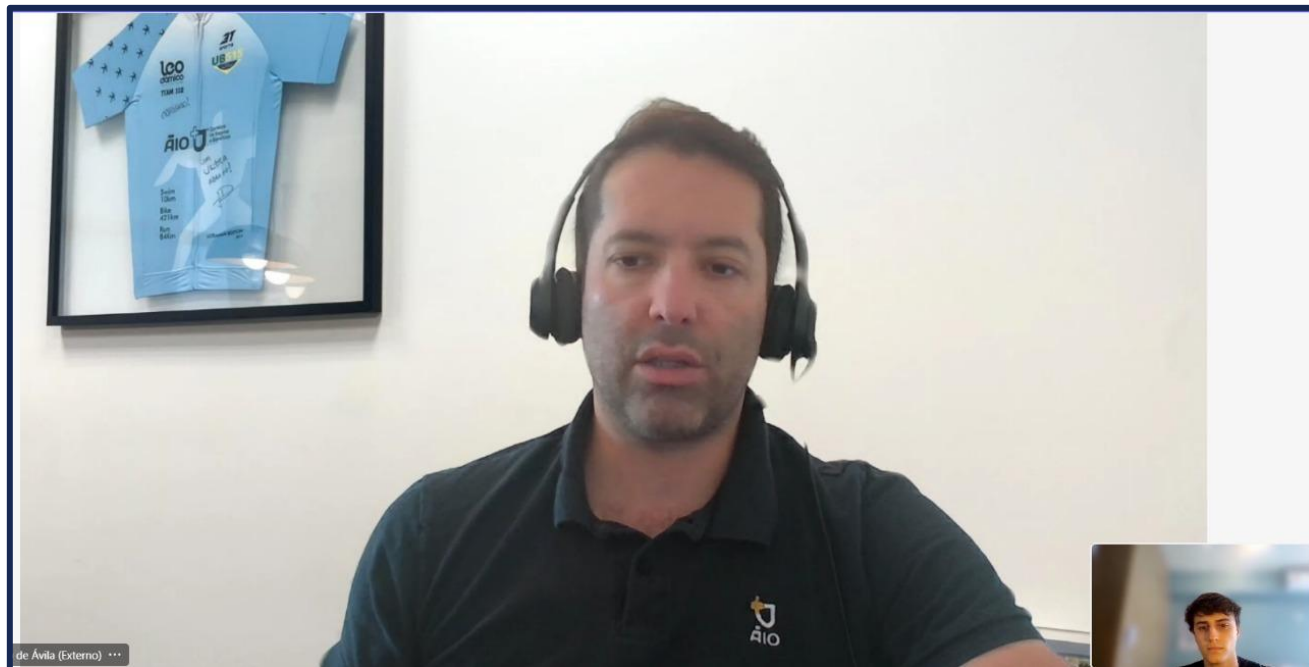


- It is important to understand that **operators have usually a regional strength** that can hardly be used on other regions;
- **Fee for service models are becoming less used**, as operators and providers are trying to align interests;
- **Rede D'Or had signed a non-compete with Fleury for diagnostics** after it sold its diagnostics BU to them, but that has now been waived and expired.



Call J – Luciano Ávila, Founder at AIO Corretora

Conversation happened on 03/04



- Health insurance has changed a lot in the past decades, with **individual plans having become unattractive** as they have less room to increase prices;

- **Rede D'Or** has created such a **strong** position in the states of Rio de Janeiro and São Paulo that is **now extremely hard to sell mid to high-end plans without it**;

- With Sula, Rede D'Or has **access** to how much each of the **other hospitals charge and negotiate with operators**.



Call K – Fernando Torelly, CEO at HCor

Conversation happened on 11/04



- **Santas Casas** get **60%** of their financing from **SUS** and only 40% from operators. So they do not invest in hospitality and do not attract customers;

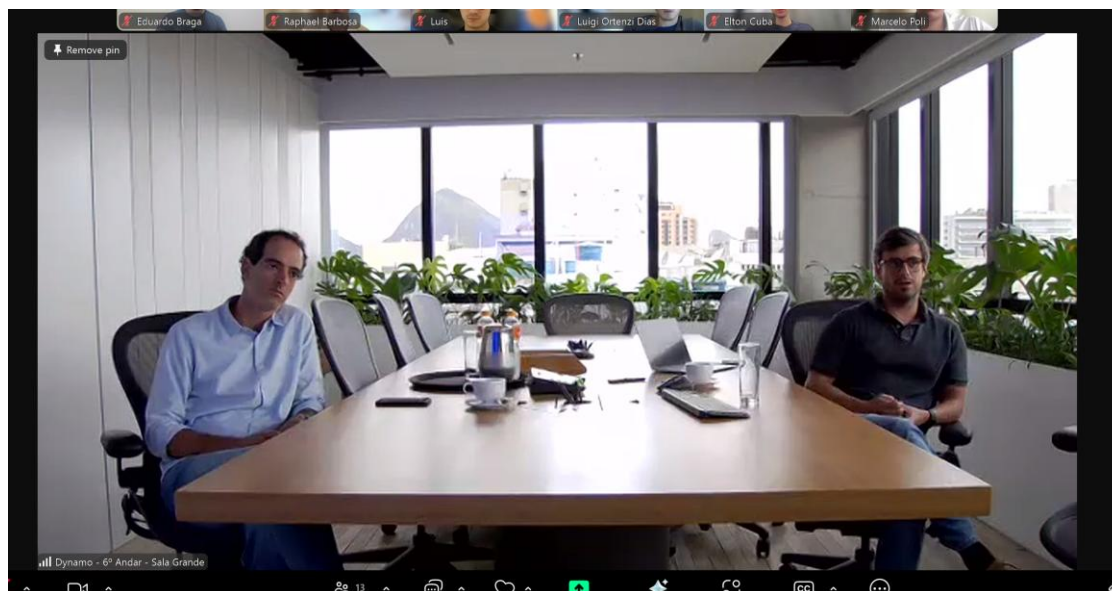
- As the industry moves toward **consolidation**, the way to stay relevant is by **either** being a **big player** or having customer captivity due to **good service**;

- As **hospitals are capex heavy**, if you do **not invest** in it for 2-3 years, your **gap to competitors can get huge**.



Call L – Dynamo, Investor

Conversation happened on 20/03

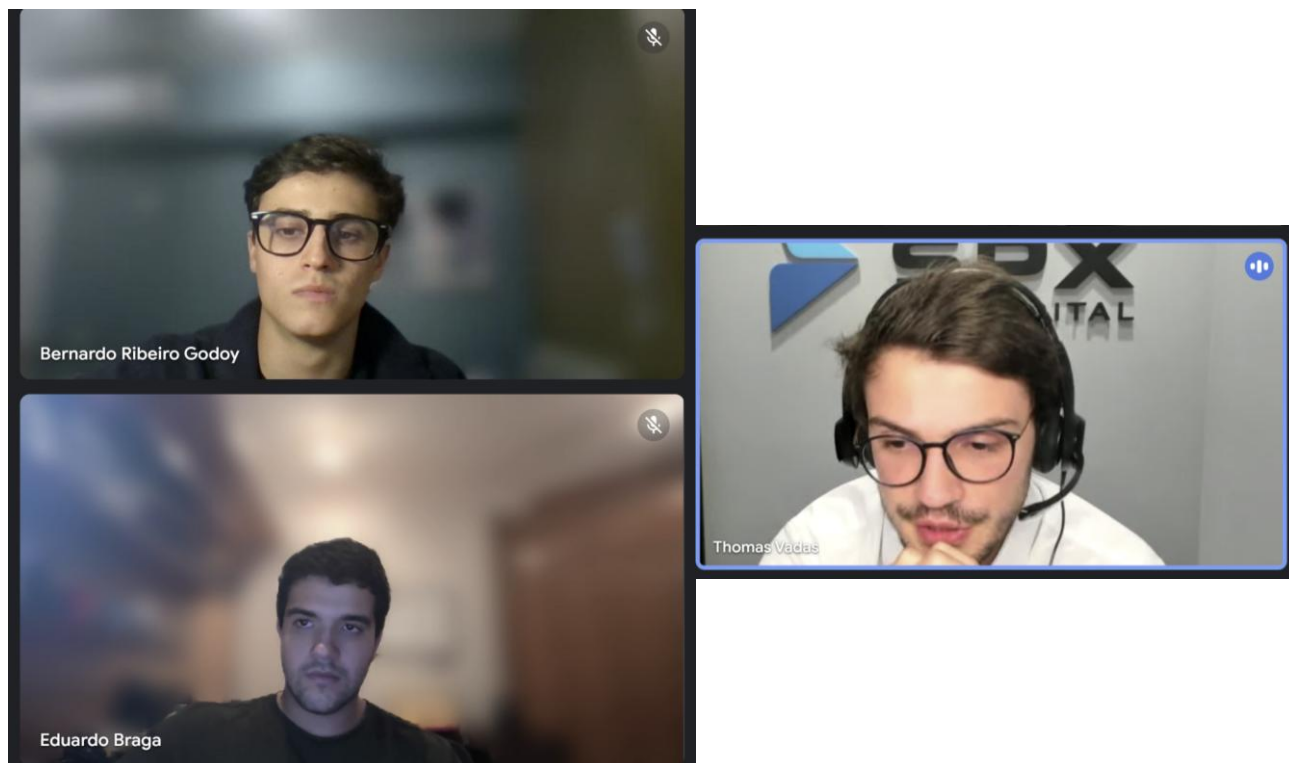


- **"The client is the doctor!"**. As the main reasons a client chooses a hospital are the doctors' indications and credentials, it is paramount to have a good medical team;
- **CP-145** may present an important risk to operators as it tries to limit MLR and plans business readjustments like individual plans;
- **Diagnostics are a good optionality for the future**, as it can be leveraged by Rede D'Or's massive hospital footprint and Sula's accreditation.



Call M – SPX, Investor

Conversation happened on 18/03

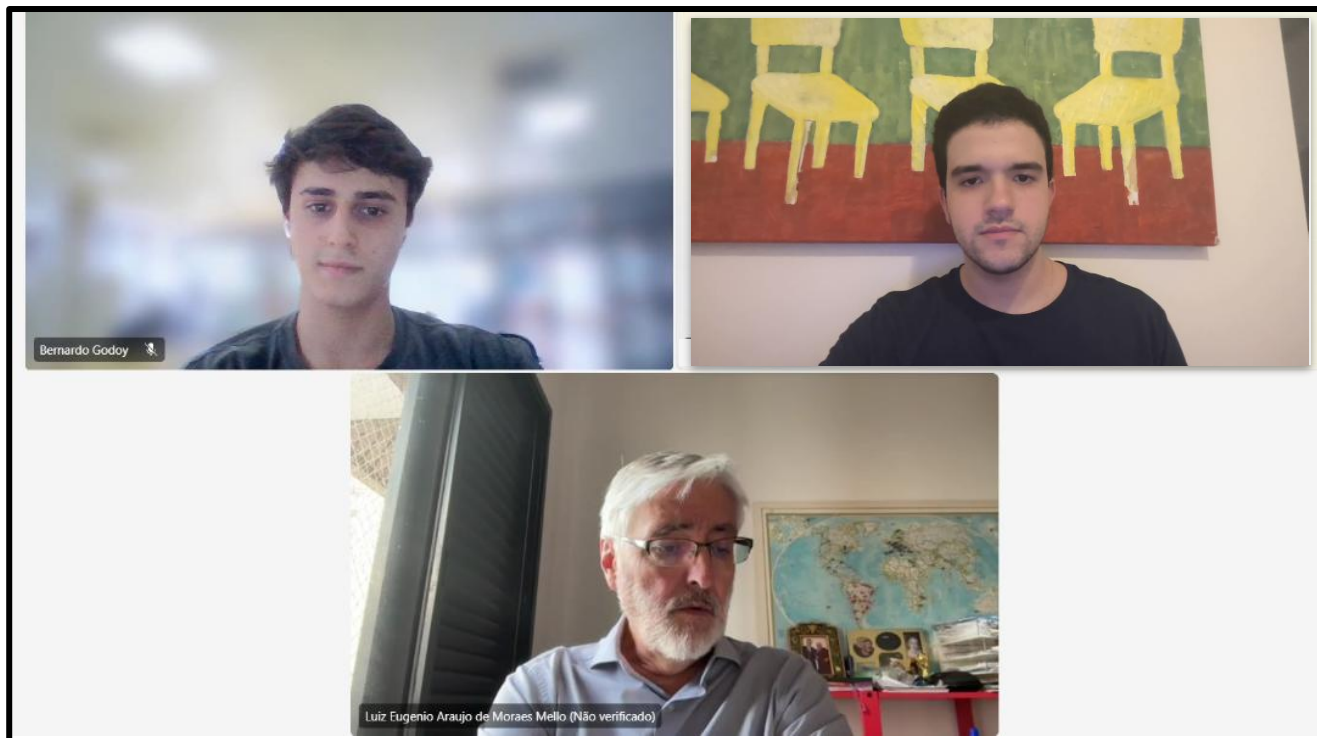


- **SPX bought Carlyle's PE operation**, so it remained with what the fund had left of Rede D'Or;
- The market is watching the stock closely and **in the short term, profits may be driven by better MLR**, but in the long-term, it is the hospitals' expansion that will drive earnings;
- In the **DCF valuation**, it is **reasonable to assume a 5-7% g on perpetuity**, as medical inflation, that is historically higher than the IPCA.



Call N – Luiz Mello, Director at IDOR

Conversation happened on 09/04



- Since its inception in 2005, **IDOR** has focused on **resonance imaging**, in an effort to develop new uses for it;
- The institute has done research in **intensive care** that can help **RDOR**, as the company has many beds for it;
- IDOR can help the company with **clinical research treatment** in **patients that would like to volunteer** for new methods not available in Brazil;
- **IDOR** also enables **RDOR** to use **new diagnostics** for special conditions.

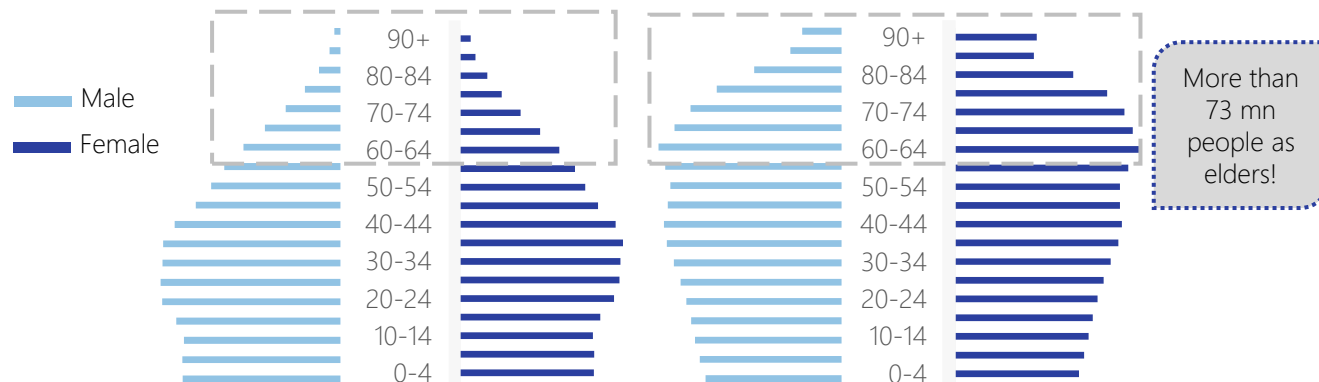


The industry has a long term driver

Opportunities may arise, but should take time

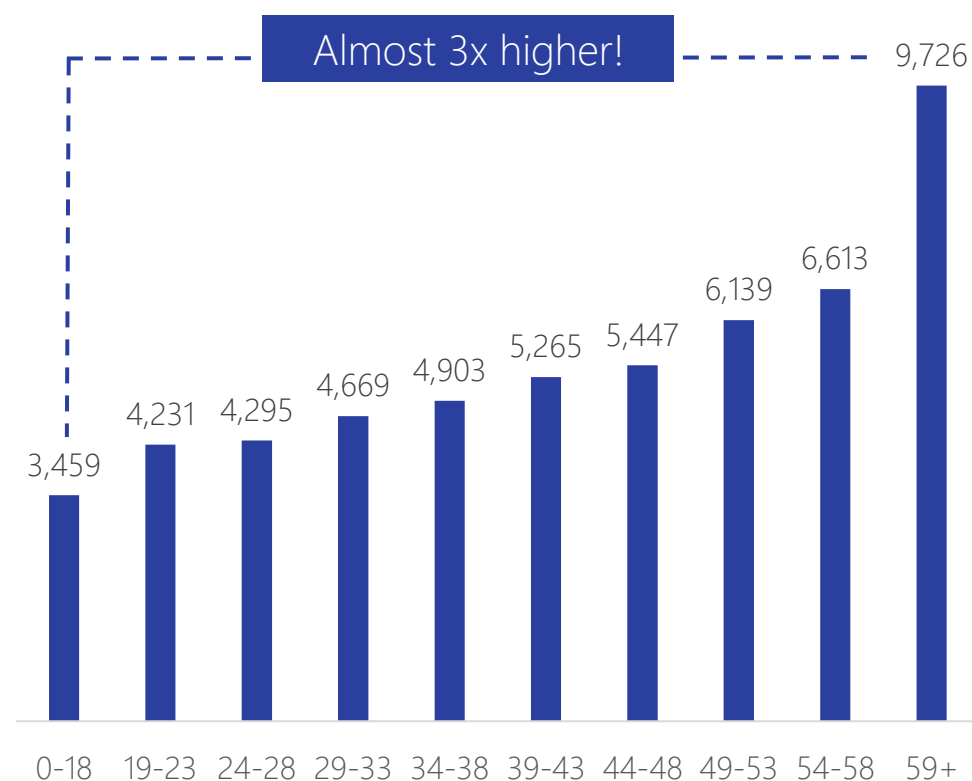
Brazil has an aging population...

Brazil age pyramid in 2022 vs. 2060 [mn people]



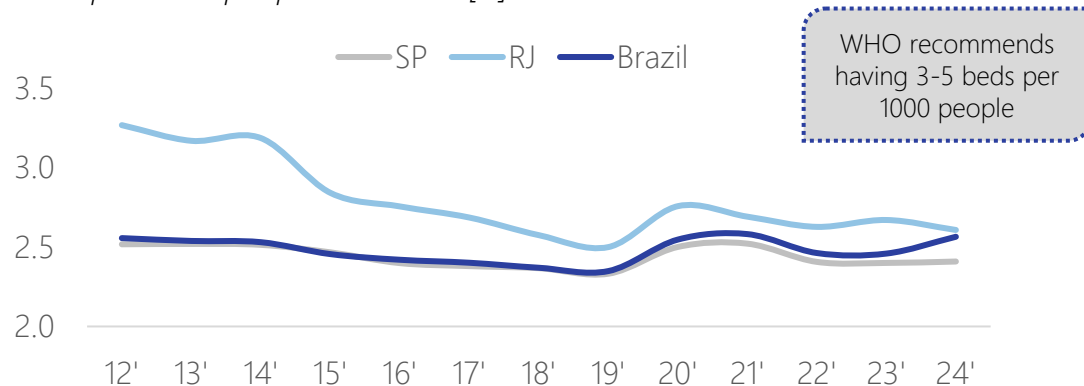
...and will also increase spending

Average health expenditure per age group in Brazil [BRL/hospitalization]



...that will need more infrastructure..

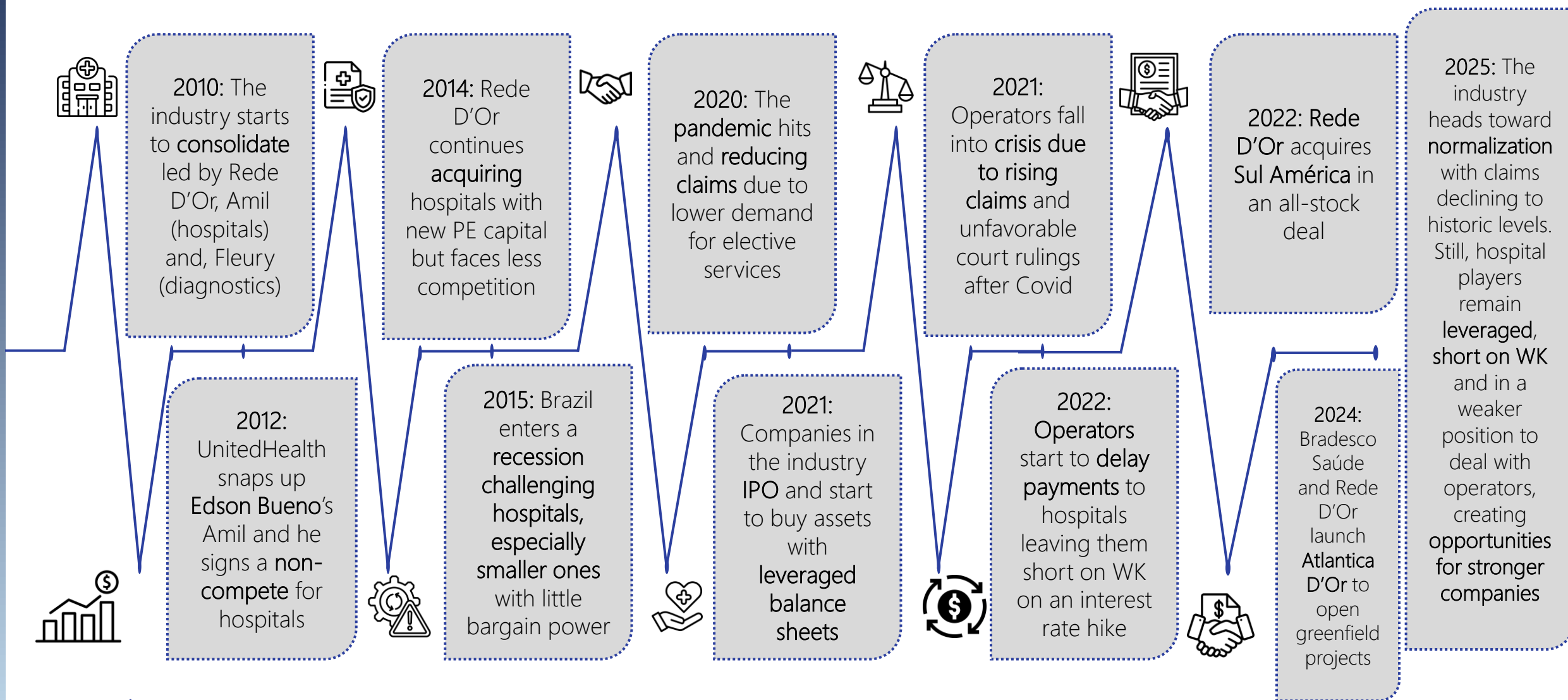
Beds per 1000 people evolution [#]





Healthcare timeline

The industry has changed significantly in the past 15 years



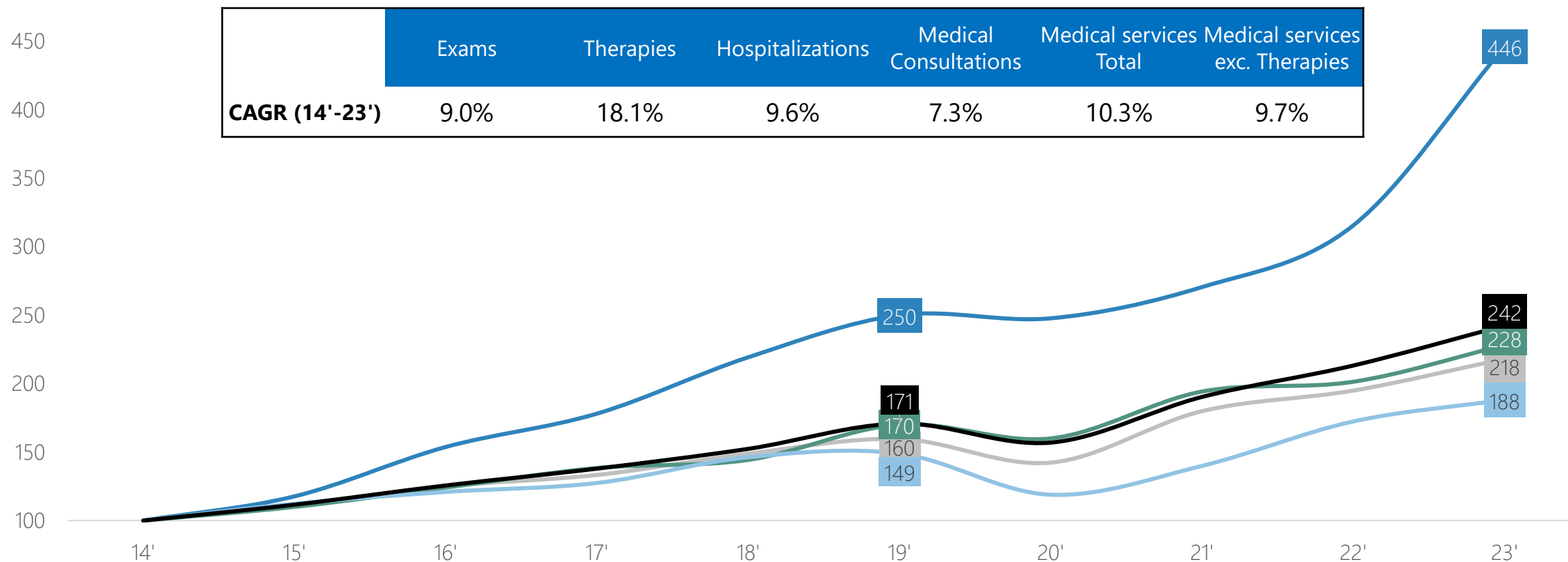


Which parts of the industry are growing more?

Therapies expenditure have exploded

Operators expenditure evolution per type of Medical Service [2014 = base 100]

— Exams — Therapies — Hospitalizations — Medical Consultations — Medical services Total



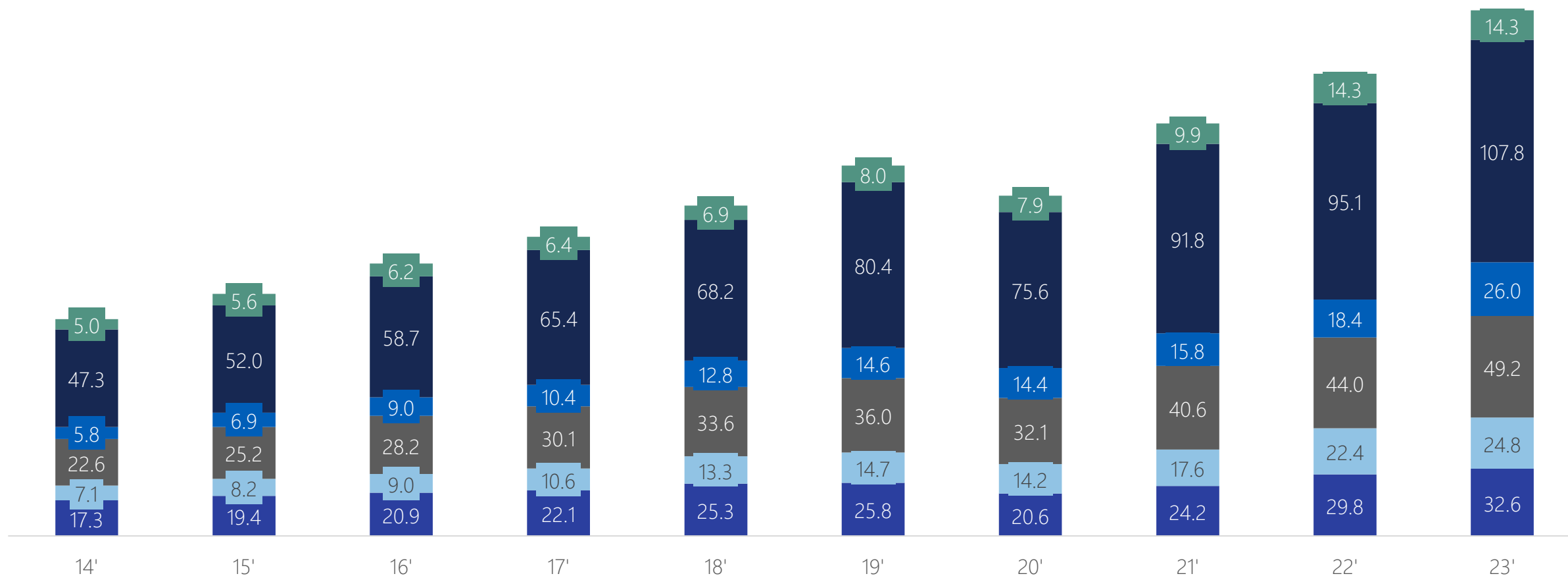


Industry spending breakdown

Different parts have shown different growth

Spending per segment [BRL bn]

■ Doctor appointments ■ Others outpatient care ■ Exams ■ Therapies ■ Hospitalization ■ Other medical expenses





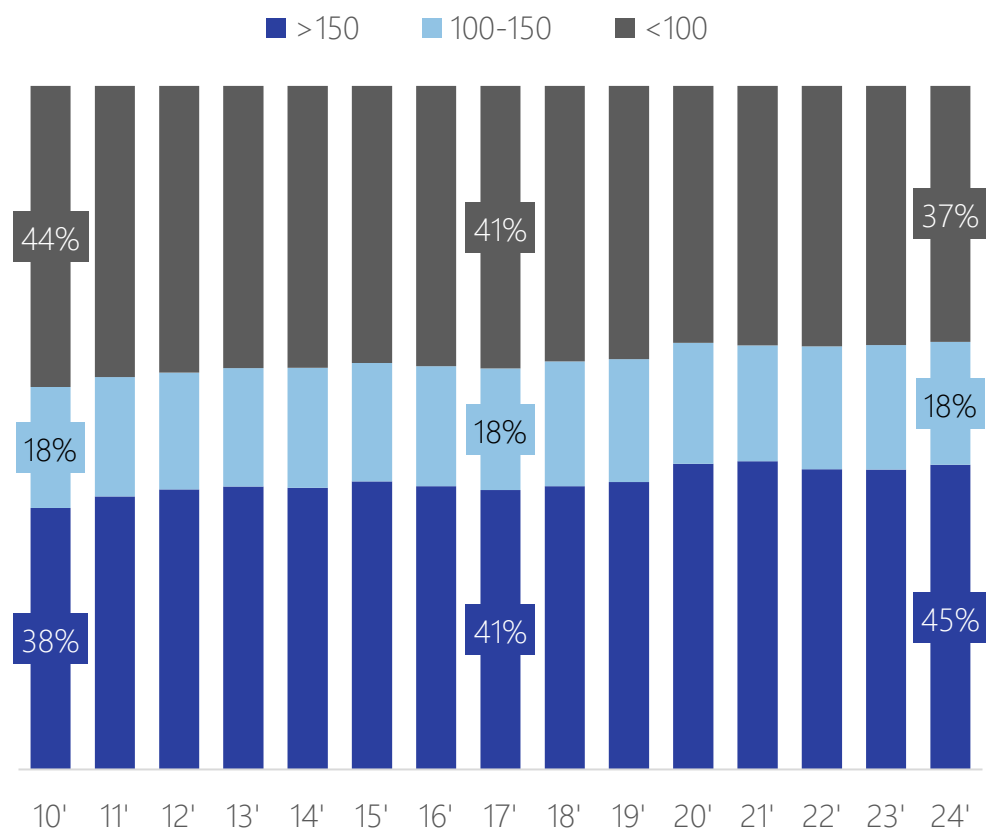
Scale will be essential going forward

In the future, smaller hospitals will suffer the most



As it is harder to survive without scale

EoP share of beds by hospital size, private and philanthropic [% ; bed]



MaterDei UF **m** G
Rede de Saúde

1983

1982

42 Years at Healthcare sector

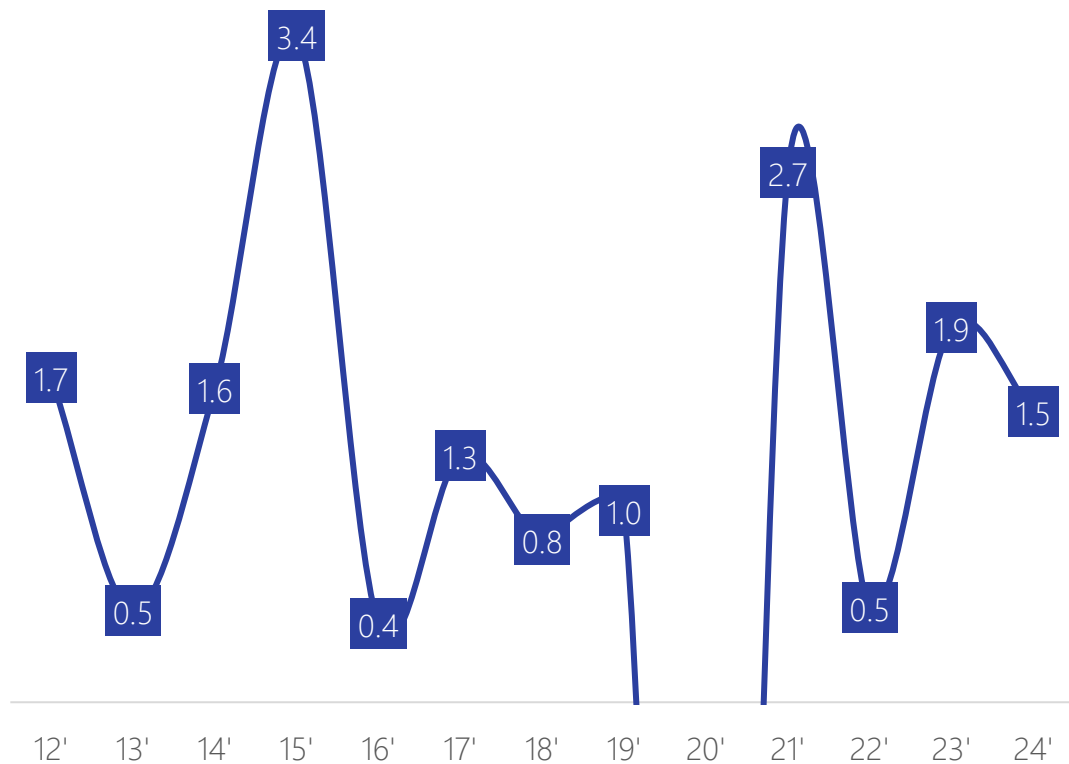
"Having **scale is extremely important** going forward because big players can **negotiate better** with suppliers and, with more beds, they can dilute costs with personnel. These is why **smaller hospitals are ceasing to exist**. Yet, they remain open because they keep putting down fires, so in the long run they are in a very weak position, since they will not have access to good technology and others."

Henrique Salvador – Chairman at Mater Dei

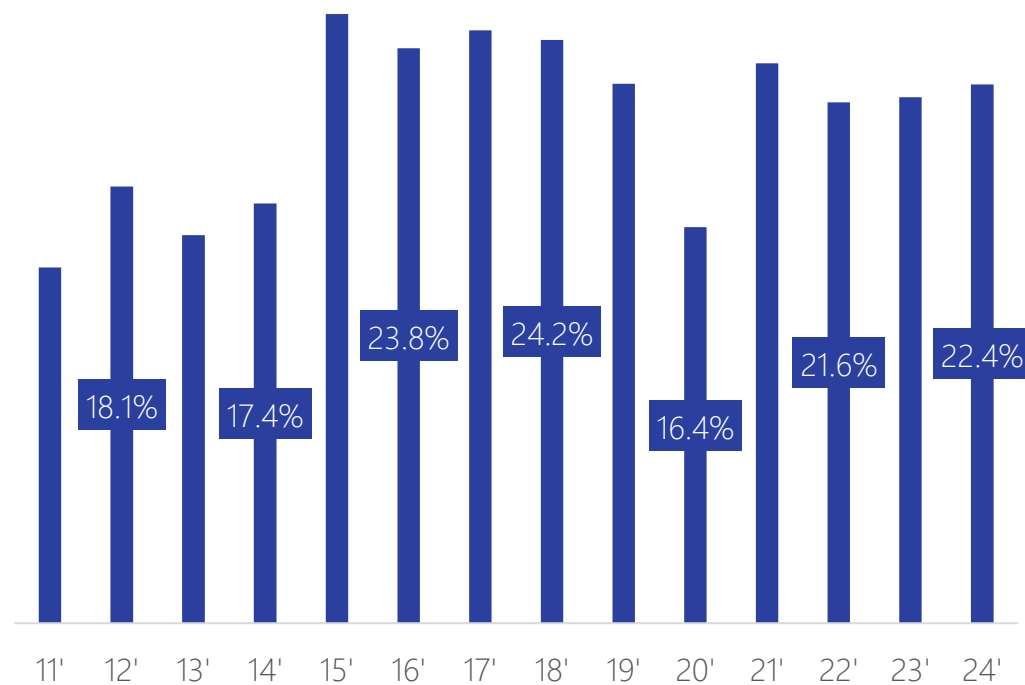


Operating Leverage

Hospitals EBIT growth / revenue growth [#]



Hospitals EBITDA margin [%]





What about verticalization?

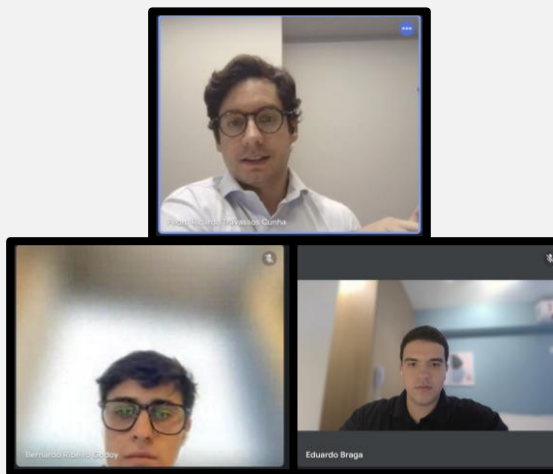
The risk of not being accredited is a barrier to expansion, so companies look for alternatives

Effects of verticalization:

Better claims control



No accreditation risk



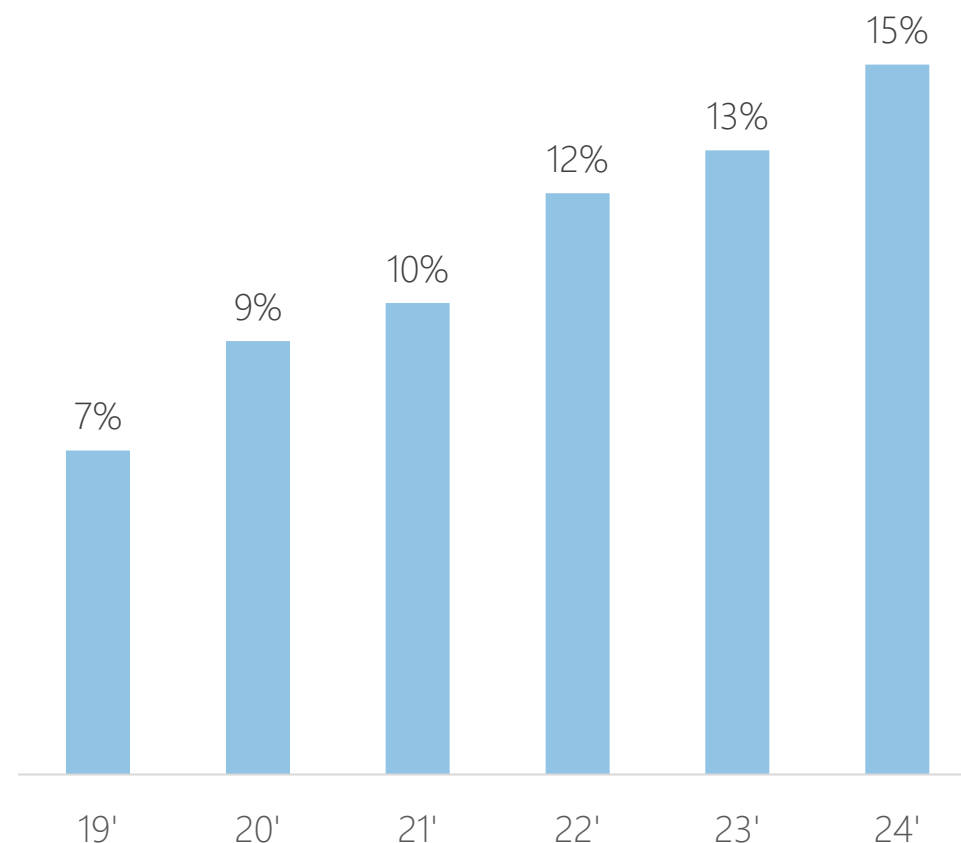
"When opening a hospital, **getting accredited by insurers is a key risk**. Insurers can also hurt hospitals through delayed payments or denied claims. Because of that, **some choose to verticalize**, so at Kora, we're building our own health insurer in Espírito Santo."

Pedro Cunha – Financial Director at Kora Saúde



The vertical model has shown intense growth recently.

Share of total hospital reimbursements paid by verticalized players [%]



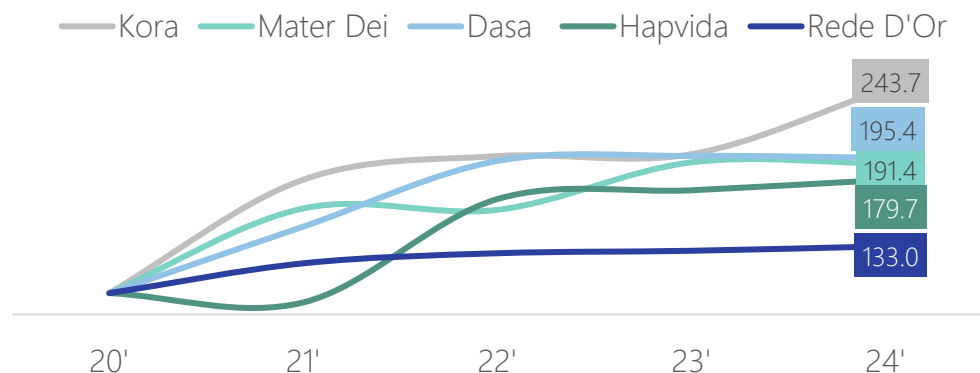


The M&A fever

Rede D'Or's main rivals used new IPO cash and leverage to expand at all costs

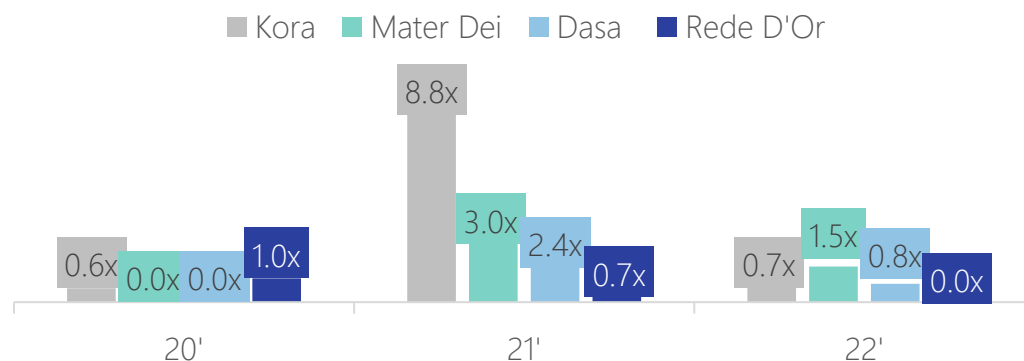
Rede D'Or was the slowest to expand in the period...

Operational Beds EoP Evolution [2020 = 100 base]



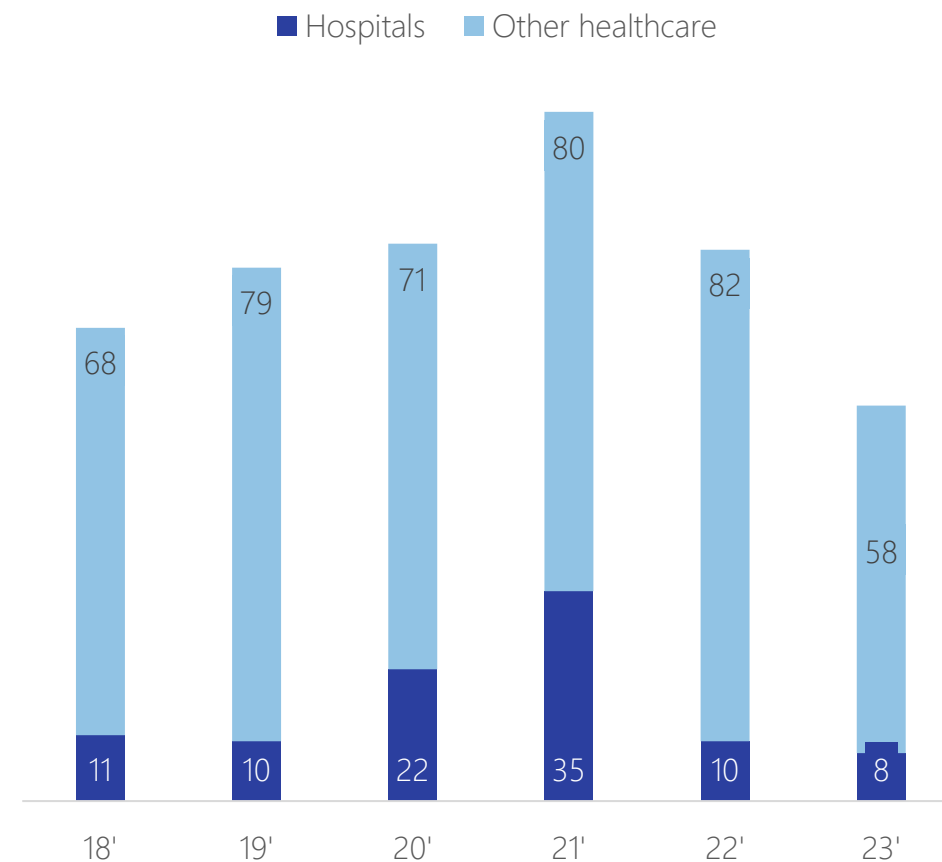
...as rivals expanded without a long term view...

M&A cash/LTM EBITDA [x]



...at a hot time for M&As, meaning more bidding for assets

Total number of M&A transactions [#]





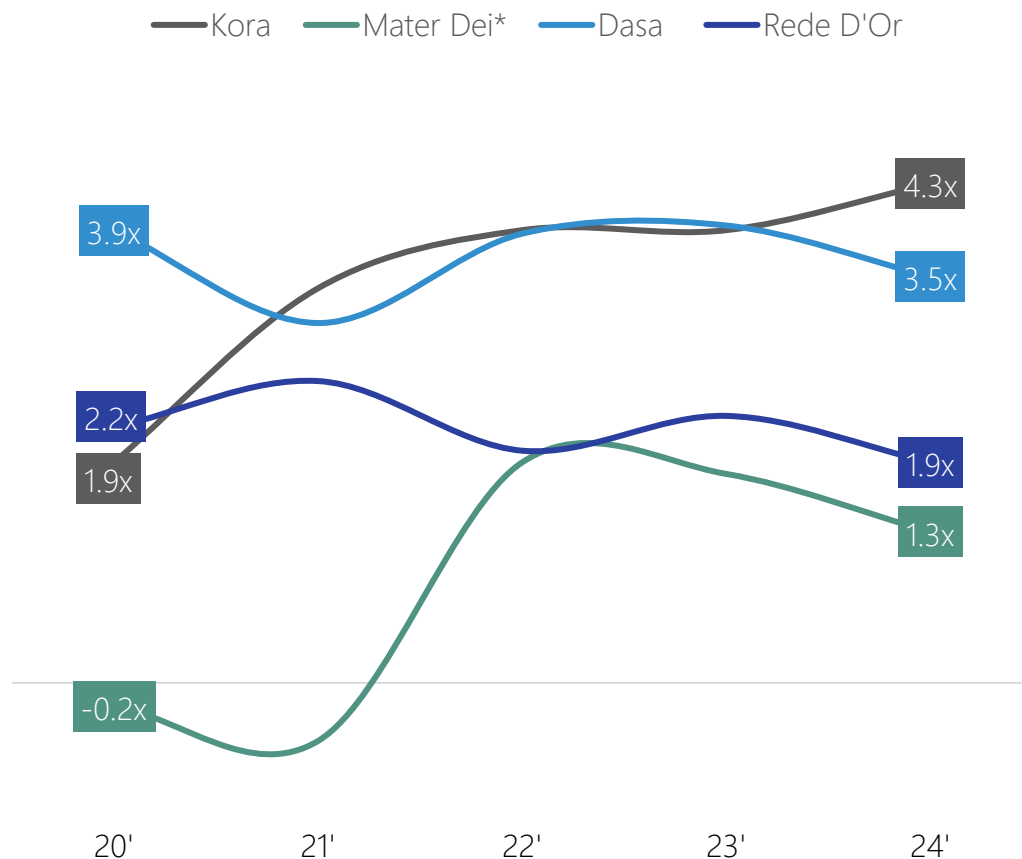
"The most important thing is...Contrarianism"

Rede D'Or did not go all in the acquisition frenzy and was rewarded for it



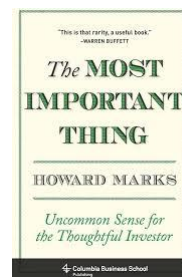
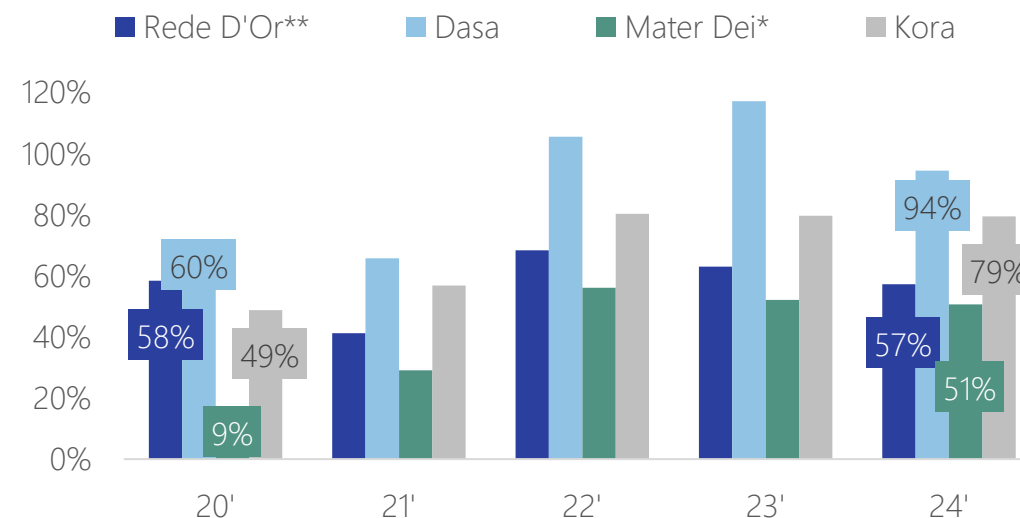
The company remained in a strong leverage position...

Net Debt/EBITDA [x]



...and did not suffer the effects of higher leverage like peers

Interest expenses/EBITDA [%]



"You must do things not just because they are the opposite of what the crowd is doing, but because you know why the crowd is wrong"

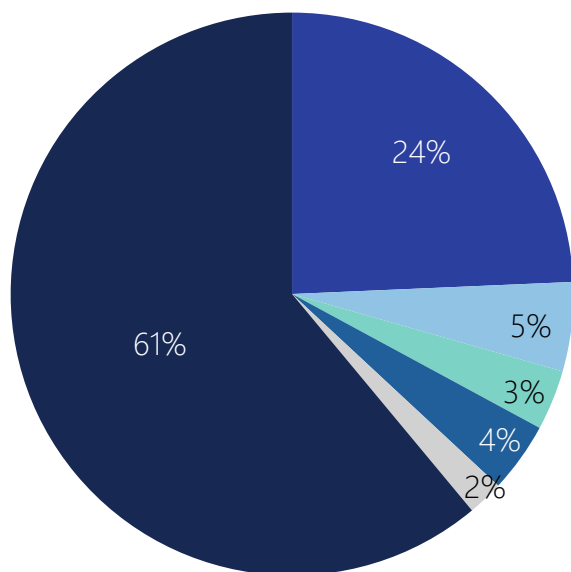
Howard Marks "The Most Important Thing"



2023 Beds market share in key regions



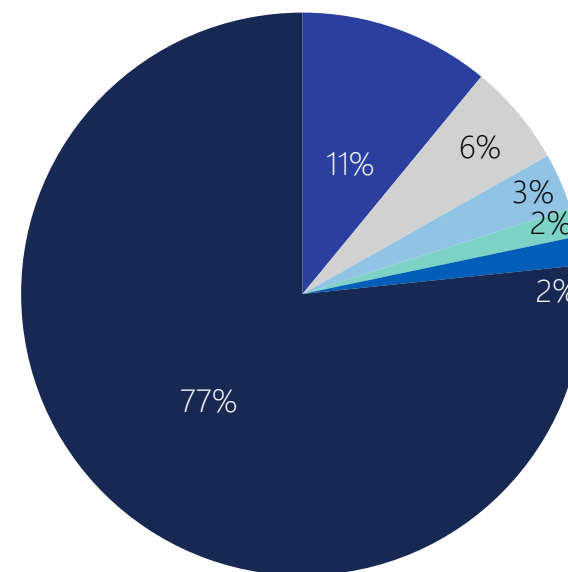
Rio de Janeiro (14,925 beds)



■ Rede D'Or ■ DASA ■ Americas ■ Rede Santa Catarina ■ Hapvida ■ Others



São Paulo (40,653 beds)

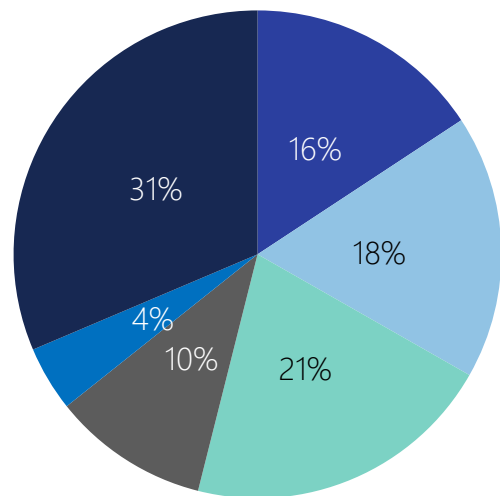


■ Rede D'Or ■ Hapvida ■ DASA ■ Americas ■ Albert Einstein ■ Others



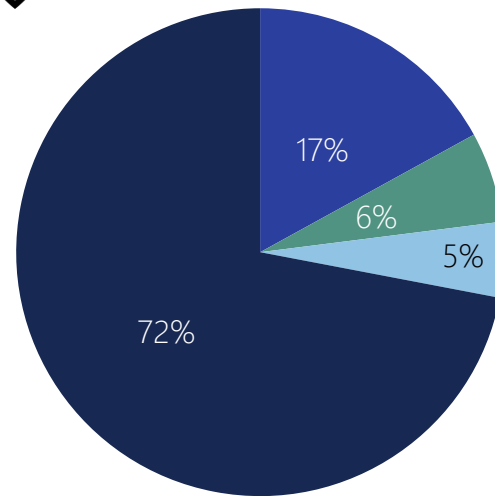
2023 Beds market share in other regions

📍 Distrito federal (3,336 beds)



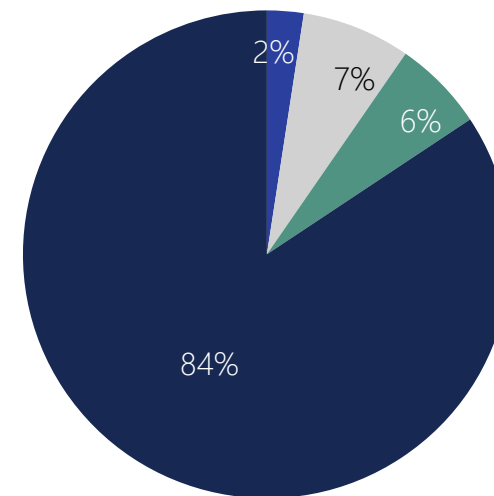
■ Rede D'Or ■ DASA ■ Grupo Santa ■ Kora ■ Sírío Libanês ■ Others

📍 Bahia (6,194 beds)



■ Rede D'Or ■ Mater Dei ■ DASA ■ Others

📍 Minas Gerais (14,130 beds)



■ Rede D'Or ■ HAPV ■ Mater Dei ■ Others

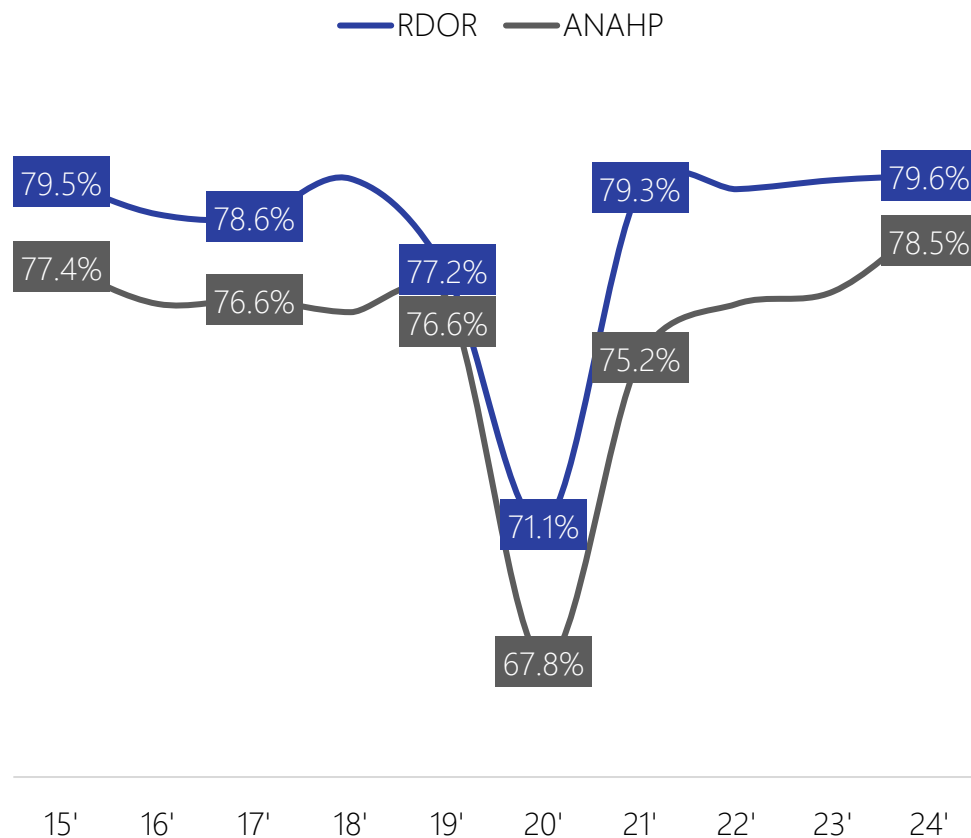


Taking advantage of a good positioning

Rede D'Or has shown it can operate better than peers

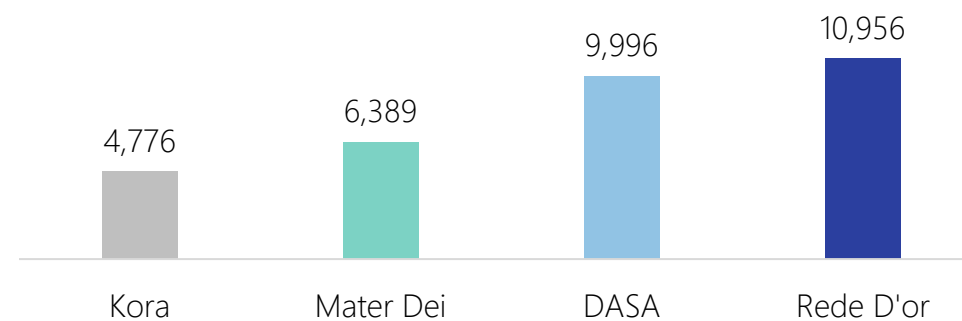
It has a higher occupation rate than them...

Beds occupancy rate [%]



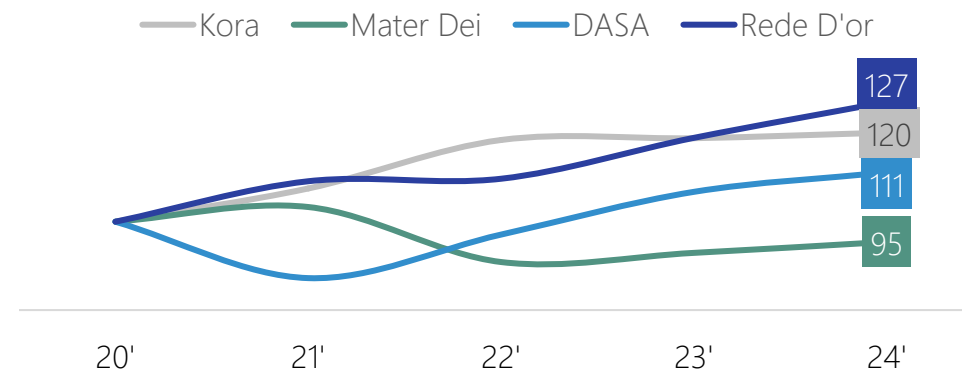
...with the highest average ticket...

Average ticket [BRL]



...and it is able to better pass on prices

Average ticket evolution [2020 = 100 base]



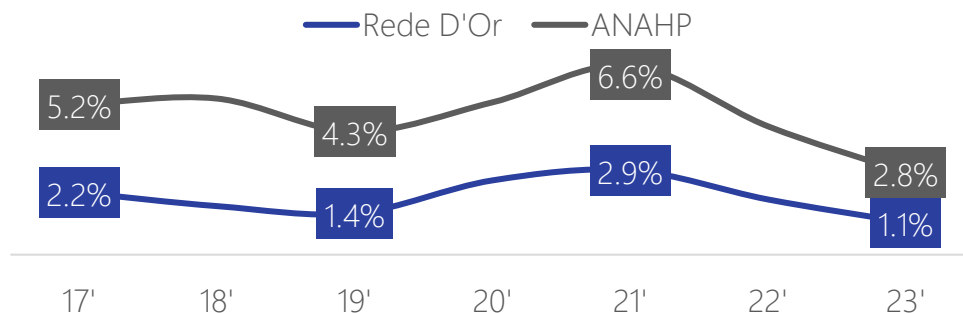


Infrastructure quality is closely watched

Besides having a great team and tech, the company provides above average infrastructure

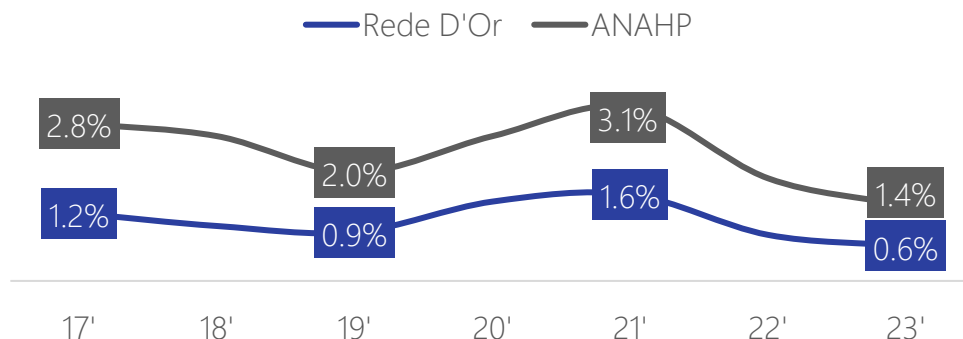
- A smaller percentage of patients with pneumonia related to ventilation shows a better infraestructure...

Percentage of patients with pneumonia related to ventilation [%]



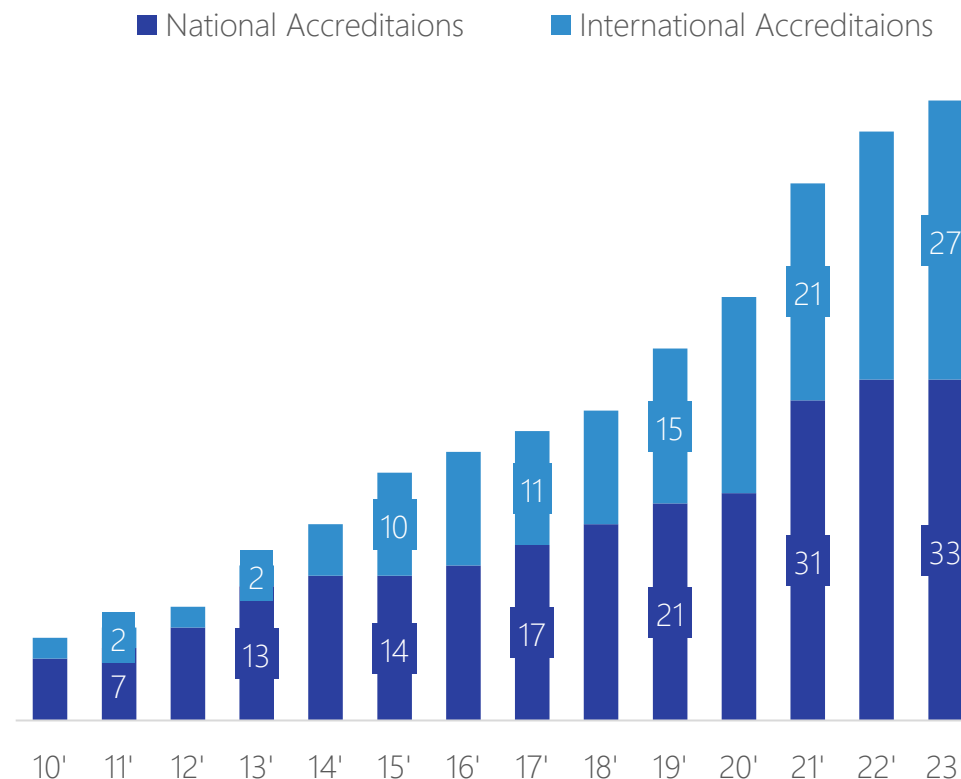
- ...and so does less catheterization blood infections.

Percentage of patients with catheterization blood infection [%]



- And national and international accreditations prove this efforts have been recognized

Number of accredited hospitals evolution [#]





Dasa and Amil have joined forces

As the industry has turned sour, the companies are trying to stay alive, but the deal is still cloudy

Valor

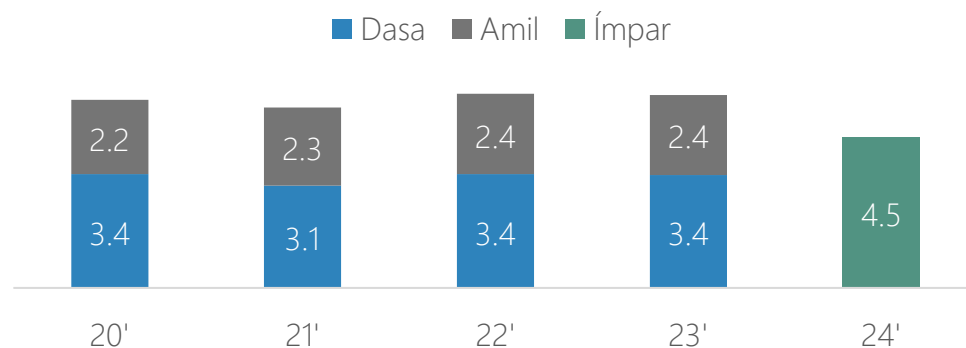
04/01/2025

Dasa and Amil finalize agreement to become partners in Ímpar

Estimated combined net revenue from operations included in the association totaled R\$10.6 billion in 2024 across 25 hospitals and 4,500 beds

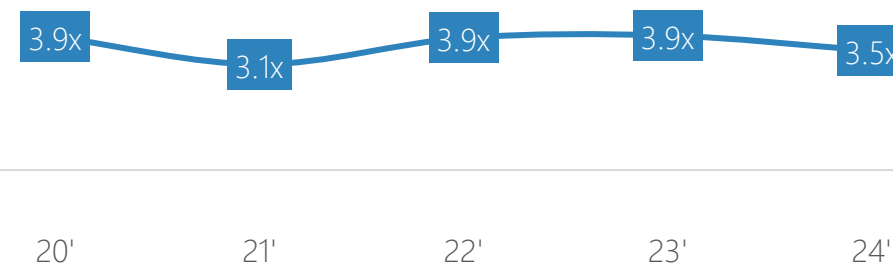
Dasa and Amil were trying to expand recently...

Number of operational beds [th]



...but Dasa has an extremely leveraged operation...

LTM Net Debt/EBITDA [x]



but doubts remain as we do not know who is calling the shots



Seripieri Júnior

Dulce Bueno

Appendix

Sources: Company's filings

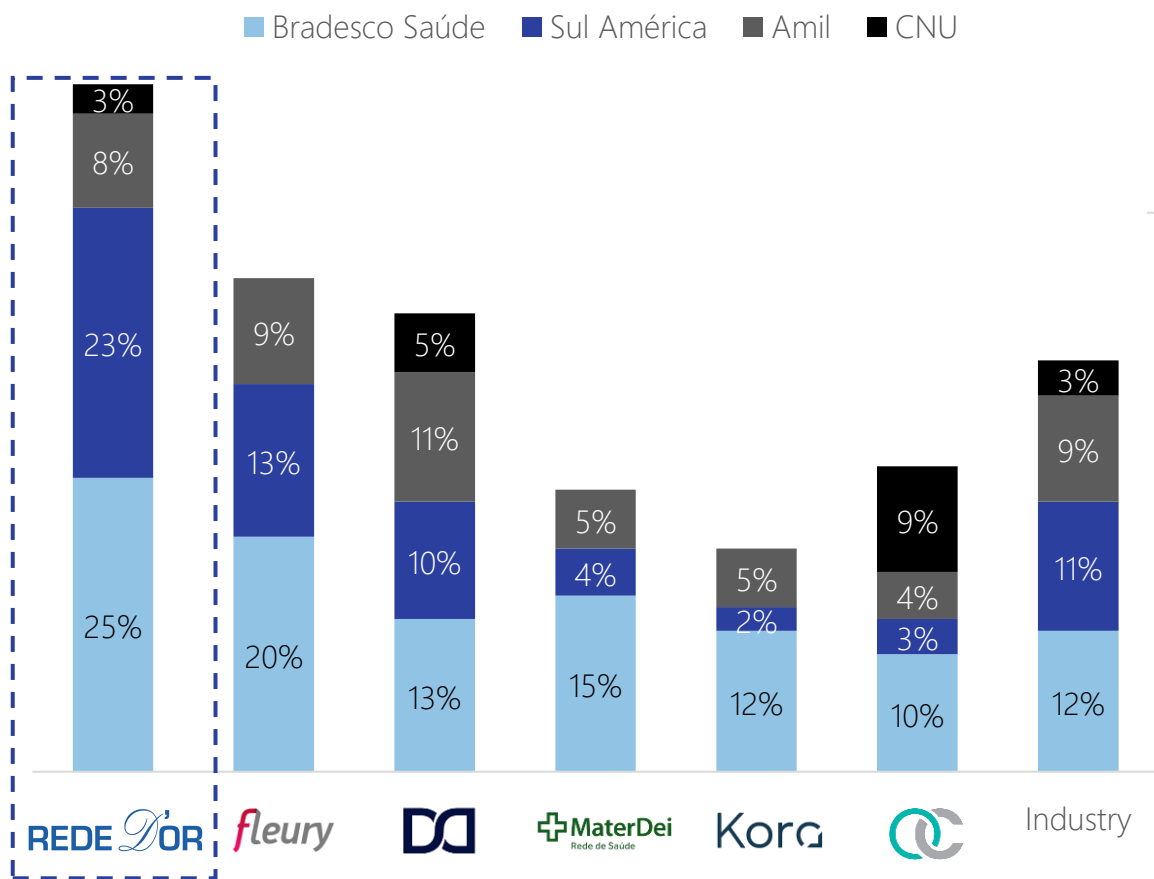


Does Amil pose a threat?

Amil and Rede D'Or have had a bumpy relation

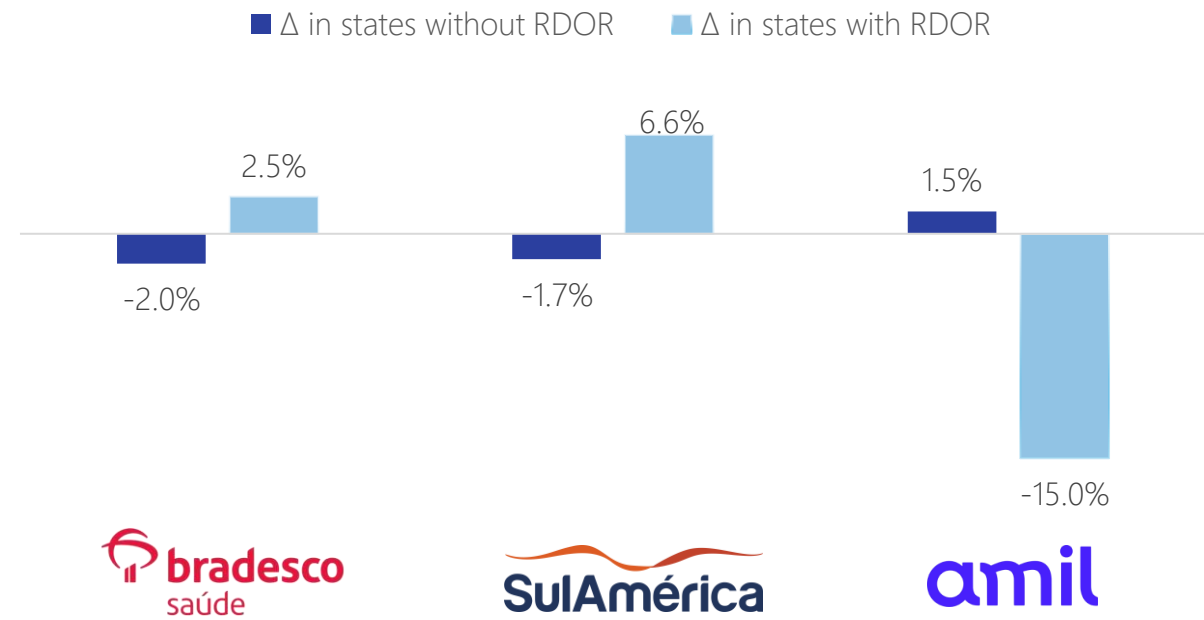
Amil is ~8% of Rede D'Or hospital revenue...

Share of revenue per company by insurer [%]



...but the last time it cut ties with D'Or, it did not end well

Amil beneficiaries evolution 2017-2019 in Brazilian states [%]



As Rede D'Or has a strong position in the industry with scale and demand from beneficiaries, it is hard to sell plans without it

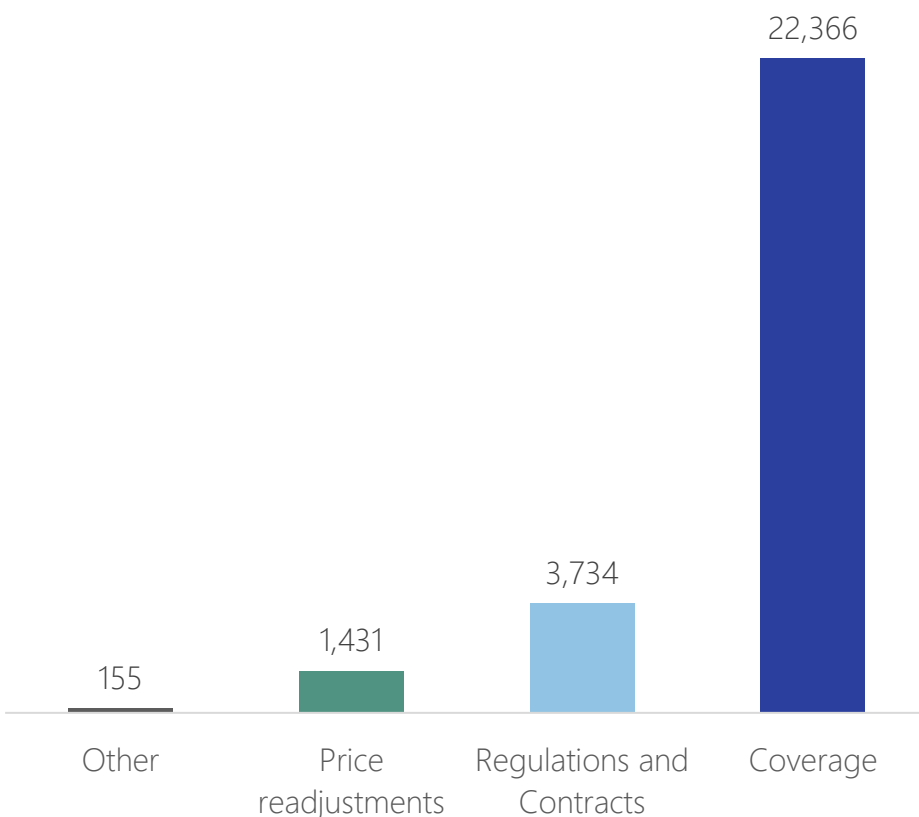


What matters to the consumer

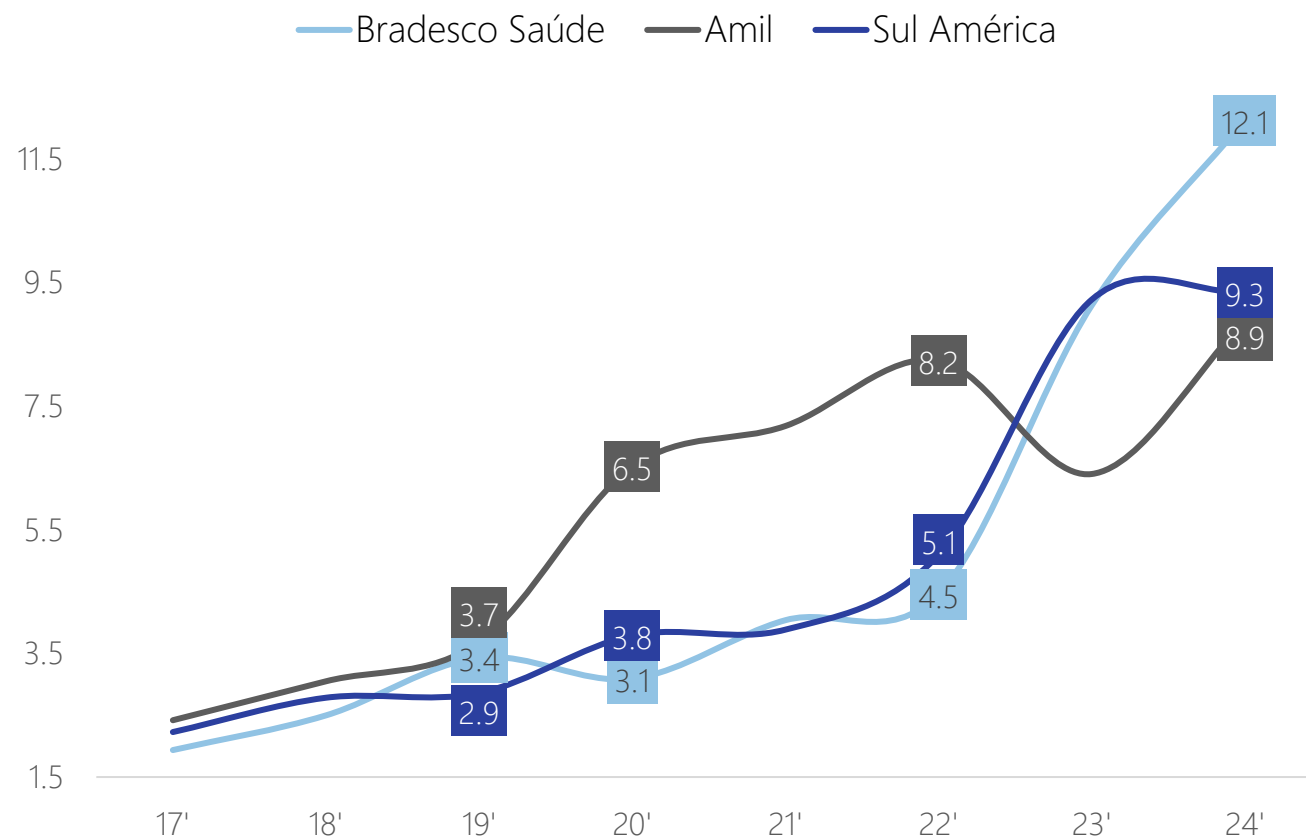
How Amil got affected by removing Rede D'Or from its coverage

Looking at what matters to the consumer, **it is clear why Amil had lots of complaints** from 19-22: **it removed RDOR from its network**

Industry consumer complaints by type in feb/25 [#]



Complaints per 1000 beneficiaries per operator [#]





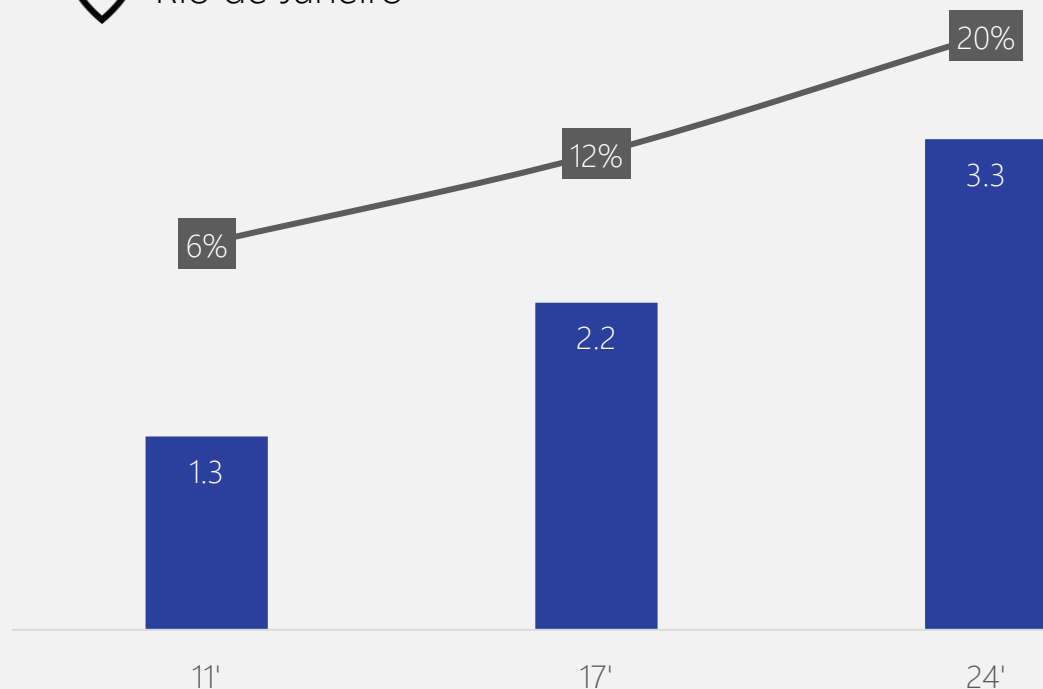
But there is even more to it...

As Rede D'Or has SulA and Bradesco on its side, it can use it as an opportunity

Rede D'Or has a strong hand against operators...

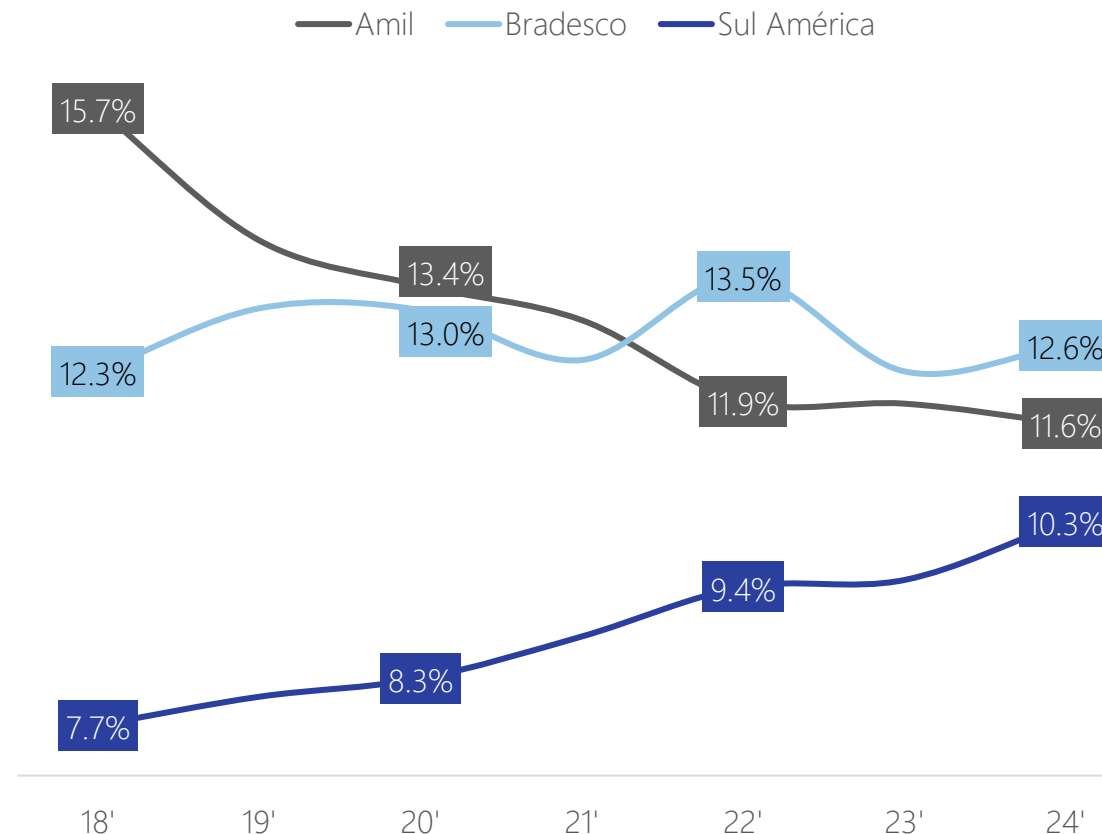
Operating beds and share of private beds in RJ [th ; %]

Rio de Janeiro



...because it can cause permanent client losses

Share of Beneficiaries Evolution per company in Rio de Janeiro city [%]





The São Luiz Campinas

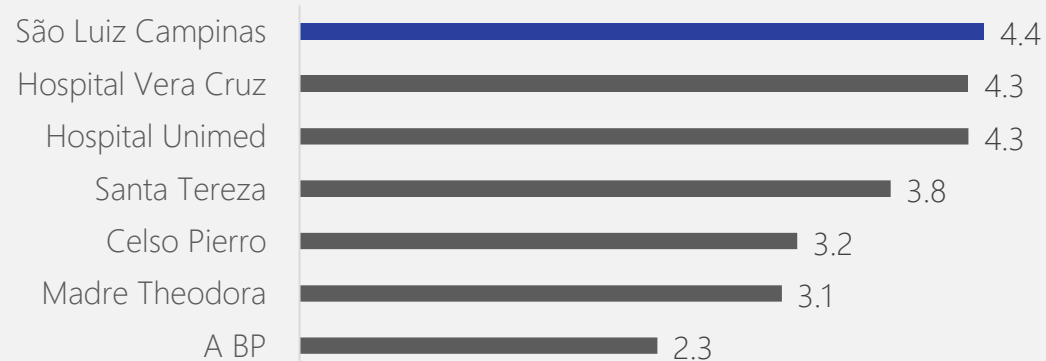
When looking at a recent opening it is clear that Rede D'Or has a strong brand



Location: **Campinas, São Paulo**
Number of Beds: **325**
Opening: **2023**
Rede D'Or Hospitals in Campinas prior to São Luiz: **0**

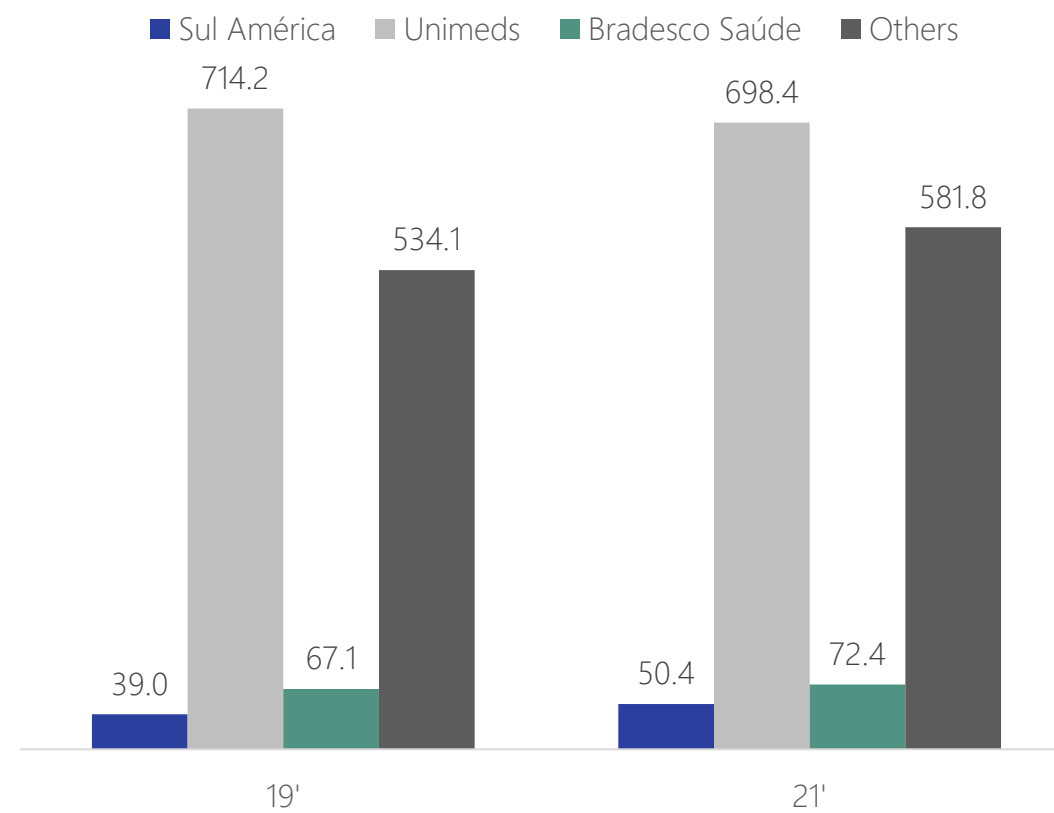
☐ The hospital is already the highest ranked in the city...

Campinas hospitals' google score [# / 5]



☐ ...that is overall dominated by Unimeds

Number of beneficiaries in Campinas per operator [th]



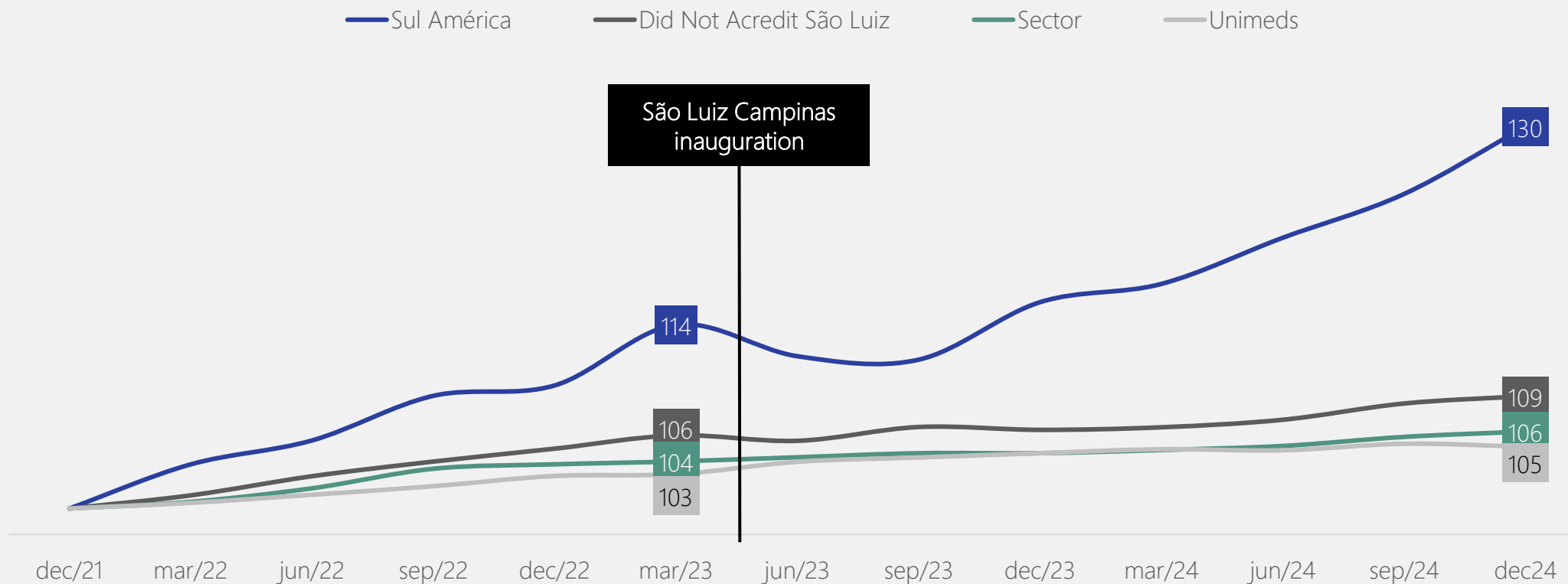


The São Luiz Campinas

...opening it is clear that Rede D'Or has a strong brand

Analyzing the Campinas opening it is clear that Rede D'Or has **reference hospitals** that can be combined with Sul América comercial team to grow

Number of beneficiaries in Campinas per operator [Dec. 2021 = 100 base]





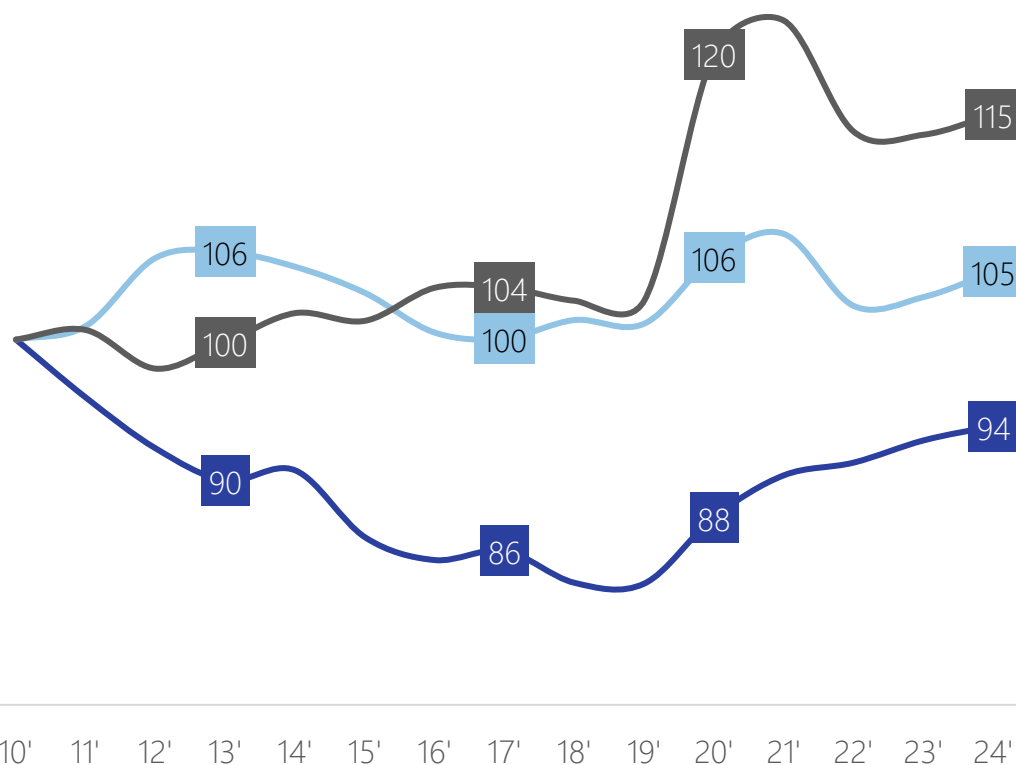
Do philanthropic hospitals pose a threat?

In a different model than private hospitals, they face other challenges

They have survived during turbulence...

EoP number of beds per segment [2010 = 100 base]

— Private — Philanthropic — Public



ahfip ASSOCIAÇÃO DOS HOSPITAIS FILANTRÓPICOS PRIVADOS

London Business School

2024 1996

40 Years as an executive

"The **top philanthropic hospitals** like the ones at Ahfip **do not face such financial challenges** as other ones, because donors are extremely engaged here. About competition, we believe that **São Paulo has very big addressable market that is able to support many different models** that suit beneficiaries, something that is not true for other places that have different competitive environments."

Jaques Rosenzvaig - CEO at Ahfip



Yet, philanthropic rivals are not top operators

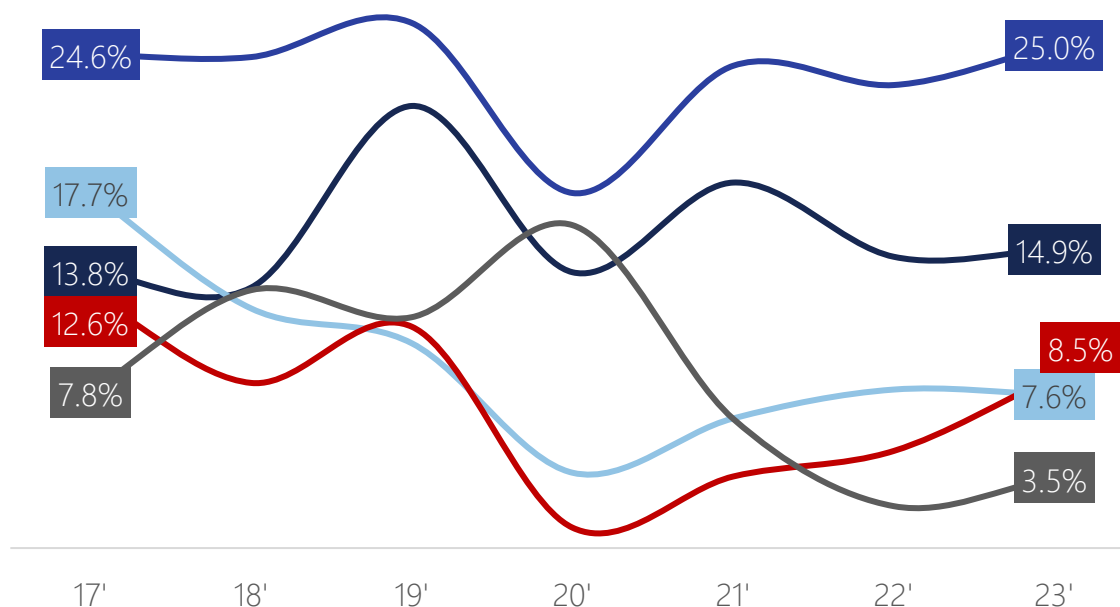
In the long run, efficiency might be a game changer



They remain with much lower margins than Rede D'Or

EBITDA Margin evolution [%]

— Rede D'Or
— Sírio Libanes
— Pequeno Príncipe
— Albert Einstein
— Beneficência Portuguesa



...and as they are facing tough scenarios, this can harm quality in the long run

FOLHA DE S.PAULO

04/02/2025

Traditional Japanes Hospital in SP lays off more than 100 employees and face crisis

Valor

30/06/2023

Philanthropic hospitals operate in the red

ABC DO ABC

07/02/2024

Oswaldo Cruz Hospital to close Vergueiro unit in São Paulo

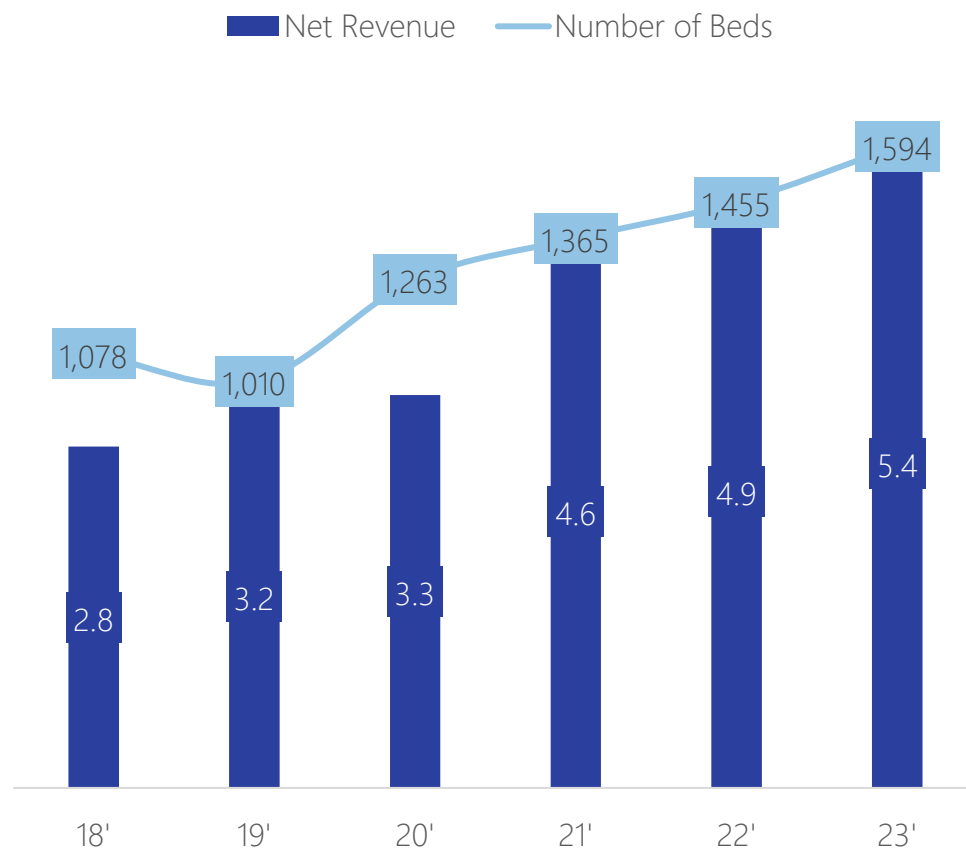


Top philanthropic name: Albert Einstein is expanding

Einstein is the main competitor for the premium segment

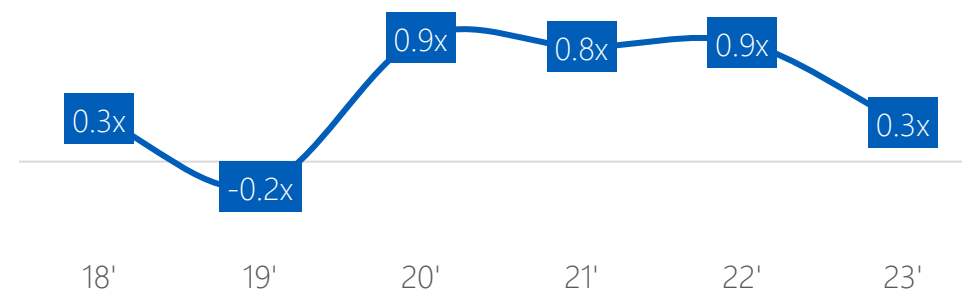
The company has had a strong expansion...

Net Revenue and total beds Evolution [BRL bn; #]



...and has a stable financial position for the expansion

Net debt/EBITDA [x]



MEDICINASA

03/07/2023

Atlântica Hospitals and Einstein announce new hospital in SP

Atlântica and Einstein have formed a joint venture to build a new hospital in Vila Mariana, in the capital of São Paulo

exame.

03/07/2023

Einstein announces BRL 1.2bn investment in new oncology hospital in SP

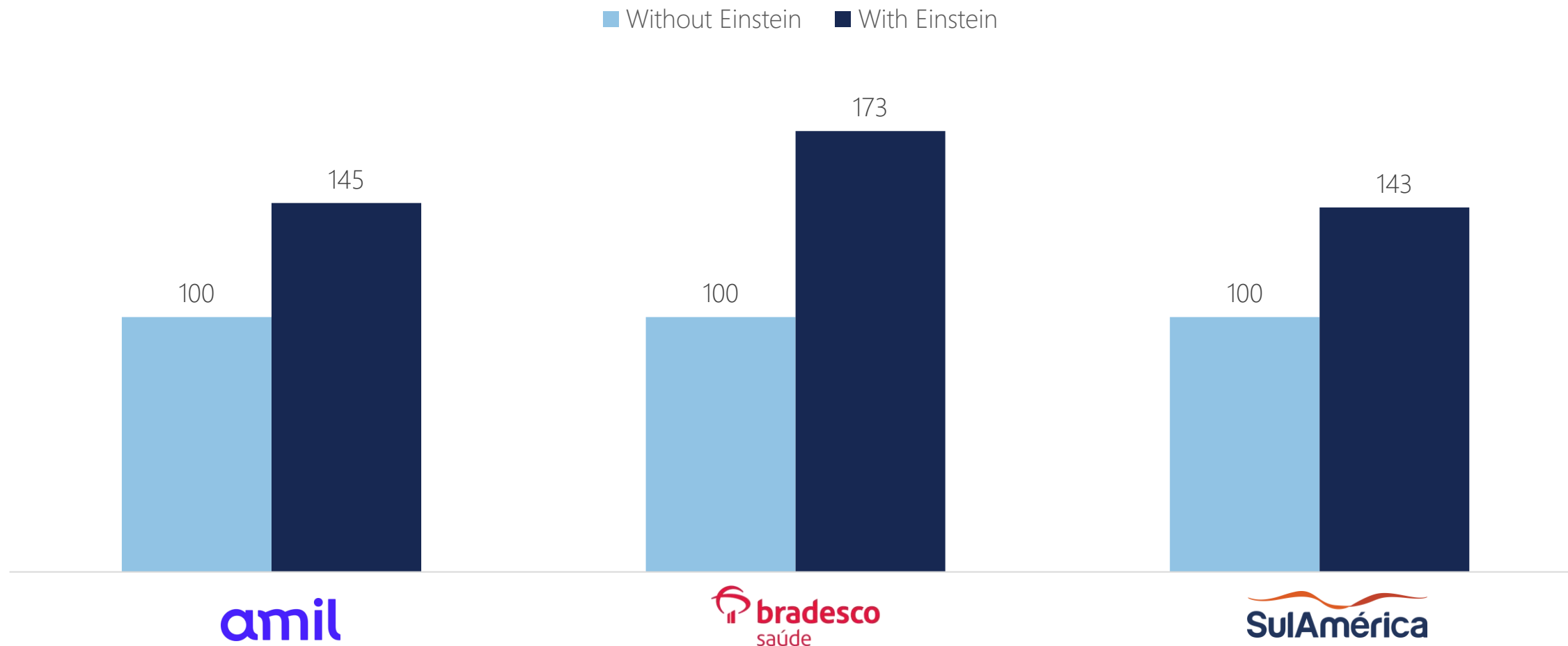
The space will unite research and service and should increase by 40% the number of cancer patients treated by the institution



However, the threat may be limited to very top clusters

Einstein is focused on very high-end customers, who can afford up to 70% premium for it

Average Price of plan [Without Einstein = 100 base]





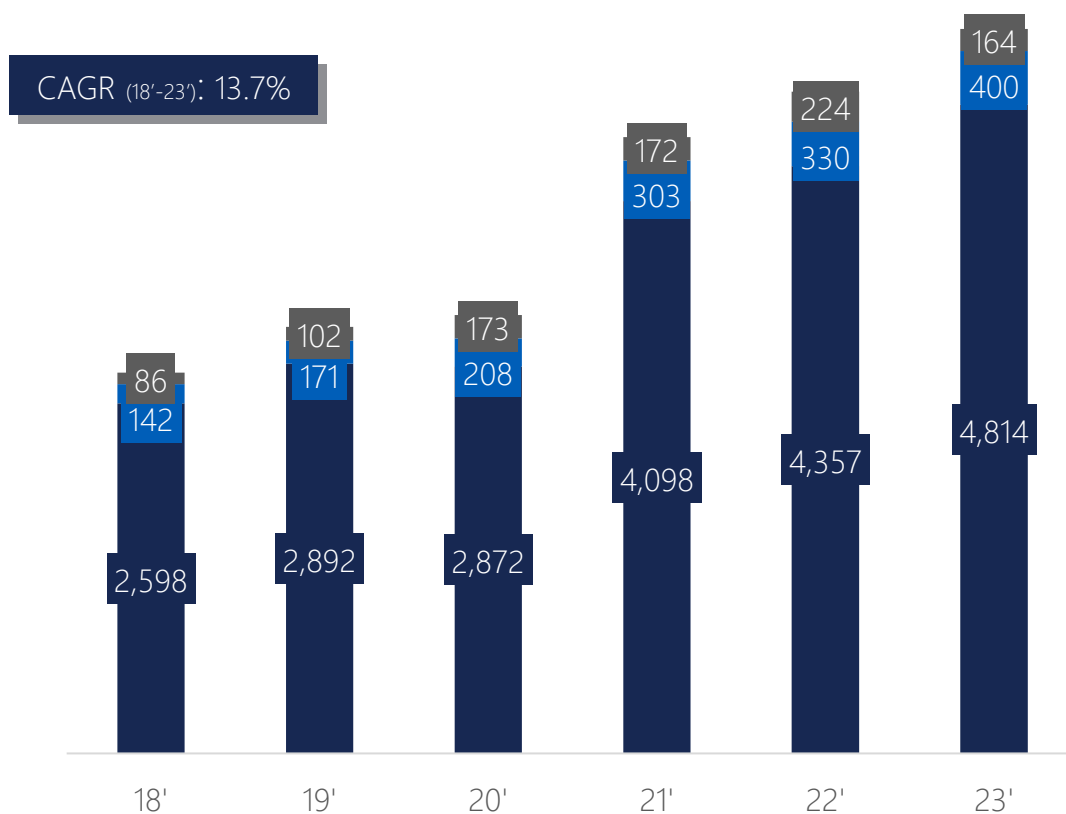
Albert Einstein: Revenue and EBITDA

The philanthropic hospital has expanded in a different mix

The company has education as well as hospitals...

Net Revenue breakdown [BRL mn]

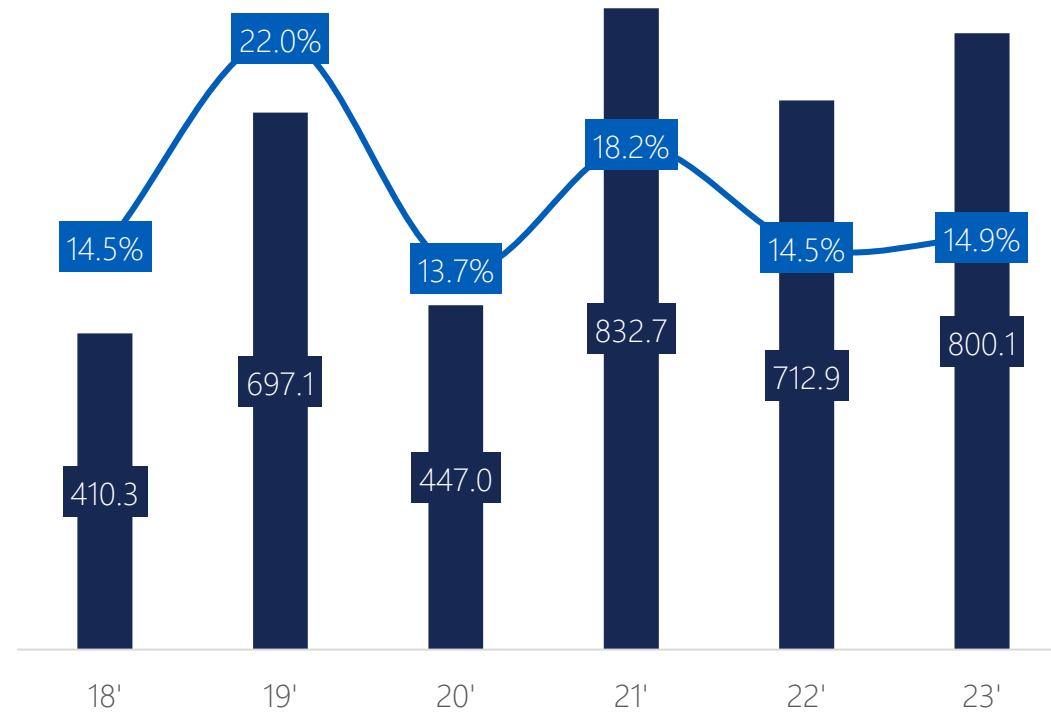
Hospitals Education Others



...what may also explain a difference in margins

EBITDA and EBITDA margin [BRL mn; %]

EBITDA EBITDA Margin





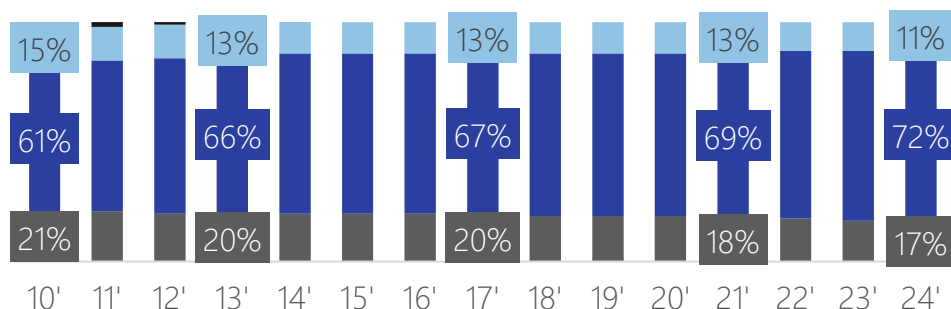
Health Insurance industry drivers

The sector shows a strong correlation with macroeconomic factors

- Different plans have shown **asymmetrical growth**, with **corporate plans increasing their share**.

Types of plan share in beneficiaries [%]

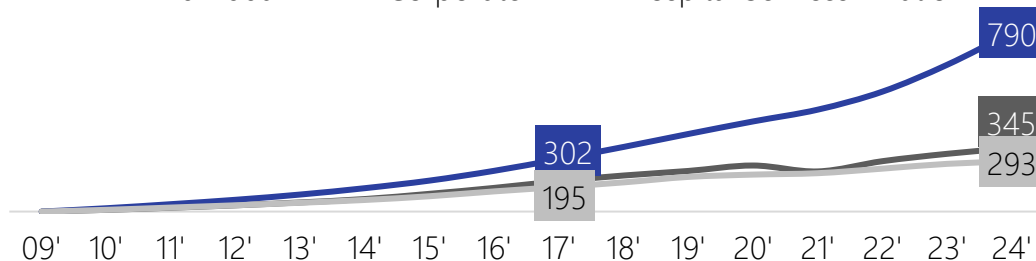
■ Individual/Family ■ Corporate group ■ Affinity group ■ Others



- Given the regulation on individual plans, **the price adjustment is much lower** than that for corporate plans...

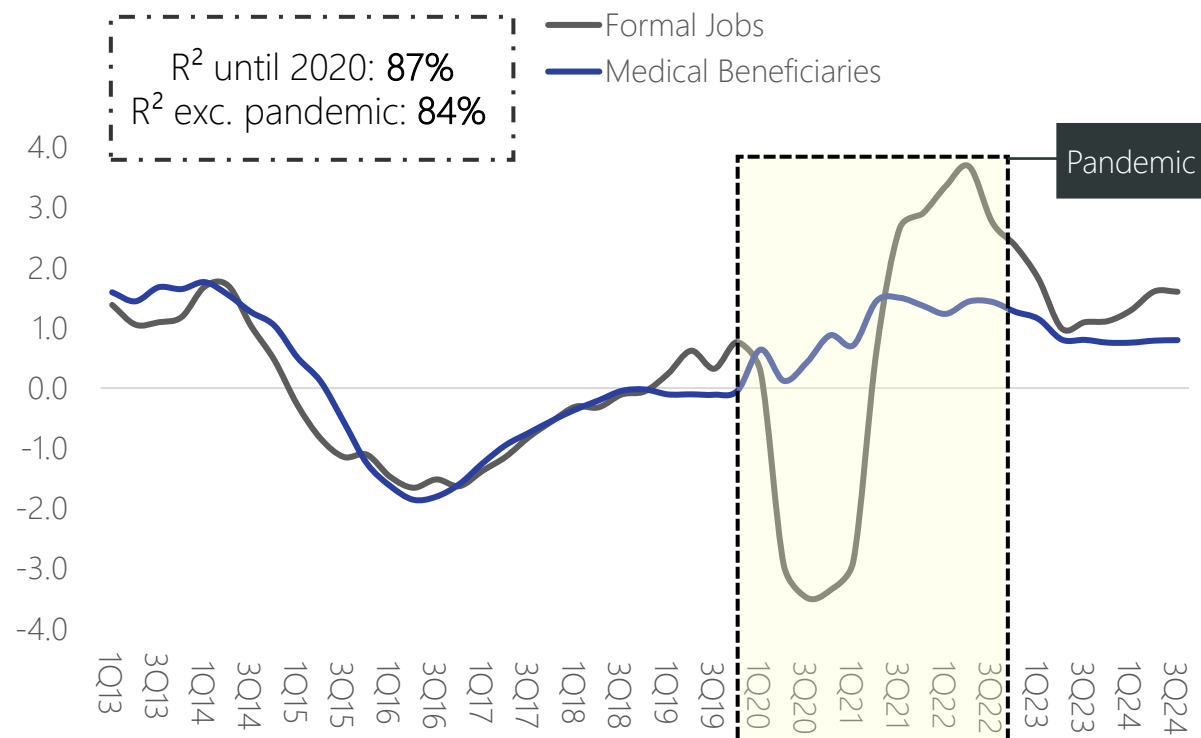
Health Plans price adjustment* vs. Inflation [2009 – 100 base]

— Individual — Corporate — Hospital Services Inflation



- ...however, **corporate plans** also have a weakness. Since they depend on formal employment, they are **sensitive to macroeconomic conditions**.

Delta in Formal Jobs vs Medical Beneficiaries LTM quarterly [mn]



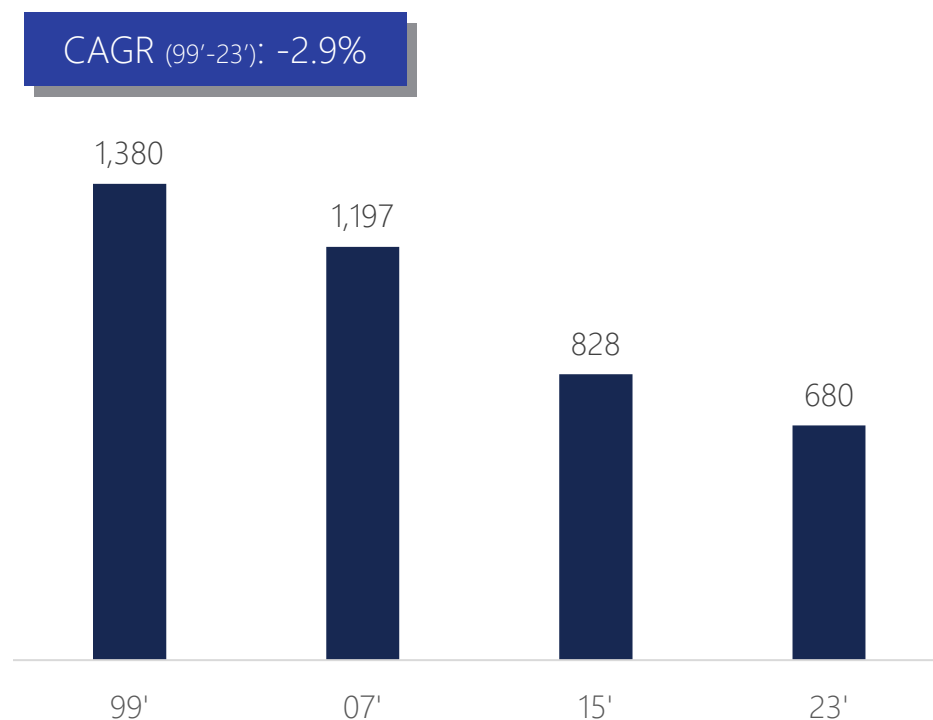


Insurance industry consolidation

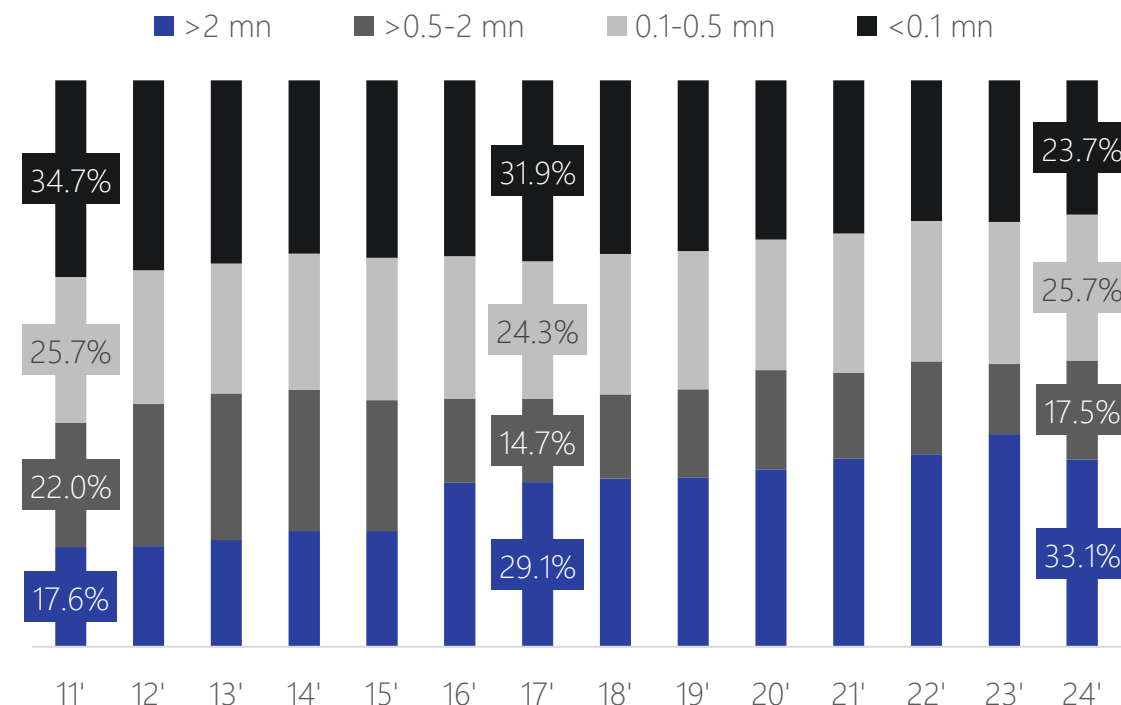
The sector's challenges favor major players

- Despite the often regional dynamics, the sector is becoming increasingly **difficult to operate without scale**, leading to a decline in smaller hospital plan operators and an ongoing consolidation among major players. In times of unpredictability and uncertainty, **the strong get stronger**.

Medical plan operators evolution [#]



Market Share by operator size in beneficiaries [Mn ; %]



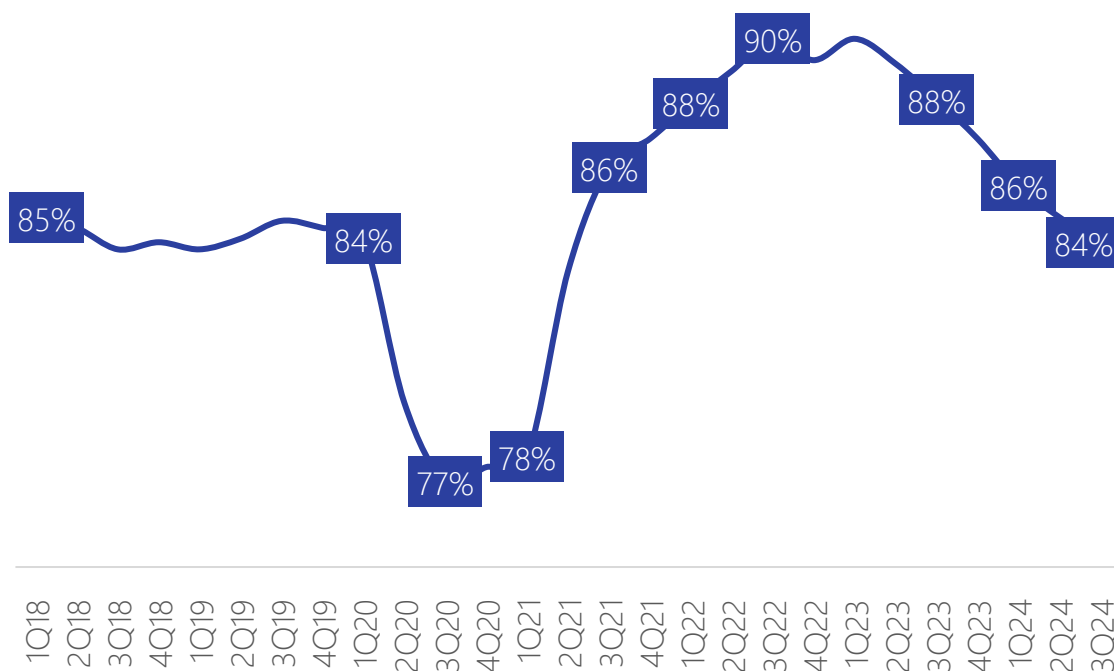


And what about the operator's health?

The industry seems to be on a normalization path going forward

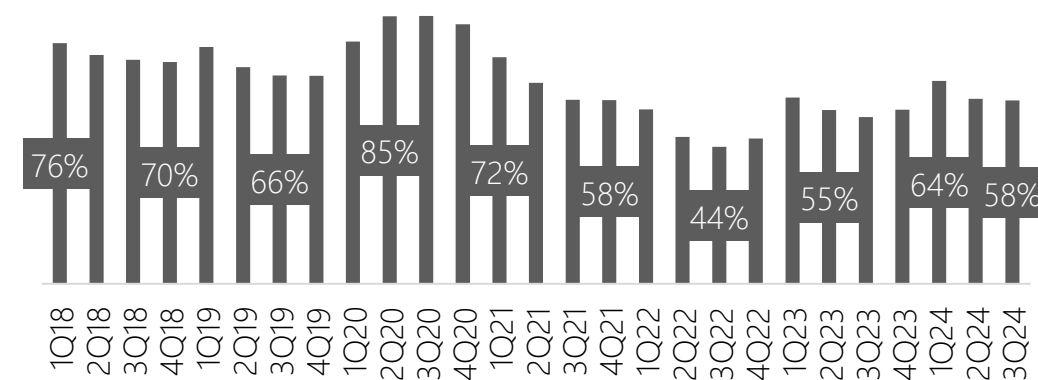
During the pandemic, fewer hospital visits reduced claims, but after Covid, new visits surged **claims to record-high levels**. Now, the MLR seems to be returning to pre-Covid levels...

Industry Medical Loss Ratio LTM quarterly [%]

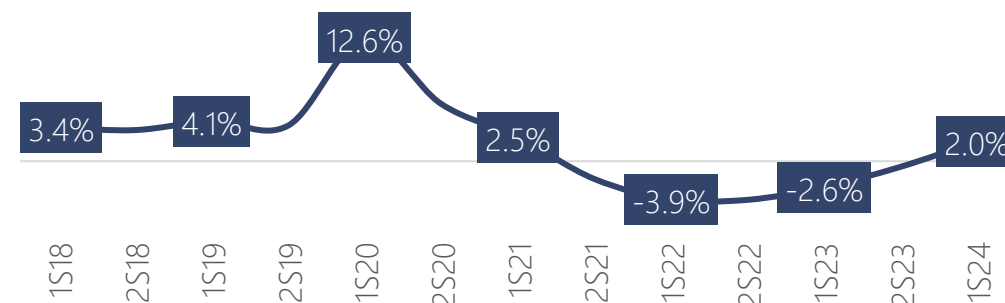


...this significantly **harmed the profitability** of the operators for a while, especially because the **industry's operating margins are low**.

Share of operators with positive accumulated EBIT for the year, quarterly [%]



Industry Total EBIT margin per semester [%]



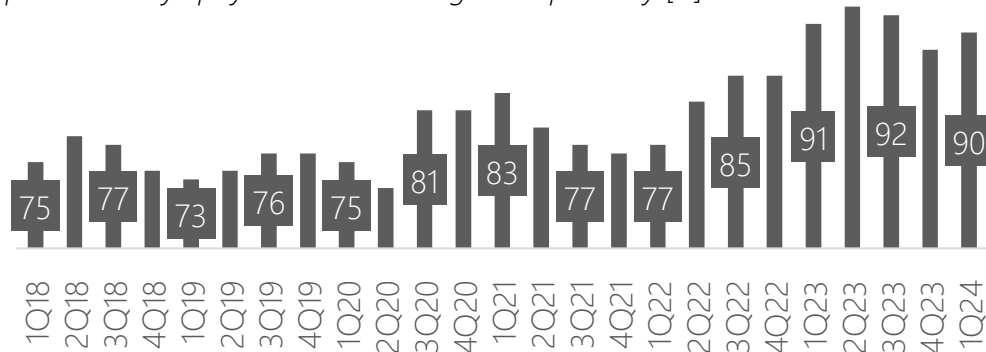


The power imbalance remains

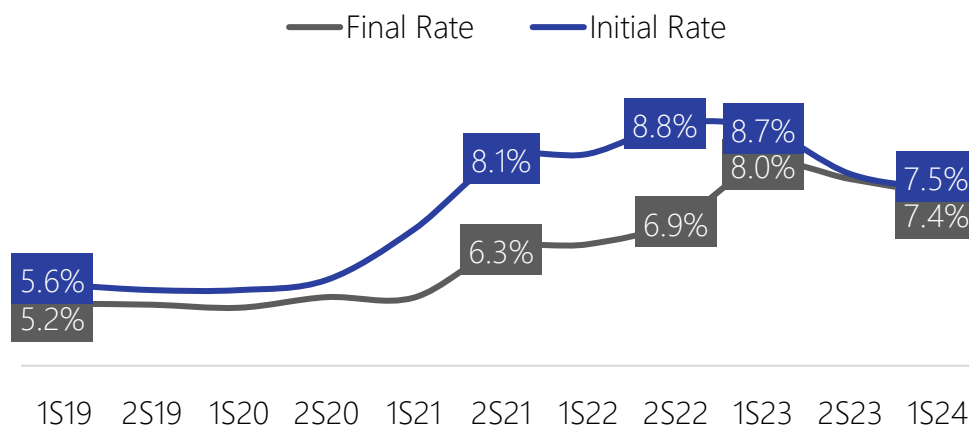
Operators usually set the rules in the industry

- In this scenario, the operators decided to **delay payments to hospitals** and increase **claim denials (glosas)** to strengthen their cash position...

Operators' days payable outstanding LTM quarterly [#]

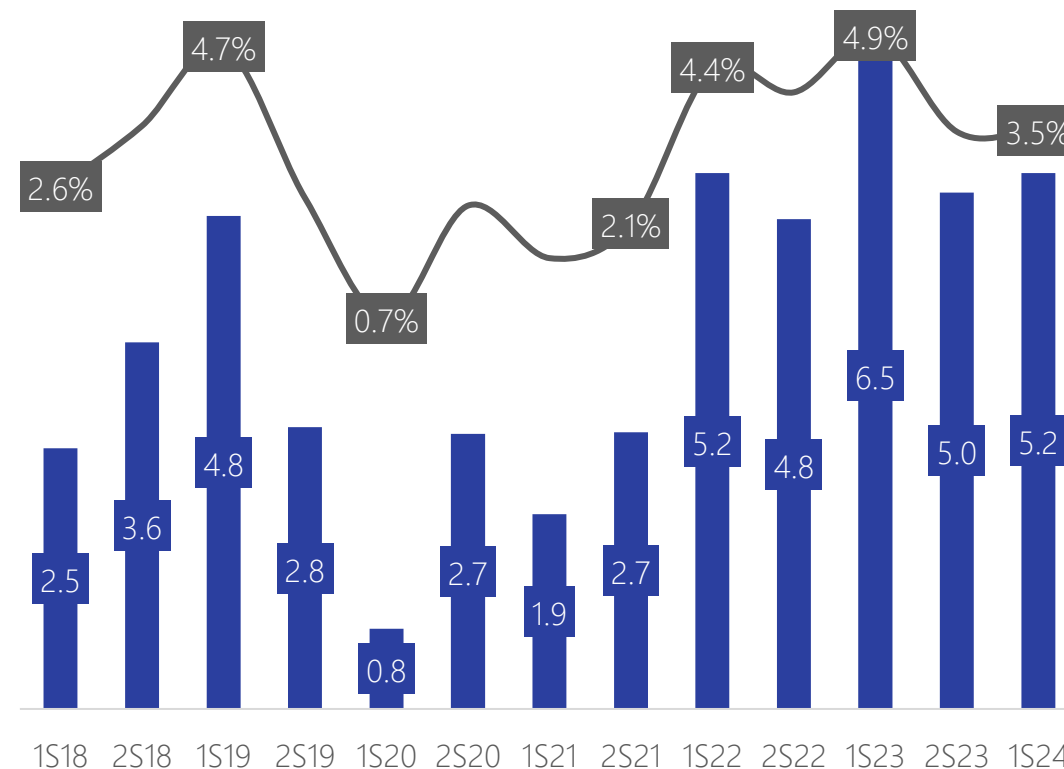


Claim denial rate per semester [%]



- ...leveraging this position to **take advantage of an increased "float"** in a high interest rate environment

Financial and equity results of the industry per semester [BRL bn ; % of contributions/premiums]



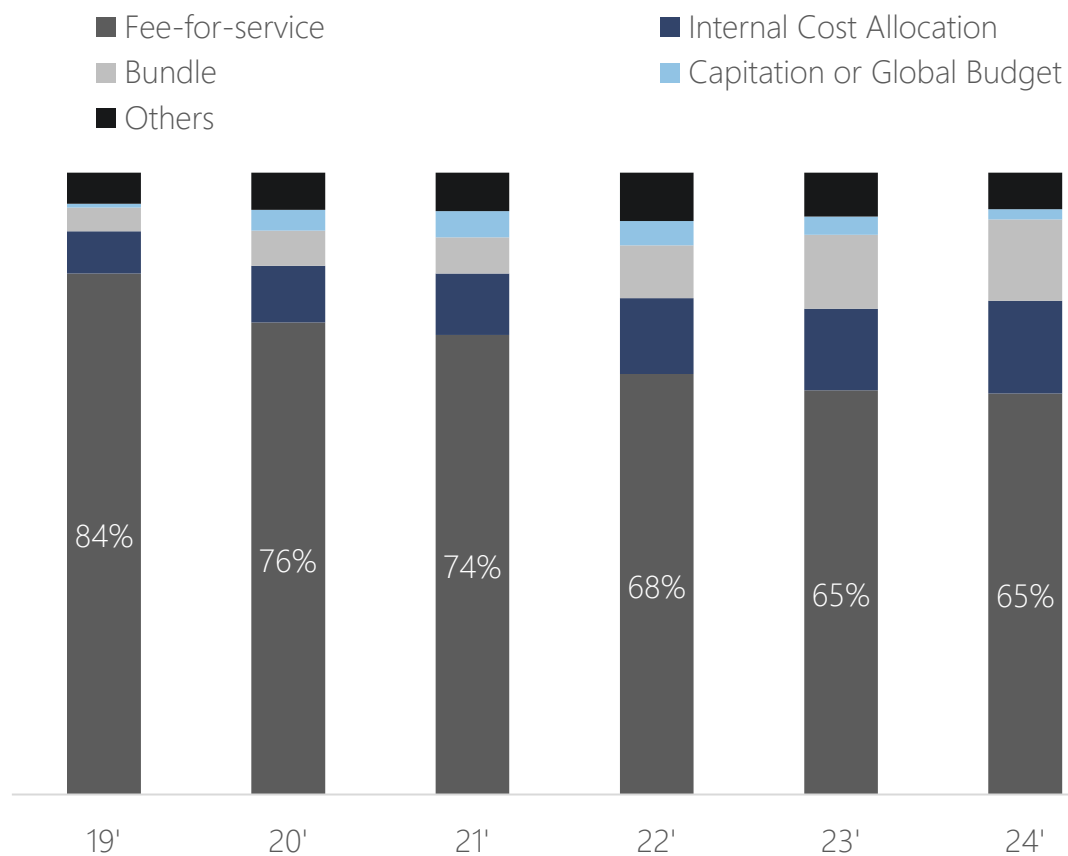


Fee-for-service is losing share

Trying to align interests of HMOs and hospitals

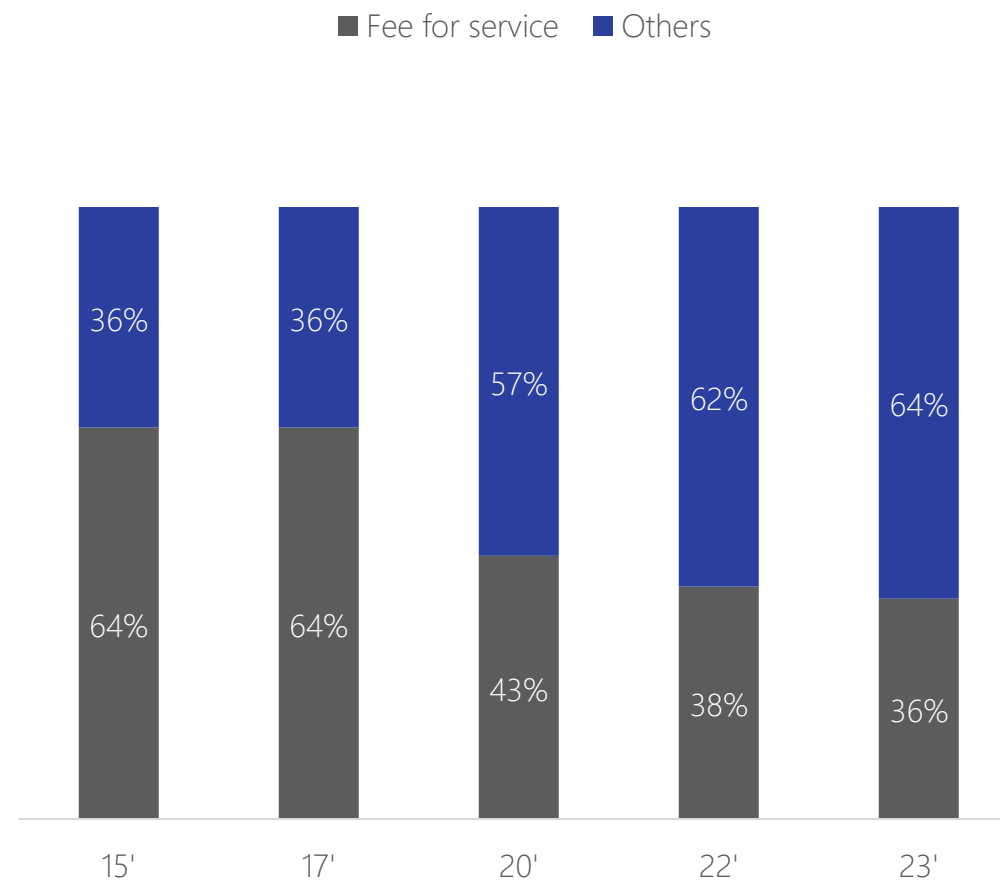
 The industry is adopting reimbursements...

Share of total hospitals reimbursement by model [%]



 ...and Rede D'Or was a pioneer of it

RDOR reimbursement by model [%]



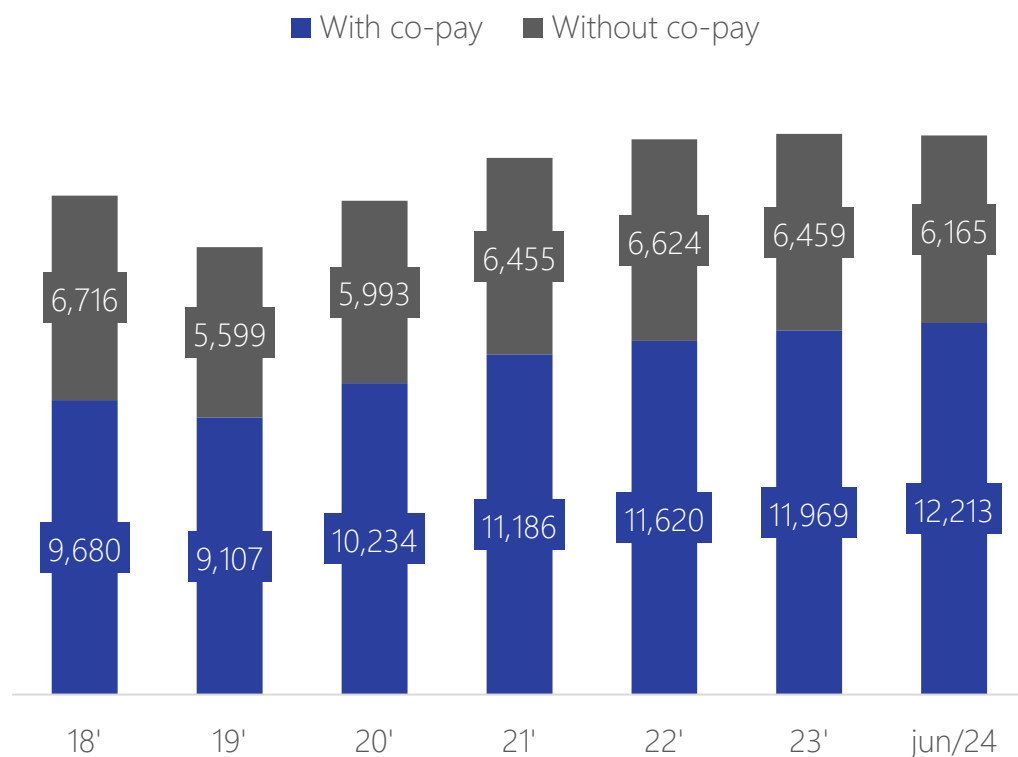


Copayments

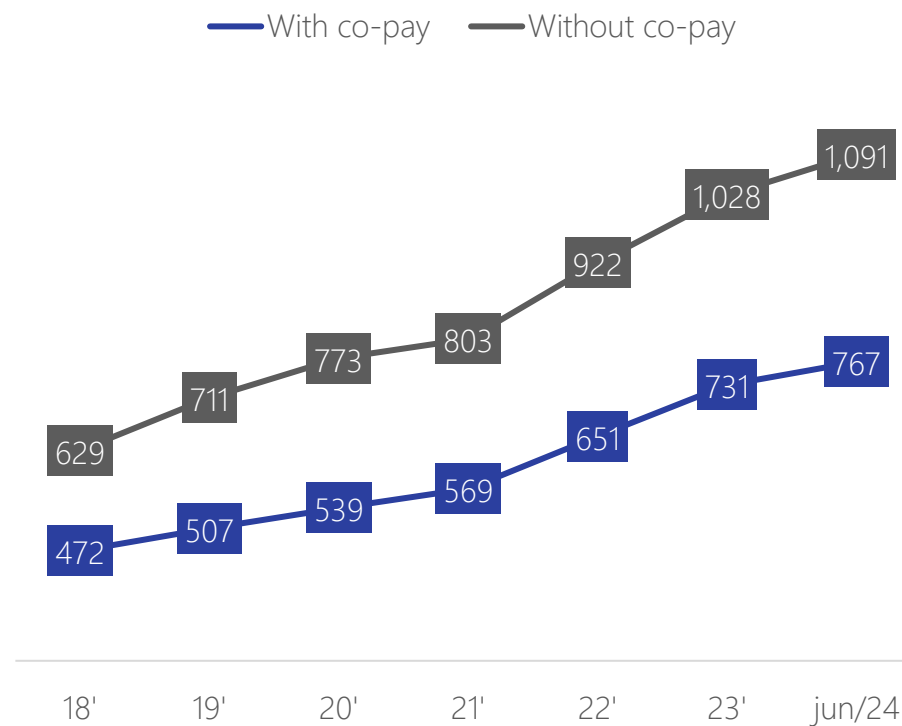
Alignment of interests and lower ticket, allowing a smaller churn

- Copayment plans have been gaining share as they allow for a lower average ticket for beneficiaries and better claims control for insurers, resulting from greater alignment of interests between both parties...

Number of plans by type [#]



Average Commercial Value (ACV) by plan type [BRL]





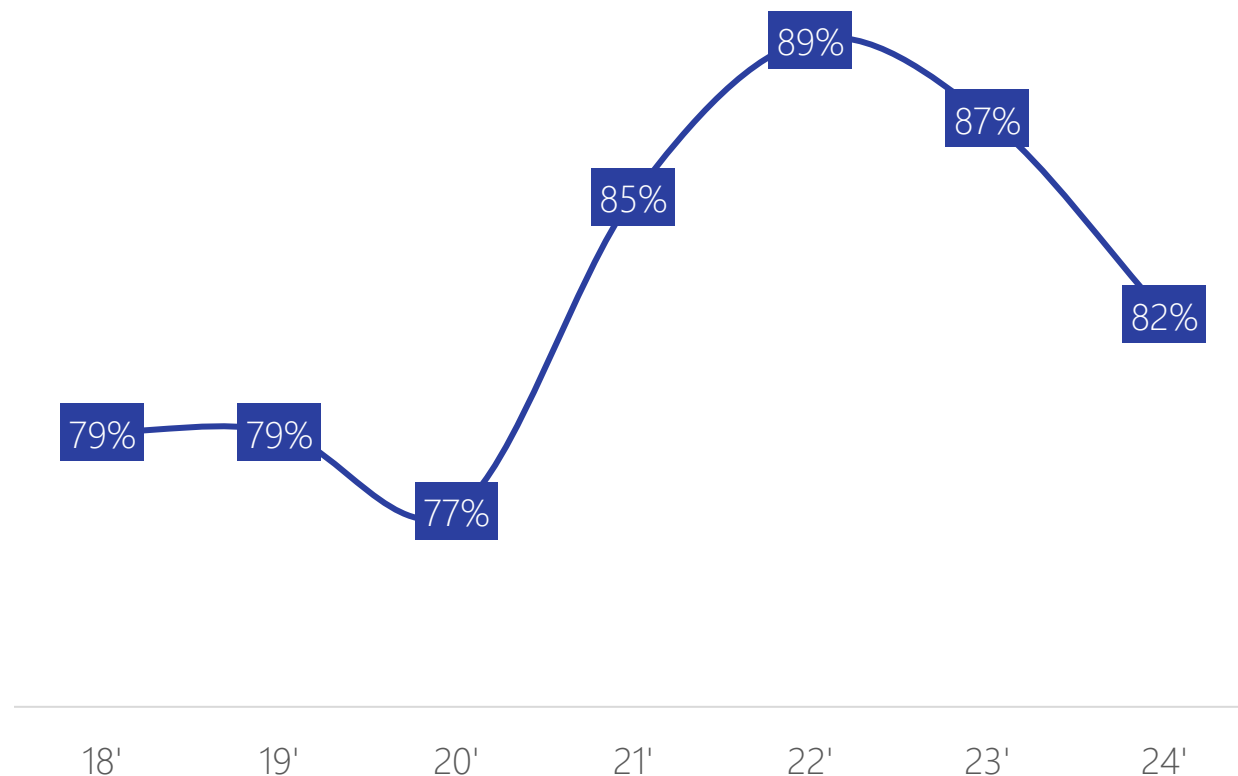
Sul América is paying close attention to frauds

The operator was a pioneer to do so



Since the pandemic, it has had to fight against a higher MLR...

Sul América's Yearly Medical Loss Ratio exc. Changes in Insurance Technical Provisions [%]



...and combating frauds is a way to do it

GLOBO

15/08/2024

SulAmérica blocked R\$73 million in fraudulent refunds in a year and a half

Valor

28/10/2024

Fraud prevention, regional expansion, portfolio expansion: SulAmérica's pillars in the healthcare sector

ESTADÃO

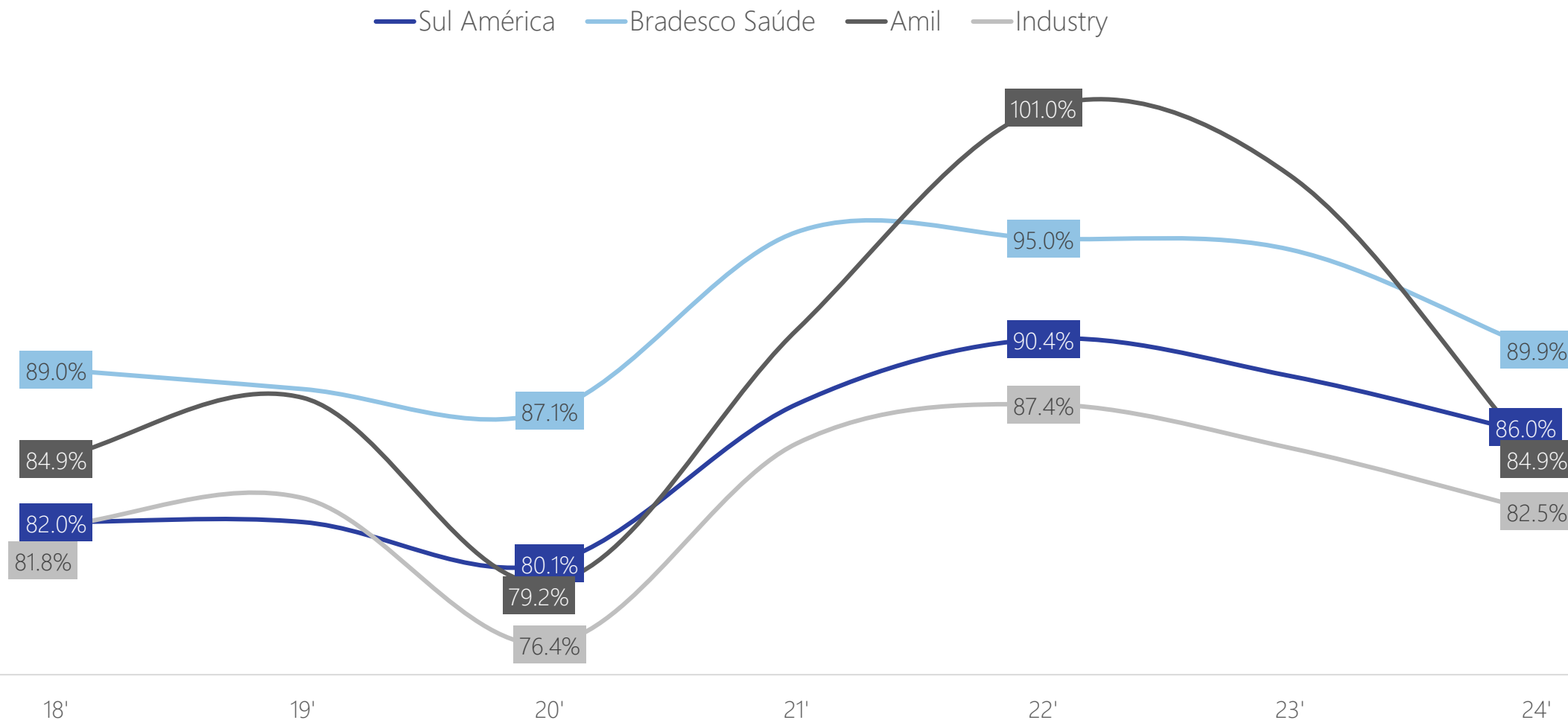
25/09/2024

Health plan: SulAmérica manages to block R\$82 thousand from the account of a user suspected of fraud



Medical Loss Ratio Comp

Medical Loss Ratio [%]

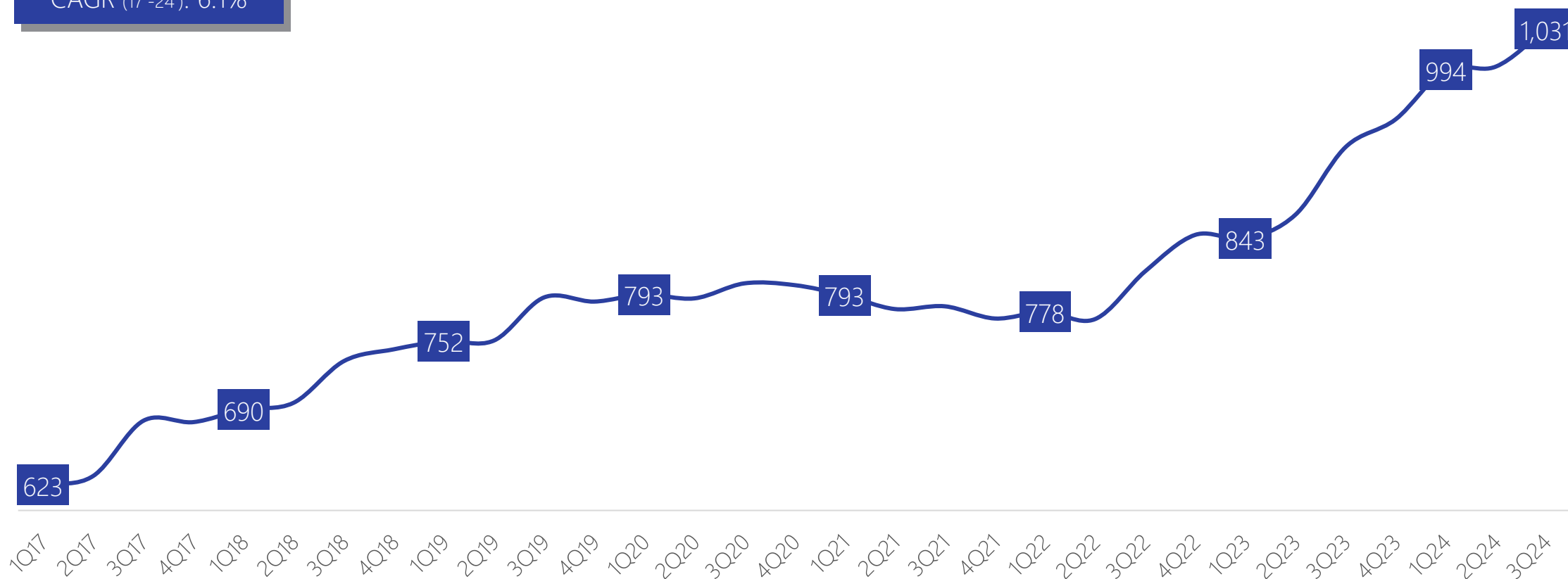




Sul América Health Average Ticket Evolution

Sul América Health Avg Ticket [BRL #]

CAGR (17'-24'): 6.1%

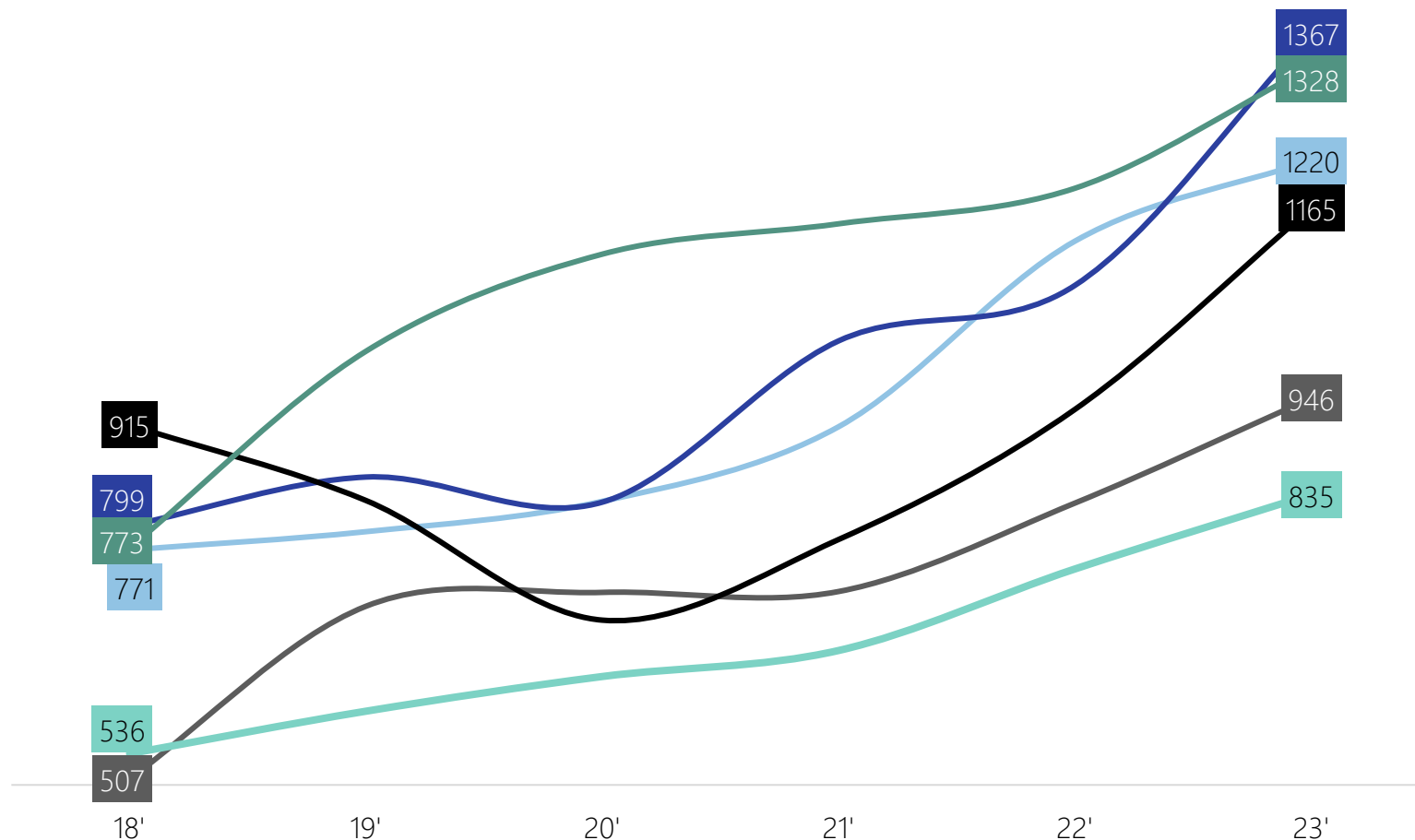




Average Commercial Value Comparison

In recent years, the premium segment has increased its average ticket above the industry average

— Bradesco — Sul América — Amil — Porto — Premium Segment — Industry



Operator	2018 ACV	2023 ACV	ACV CAGR (18'-23')
 amil	507	946	13.3%
 bradesco saúde	771	1,220	9.6%
 SulAmérica	799	1,367	11.3%
 Porto Saúde	915	1,165	4.9%
Premium Segment	773	1,328	11.4%
Industry Avg.	536	835	9.3%

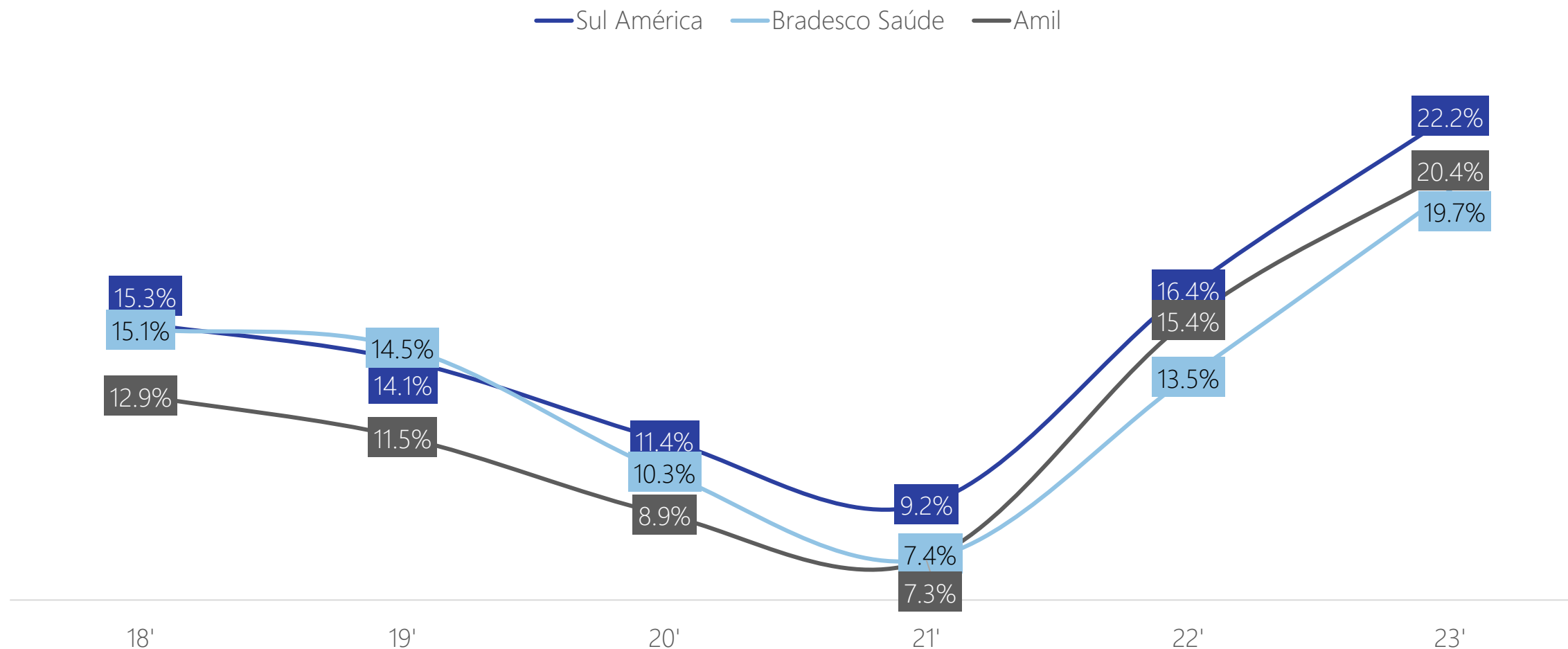
Amil's ACV growth is driven by a shift to a premium plan mix, a trend expected to accelerate with its acquisition by Seripieri.



Price adjustments by insurer

Sul América has higher adjustments

YoY Price adjustments in group plans yearly [%]

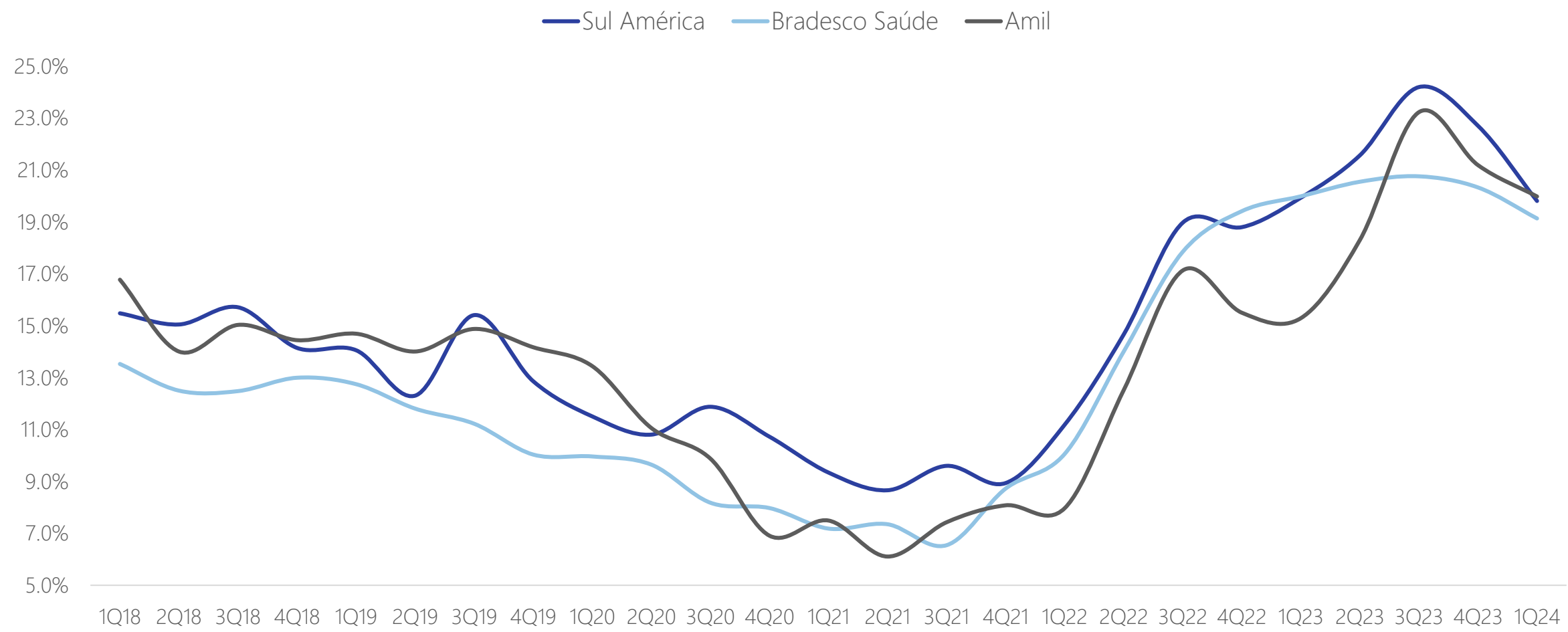




Price adjustments by insurer

Sul América has higher adjustments

YoY Price adjustments in group plans quarterly [%]

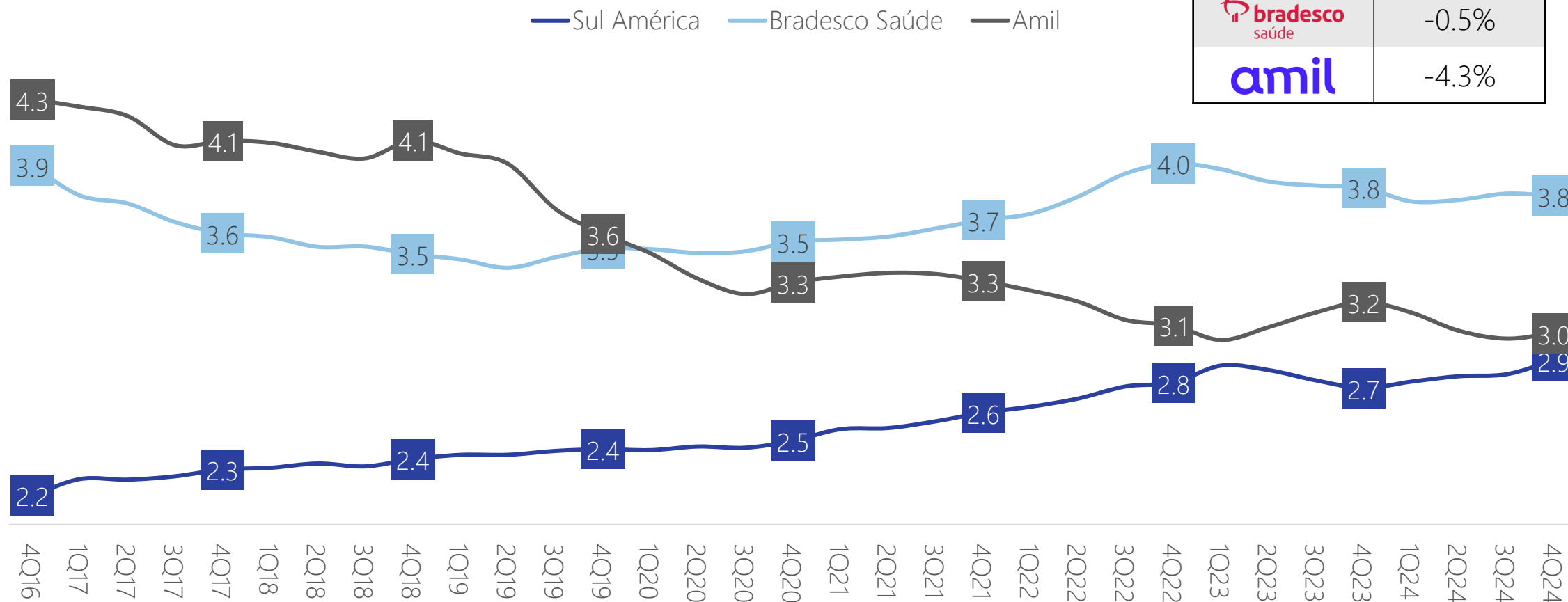




Beneficiaries by insurer evolution (pro forma)

Beneficiaries [mn]

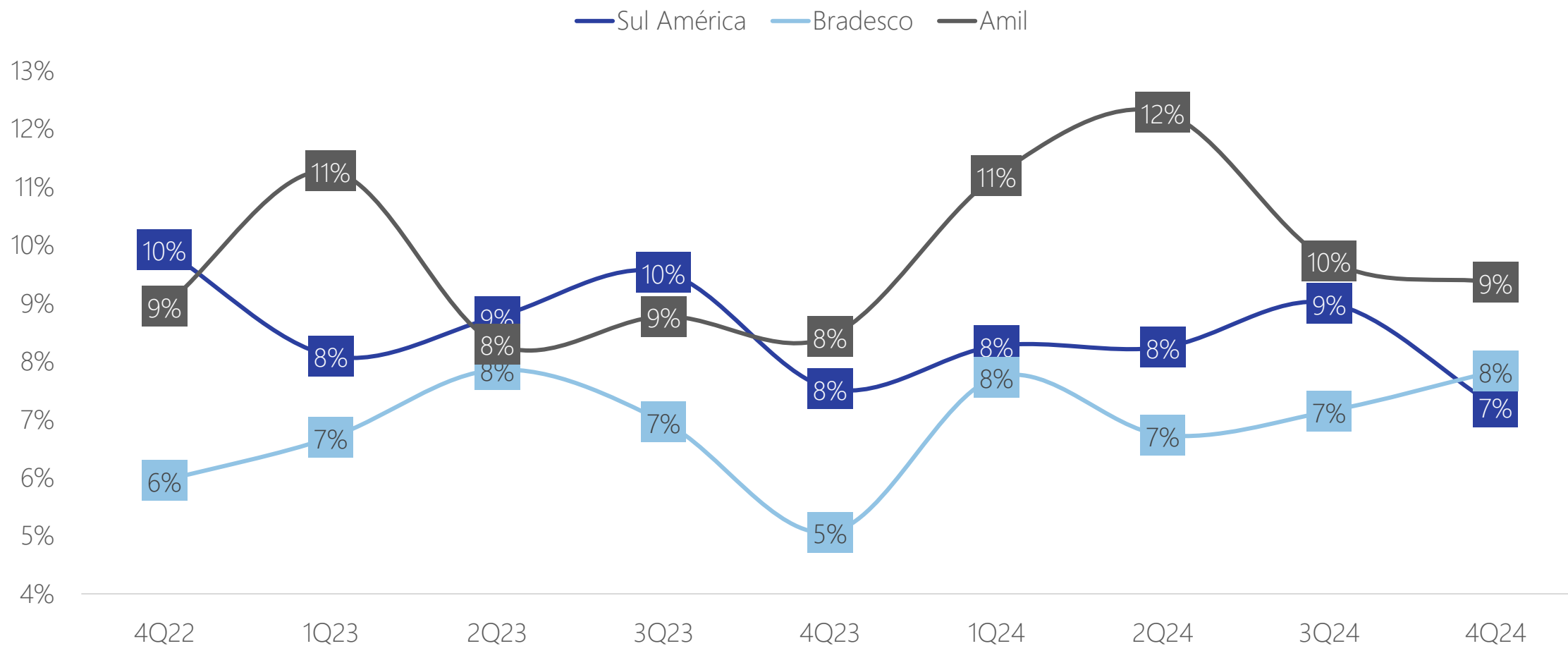
Operator	CAGR (16'-24')
 SulAmérica	3.7%
 bradesco saúde	-0.5%
 amil	-4.3%





Churn by insurer

Churn rate [% of beneficiaries]





ESG: Environmental

Taking responsibility for a better planet

Rede D'Or has several strategies aimed at the environment and positive impact...

Main Objectives:



Adopt **low water consumption** equipment for 90% of the system



Achieve a **30% recyclable waste** rate by 2030



Adopt **high-performance LED** lamps in at least 90% of the system



The company assesses its **suppliers for environmental risks**

Rede D'Or is part of B3's group of sustainable companies

ICO2 B3

Environmental

E

Social

S

Governance

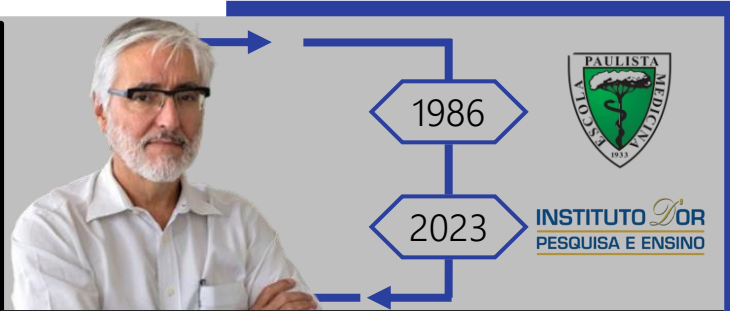
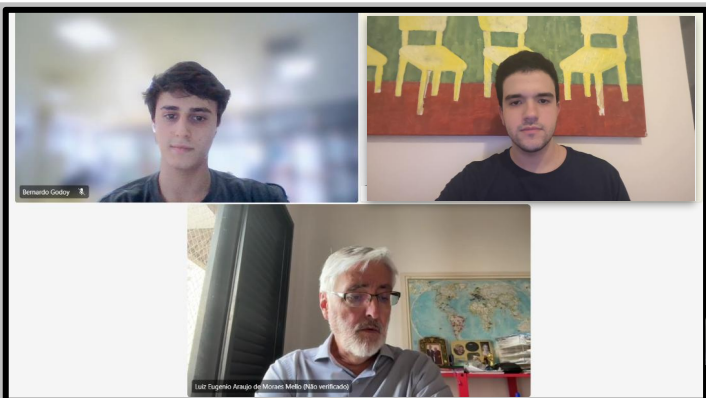
G



ESG: Social

A healthcare company with an institute focused on research and technology

We talked to the one who knows best about IDOR's efforts to change society...



Luiz Mello – Science and Innovation Director at IDOR

"From the start, IDOR has focused on resonance imaging, but it has not stopped there. The institute has also done much research on intensive care and other treatments. We have developed as well Open D'Or to invest in companies that have a promising use of technology. But the main benefit for us and RDOR is being able to use Technologies that for a company do not make sense, but do for research, such as the Nanostring GeoMx."

Environmental
E

Social
S

Governance
G

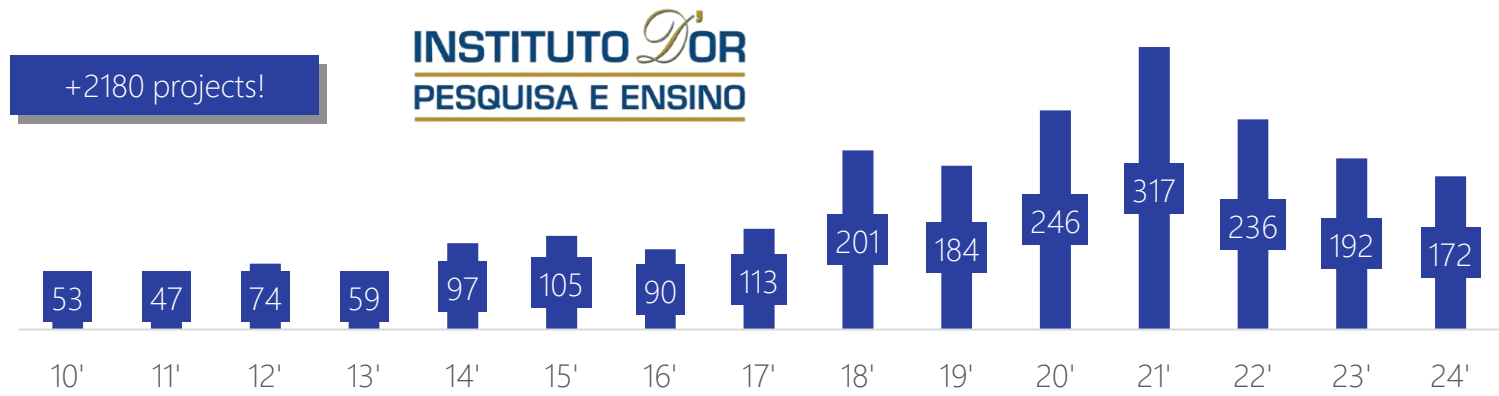


ESG: Social

Taking a look at the big numbers

 IDOR has done incredible work in the past 15 years!

Research projects per year [#]



+100 PHDs working together



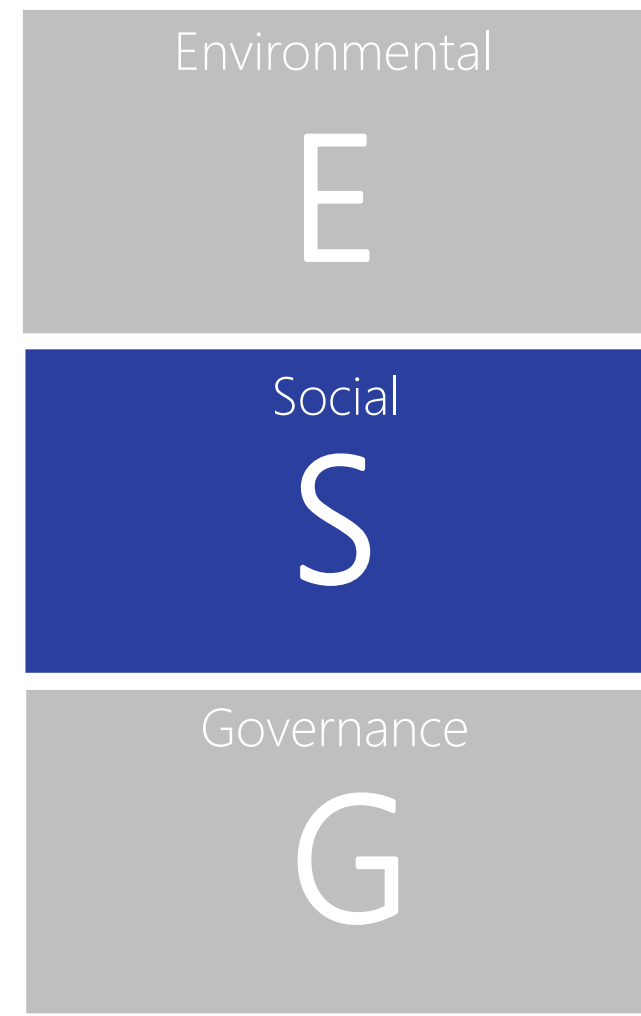
+2,300 journals publications



Partnerships with +80 countries



+10 developing products



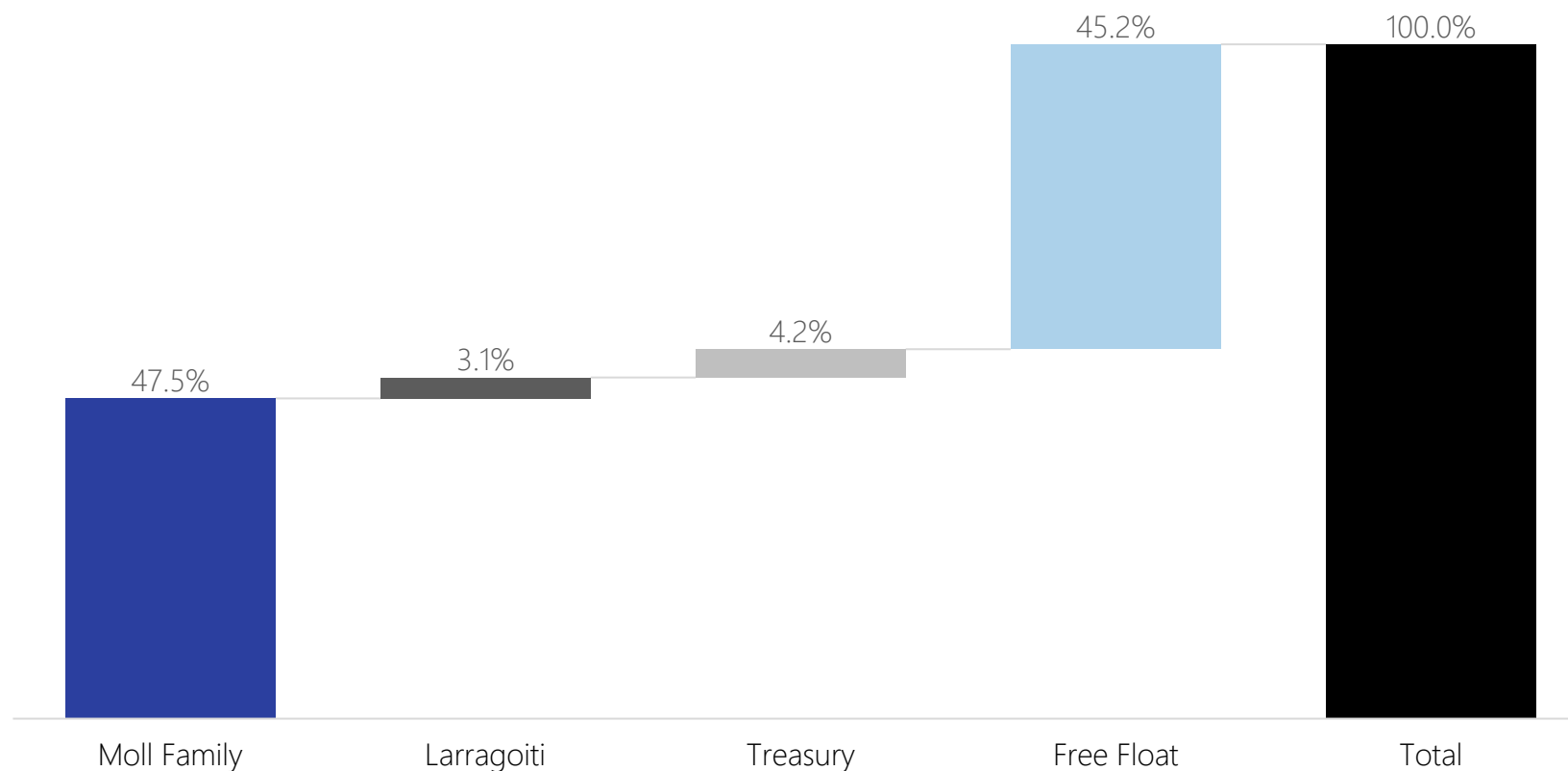


ESG: Governance

A business with a family in charge

- The Moll Family has set the pace from day one

Share ownership breakdown [%]



Environmental

E

Social

S

Governance

G



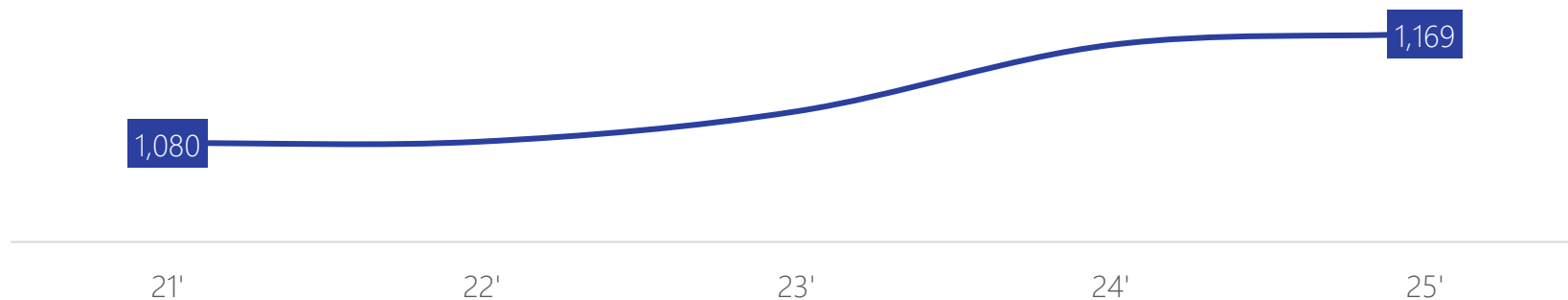
ESG: Governance

A business with a family in charge



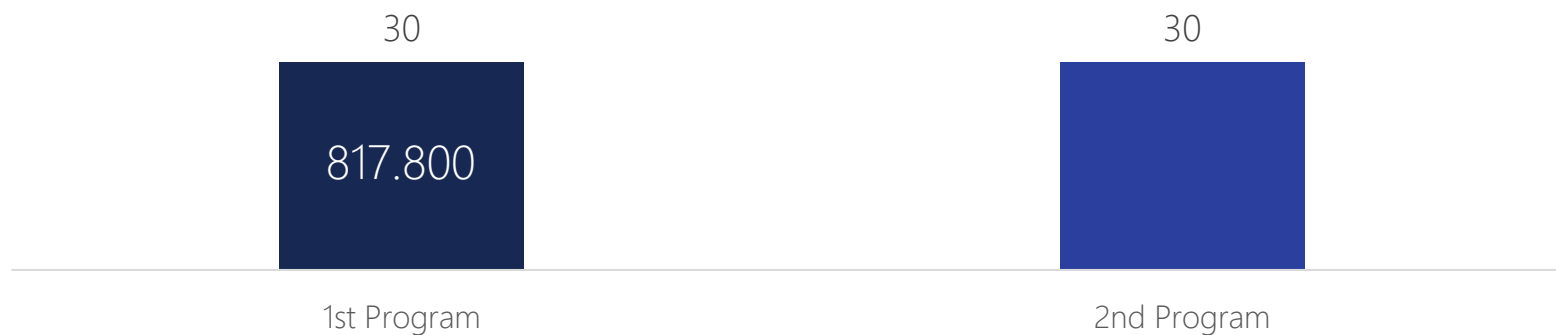
The controlling Family has raised their bets in the company...

Moll family number of shares [mn]



...and also started share buybacks at the current stock price

Buyback shares and total value [mn; BRL mn]



Environmental

E

Social

S

Governance

G



A top execution team at the helm

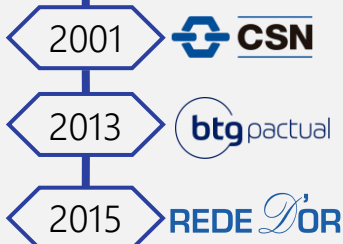
The company has a top tier management

Years at Rede D'Or [#]



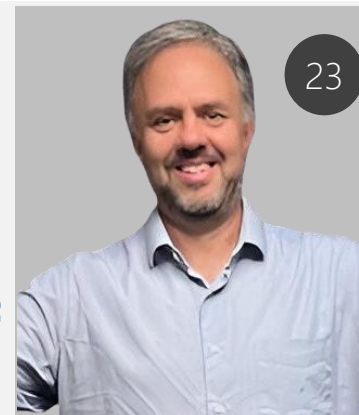
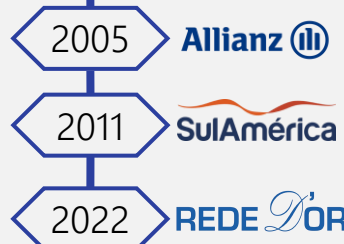
Otávio Lazcano
CFO at Rede D'Or

10



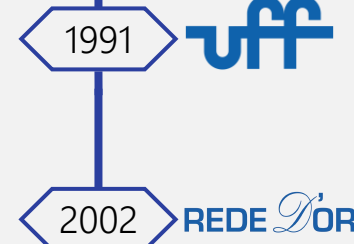
Raquel Reis
CEO at Sul América

14



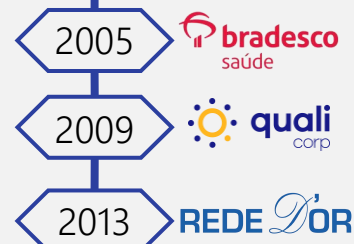
Jamil Muanis
VP at Rede D'Or

23



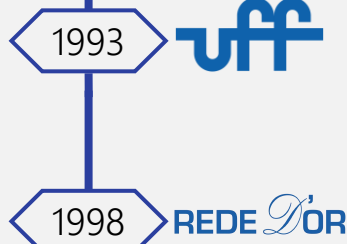
Heráclito Gomes
Vice Chairman

12



Rodrigo Gavina
VP at Rede D'Or

27



Leandro Tavares
VP at Rede D'Or

7



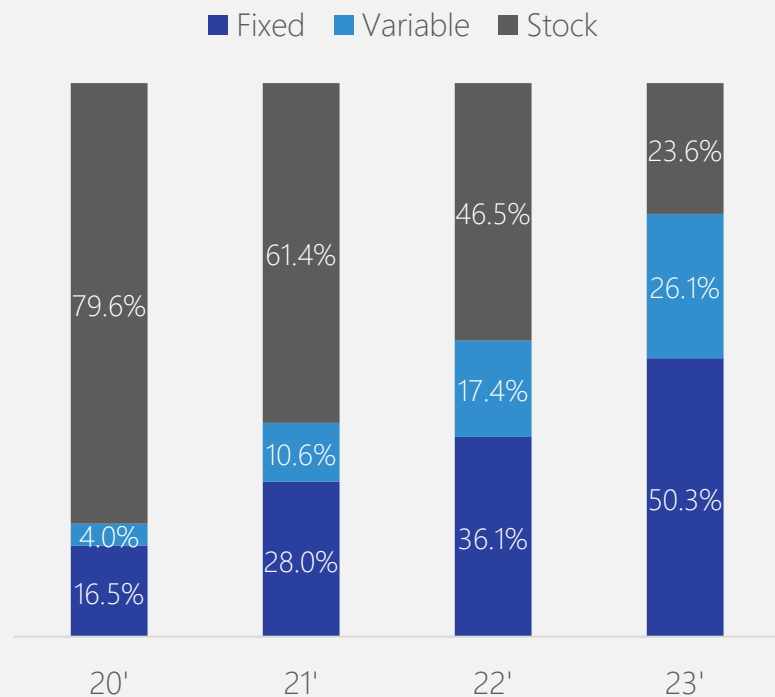


Consistency and in line with shareholders

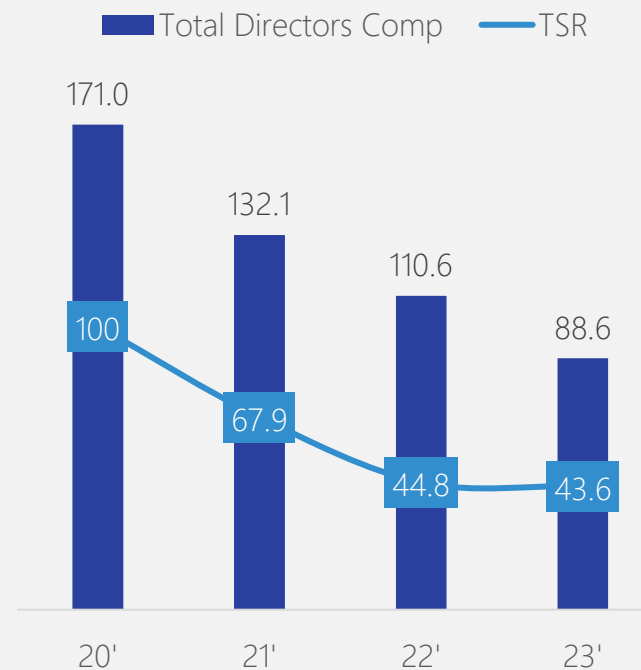
As the performance goals were not hit, compensation adapted to it

The company has maintained compensation in line with shareholder return

Directors compensation breakdown [%]



Total Directors compensation; Total Shareholder Return [BRL mn; %]



Previous year max. expected pay vs. effectively paid [BRL mn]



Compensation based mostly on Financial Performance, EBITDA and NPS



What about new M&As?

Even as assets have worsened, there are still some potential targets for Rede D'Or

Potential Target	Owner	Target Operating beds 25'	Target City/Region	RDOR hospitals in the region	RDOR operating beds in the region	Non-Public beds in the region	RDOR Share
 Promater	Américas	66	Natal/RN	0	0	4,050	0%
 NOVE DE JULHO	Ímpar	294	São Paulo/SP	28	3,770	29,000	13.0%
 Anchieta Kora Saúde	Kora	410	Brasília/DF	4	555	7,160	7.8%
 São Domingos	Américas	353	São Luiz/MA	1	170	4,550	3.7%

M&A may be an option as rivals face extreme hurdles

Valor

24/07/2024

Hospitals owned by family groups and investment funds become acquisition targets

Valor

19/12/2023

Hospital Anchieta, part of Kora Saúde, carries out three transactions to reduce debt

exame.

07/05/2024

Amil considers selling hospitals and health insurance portfolio in the Northeast



M&A Simulation – Nove de Julho

We still stressed this for other scenarios that Rede D'Or could acquire it

IRR Sensitivity [EBITDA %; EV/bed aquired]

EV/Bed

Hospital	Nove de Julho
EV/Bed	4.0
Initial EBITDA Margin	15%
Post-synergies EBITDA Margin	30%
WK/Revenue	25%
Initial Occupancy Rate	82%
Initial Avg Ticket	11,000
Operating Beds	294
Maintenance Capex / Revenue	2.5%
ticket growth	5%
perpetuity g	5%
Leverage	75%
Depreciation Rate	9.5%
Tax Rate	34.0%

Mature EBITDA
Margin

	6	5	4	3	2
20%	11%	13%	17%	21%	27%
25%	15%	18%	22%	26%	33%
30%	19%	22%	25%	30%	37%
35%	21%	24%	28%	33%	40%
40%	23%	26%	30%	35%	42%

	Y0	2026	2027	2028	2029	2030	2031	2032	2033	2034	2035
Ticket		11,000	11,550	12,128	12,734	13,371	14,039	14,741	15,478	16,252	17,065
Operating Beds		294	294	294	294	294	294	294	294	294	294
Occupancy Rate		82%	83%	84%	85%	85%	85%	85%	85%	85%	85%
Net Revenue (BRL mn)		955	1,015	1,078	1,146	1,203	1,263	1,326	1,392	1,462	1,535
Capex	-294	-23.9	-25.4	-27.0	-28.6	-30.1	-31.6	-33.2	-34.8	-36.6	-38.4
EBITDA Margin		15%	20.0%	25.0%	30.0%	30.0%	30.0%	30.0%	30.0%	30.0%	30.0%
D&A		-114	-116	-119	-122	-125	-128	-131	-134	-137	-141
WK		239	254	270	286	301	316	332	348	366	384
Delta WK		239	15	16	17	14	15	16	17	17	18
Financial Result		-123	-123	-123	-123	-123	-123	-123	-123	-123	-123
EBT		-94.3	-37.0	27.1	98.5	112.8	127.9	143.7	160.3	177.7	196.0
Tax Rate		34.0%	34.0%	34.0%	34.0%	34.0%	34.0%	34.0%	34.0%	34.0%	34.0%
Lucro liquido		-62	-24	18	65	74	84	95	106	117	129
FCFE	-294	-211	52	94	141	155	165	177	188	201	214
Debt	882	882	882	882	882	882	882	882	882	882	882
Kd	14%	14%	14%	14%	14%	14%	14%	14%	14%	14%	14%
PP&E	1,176	1,086	995	903	810	715	619	522	423	322	219



M&A track record

Date	Hospital	EV (mn)	# of beds	Revenues (mn)	EV/bed (R\$000)	EV/revenues	Revenue/bed (th)
Nov-07	Hope-Esperança	n.a.	n.a.	100	n.a.	n.a.	n.a.
Apr-10	Hospital Brasil	n.a.	249	250	n.a.	n.a.	1,004
Sep-10	São Luiz	1,030	803	630	1,283	1.6x	785
Sep-10	Hospital Assunção	n.a.	n.a.	70	n.a.	n.a.	n.a.
Jan-10	Hospital Rio de Janeiro	n.a.	n.a.	n.a.	n.a.	n.a.	n.a.
Feb-11	Hospital Prontolinda	n.a.	120	n.a.	n.a.	n.a.	n.a.
Dec-11	Hospital Vivalle	n.a.	60	n.a.	n.a.	n.a.	n.a.
Dec-11	Nossa Senhora de Lourdes	300	n.a.	175	n.a.	1.7x	n.a.
May-12	Santa Lúcia	n.a.	n.a.	500	n.a.	n.a.	n.a.
Jan-12	Hospital do Coração	n.a.	n.a.	n.a.	n.a.	n.a.	n.a.
Jan-13	Hospite Norte D'Or	n.a.	n.a.	93	n.a.	n.a.	n.a.
Jan-13	Hospital IFOR	85	n.a.	81	n.a.	1.1x	n.a.
Dec-14	Hospital Villa-Lobos	123	109	162	1,132	0.8x	1,486
Dec-14	Sino-Brasileiro	346	145	167	2,386	2.1x	1,149
Jan-15	Maternidade Bartira	118	108	109	1,096	1.1x	1,008
Jan-15	Hospital Fluminense	n.a.	n.a.	n.a.	n.a.	n.a.	n.a.
Jan-16	Alpha-Med	54	n.a.	53	n.a.	1.0x	n.a.
Jan-16	Ribeirão Pires	196	115	137	1,705	1.4x	1,194
Apr-16	Hospital Memorial São José	141	155	183	912	0.8x	1,181
Apr-17	São Vicente (Sator)	125	97	153	1,291	0.8x	1,581
Jan-18	UDI	441	154	354	2,867	1.2x	2,301
Jan-18	São Rafael	810	356	263	2,275	3.1x	738
Jan-18	Samer	78	125	104	627	0.8x	829
Jan-19	Rio Mar	90	89	64	1,009	1.4x	724
Jan-19	São Lucas	314	200	342	1,570	0.9x	1,711
Jan-19	Cardio Pulmonar	231	180	n.a.	1,282	n.a.	n.a.
Jan-19	Hospital Pro Criança Cardiac	n.a.	n.a.	n.a.	n.a.	n.a.	n.a.
Jun-19	Perinatal	802	295	268	2,719	3.0x	910
Jun-19	Hospital Aviccena	93	110	93	849	1.0x	849
Aug-19	Qualicorp (10%)	n.a.	n.a.	n.a.	n.a.	n.a.	n.a.
Nov-19	Hospital Santa Cruz	361	171	128	2,112	2.8x	750
Dec-19	Hospital São Carlos	215	123	109	1,744	2.0x	884

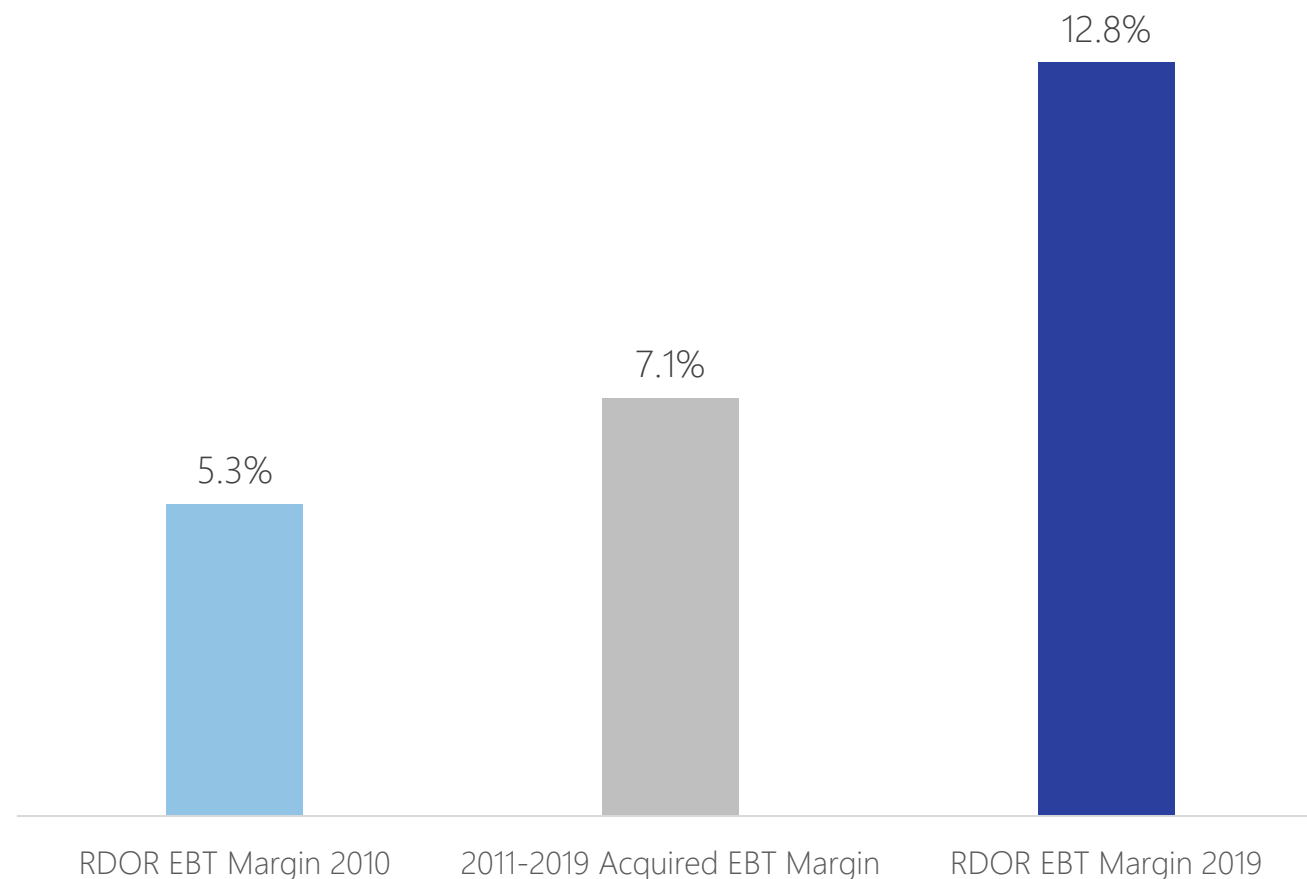
Date	Hospital	EV (mn)	# of beds	Revenues (mn)	EV/bed (R\$000)	EV/revenues	Revenue/bed (th)
Feb-20	Hospital Aliança	1,000	209	352	4,785	2.8x	1,686
Oct-20	Clínica São Lucas	56	58	55	965	1.0x	952
Nov-20	ClivaleMais (BA)	n.a.	n.a.	n.a.	n.a.	n.a.	n.a.
Nov-20	Hospital Balbino	91	141	81	646	1.1x	575
Nov-20	Hospital Central de Guaianases	104	153	n.a.	679	n.a.	n.a.
Nov-20	Cardio Pulmonar (remaining stake)	283	181	n.a.	1,562	n.a.	n.a.
Nov-20	Hospital América	468	112	145	4,179	3.2x	1,296
Dec-20	Hospital de Clínicas Antonio Afonso	26	60	n.a.	n.a.	n.a.	n.a.
Apr-21	Hospital Biocor	750	350	300	2,143	2.5x	857
Apr-21	Hospital Nossa Senhora das Neves (HNSN)	550	235	320	2,340	1.7x	1,362
Jun-21	Hospital Serra Mayor	130	102	38	1,275	3.4x	375
Jul-21	Proncor	290	136	57	2,132	5.1x	418
Jul-21	Hospital Santa Emília	201	144	36	1,397	5.6x	248
Aug-21	Hospital Aliança	1,150	209	700	5,502	1.6x	3,349
Sep-21	Hospital Novo Atibaia	296	166	283	1,785	1.0x	1,705
Oct-21	Hospital Aeroporto	230	85	n.a.	2,706	n.a.	n.a.
Oct-21	Hospital Santa Isabel	280	119	n.a.	2,353	n.a.	n.a.
Nov-21	Hospital Arthur Ramos	372	176	n.a.	2,113	n.a.	n.a.
Jan-22	Hospital Santa Marina	25	30	n.a.	833	n.a.	n.a.



M&A Operational synergies

Hospital	Year	Year of the acquisition			Value Paid	% Acquired	EV 100%
		Revenue (BRL mn)	EBT (BRL mn)	EBT Margin			
Hospital e Maternidade São Luiz	2010	723.1	-12.4	-1.7%	748.4	74.6%	1,003.5
Hospital Brasil	2010	262.6	19.6	7.5%	178.7	88.9%	201.0
Hospital Assunção	2010	79.4	-24.5	-30.9%	20.6	100.0%	20.6
Hospital Prontolinda	2010	50.2	7.9	15.7%	31.4	50.0%	62.8
Clínica Radiológica Menezes da Costa	2010	19.1	5.1	26.6%		100.0%	0.0
Centro Hospital São Marcos	2011	65.9	1.5	2.3%	11.0	100.0%	11.0
Vivalle	2011	54.9	5.9	10.7%	67.8	100.0%	67.8
Santa Luzia Participações	2012	296.2	14.6	4.9%	502.2	74.6%	673.3
Siniscal	2012	168.5	-153.5	-91.1%	22.0	100.0%	22.0
Hospital Esperança	2012	196.4	4.1	2.1%	73.4	50.0%	146.7
Hospital Norte D'or Cascadura	2013				7.5	70.0%	10.6
Hospital IFOR	2014	80.5	0.4	0.5%	84.7	100.0%	84.7
Hospital Fluminense	2015	73.4	4.5	6.1%	0.7	50.0%	1.4
Hospital Villa-Lobos	2015	162.1	35.5	21.9%	123.5	100.0%	123.5
Sino Brasileiro	2015	166.6	42.0	25.2%	345.9	100.0%	345.9
Cardial Serviços Médicos	2015	51.2	11.2	21.9%	16.9	23.5%	72.1
Hospital Santa Helena	2015	140.4	21.7	15.5%	331.2	95.6%	346.6
Hospital Bartira	2015	108.9	15.4	14.2%	118.4	100.0%	118.4
Oncoholding	2015	214.1	46.5	21.7%	64.5	37.5%	172.0
NEOH	2016	38.3	12.8	33.5%	35.5	100.0%	35.5
Sator	2017	153.4	3.0	2.0%	125.2	100.0%	125.2
Hospital Samer	2018	103.6	14.6	14.1%	78.3	100.0%	78.3
Hospital São Rafael	2018	262.7	-14.7	-5.6%	607.5	75.0%	810.0
Laboratório Richet	2018	100.1	12.8	12.8%	192.0	100.0%	192.0
Hospital UDI	2018	354.3	86.7	24.5%	441.5	100.0%	441.5
Hospital Avicenna	2019	93.4	17.6	18.9%	71.7	100.0%	71.7
Hospital São Lucas	2019	342.3	73.3	21.4%	235.5	75.0%	314.0
Hospital Clínicas Rio Mar	2019	64.4	-20.6	-32.0%	85.3	95.3%	89.5
Hospital Santa Cruz	2020	128.2	13.2	10.3%	361.1	100.0%	361.1
Hospital Aliança	2020	352.4	2.2	0.6%	800.0	80.0%	1,000.0
Hospital São Carlos	2020	108.7	13.1	12.1%	160.9	75.0%	214.6
Cardio Pulmonar	2020	-11.7	-11.7	100.0%	147.0	100.0%	147.0
Clinica São Lucas	2020	55.2	-0.3	-0.5%	56.0	100.0%	56.0
Balbino	2021	104.6	-14.3	-13.7%	88.6	100.0%	88.6
Serra Mayor	2021	59.4	-23.2	-39.1%	87.0	100.0%	87.0
América	2021	212.7	29.4	13.8%	468.9	100.0%	468.9
Biocor	2021	245.2	-3.8	-1.6%	391.7	51.0%	768.1
Proncor	2021	121.4	-23.6	-19.5%	113.5	51.0%	222.6
Santa Emília	2021	62.0	24.8	40.0%	168.1	100.0%	168.1
N.S das Neves	2021	261.8	-6.1	-2.3%	197.0	51.0%	386.3
Novo Atibaia	2021	243.2	38.0	15.6%	164.6	100.0%	164.6
Arthur Ramos	2022	331.1	75.9	22.9%	356.3	100.0%	356.3
Santa Isabel	2022	123.0	-7.8	-6.4%	280.0	100.0%	280.0
Aeroporto	2022	109.7	0.6	0.5%	218.1	100.0%	218.1
Average		161.2	7.8	4.9%	201.9	81.7%	241.6
Soma		6,932.7	337.4	4.9%	8,680.1	81.7%	10,628.9

Synergies



Appendix

Sources: Company's filings



Rede D'Or has expanded fiercely

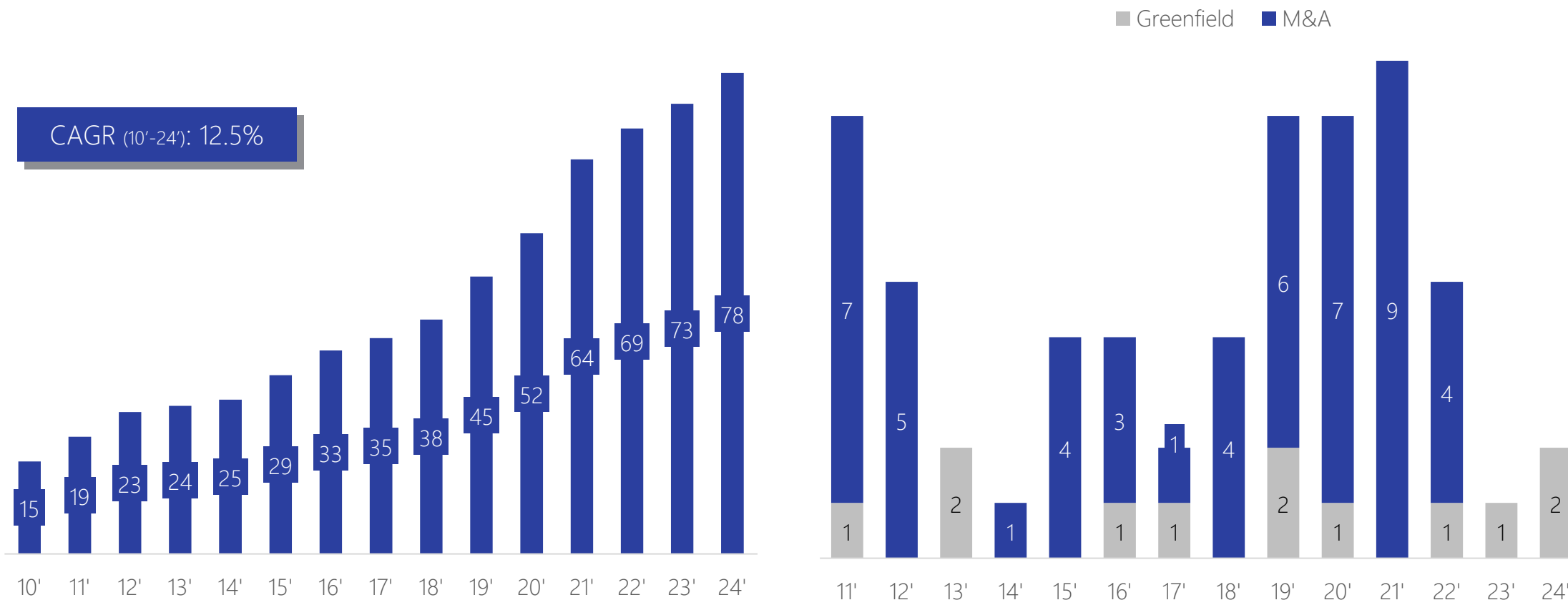
By combining organic and M&A opportunities, it became the top player

 The company kept a steady rhythm...

Number of hospitals [x]

 ...that will now be more focused on organic expansion

New hospitals per type





The Sul América acquisition analysis breakdown

A masterstroke of capital allocation

New RDOR3 Shares	A	308,304,834
RDOR3 at 23 Dec. 2022	B	BRL 28.83
SULA11 Mkt. Cap. B3	C	BRL 8,648 mn
Deal Implied Equity Value (P)	$D = A \times B$	BRL 8,888 mn
Revenue 23' (1 Year Fwd)	E	BRL 26,966 mn
Historical Net Margin	F	4.3%
Normalized Earning (E)	G	BRL 1,159 mn
Normalized P/E	D/G	7.67x
SULA11 10'-20' Avrg. P/E FWD	H	9.78x



Good Capital Allocation with the Sula deal

Even if the deal closed at the announcement day, it would have created a lot of value

It took a while for the deal to finally close...

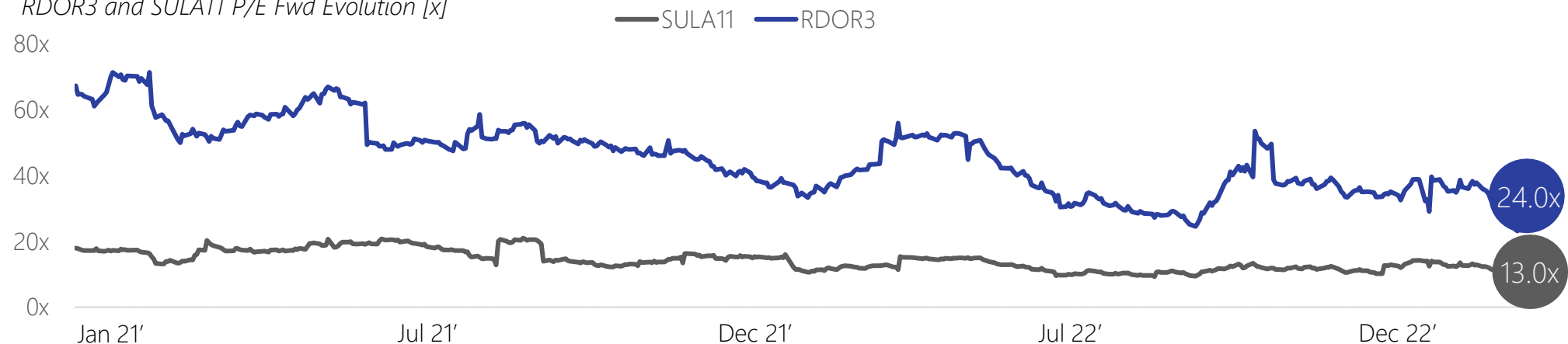


$$\begin{matrix} P \\ E \end{matrix} = \begin{matrix} \text{New Shares} \\ 2023 \text{ Rev.} \end{matrix} \times \begin{matrix} \text{Share Price Deal Announcement} \\ \text{His. Net Margin} \end{matrix}$$

P/E Fwd 23' = 12.5x

...and as Rede D'Or had a higher multiple, it could also create value

RDOR3 and SULA11 P/E Fwd Evolution [x]



REDE D'OR

SulAmérica

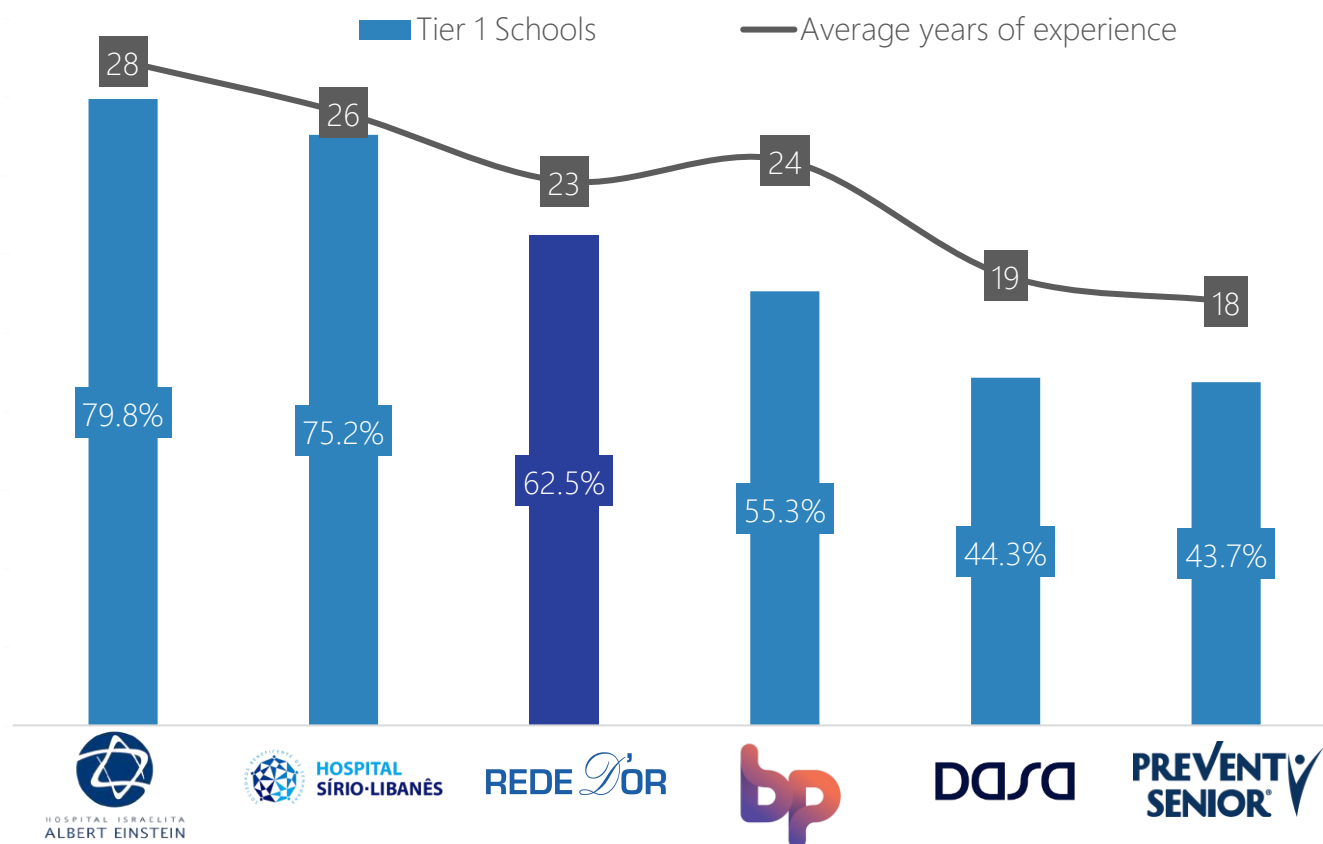


Doctors analysis breakdown

We sought to understand how well positioned is Rede D'Or with its doctors

- As doctors are the ones who bring clients to a hospital, we believe a good team is essential

Percentage of doctors from Tier 1 schools in SP and RJ hospitals, years of experience in the industry [%; #]



Rio de Janeiro



São Paulo



Tier 1 schools considered



HARVARD
MEDICAL SCHOOL

USP



HOSPITAL ISRAELITA
ALBERT EINSTEIN

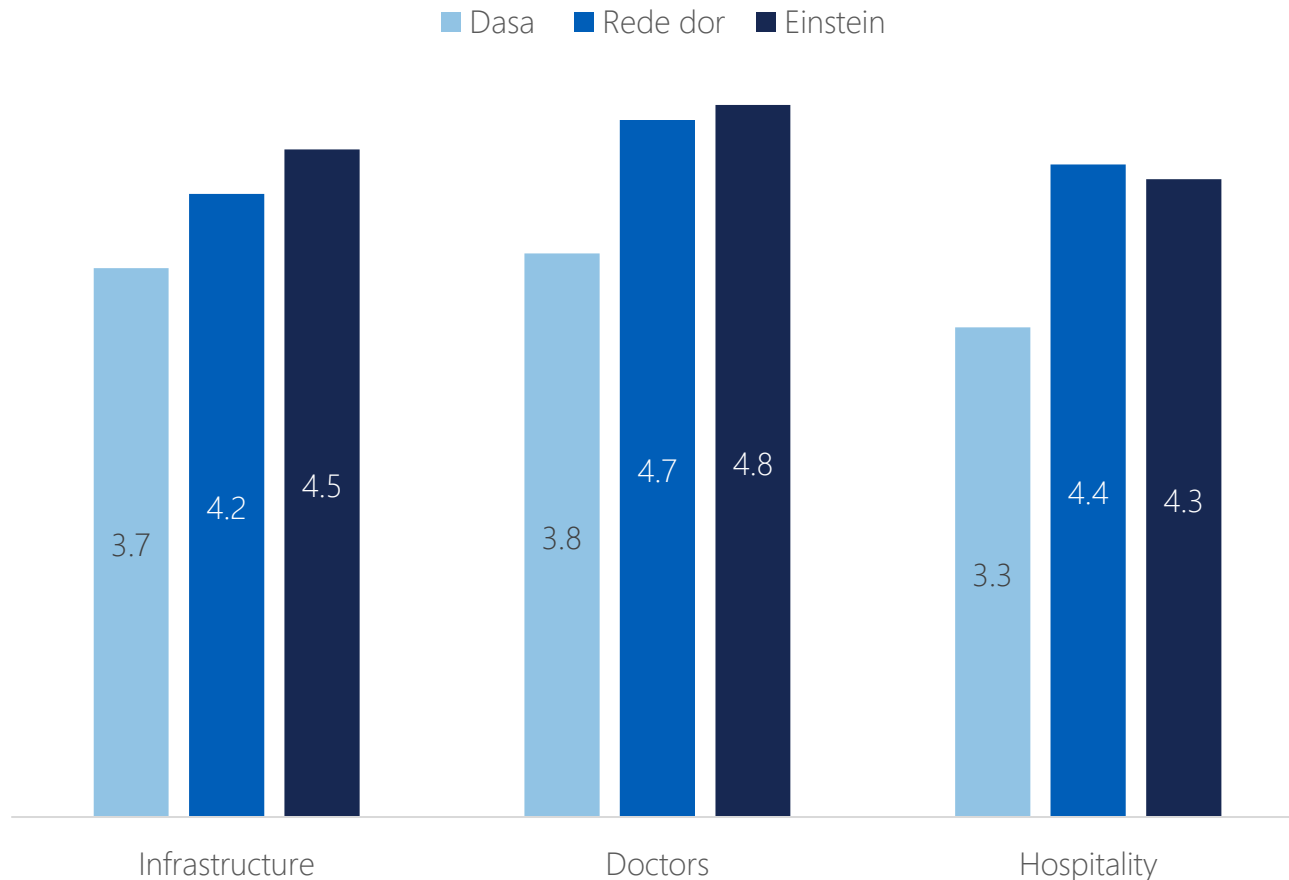




Google analysis breakdown

We sought to understand Rede D'Or's reviews

Average Google review of post with related words [# / 5]

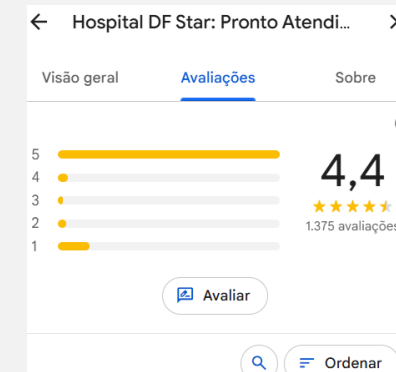
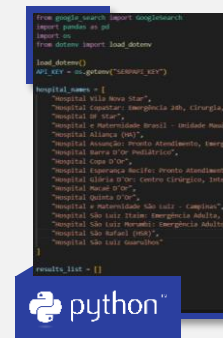


By using a scraping code, we were able to see how reviews from google maps with specific words were rated. We used the following key-words:

Infrastructure: infrastructure, building, technology

Doctors: Doctors, team, medication, surgeon

Hospitality: hospitality, hotel, service, rooms

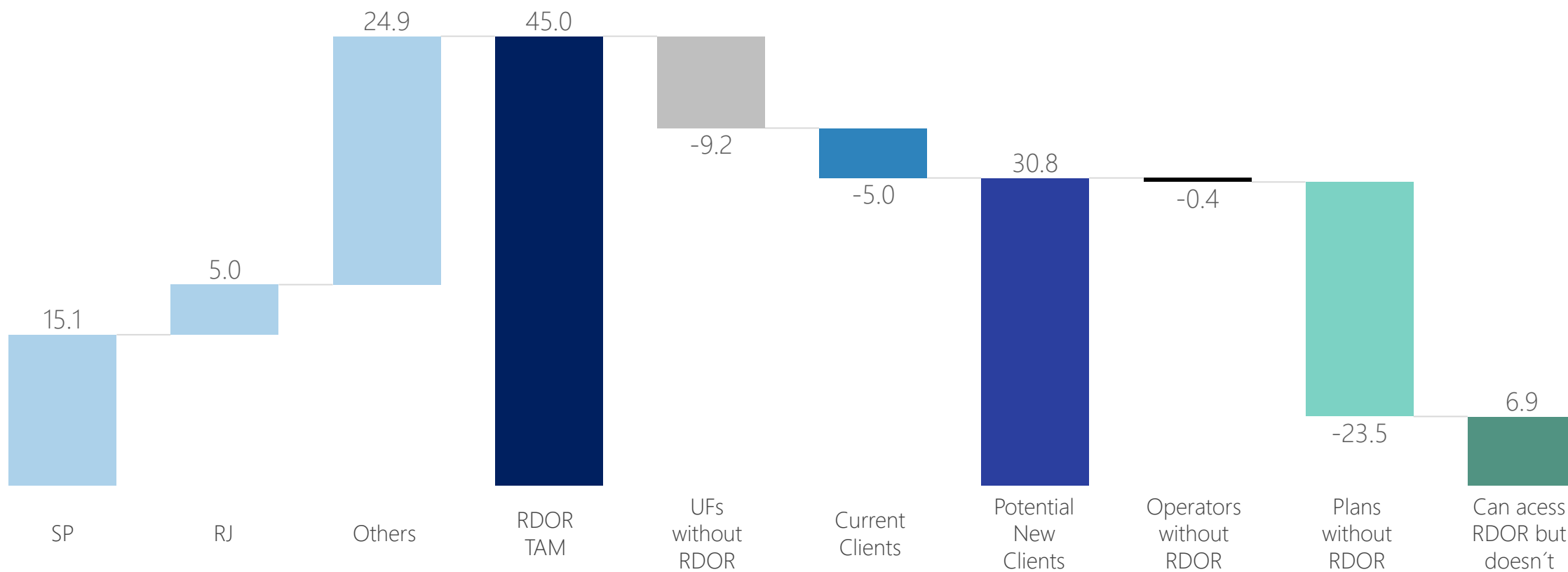




Calculating Rede D'Or's TAM

We sought to understand how many beneficiaries can attend Rede D'Or facilities

Company's TAM and potential new clients [mn]

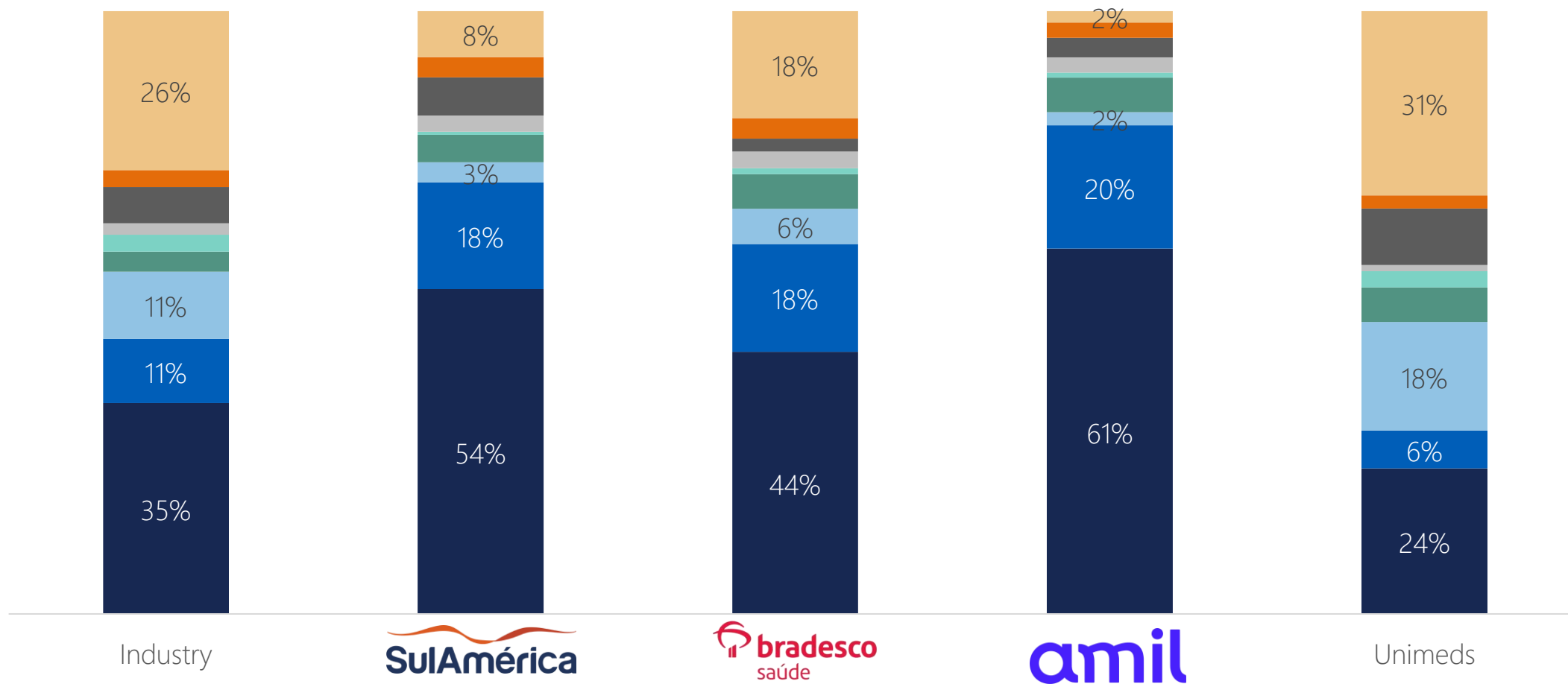




Beneficiary Distribution

Share of beneficiary base per state [%]

■ São Paulo ■ Rio de Janeiro ■ Minas Gerais ■ Bahia ■ Ceará ■ Distrito Federal ■ Paraná ■ Pernambuco ■ Others





Unit Economics breakdown

	Y0	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	Y9	Y10
	31/12/2024	31/12/2025	31/12/2026	31/12/2027	31/12/2028	31/12/2029	31/12/2030	31/12/2031	31/12/2032	31/12/2033	31/12/2034
Beds Capacity		180	180	180	180	180	180	180	180	180	180
Operating Beds		90	140	180	180	180	180	180	180	180	180
Occupancy Rate		60%	75%	85%	85%	85%	85%	85%	85%	85%	85%
Avg ticket (day)		10,800	11,340	11,907	12,502	13,127	13,784	14,473	15,197	15,957	16,754
Net Revenue (mn)		189	386	590	620	651	683	717	753	791	831
Expansion Capex	-360	0	0	0	0	0	0	0	0	0	0
Maintenance Capex		-9	-10	-15	-15	-16	-17	-18	-19	-20	-21
D&A		-34.7	-35.5	-35.6	-36.1	-36.1	-36.2	-36.3	-36.4	-36.5	-36.6
WK		47.2	96.4	147.6	154.9	162.7	170.8	179.4	188.3	197.7	207.6
Delta WK		-47.2	-49.2	-51.1	-7.4	-7.7	-8.1	-8.5	-9.0	-9.4	-9.9
WK (% of net revenue)		25%	25%	25%	25%	25%	25%	25%	25%	25%	25%
Financial Result		-38	-38	-38	-38	-38	-38	-38	-38	-38	-38
EBT		-82	-35	74	81	89	97	105	114	123	133
Tax Rate		30%	30%	30%	30%	30%	30%	30%	30%	30%	30%
Net Income		-82	-35	74	69	62	68	74	80	86	93
FCFE	-90	-103	-58	44	83	74	79	84	88	94	99
Income Reserves		-25	-35	-13	12	38	67	99	133	170	210
Debt	270	270	270	270	270	270	270	270	270	270	270
Kd	14%	14%	14%	14%	14%	14%	14%	14%	14%	14%	14%
PP&E	360.0	333.9	308.0	287.2	266.6	246.8	227.6	209.3	191.7	175.0	159.2
EBITDA Margin		-5%	10%	25%	25%	25%	25%	25%	25%	25%	25%

Net revenue / Gross revenue	90%
Beds	180
Capex / Bed	2.0
Initial avg ticket	10,800
Ticket Growth	5%
Taxes (%EBT)	30%
Mature maintenance Capex g perpetuity	2.5%
Initial Debt / Capex	5.0%
Depreciation Rate	75%
	10%



Expansion analysis breakdown - A



1st Step: Understand the profile of the cities in which the company owns hospitals

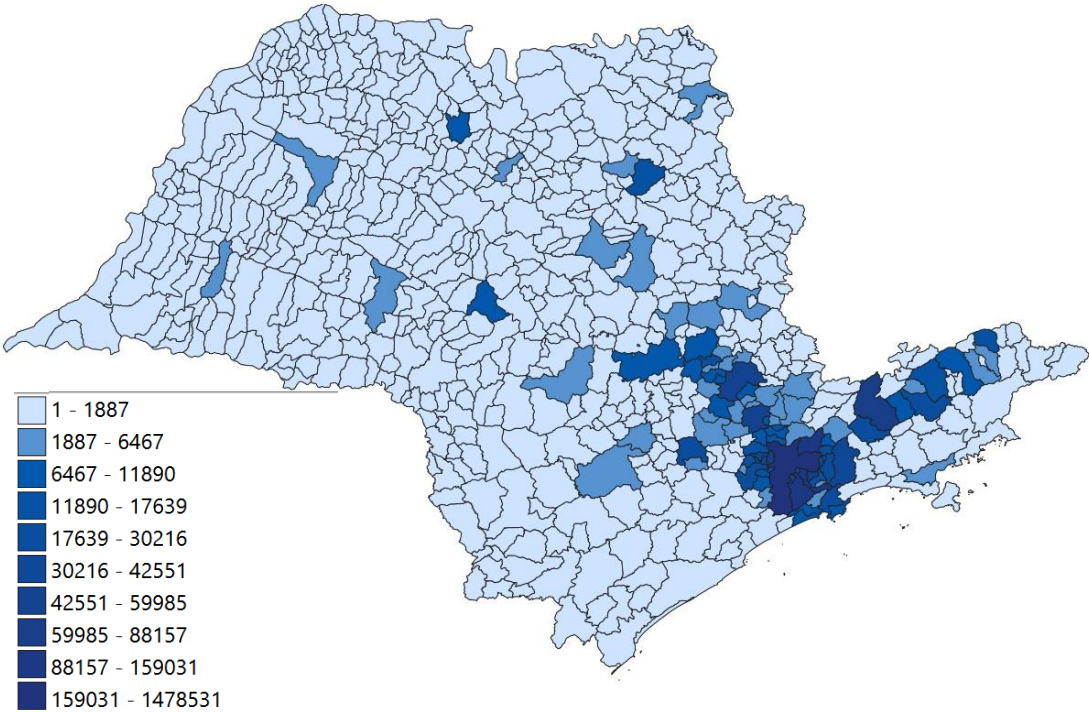
City	RDOR operating beds	Beneficiaries	PIB per capita (BRL th)	Non-public Beds per 1000 Beneficiaries
São Paulo	1,764	5,817,750	67	4.3
Carapicuíba	41	142,140	17	0.3
São Bernardo do Campo	189	457,663	69	2.8
Santo André	454	423,055	45	2.3
Mauá	115	203,000	43	1.4
Jacareí	44	104,000	68	4.2
Ribeirão pires	125	52,600	31	2.4
Campinas	174	600,354	59	4.0
Osasco	175	347,000	123	1.1
São caetano	267	105,000	96	7.5
Atibaia	87	75,350	58	4.5
São José dos Campos	224	318,379	61	3.7
Guarulhos	55	625,740	55	2.4
Barueri	58	157,283	68	1.0



Expansion analysis breakdown - B

2nd Step: Understand the presence of Sul América and Bradesco Saúde by region

Bradesco's and Sul América's medical beneficiaries distribution by city [mn]



Metropolitan Region	São Paulo	Santos	Campinas	Vale do Paraíba / North Coast	Jundiaí	Piracicaba	Sorocaba
SulA beneficiaries (th)	1,209	30	64	124	38	7	23
Bradesco Beneficiaries (th)	1,244	38	84	87	39	26	27
Region's beneficiaries (th)	9,725	740	1,485	793	444	680	756
SulA + Bradesco Share	25.2%	9.1%	9.9%	26.6%	17.3%	4.8%	6.7%



Expansion analysis breakdown - C

3rd Step: Filtering the cities

Filter	Rule
Beneficiaries in the city	>52,000
City's BRL GDP per capita	>32,000
Non-public beds per 1000 beneficiaries	<7.5
SulA+ Bradesco Share	>5%
Reference Hospitals in the city	0

Attractive Cities
Americana
Araçatuba
Araraquara
Bauru
Cotia
Guarujá
Indaiatuba
Mogi das Cruzes
Mogi Guaçu
Ribeirão Preto
Santos
São Carlos
Sorocaba
Suzano
Taubaté

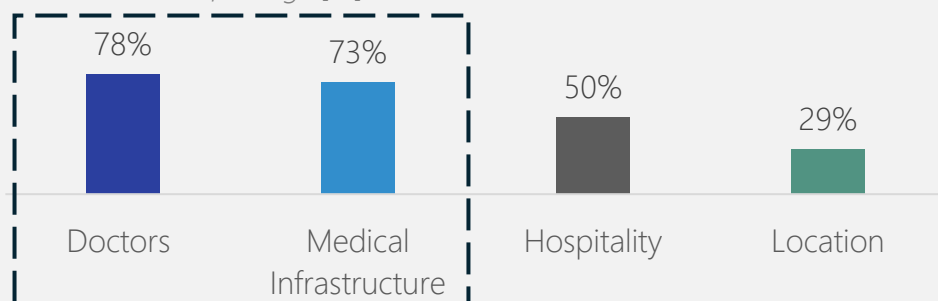


Field research I

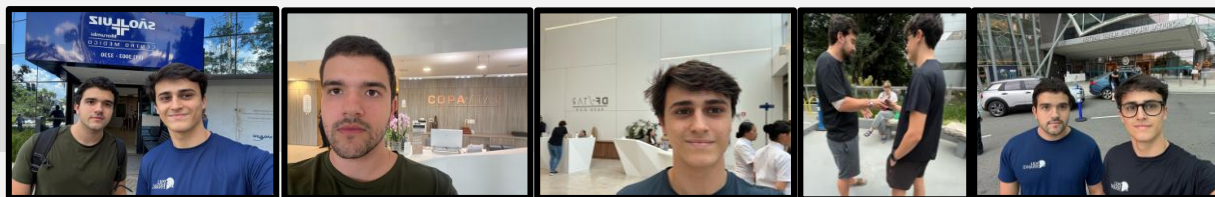
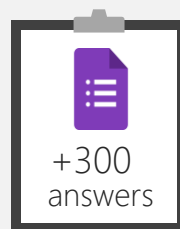
We sought to understand the reasons someone chooses a hospital

"What are the main reasons that you go to a hospital?"

Field research findings [%]

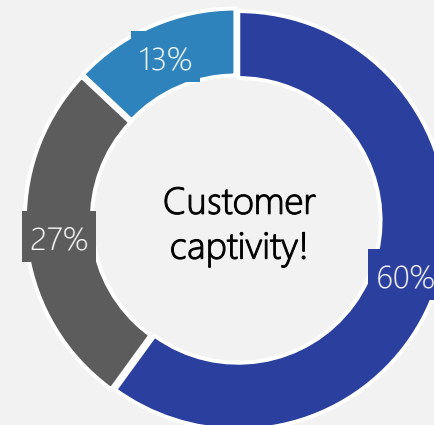


Hospital Albert Einstein
Hospital São Luiz Morumbi
Hospital São Luiz Itaim
Hospital Sírio Libanês
Hospital HCor
Hospital Vila Nova Star
Hospital Copa D'Or
Hospital Copa Star
Hospital DF Star



"What keeps you going to the same hospital?"

- Doctors Indication
- Habit
- HMO Coverage



- **The client is the doctor!** As the doctors are the one able to bring beneficiaries, they are the hospitals true clientes;
- In **medium to high tickets**, hospital's **brand** becomes **more important**.



Field research II

We visited Vila Nova Star to understand more about doctors' relation with D'Or



REDE D'OR

1999

2024

- **Doctors** have a very important role to **bring clients** to their hospitals, **especially for Rede D'Or**, since it does not have an extremely estabilished brand such as Einstein and Sírio;
- The company has made serious effort to bring top doctors, with **90%** of its team in **Vila Nova Star** coming from **Hospital das Clínicas** (USP);
- For **Vila Nova Star**, Rede D'Or brought some **top technology** to offer to premium clients and **compete with Einstein and Sírio** in the long run.



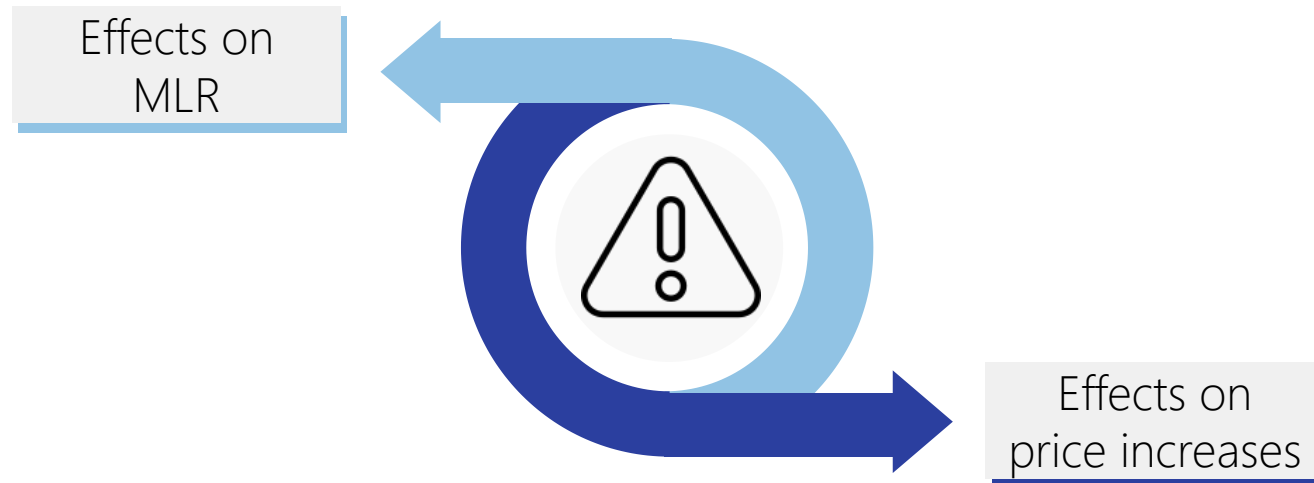
Main regulatory concerns

Regulation such as CP-145 may have two effects on the thesis



So we tried to understand the effects it may actually have on our model

Effects on IRR



Individual plans max.. price increase formula

$$\text{Max. Price Increase} = (0.8 \times \text{IVDA}) + (0.2 \times \text{IPCA exc. Health plans})$$

Delta Health Ticket Growth	Delta MLR					
	23.4%	(-) 8 p.p	(-) 4 p.p	0	(+) 4 p.p	(+) 8 p.p
	(+) 6 p.p	38%	36%	33%	31%	28%
	(+) 3 p.p	33%	31%	28%	26%	23%
	0	28%	26%	23%	21%	17%
	(-) 3 p.p	23%	21%	18%	15%	12%
	(-) 6 p.p	18%	15%	12%	9%	6%

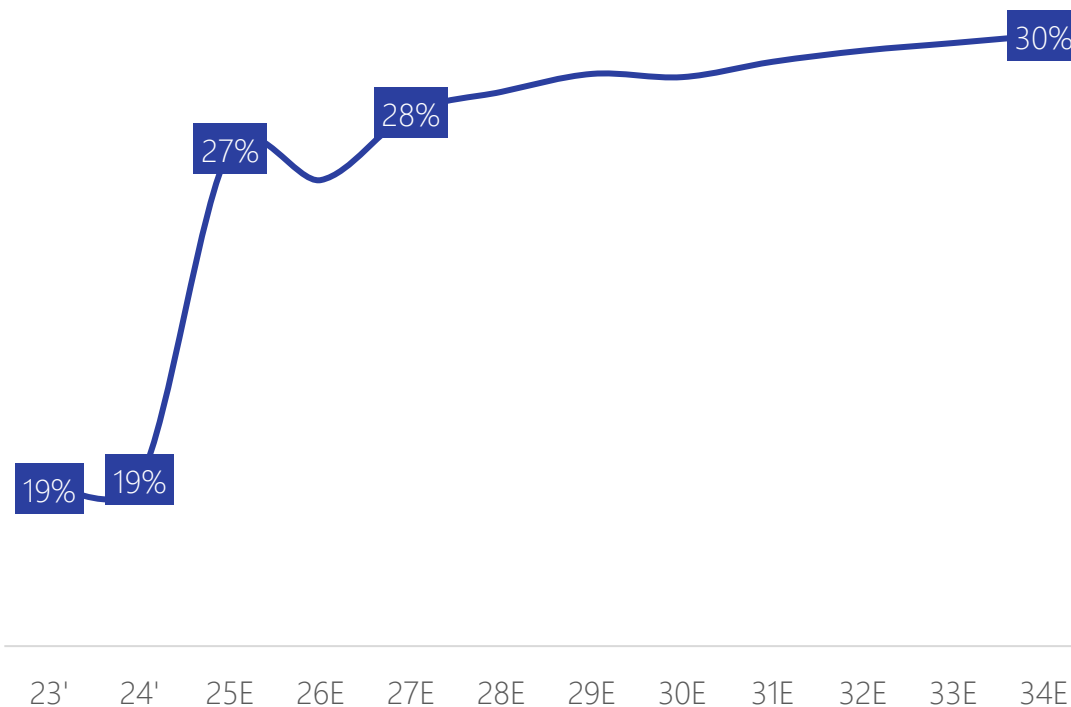


The end of JCP: a regulatory risk

The end of the benefit should impact earnings

So we stressed our model to see the impacts it may have if in fact the benefit ends

Tax rate (% EBT)



Effects on IRR

5y Exit P/E fwd 1y

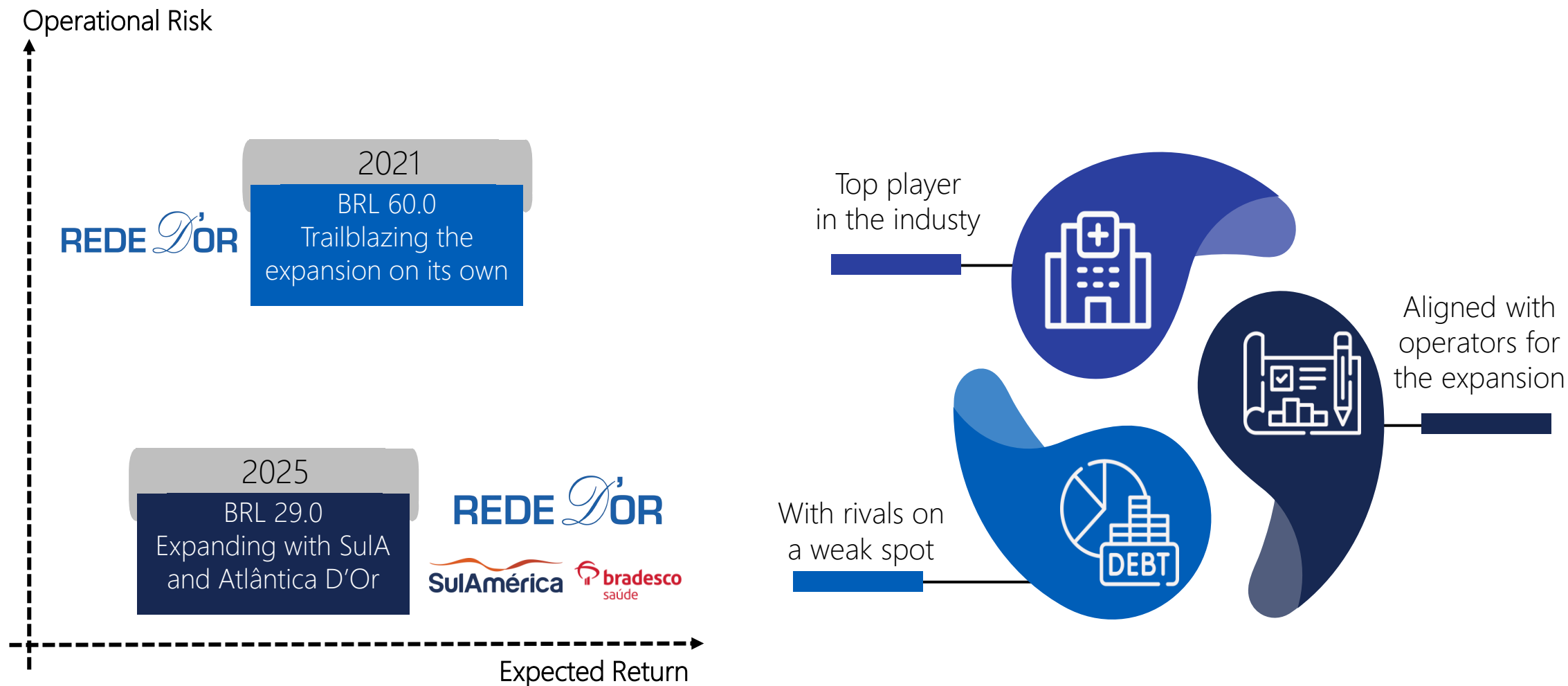
IRR without JCP: 21.6%	9.5x	11.0x	12.5x	14.0x	15.5x	17.0x	18.5x
34%	13%	16%	19%	22%	24%	26%	28%
32%	14%	17%	20%	22%	25%	27%	29%
30%	15%	18%	21%	23%	25%	27%	29%
28%	16%	19%	21%	24%	26%	28%	30%
26%	16%	19%	22%	25%	27%	29%	31%
24%	17%	20%	23%	25%	28%	30%	32%
22%	18%	21%	23%	26%	28%	30%	32%

IRR – Ke > 4% in 73% of the stressed cases!



It is not only what we see, it is also **why** we see it now

Above our upside, we see a derisking operation as the main reason for our buy





NPV São Luiz Campinas – Atlântica D'Or

Year	2024	2025	2026	2027	2028	2029	2030	2031	2032	2033
Capex paid back	246.5	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
Bed Capacity (São Luiz Campinas)	325	325	325	325	325	325	325	325	325	325
% Operating Beds	54%	69%	89%	90%	90%	90%	90%	90%	90%	90%
Operating Beds	174	224	289	294	294	294	294	294	294	294
Average Ticket	8,030	9,827	11,141	11,698	12,283	12,898	13,542	14,220	14,930	15,677
Occupancy Rate	59.0%	73.0%	81.4%	82.5%	83.8%	85.3%	86.5%	87.2%	87.4%	87.4%
EBITDA Margin	-10%	-3%	10%	22%	29%	29%	29%	29%	29%	29%
Atlantica Share	50%	50%	50%	50%	50%	50%	50%	50%	50%	50%
Tax Rate	34%	34%	34%	34%	34%	34%	34%	34%	34%	34%
Ke	15.7%	15.7%	15.7%	15.7%	15.7%	15.7%	15.7%	15.7%	15.7%	15.7%
Cash Flows (BRL mn)	248.9	3.3	-30.0	-72.7	-104.3	-111.4	-118.6	-125.6	-132.1	-138.8

24' NPV = -428 BRL mn



Exit P/E

Fair P/E Formula Used:

$$\frac{P_0}{EPS_0} = \left(\frac{\left(1 - \frac{g}{ROE_{hg}}\right) * (1 + g) * \left(1 - \frac{(1 + g)^n}{(1 + k_{e,hg})^n}\right)}{k_{e,hg} - g} + \frac{\left(1 - \frac{g_n}{ROE_{st}}\right) * (1 + g)^n * (1 + g_n)}{(k_{e,st} - g_n) * (1 + k_{e,st})^n} \right)$$

Fair P/E Fwd 29E	12.3x
ROE 25E-29E	22.1%
ROE 30E	28.3%
g (25E-29E)	24.8%
Perpetuity g	5.0%
Ke	15.7%
n (years of high growth phase)	10

--> Avg ROE
--> ROE
--> Avg earnings growth
--> Premise
--> Premise
--> Premise

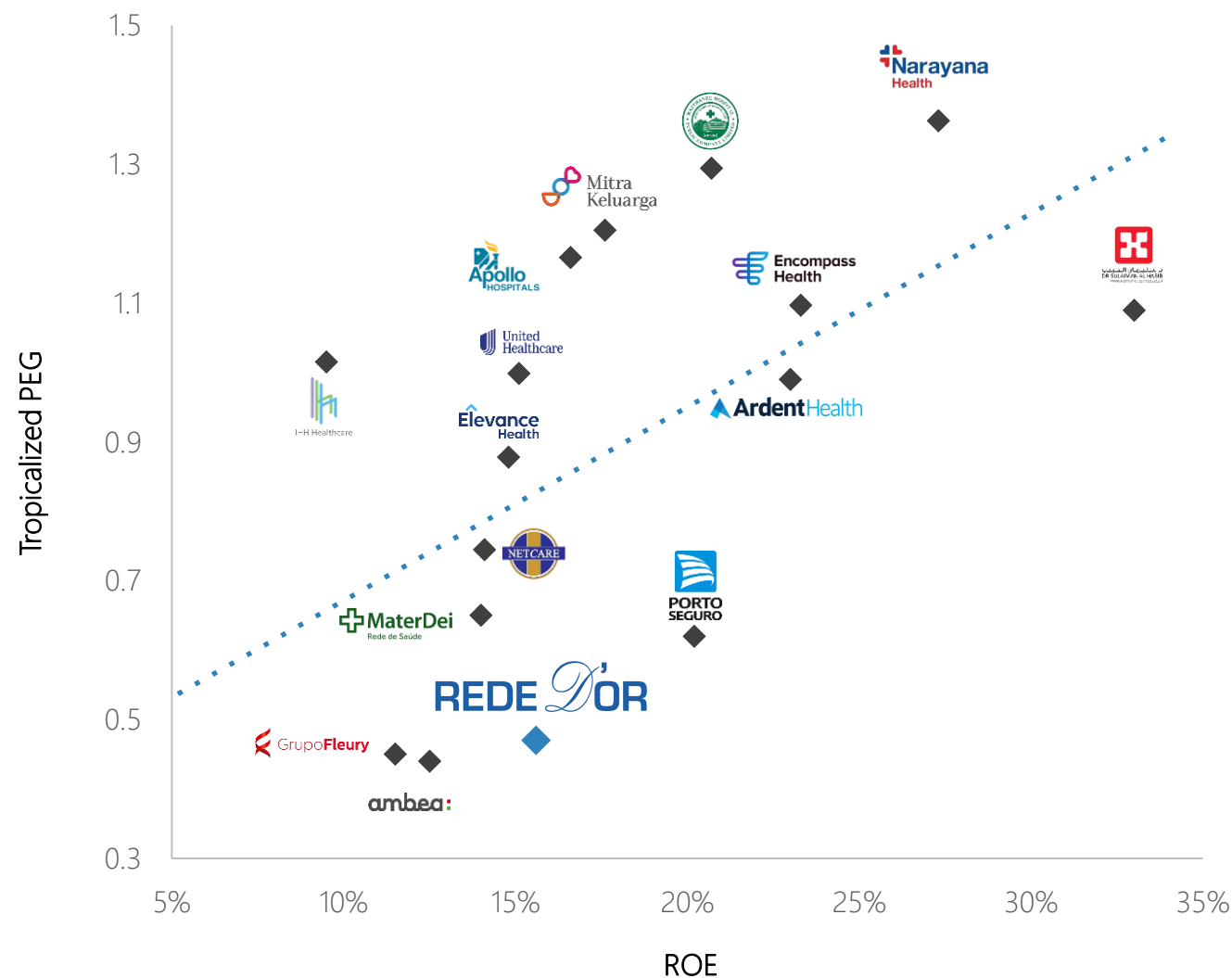
Exit P/E Calculation:

Exit Multiple 29E	Weight	14.0x
Fair P/E	60%	12.3x
Hist. 1y Fwd Average	40%	16.6x



Multiples – International Peers

Tropicalized PEG Ratio 1Y Fwd [x] vs ROE 1y Fwd [%]



Multiples Comp – International Peers

Stock	Region	(A) CRP	(B) Brazil CRP - CRP	(C) P/E Fwd NTM	(D) P/E ⁻¹	(E) B + D	(F) 1/E
IHH	Singapore	0.0%	3.3%	15.15	0.07	9.9%	10.10
NTC	South Africa	4.0%	-0.7%	11.01	0.09	8.4%	11.96
BDMS	Thailand	2.1%	1.2%	16.19	0.06	7.4%	13.56
APOLLOHOSP	India	2.9%	0.4%	40.41	0.02	2.9%	34.79
FORTIS	India	2.9%	0.4%	50.06	0.02	2.4%	41.71
1515	China	0.9%	2.4%	6.64	0.15	17.5%	5.73
NH	India	2.9%	0.4%	34.41	0.03	3.3%	30.25
KPJ	Malaysia	1.6%	1.7%	19.75	0.05	6.8%	14.79
ELV	US	0.0%	3.3%	8.85	0.11	14.6%	6.85
UNH	US	0.0%	3.3%	11.03	0.09	12.4%	8.09
LHC	South Africa, UK, Europe, US	2.5%	0.8%	10.36	0.10	10.4%	9.57
RJH	Thailand	2.1%	1.2%	8.45	0.12	13.0%	7.67
543220	India	2.9%	0.4%	48.78	0.02	2.5%	40.81
300015	China, Europe, US	1.0%	2.3%	18.66	0.05	7.7%	13.06
4013	Saudi Arabia	0.8%	2.5%	19.54	0.05	7.6%	13.13
MIKA	Indonesia	2.5%	0.8%	20.70	0.05	5.6%	17.89
RAINBOW	India	2.9%	0.4%	39.46	0.03	2.9%	34.08
EHC	US, Puerto Rico	0.5%	2.8%	13.20	0.08	10.4%	9.64

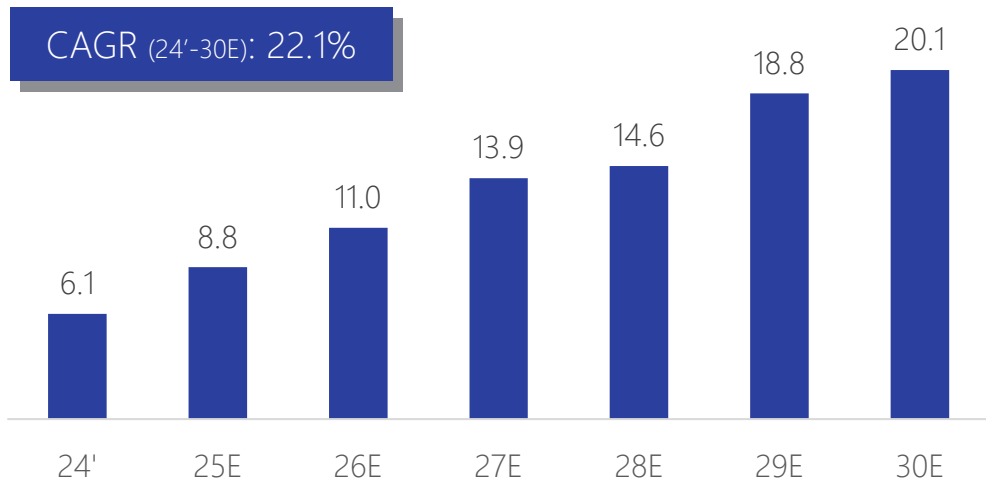


Exit: EV/EBITDA

We also analyzed and sensitized our IRR analysis with an exit multiple in 2030

5y IRR indicates a good investment...

Adj. EBITDA [BRL bn]



And our sensitivity analysis reinforces our thesis...

IRR Sensitivity Analysis [% ; x]

5y Exit EV/EBITDA fwd 1y							
22.6%	5.0	6.0	7.0	8.0	9.0	10.0	11.0
13.1%	5%	8%	11%	13%	15%	18%	20%
16.1%	7%	11%	14%	16%	19%	21%	23%
19.1%	10%	14%	17%	19%	22%	24%	26%
22.1%	13%	16%	20%	23%	25%	28%	30%
25.1%	16%	20%	23%	26%	29%	31%	33%
28.1%	19%	23%	26%	29%	32%	35%	37%
31.1%	22%	26%	29%	33%	35%	38%	41%

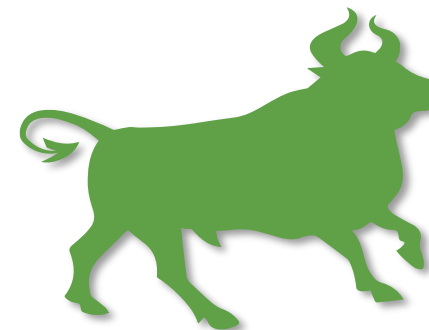
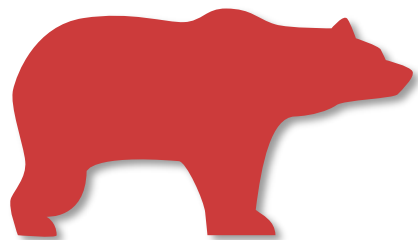


IRR – Ke > 4% in 55% of the stressed cases!



Bull, Base and Bear

We stressed scenarios Rede D'Or can face in our model



Bear Case

Delta hospitals and insurance ticket (-2p.p)

No G&A Dillutions

MLR flat from 2024

Delta occupancy rate (-5 p.p)

Perpetuity g: 2%



-16.0%
Downside

Base Case

Hospitals ticket growth: IPCA + 1%

Insurance health ticket growth: IPCA + 4%

G&A/Revenue 2034: 81%

MLR 2034: 80%

Occupancy rate 2034: 87%



37.4%
Upside

Bull Case

Delta ticket hospitals (+1 p.p)

Delta insurance ticket (+1p.p)

MLR delta (-1.5 p.p)

Delta personnel/revenue (-1p.p)

Delta matmed/revenue (-1p.p)



68.6%
Upside

By analyzing different scenarios, we still believe in our **buy thesis**...



Consolidated Income Statement - A

	[Unit]	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Gross Revenue	[BRL mn]	54,779	61,860	73,822	84,183	94,870	105,378	114,005	124,899	136,636	149,197	162,803
Growth YoY	[%]	10%	13%	19%	14%	13%	11%	8%	10%	9%	9%	9%
Taxes & Others	[BRL mn]	(3,453)	(3,735)	(4,560)	(5,204)	(5,962)	(6,660)	(7,223)	(7,930)	(8,681)	(9,467)	(10,312)
as % Gross Revenue	[%]	6%	6%	6%	6%	6%	6%	6%	6%	6%	6%	6%
Net Revenue	[BRL mn]	51,325	58,125	69,262	78,979	88,908	98,719	106,782	116,969	127,955	139,729	152,491
Growth YoY	[%]	9%	13%	19%	14%	13%	11%	8%	10%	9%	9%	9%
Changes in technical reserves	[BRL mn]	(774)	(992)	(1,168)	(1,235)	(1,380)	(1,525)	(1,668)	(1,824)	(1,994)	(2,180)	(2,383)
COGS	[BRL mn]	(40,965)	(46,117)	(54,334)	(60,783)	(67,954)	(74,711)	(81,015)	(87,943)	(95,534)	(104,189)	(113,515)
Cost with hospitals services	[BRL mn]	(21,271)	(23,224)	(28,167)	(31,830)	(36,349)	(40,222)	(43,773)	(47,745)	(52,158)	(56,773)	(61,690)
Operating costs	[BRL mn]	(19,694)	(22,893)	(26,167)	(28,953)	(31,605)	(34,489)	(37,242)	(40,198)	(43,376)	(47,416)	(51,826)
Growth YoY	[%]	6%	13%	18%	12%	12%	10%	8%	9%	9%	9%	9%
Gross Profit	[BRL mn]	9,587	11,015	13,761	16,960	19,574	22,482	24,099	27,201	30,427	33,360	36,593
Margin	[%]	19%	19%	20%	21%	22%	23%	23%	23%	24%	24%	24%
Growth YoY	[%]	28%	15%	25%	23%	15%	15%	7%	13%	12%	10%	10%
SG&A	[BRL mn]	(3,021)	(3,204)	(3,740)	(4,159)	(4,545)	(4,920)	(5,248)	(5,666)	(6,110)	(6,660)	(7,255)
G&A	[BRL mn]	(2,943)	(3,090)	(3,603)	(4,002)	(4,416)	(4,776)	(5,091)	(5,494)	(5,922)	(6,455)	(7,031)
Sales	[BRL mn]	(78)	(114)	(138)	(157)	(129)	(143)	(157)	(172)	(188)	(205)	(224)
Growth YoY	[%]	22%	6%	17%	11%	9%	8%	7%	8%	8%	9%	9%
Other operating income (expenses) & equity pickup	[BRL mn]	(55)	(411)	(500)	(571)	(654)	(729)	(799)	(877)	(960)	(1,047)	(1,141)



Consolidated Income Statement - B

	[Unit]	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
EBIT	[BRL mn]	6,510	7,400	9,520	12,230	14,374	16,833	18,052	20,658	23,357	25,653	28,197
Margin	[%]	13%	13%	14%	15%	16%	17%	17%	18%	18%	18%	18%
Growth YoY	[%]	45%	14%	29%	28%	18%	17%	7%	14%	13%	10%	10%
Financial Results	[BRL mn]	(1,641)	(2,320)	(2,269)	(1,881)	(1,790)	(1,436)	(1,257)	(877)	(772)	(561)	(524)
Financial Revenues	[BRL mn]	8,437	4,359	4,506	4,288	4,671	5,135	5,414	5,847	6,065	6,336	6,437
Financial Expenses	[BRL mn]	(10,078)	(6,679)	(6,775)	(6,169)	(6,461)	(6,570)	(6,671)	(6,724)	(6,837)	(6,897)	(6,961)
Growth YoY	[%]	-10%	41%	-2%	-17%	-5%	-20%	-12%	-30%	-12%	-27%	-7%
EBT	[BRL mn]	4,869	5,080	7,251	10,350	12,585	15,397	16,795	19,781	22,585	25,092	27,673
Margin	[%]	9%	9%	10%	13%	14%	16%	16%	17%	18%	18%	18%
Growth YoY	[%]	82%	4%	43%	43%	22%	22%	9%	18%	14%	11%	10%
Taxes	[BRL mn]	(933)	(1,393)	(1,932)	(2,932)	(3,630)	(4,510)	(4,906)	(5,855)	(6,747)	(7,544)	(8,375)
Tax Rate	[%]	19%	27%	27%	28%	29%	29%	29%	30%	30%	30%	30%
Net Income	[BRL mn]	3,936	3,687	5,319	7,418	8,954	10,887	11,889	13,927	15,837	17,548	19,299
Margin	[%]	8%	6%	8%	9%	10%	11%	11%	12%	12%	13%	13%
Growth YoY	[%]	82%	-6%	44%	39%	21%	22%	9%	17%	14%	11%	10%



Balance Sheet - Assets

	[Unit]	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Assets	[BRL mn]	106,974	114,962	123,435	131,908	140,054	146,705	153,090	159,635	165,271	170,603	175,458
Current Assets	[BRL mn]	53,075	58,929	65,309	71,698	77,897	83,240	88,152	92,961	96,591	99,880	103,111
Cash and cash equivalents	[BRL mn]	6,571	10,715	14,246	18,440	21,989	24,980	27,696	30,035	31,034	31,550	31,793
Marketable securities	[BRL mn]	32,067	32,067	32,067	32,067	32,067	32,067	32,067	32,067	32,067	32,067	32,067
Accounts receivable of hospital services	[BRL mn]	8,193	9,476	11,678	13,332	15,398	17,219	18,876	20,732	22,699	24,749	26,948
Accounts receivable of insurance and ASO	[BRL mn]	2,327	2,710	3,184	3,578	3,998	4,386	4,797	5,247	5,737	6,272	6,856
Inventories	[BRL mn]	913	958	1,130	1,277	1,441	1,584	1,710	1,876	2,050	2,238	2,442
Taxes recoverable	[BRL mn]	1,225	1,225	1,225	1,225	1,225	1,225	1,225	1,225	1,225	1,225	1,225
Reinsurance assets	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Derivative financial instruments	[BRL mn]	174	174	174	174	174	174	174	174	174	174	174
Related parties	[BRL mn]	192	192	192	192	192	192	192	192	192	192	192
Dividends receivable	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Receivables from disposal of properties	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Deferred acquisitions costs	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Assets held for sale	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Other	[BRL mn]	1,413	1,413	1,413	1,413	1,413	1,413	1,413	1,413	1,413	1,413	1,413
Non-current Assets	[BRL mn]	53,899	56,033	58,126	60,210	62,157	63,465	64,939	66,674	68,680	70,723	72,347
Related parties	[BRL mn]	62	62	62	62	62	62	62	62	62	62	62
Marketable securities	[BRL mn]	1,852	1,852	1,852	1,852	1,852	1,852	1,852	1,852	1,852	1,852	1,852
Accounts receivable of insurance and ASO	[BRL mn]	1,793	2,188	2,572	2,890	3,229	3,543	3,875	4,238	4,634	5,066	5,538
Taxes recoverable	[BRL mn]	479	479	479	479	479	479	479	479	479	479	479
Deposit for acquisition of property	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Judicial deposits	[BRL mn]	2,770	2,770	2,770	2,770	2,770	2,770	2,770	2,770	2,770	2,770	2,770
Reinsurance assets	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Deferred income tax and social contribution	[BRL mn]	3,657	3,657	3,657	3,657	3,657	3,657	3,657	3,657	3,657	3,657	3,657
Derivative financial instruments	[BRL mn]	3,551	3,551	3,551	3,551	3,551	3,551	3,551	3,551	3,551	3,551	3,551
Investments in subsidiaries, affiliates and jointly controlled subsidiaries	[BRL mn]	2,484	2,484	2,484	2,484	2,484	2,484	2,484	2,484	2,484	2,484	2,484
Property and equipments	[BRL mn]	14,978	16,431	17,912	19,512	20,877	21,765	22,820	24,106	25,631	27,155	28,221
Intangible assets	[BRL mn]	17,610	17,610	17,610	17,610	17,610	17,610	17,610	17,610	17,610	17,610	17,610
Right of use - leases	[BRL mn]	3,053	3,339	3,568	3,733	3,977	4,083	4,169	4,255	4,341	4,427	4,513
Deferred acquisitions costs	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Others	[BRL mn]	1,610	1,610	1,610	1,610	1,610	1,610	1,610	1,610	1,610	1,610	1,610



Balance Sheet – Liabilities

	[Unit]	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Liabilities	[BRL mn]	80,674	86,451	91,998	96,762	100,879	104,263	107,677	111,436	114,697	118,274	122,163
Current Liabilities	[BRL mn]	18,732	20,189	21,557	22,806	24,129	25,437	26,774	28,267	29,621	31,123	32,754
Trade account payable	[BRL mn]	1,535	1,800	2,259	2,688	3,217	3,719	4,264	4,896	5,311	5,791	6,307
Derivative financial instruments	[BRL mn]	661	661	661	661	661	661	661	661	661	661	661
Loans, financing and debentures	[BRL mn]	3,915	3,915	3,915	3,915	3,915	3,915	3,915	3,915	3,915	3,915	3,915
Related parties	[BRL mn]	12	12	12	12	12	12	12	12	12	12	12
Salaries, provisions and social charges	[BRL mn]	1,109	1,109	1,109	1,109	1,109	1,109	1,109	1,109	1,109	1,109	1,109
Tax liabilities	[BRL mn]	883	883	883	883	883	883	883	883	883	883	883
Accounts payable for acquisitions	[BRL mn]	465	465	465	465	465	465	465	465	465	465	465
Dividends payable	[BRL mn]	69	69	69	69	69	69	69	69	69	69	69
Insurance liabilities	[BRL mn]	8,461	9,590	10,449	11,232	11,972	12,756	13,528	14,371	15,290	16,294	17,390
Managed health	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Deferred gain on the disposal of real estate	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Leases	[BRL mn]	776	839	890	926	980	1,003	1,022	1,041	1,060	1,079	1,098
Other	[BRL mn]	845	845	845	845	845	845	845	845	845	845	845
Non-current Liabilities	[BRL mn]	61,943	66,262	70,441	73,956	76,750	78,826	80,903	83,169	85,076	87,151	89,409
Derivative financial instruments	[BRL mn]	1,546	1,546	1,546	1,546	1,546	1,546	1,546	1,546	1,546	1,546	1,546
Loans, financing and debenture	[BRL mn]	34,955	37,119	39,402	41,220	42,345	42,771	43,235	43,750	43,750	43,750	43,750
Related parties	[BRL mn]	4	4	4	4	4	4	4	4	4	4	4
Tax obligations	[BRL mn]	186	186	186	186	186	186	186	186	186	186	186
Accounts payable for acquisitions	[BRL mn]	288	288	288	288	288	288	288	288	288	288	288
Insurance liabilities	[BRL mn]	17,247	19,180	20,898	22,465	23,944	25,511	27,056	28,741	30,581	32,589	34,780
Managed health	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Deferred income tax and social contribution	[BRL mn]	225	225	225	225	225	225	225	225	225	225	225
Provision for lawsuits	[BRL mn]	3,359	3,359	3,359	3,359	3,359	3,359	3,359	3,359	3,359	3,359	3,359
Deferred gain on the disposal of real estate	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Leases	[BRL mn]	2,826	3,049	3,228	3,356	3,546	3,629	3,697	3,764	3,831	3,898	3,965
Other	[BRL mn]	1,306	1,306	1,306	1,306	1,306	1,306	1,306	1,306	1,306	1,306	1,306



Balance Sheet – Equity

	[Unit]	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Equity (inc. noncontrolling interests)	[BRL mn]	26,299	28,511	31,437	35,146	39,175	42,441	45,414	48,199	50,575	52,329	53,294
Capital	[BRL mn]	15,711	15,711	15,711	15,711	15,711	15,711	15,711	15,711	15,711	15,711	15,711
Share issue costs	[BRL mn]	(253)	(253)	(253)	(253)	(253)	(253)	(253)	(253)	(253)	(253)	(253)
Capital reserves	[BRL mn]	4,961	4,961	4,961	4,961	4,961	4,961	4,961	4,961	4,961	4,961	4,961
Treasury shares	[BRL mn]	(1,459)	(1,459)	(1,459)	(1,459)	(1,459)	(1,459)	(1,459)	(1,459)	(1,459)	(1,459)	(1,459)
Income reserves	[BRL mn]	5,177	7,389	10,314	14,023	18,053	21,319	24,291	27,076	29,452	31,207	32,172
Retained earnings	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
Future capital contribution	[BRL mn]	4	4	4	4	4	4	4	4	4	4	4
Other comprehensive income	[BRL mn]	159	159	159	159	159	159	159	159	159	159	159
Noncontrolling interests	[BRL mn]	1,999	1,999	1,999	1,999	1,999	1,999	1,999	1,999	1,999	1,999	1,999

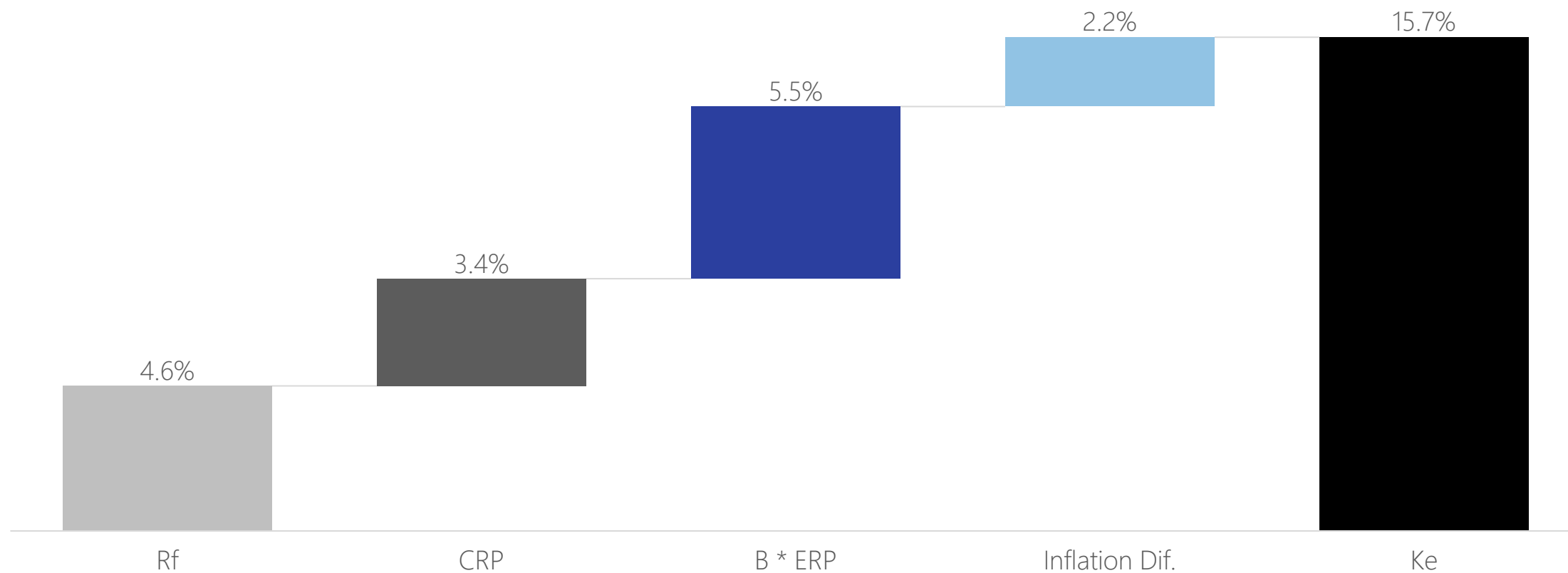


FCFE

	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Adjusted Net Income	3,723	5,356	7,455	8,991	10,923	11,925	13,963	15,874	17,585	19,335
D&A ex-IFRS	1,800	1,987	2,176	2,370	2,523	2,649	2,801	2,978	3,176	3,355
Delta Working Capital	-1,841	-2,773	-2,084	-2,460	-2,163	-1,983	-2,202	-2,612	-2,726	-2,943
Capex	-2,884	-3,044	-3,307	-3,213	-2,842	-3,094	-3,430	-3,795	-3,941	-3,606
Delta Debt	2,163	2,283	1,819	1,124	426	464	515	0	0	0
Delta Insurance Liabilities	1,845	2,577	2,351	2,220	2,350	2,318	2,528	2,759	3,012	3,287
(=) Free Cash Flow to Equity	4,807	6,386	8,409	9,033	11,218	12,280	14,174	15,204	17,105	19,429



Ke





FCFF

	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
NOPAT	5,370	6,984	8,766	10,228	11,902	12,778	14,544	16,379	17,940	19,664
Capex	-2,884	-3,044	-3,307	-3,213	-2,842	-3,094	-3,430	-3,795	-3,941	-3,606
Delta WK and liabilities	4	-196	266	-240	188	335	325	148	285	344
D&A	1,800	1,987	2,176	2,370	2,523	2,649	2,801	2,978	3,176	3,355
FCFF (BRL mn)	4,291	5,731	7,901	9,145	11,770	12,668	14,240	15,709	17,460	19,758



WACC

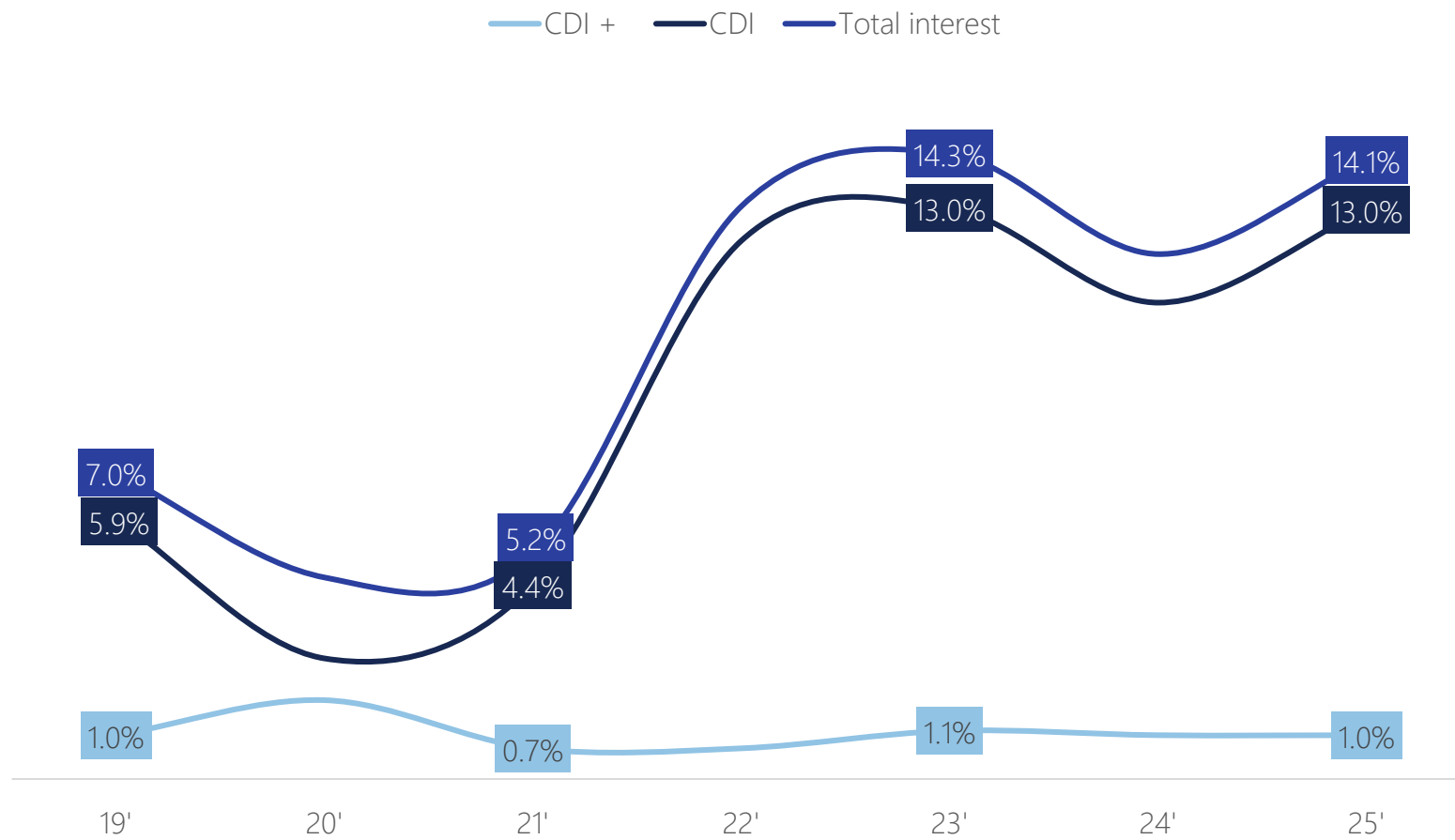
WACC	14.0%
Ke	15.7%
Kd	11.4%
D/D+E	40.4%

$$\text{WACC} = K_e \times (E/E+D) + K_d \times (D/D+E)$$



Kd

CDI+, CDI and Total interest evolution [%]



Interest	14.1%
24' Tax Rate	19.0%
Kd	11.4%

$$Kd = \text{Interest} \times (1 - \text{Tax Rate})$$



Highlights: ROIC and ROE

	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
ROIC	15.4%	18.6%	21.9%	24.3%	27.1%	28.3%	31.0%	33.7%	35.6%	37.9%
NOPAT Margin	10.3%	11.0%	11.8%	12.1%	12.5%	12.3%	12.7%	13.0%	13.0%	13.0%
IC Turnover	1.5	1.7	1.9	2.0	2.2	2.3	2.4	2.6	2.7	2.9
ROE	14.4%	18.9%	23.5%	25.2%	27.8%	28.2%	30.9%	33.3%	35.4%	37.9%
Net Margin	6.3%	7.7%	9.4%	10.1%	11.0%	11.1%	11.9%	12.4%	12.6%	12.7%
Asset Turnover	0.5	0.6	0.6	0.6	0.7	0.7	0.7	0.8	0.8	0.9
Leverage	4.5	4.4	4.2	3.9	3.8	3.6	3.5	3.5	3.4	3.4



Adjusted ROIC

Year	2023	2022	2021	2020	2019	2018	2017	2016	2015	2014
Adjusted ROIC	18.0%	19.4%	22.0%	15.6%	21.6%	22.4%	30.1%	24.9%	28.8%	
Net Income	2,114	1,262	1,678	459	1,192	1,180	979	816	750	323
Financial Result	3,015	2,574	1,636	1,154	1,077	632	583	500	470	343
NOPAT	5,129	3,836	3,314	1,613	2,268	1,811	1,561	1,316	1,220	666
Business Combination Costs	-47	25	55	33	37	36	39	29	48	29
Laws and Incentives	22	4	18	3	18	16	10	1	1	12
COVID-19 Pandemic		369	581	348						
Other Non-recurring and/or non-operating expenses	-421	61	-106	-79	31	22	44	-21	-61	11
(=) Non-recurring and/or non-operating expenses	-350	459	547	305	85	74	92	9	-12	52
Net income/loss before financial result of newly opened hospitals	76	11	15	150	100	-32	15	23		
Net income before financial result of recent acquisitions	-47	-153	-215	-77	-265	-65	-103	-192	-59	-2
Adjusted NOPAT	4,712	4,138	3,660	1,991	2,188	1,788	1,765	1,156	1,149	716
Working capital	-8,428	7,057	6,515	4,254	3,445	2,578	2,220	1,519	1,455	831
Investments in subsidiaries, associates, and joint ventures	2,564	2,465	2,326	1,089	1,227	145	90	78	111	150
Property, plant and equipment (PPE)	12,909	10,990	9,097	7,369	6,439	4,995	3,956	3,112	2,667	1,503
Intangible assets	16,450	11,323	10,631	7,810	5,366	4,813	3,557	3,332	2,712	1,847
Other assets and liabilities	-13	-2,473	-2,482	-371	-452	-177	-567	-724	-655	-438
Invested capital	23,482	30,362	27,087	20,151	12,025	13,293	13,257	7,187	6,290	3,893
Real estate investments	-1,294	-1,019	-1,098	-837	-810	-358	-274	-263	-127	-80
Investments in newly opened hospitals	-1,810	-687	-312	-1,135	-1,006	-629	-707	-520	-208	-68
Investments in recent acquisitions	9,667	-4,253	-5,638	-3,325	-2,148	-1,874	-644	-1,403	-922	-96
Portion of goodwill utilized for tax purposes	-1,120	-930	-830	-768	-694	-609	-550	-476	-398	-288
Adjusted invested capital	28,926	23,472	19,208	14,086	11,368	8,884	7,085	4,655	4,626	3,362



Revenue Build-Up: Hospitals and Oncology A

1 - Gross Revenue	Unit	2023	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
1.1 - Hospitals and others	[BRL mn]	25,693	28,096	30,762	37,911	43,280	49,987	55,898	61,279	67,304	73,690	80,346	87,483
Growth YoY	[%]	9.6%	9.4%	9.5%	23.2%	14.2%	15.5%	11.8%	9.6%	9.8%	9.5%	9.0%	8.9%
Total Beds	[#]	11,737	13,054	14,030	14,976	15,628	16,686	17,046	17,406	17,766	18,126	18,486	18,846
Addition of beds	[#]	250	1,317	976	947	652	1,058	360	360	360	360	360	360
% Operating beds	[%]	82%	76%	78%	81%	83%	84%	85%	86%	87%	89%	90%	92%
Operating beds	[#]	9,598	9,857	10,977	12,177	12,977	14,097	14,497	14,897	15,497	16,097	16,697	17,297
Addition of operating beds	[#]	129	259	1,120	1,200	800	1,120	400	400	600	600	600	600
Occupancy rate	[%]	79.5%	79.6%	76.4%	80.8%	81.6%	83.2%	84.4%	85.9%	86.9%	87.4%	87.4%	87.5%
Price per day	[BRL #]	9,379	9986	10,354	10,872	11,415	11,986	12,585	13,214	13,875	14,569	15,297	16,062
Growth YoY	[%]	8.1%	6.5%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%
1.2 - Oncology (infusions)	[BRL mn]	2,736	3,197	3,538	4,360	4,977	5,749	6,428	7,047	7,740	8,474	9,240	10,061
% hospitals and others	[%]	10.7%	11.4%	11.5%	11.5%	11.5%	11.5%	11.5%	11.5%	11.5%	11.5%	11.5%	11.5%



Revenue Build-Up: Hospitals and Oncology B

	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Deductions from gross revenue	(3,505)	(3,842)	(4,734)	(5,405)	(6,242)	(6,981)	(7,652)	(8,405)	(9,202)	(10,034)	(10,925)
% of gross revenue	11.2%	11.2%	11.2%	11.2%	11.2%	11.2%	11.2%	11.2%	11.2%	11.2%	11.2%
<i>Glosas (disallowances)</i>	(1,752)	(1,921)	(2,367)	(2,702)	(3,121)	(3,490)	(3,826)	(4,202)	(4,601)	(5,017)	(5,462)
% of gross revenue	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%
<i>Taxes on revenue</i>	(1,752)	(1,921)	(2,367)	(2,702)	(3,121)	(3,490)	(3,826)	(4,202)	(4,601)	(5,017)	(5,462)
% of gross revenue	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%	5.6%
Net Revenue	27,788	30,458	37,536	42,852	49,494	55,345	60,673	66,639	72,962	79,552	86,618



Revenue Build-Up: Sul América A

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
1.3 - Sul América Gross Revenue	[BRL mn]	29,455	35,792	42,119	47,990	53,626	59,257	64,811	70,867	77,478	84,695	92,572
% of net revenue		101%	101%	101%	101%	101%	101%	101%	101%	101%	101%	101%
Health Beneficiaries (ex-ASO)	[#]	2,332,207	2,532,207	2,732,207	2,832,207	2,932,207	2,972,207	3,012,207	3,052,207	3,092,207	3,132,207	3,172,207
Addition (ex-ASO)	[#]	103,343	200,000	200,000	100,000	100,000	40,000	40,000	40,000	40,000	40,000	40,000
Health Avg Monthly Ticket (ex-ASO)	[BRL #]	1,019	1,111	1,211	1,308	1,413	1,526	1,648	1,780	1,922	2,076	2,242
Growth YoY	[%]	13.8%	9.0%	9.0%	8.0%	8.0%	8.0%	8.0%	8.0%	8.0%	8.0%	8.0%
Dental Beneficiaries (ex-ASO)	[#]	2,393,860	2,553,860	2,713,860	2,833,860	2,953,860	3,073,860	3,153,860	3,233,860	3,313,860	3,393,860	3,473,860
Addition (ex-ASO)	[#]	215,933	160,000	160,000	120,000	120,000	120,000	80,000	80,000	80,000	80,000	80,000
Dental Avg Monthly Ticket (ex-ASO)	[BRL #]	22	23	23	24	24	25	25	26	26	27	27
Growth YoY	[%]	6.9%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%
ASO Clients	[#]	576,399	588,399	600,399	612,399	624,399	636,399	648,399	660,399	672,399	684,399	696,399
Addition	[#]	49,511	12,000	12,000	12,000	12,000	12,000	12,000	12,000	12,000	12,000	12,000
ASO Avg Monthly Ticket	[BRL #]	14	14	15	15	16	17	17	18	19	19	20
Growth YoY	[%]	-18.9%	4.0%	4.0%	4.0%	4.0%	4.0%	4.0%	4.0%	4.0%	4.0%	4.0%
ASO net revenue	[BRL mn]	91	99	105	111	118	125	133	140	149	157	167
Other Health & Dental Insurance Revenues	[BRL mn]	121	123	125	128	131	133	136	139	141	144	147
Growth YoY	[%]	-	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%
Life & Accidents Net Retained Premiums	[BRL mn]	353	663	780	889	994	1,099	1,202	1,315	1,438	1,572	1,719
% of Dental and Health Net Premiums (ex-ASO)	[%]	1.7%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%	2.0%
Insurance Net Revenues	[BRL mn]	28,924	34,009	40,021	45,600	50,955	56,305	61,582	67,337	73,619	80,477	87,961
Growth YoY	[%]	11.0%	17.6%	17.7%	13.9%	11.7%	10.5%	9.4%	9.3%	9.3%	9.3%	9.3%



Revenue Build-Up: Sul América B

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Health & Dental (Insurance)	[BRL mn]	28,571	33,347	39,241	44,711	49,960	55,207	60,380	66,022	72,181	78,905	86,243
Net Premiums	[BRL mn]	20,824	33,125	39,011	44,471	49,712	54,948	60,112	65,743	71,891	78,603	85,929
ASO	[BRL mn]	91	99	105	111	118	125	133	140	149	157	167
Others	[BRL mn]	121	123	125	128	131	133	136	139	141	144	147
Life & Personal Accident (Insurance)	[BRL mn]	353	663	780	889	994	1,099	1,202	1,315	1,438	1,572	1,719
Other Revenues and Discontinued insurances	[BRL mn]	236	0	0	0	0	0	0	0	0	0	0
Pension Net Revenues	[BRL mn]	864	1,088	1,281	1,459	1,631	1,802	1,971	2,155	2,356	2,575	2,815
% of Insurance Net Revenues	[%]	3.0%	3.2%	3.2%	3.2%	3.2%	3.2%	3.2%	3.2%	3.2%	3.2%	3.2%
Other health plans and insurance revenues	[BRL mn]	236	340	400	456	510	563	616	673	736	805	880
% of Insurance Net Revenues	[%]	0.8%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%
Sul América Net Revenue	[BRL mn]	30,023	35,438	41,702	47,515	53,095	58,670	64,169	70,165	76,711	83,857	91,656



Revenue Build-Up: Intercompany

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
1.4 - Gross Revenue Intercompany	[BRL mn]	-6,834	-8,232	-10,568	-12,064	-14,491	-16,205	-19,131	-21,012	-23,006	-25,084	-27,312
% of hospitals, oncology and others gross revenues from Sula	[%]	21.8%	24.0%	25.0%	25.0%	26.0%	26.0%	28.0%	28.0%	28.0%	28.0%	28.0%



COGS Build-Up: Hospitals and Oncology

		2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
3.1 - Hospitals, oncology and others COGS	[BRL mn]	(21,271)	(23,224)	(28,167)	(31,830)	(36,349)	(40,222)	(43,773)	(47,745)	(52,158)	(56,773)	(61,690)
% of hospitals and oncology net revenue	[%]	76.5%	76.3%	75.0%	74.3%	73.4%	72.7%	72.1%	71.6%	71.5%	71.4%	71.2%
Personnel	[BRL mn]	(7,555)	(8,253)	(10,090)	(11,432)	(13,107)	(14,541)	(15,939)	(17,508)	(19,167)	(20,898)	(22,754)
% of hospitals and oncology net revenue	[%]	27.2%	27.1%	26.9%	26.7%	26.5%	26.3%	26.3%	26.3%	26.3%	26.3%	26.3%
Materials and Medicine	[BRL mn]	(6,060)	(6,640)	(8,070)	(9,085)	(10,344)	(11,401)	(12,317)	(13,328)	(14,592)	(15,910)	(17,324)
% of hospitals and oncology net revenue	[%]	21.8%	21.8%	21.5%	21.2%	20.9%	20.6%	20.3%	20.0%	20.0%	20.0%	20.0%
Third-party Services	[BRL mn]	(5,516)	(5,939)	(7,320)	(8,356)	(9,651)	(10,792)	(11,831)	(12,995)	(14,228)	(15,513)	(16,891)
% of hospitals and oncology net revenue	[%]	19.8%	19.5%	19.5%	19.5%	19.5%	19.5%	19.5%	19.5%	19.5%	19.5%	19.5%
Utilities and Services	[BRL mn]	(431)	(457)	(563)	(643)	(742)	(830)	(910)	(1,000)	(1,094)	(1,193)	(1,299)
% of hospitals and oncology net revenue	[%]	1.5%	1.5%	1.5%	1.5%	1.5%	1.5%	1.5%	1.5%	1.5%	1.5%	1.5%
Rents	[BRL mn]	(103)	(122)	(150)	(171)	(198)	(221)	(243)	(267)	(292)	(318)	(346)
% of hospitals and oncology net revenue	[%]	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%



COGS Build-Up: Insurance A

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
3.2 - Insurance and Pension Changes in Technical Reserves	[BRL mn]	(774)	(992)	(1,168)	(1,235)	(1,380)	(1,525)	(1,668)	(1,824)	(1,994)	(2,180)	(2,383)
% of Sul América net revenue	[%]	2.6%	2.8%	2.8%	2.6%	2.6%	2.6%	2.6%	2.6%	2.6%	2.6%	2.6%
Insurance	[BRL mn]	(71)	(106)	(125)	(143)	(159)	(176)	(193)	(210)	(230)	(252)	(275)
% of Sul América net revenue	[%]	0.2%	0.3%	0.3%	0.3%	0.3%	0.3%	0.3%	0.3%	0.3%	0.3%	0.3%
Pension	[BRL mn]	(703)	(886)	(1,043)	(1,093)	(1,221)	(1,349)	(1,476)	(1,614)	(1,764)	(1,929)	(2,108)
% of Sul América net revenue	[%]	2.3%	2.5%	2.5%	2.3%	2.3%	2.3%	2.3%	2.3%	2.3%	2.3%	2.3%



COGS Build-Up: Insurance B

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
3.3- Insurance and Pension Costs	[BRL mn]	(26,190)	(30,187)	(35,123)	(39,564)	(43,708)	(48,298)	(52,825)	(57,761)	(63,149)	(69,031)	(75,452)
% of Sul América net revenue	[%]	87.2%	85.2%	84.2%	83.3%	82.3%	82.3%	82.3%	82.3%	82.3%	82.3%	82.3%
Insurance	[BRL mn]	(25,675)	(29,604)	(34,437)	(38,782)	(42,826)	(47,324)	(51,759)	(56,596)	(61,875)	(67,639)	(73,929)
Claims	[BRL mn]	(23,822)	(27,478)	(31,934)	(35,931)	(39,640)	(43,804)	(47,909)	(52,386)	(57,273)	(62,608)	(68,430)
MLR adjusted (inc. delta in insurance technical reserves)	[%]	82.6%	81.1%	80.1%	79.1%	78.1%	78.1%	78.1%	78.1%	78.1%	78.1%	78.1%
MLR reported	[%]	82.0%	82.7%	81.7%	80.7%	79.7%	79.7%	79.7%	79.7%	79.7%	79.7%	79.7%
MLR as % of insurance net revenues	[%]	82.9%	80.8%	79.8%	78.8%	77.8%	77.8%	77.8%	77.8%	77.8%	77.8%	77.8%
Acquisition Costs	[BRL mn]	(1,853)	(2,126)	(2,502)	(2,851)	(3,186)	(3,520)	(3,850)	(4,210)	(4,603)	(5,031)	(5,499)
% of Sul América net revenue	[%]	6.2%	6.0%	6.0%	6.0%	6.0%	6.0%	6.0%	6.0%	6.0%	6.0%	6.0%
Pension	[BRL mn]	(123)	(158)	(186)	(212)	(245)	(270)	(296)	(323)	(353)	(386)	(422)
% of pension revenue	[%]	0.4%	0.4%	0.4%	0.4%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%
Other operating costs	[BRL mn]	(392)	(425)	(500)	(570)	(637)	(704)	(770)	(842)	(921)	(1,006)	(1,100)
% of Sul América net revenue	[%]	1.3%	1.2%	1.2%	1.2%	1.2%	1.2%	1.2%	1.2%	1.2%	1.2%	1.2%



COGS Build-Up: Intercompany

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
3.4 - Intercompany Claims (Operating Costs)	[BRL mn]	(6,495)	(7,294)	(8,956)	(10,611)	(12,103)	(13,809)	(15,582)	(17,562)	(19,774)	(21,616)	(23,626)
% of claims paid to RDOR's hospitals	[%]	27.3%	26.5%	28.0%	29.5%	30.5%	31.5%	32.5%	33.5%	34.5%	34.5%	34.5%



SG&A Build-Up: Hospitals and Oncology

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
4.1- Hospitals and Oncology SG&A	[BRL mn]	(1,095)	(1,553)	(1,877)	(2,100)	(2,326)	(2,546)	(2,730)	(2,999)	(3,283)	(3,580)	(3,898)
% of hospitals and oncology net revenue	[%]	3.9%	5.1%	5.0%	4.9%	4.7%	4.6%	4.5%	4.5%	4.5%	4.5%	4.5%
G&A	[BRL mn]	(1,146)	(1,188)	(1,426)	(1,586)	(1,782)	(1,937)	(2,063)	(2,266)	(2,481)	(2,705)	(2,945)
% of hospitals and oncology net revenue	[%]	4.1%	3.9%	3.8%	3.7%	3.6%	3.5%	3.4%	3.4%	3.4%	3.4%	3.4%
Personnel	[BRL mn]	(763)	(761)	(901)	(986)	(1,089)	(1,162)	(1,213)	(1,333)	(1,459)	(1,591)	(1,732)
% of hospitals and oncology net revenue	[%]	2.7%	2.5%	2.4%	2.3%	2.2%	2.1%	2.0%	2.0%	2.0%	2.0%	2.0%
Third-party services	[BRL mn]	(171)	(152)	(188)	(214)	(247)	(277)	(303)	(333)	(365)	(398)	(433)
% of hospitals and oncology net revenue	[%]	0.6%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%
Travel and accommodation	[BRL mn]	(70)	(61)	(75)	(86)	(99)	(111)	(121)	(133)	(146)	(159)	(173)
% of hospitals and oncology net revenue	[%]	0.3%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%
Others	[BRL mn]	65	-	-	-	-	-	-	-	-	-	-
% of hospitals and oncology net revenue	[%]	-0.2%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%
Commercial expenses	[BRL mn]	(37)	(61)	(75)	(86)	(49)	(55)	(61)	(67)	(73)	(80)	(87)
% of hospitals and oncology net revenue	[%]	0.1%	0.2%	0.2%	0.2%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%
Equity pickup	[BRL mn]	27	-	-	-	-	-	-	-	-	-	-
% of hospitals and oncology net revenue	[%]	-0.1%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%
Other operating income (expenses)	[BRL mn]	(17)	(305)	(375)	(429)	(495)	(553)	(607)	(666)	(730)	(796)	(866)
% of hospitals and oncology net revenue	[%]	0.1%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%



SG&A Build-Up: Sul América

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
4.2 - Insurance and Pension SG&A	[BRL mn]	(1,385)	(2,062)	(2,364)	(2,630)	(2,873)	(3,103)	(3,317)	(3,544)	(3,786)	(4,127)	(4,498)
% of Sul América net revenue	[%]	4.6%	5.8%	5.7%	5.5%	5.4%	5.3%	5.2%	5.1%	4.9%	4.9%	4.9%
G&A	[BRL mn]	(1,797)	(1,902)	(2,176)	(2,417)	(2,634)	(2,839)	(3,028)	(3,229)	(3,441)	(3,750)	(4,086)
% of Sul América net revenue	[%]	6.0%	5.4%	5.2%	5.1%	5.0%	4.8%	4.7%	4.6%	4.5%	4.5%	4.5%
Personnel	[BRL mn]	(857)	(957)	(1,084)	(1,188)	(1,274)	(1,349)	(1,412)	(1,473)	(1,534)	(1,677)	(1,833)
% of Sul América net revenue	[%]	2.9%	2.7%	2.6%	2.5%	2.4%	2.3%	2.2%	2.1%	2.0%	2.0%	2.0%
Third-party services	[BRL mn]	(356)	(354)	(417)	(475)	(531)	(587)	(642)	(702)	(767)	(839)	(917)
% of Sul América net revenue	[%]	1.2%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%	1.0%
Travel and accommodation	[BRL mn]	(8)	(11)	(13)	(14)	(16)	(18)	(19)	(21)	(23)	(25)	(27)
% of Sul América net revenue	[%]	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%
Provision of contingencies and others	[BRL mn]	(419)	(425)	(500)	(570)	(637)	(704)	(770)	(842)	(921)	(1,006)	(1,100)
% of Sul América net revenue	[%]	1.4%	1.2%	1.2%	1.2%	1.2%	1.2%	1.2%	1.2%	1.2%	1.2%	1.2%
Commercial expenses	[BRL mn]	(41)	(53)	(63)	(71)	(80)	(88)	(96)	(105)	(115)	(126)	(137)
% of Sul América net revenue	[%]	0.1%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%
Equity pickup	[BRL mn]	21	-	-	-	-	-	-	-	-	-	-
% of Sul América net revenue	[%]	-0.1%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%
Other operating income (expenses)	[BRL mn]	(87)	(106)	(125)	(143)	(159)	(176)	(193)	(210)	(230)	(252)	(275)
% of Sul América net revenue	[%]	0.3%	0.3%	0.3%	0.3%	0.3%	0.3%	0.3%	0.3%	0.3%	0.3%	0.3%



D&A Build-Up

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
D&A	[BRL mn]	(1,969)	(2,182)	(2,398)	(2,612)	(2,829)	(3,005)	(3,143)	(3,305)	(3,492)	(3,700)	(3,891)
D&A ex. right of use amortization	[BRL mn]	(1,633)	(1,800)	(1,987)	(2,176)	(2,370)	(2,523)	(2,649)	(2,801)	(2,978)	(3,176)	(3,355)
Goodwill	[BRL mn]	(80)	(80)	(80)	(80)	(80)	(80)	(80)	(80)	(80)	(80)	(80)
COGS D&A	[BRL mn]	(1,607)	(1,813)	(1,974)	(2,143)	(2,306)	(2,437)	(2,533)	(2,648)	(2,785)	(2,941)	(3,076)
G&A D&A	[BRL mn]	(362)	(369)	(425)	(469)	(522)	(569)	(610)	(657)	(707)	(760)	(815)
Fixed Assets (Consolidated)	[BRL mn]	38,125	39,863	41,573	43,338	44,947	45,941	47,083	48,455	50,065	51,676	52,828
Investments in subsidiaries, affiliates and jointly controlled subsidiaries	[BRL mn]	2,484	2,484	2,484	2,484	2,484	2,484	2,484	2,484	2,484	2,484	2,484
Property and equipments	[BRL mn]	14,978	16,431	17,912	19,512	20,877	21,765	22,820	24,106	25,631	27,155	28,221
Intangible assets	[BRL mn]	17,610	17,610	17,610	17,610	17,610	17,610	17,610	17,610	17,610	17,610	17,610
Right of use - leases	[BRL mn]	3,053	3,339	3,568	3,733	3,977	4,083	4,169	4,255	4,341	4,427	4,513
5.1 - Hospitals COGS												
D&A	[BRL mn]	(1,607)	(1,813)	(1,974)	(2,143)	(2,306)	(2,437)	(2,533)	(2,648)	(2,785)	(2,941)	(3,076)
% Fixed Assets BoP	[%]	1.1%	1.2%	1.2%	1.3%	1.3%	1.3%	1.4%	1.4%	1.4%	1.4%	1.5%
D&A ex right of use amortization	[BRL mn]	(1,270)	(1,432)	(1,563)	(1,707)	(1,848)	(1,954)	(2,039)	(2,144)	(2,271)	(2,416)	(2,540)
% PP&E BoP	[%]	2.3%	2.3%	2.3%	2.3%	2.3%	2.3%	2.3%	2.3%	2.3%	2.3%	2.3%
Right of use amortization	[BRL mn]	(336)	(382)	(411)	(436)	(458)	(483)	(494)	(504)	(514)	(525)	(535)
% of right of use BoP	[%]	3.1%	3.0%	3.0%	3.0%	3.0%	3.0%	3.0%	3.0%	3.0%	3.0%	3.0%
5.2 - Hospitals G&A												
D&A	[BRL mn]	(207)	(213)	(263)	(300)	(346)	(387)	(425)	(466)	(511)	(557)	(606)
% of hospitals and oncology net revenue	[%]	0.7%	0.7%	0.7%	0.7%	0.7%	0.7%	0.7%	0.7%	0.7%	0.7%	0.7%
5.3 - Insurance and Pension												
D&A	[BRL mn]	(156)	(155)	(162)	(169)	(176)	(181)	(185)	(190)	(196)	(203)	(209)
% Fixed Assets BoP	[%]	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%



Financial Result Build-Up

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Financial Result	[BRL mn]	(1,641)	(2,320)	(2,269)	(1,881)	(1,790)	(1,436)	(1,257)	(877)	(772)	(561)	(524)
% of Selic	[%]	-86.8%	-94.9%	-99.4%	-101.8%	-105.0%	-92.4%	-88.9%	-65.9%	-57.8%	-39.6%	-33.8%
Financial Revenues	[BRL mn]	8,437	4,359	4,506	4,288	4,671	5,135	5,414	5,847	6,065	6,336	6,437
% of Selic	[%]	224.3%	74.1%	75.1%	78.5%	78.9%	81.3%	81.6%	84.1%	84.8%	87.6%	88.4%
Financial applications and Monetary updates	[BRL mn]	2,585	4,068	4,160	3,893	4,226	4,641	4,880	5,262	5,425	5,637	5,675
% of Selic	[%]	68.8%	70.0%	70.0%	72.0%	72.0%	74.0%	74.0%	76.0%	76.0%	78.0%	78.0%
% of financial revenues	[BRL mn]	30.6%	93.3%	92.3%	90.8%	90.5%	90.4%	90.1%	90.0%	89.5%	89.0%	88.2%
Financial Expenses	[BRL mn]	(10,078)	(6,679)	(6,775)	(6,169)	(6,461)	(6,570)	(6,671)	(6,724)	(6,837)	(6,897)	(6,961)
% of Selic	[%]	270.8%	124.0%	126.5%	130.5%	131.3%	130.5%	131.3%	130.2%	131.1%	132.2%	133.5%
Interest and FX (+Capitalized Interest)	[BRL mn]	(3,644)	(5,958)	(5,973)	(5,297)	(5,520)	(5,562)	(5,616)	(5,615)	(5,672)	(5,672)	(5,672)
% of Selic (ex Capitalized Interest and FX)	[%]	85.7%	114.3%	115.2%	116.3%	116.3%	114.5%	114.5%	112.6%	112.6%	112.6%	112.6%
Spread to Selic (- rate paid - selic)	[%]	-0.4%	0.5%	0.5%	0.5%	0.5%	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%
Capitalized Interest	[BRL mn]	(176)	200	200	200	200	200	200	200	200	200	200
Taxes and charges	[BRL mn]	(159)	(232)	(277)	(316)	(356)	(395)	(427)	(468)	(512)	(559)	(610)
% of net revenue	[%]	0.3%	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%	0.4%
Leases	[BRL mn]	(462)	(489)	(525)	(555)	(585)	(614)	(627)	(640)	(653)	(666)	(679)
% Right of use Avg	[%]	4.1%	3.8%	3.8%	3.8%	3.8%	3.8%	3.8%	3.8%	3.8%	3.8%	3.8%
Other financial revenues/expenses	[BRL mn]	314	291	346	395	445	494	534	585	640	699	762
% of net revenue	[%]	0.6%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%	0.5%
FX effects	[BRL mn]	(276)	-	-	-	-	-	-	-	-	-	-
% of net revenue	[%]	0.5%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%



Taxes Build-Up

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Tax Paid	[BRL mn]	(933)	(1,393)	(1,932)	(2,932)	(3,630)	(4,510)	(4,906)	(5,855)	(6,747)	(7,544)	(8,375)
Tax Benefits	[BRL mn]	1,508	334	534	587	649	725	804	871	931	987	1,034
EBT	[BRL mn]	4,869	5,080	7,251	10,350	12,585	15,397	16,795	19,781	22,585	25,092	27,673
Initial Tax Rate	[%]	34%	34%	34%	34%	34%	34%	34%	34%	34%	34%	34%
% EBT (effective rate)	[%]	19%	27%	27%	28%	29%	29%	29%	30%	30%	30%	30%
Maximum Interest on Equity	[BRL mn]	1,274	982	1,570	1,727	1,908	2,131	2,365	2,561	2,739	2,904	3,042
(i) 50% x EBT	[BRL mn]	1,337	2,434	2,540	3,626	5,175	6,292	7,699	8,397	9,891	11,292	12,546
(ii) TJLP x Shareholders Equity	[BRL mn]	1,274	1,471	1,602	1,727	1,908	2,131	2,365	2,561	2,739	2,904	3,042
(iii) Total Dividends	[BRL mn]	1,381	982	1,570	2,520	3,375	5,266	6,163	7,749	9,402	11,062	12,875



Net Income Adjustments

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
(+) Net income reported	[BRL mn]	3,936	3,687	5,319	7,418	8,954	10,887	11,889	13,927	15,837	17,548	19,299
(-) Capitalized Interest	[BRL mn]	176	(200)	(200)	(200)	(200)	(200)	(200)	(200)	(200)	(200)	(200)
(+) EBITDA Adjustments * 0.66	[BRL mn]	(197)	26	26	26	26	26	26	26	26	26	26
(+) Goodwill * 0.66	[BRL mn]	210	210	210	210	210	210	210	210	210	210	210
(=)Adjusted Net Income	[BRL mn]	3,773	3,723	5,356	7,455	8,991	10,923	11,925	13,963	15,874	17,585	19,335



Payout

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
(-) Dividends	[BRL mn]	(1,381)	(1,475)	(2,394)	(3,709)	(4,925)	(7,621)	(8,916)	(11,141)	(13,462)	(15,793)	(18,334)
<i>Payout Rate</i>	[%]	36.6%	39.6%	44.7%	49.8%	54.8%	69.8%	74.8%	79.8%	84.8%	89.8%	94.8%



Capex Build-Up

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Capex	[BRL mn]	2,141	2,884	3,044	3,307	3,213	2,842	3,094	3,430	3,795	3,941	3,606
Maintenance Capex	[BRL mn]	457	818	1,015	1,229	1,455	1,681	1,904	2,145	2,407	2,691	2,977
% of hospitals and oncology net revenue	[%]	1.6%	2.7%	2.7%	2.9%	2.9%	3.0%	3.1%	3.2%	3.3%	3.4%	3.4%
% of fixed assets	[%]	1.3%	2.1%	2.5%	3.0%	3.4%	3.7%	4.1%	4.6%	5.0%	5.4%	5.8%
% of total capex	[%]	21.3%	28.4%	33.3%	37.1%	45.3%	59.1%	61.5%	62.5%	63.4%	68.3%	82.5%
Expansion Capex	[BRL mn]	2,722	2,066	2,029	2,079	1,758	1,161	1,190	1,285	1,388	1,249	630
Total Capex by bed	[BRL mn]	-	2.0	2.1	2.3	2.5	2.7	2.9	3.1	3.4	3.6	3.9
Months it takes to build it	[#]	-	24	24	24	24	24	24	24	24	24	24
Capex by bed per quarter	[BRL mn]	-	0.3	0.3	0.3	0.3	0.3	0.4	0.4	0.4	0.5	0.5
Beds added	[#]	1,317	976	947	652	1,058	360	360	360	360	360	360
Beds being built	[#]	-	2,114	1,880	1,848	1,519	941	810	810	810	743	405

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
M&A Capex	[BRL mn]	(1,038)	0	0	0	0	0	0	0	0	0	0



PP&E and Right of use Build-Up

	Unit	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
PP&E BoP	[BRL mn]	14,978	16,431	17,912	19,512	20,877	21,765	22,820	24,106	25,631	27,155
D&A ex-right of use	[BRL mn]	(1,432)	(1,563)	(1,707)	(1,848)	(1,954)	(2,039)	(2,144)	(2,271)	(2,416)	(2,540)
Capex	[BRL mn]	2,884	3,044	3,307	3,213	2,842	3,094	3,430	3,795	3,941	3,606
PP&E EoP	[BRL mn]	16,431	17,912	19,512	20,877	21,765	22,820	24,106	25,631	27,155	28,221

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Right of use BoP	[BRL mn]	3,746	3,053	3,339	3,568	3,733	3,977	4,083	4,169	4,255	4,341	4,427
Right of use EoP per Bed Avg (ex-Amortization)	[BRL mn]	0.23	0.25	0.25	0.25	0.25	0.25	0.25	0.25	0.25	0.25	0.25
Right of use added ex. D&A	[BRL mn]	(202)	823	802	770	878	771	766	781	797	813	830
Right of use amortization (ex Sula)	[BRL mn]	(336)	(382)	(411)	(436)	(458)	(483)	(494)	(504)	(514)	(525)	(535)
Sula right of use amortization	[BRL mn]	(156)	(155)	(162)	(169)	(176)	(181)	(185)	(190)	(196)	(203)	(209)
Right of use EoP	[BRL mn]	3,053	3,339	3,568	3,733	3,977	4,083	4,169	4,255	4,341	4,427	4,513



Debt Build-Up

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Non-Current												
Loans, financing and debentures	[BRL mn]	34,955	37,119	39,402	41,220	42,345	42,771	43,235	43,750	43,750	43,750	43,750
% of EBITDA ex-IFRS LTM	[%]	455%	426%	359%	298%	262%	228%	215%	192%	170%	155%	142%
Delta Debt / Capex LTM	[%]	154%	75%	75%	55%	35%	15%	15%	15%	0%	0%	0%



Working Capital Build-Up

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Working Capital ex-insurance liabilities	[BRL mn]	11,691	13,532	16,305	18,389	20,849	23,012	24,994	27,197	29,808	32,535	35,478
% of net revenue	[%]	23%	23%	24%	23%	23%	23%	23%	23%	23%	23%	23%
Working Capital with insurance liabilities	[BRL mn]	(15,234)	(15,238)	(15,042)	(15,308)	(15,068)	(15,255)	(15,590)	(15,915)	(16,063)	(16,348)	(16,693)
% of net revenue	[%]	-30%	-26%	-22%	-19%	-17%	-15%	-15%	-14%	-13%	-12%	-11%
Working Capital inc. insurance liabilities exc. Technical Provisions	[BRL mn]	5,441	6,267	7,756	8,648	9,965	10,984	11,840	12,813	14,083	15,344	16,688
% of net revenue	[%]	11%	11%	11%	11%	11%	11%	11%	11%	11%	11%	11%
Accounts receivable of hospital services (Current Assets)	[BRL mn]	8,193	9,476	11,678	13,332	15,398	17,219	18,876	20,732	22,699	24,749	26,948
Days of hospitals and oncology net revenue	[#]	106	112	112	112	112	112	112	112	112	112	112
Accounts receivable of insurance and ASO (Current Assets)	[BRL mn]	2,327	2,710	3,184	3,578	3,998	4,386	4,797	5,247	5,737	6,272	6,856
Days of Sul América Revenues	[#]	28	28	27	27	27	27	27	27	27	27	27
Accounts receivable of insurance and ASO (Non-current assets)	[BRL mn]	1,793	2,188	2,572	2,890	3,229	3,543	3,875	4,238	4,634	5,066	5,538
Days of Sul América Revenues	[#]	21	22	22	22	22	22	22	22	22	22	22
Inventory	[BRL mn]	913	958	1,130	1,277	1,441	1,584	1,710	1,876	2,050	2,238	2,442
Days of Revenue	[#]	6	6	6	6	6	6	6	6	6	6	6
Trade account payable	[BRL mn]	1,535	1,800	2,259	2,688	3,217	3,719	4,264	4,896	5,311	5,791	6,307
Days of COGS	[#]	13	14	15	16	17	18	19	20	20	20	20
Insurance Liabilities - Technical Provisions	[BRL mn]	6,250	7,265	8,549	9,741	10,884	12,027	13,155	14,384	15,726	17,191	18,789
% of Sul América Net Revenue	[%]	20.8%	20.5%	20.5%	20.5%	20.5%	20.5%	20.5%	20.5%	20.5%	20.5%	20.5%
Insurance Technical Provisions	[BRL mn]	7,138	7,969	9,261	10,420	11,496	12,703	13,894	15,192	16,609	18,156	19,845
% of Sul América insurance claims	[%]	30.0%	29.0%	29.0%	29.0%	29.0%	29.0%	29.0%	29.0%	29.0%	29.0%	29.0%
Pension Technical Provisions	[BRL mn]	13,536	13,536	13,536	13,536	13,536	13,536	13,536	13,536	13,536	13,536	13,536

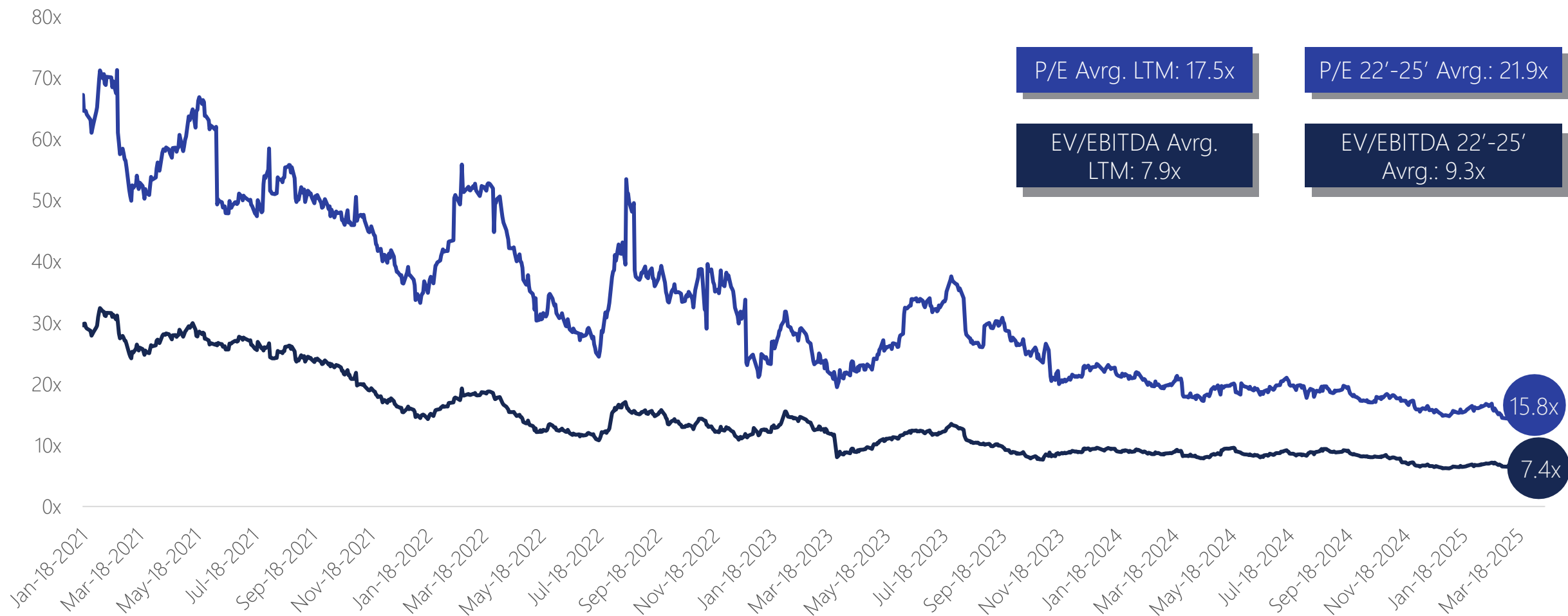


Historic multiples

We believe Rede D'Or is trading at a discount from the past two years

Fwd multiples history [x]

— P/E Fwd 1 yr. — EV/EBITDA Fwd. 1 yr.



Appendix

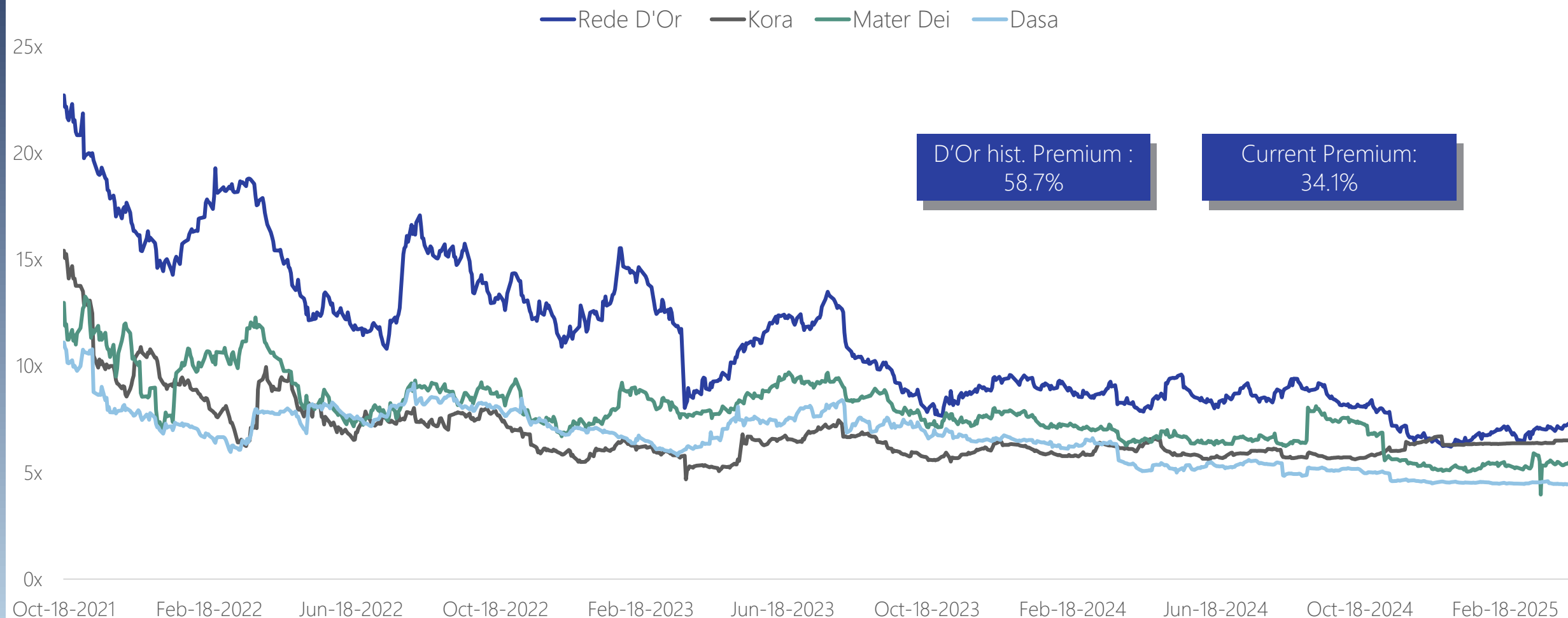
Sources: Capital IQ



Historic multiples vs. peers

As it trades at a lower premium than average, we have one more reason for our **buy**

EV/EBITDA Fwd. 1 Year [x]



Appendix

Sources: Capital IQ

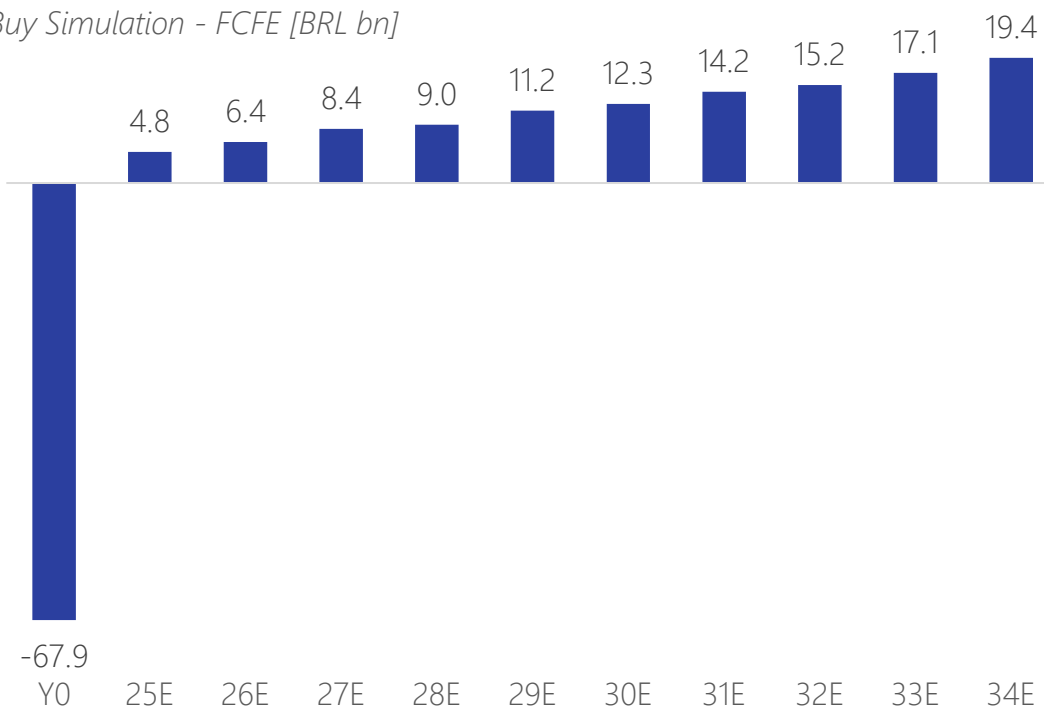


DCF: a company for the long-run

We believe Rede D'Or is at an attractive valuation for a high-quality company

Even if the company was **private**, it would be a **good deal**...

Buy Simulation - FCFE [BRL bn]



Duration = 7.1 years

IRR = 19.0%

Spread = 3.3%

...and when we **stress our g and Ke**, we maintain our **thesis**

		Ke							
		37.4%	18.7%	17.7%	16.7%	15.7%	14.7%	13.7%	12.7%
Perpetuity growth (g)	2.0%	-6%	2%	11%	22%	34%	48%	66%	
	3.0%	-3%	5%	15%	26%	39%	56%	75%	
	4.0%	-1%	8%	19%	31%	46%	64%	87%	
	5.0%	2%	12%	24%	37%	54%	75%	101%	
	6.0%	6%	16%	29%	45%	64%	88%	120%	
	7.0%	10%	22%	36%	54%	76%	106%	146%	
	8.0%	15%	28%	45%	65%	93%	130%	182%	

BUY (Returns > 20%) = 70%

HOLD (10% < Returns < 20%) = 12%

SELL (Returns < 10%) = 18%



EBITDA Adjustments

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
EBITDA ex-IFRS	[BRL mn]	6,370	8,712	10,982	13,851	16,159	18,741	20,074	22,819	25,681	28,162	30,873
(+/-) EBITDA from maturing hospitals	[BRL mn]	1	20	20	20	20	20	20	20	20	20	20
(+) Despesas com Stock Option Plan e Plano de Ações Restritas	[BRL mn]	130	120	120	120	120	120	120	120	120	120	120
(+) Business Combination Costs	[BRL mn]	54	20	20	20	20	20	20	20	20	20	20
(+) Laws and Incentives	[BRL mn]	24	-	-	-	-	-	-	-	-	-	-
(+) COVID-19 Pandemic	[BRL mn]	-	-	-	-	-	-	-	-	-	-	-
(+) Other Non-recurring and/or non-operating expenses	[BRL mn]	(378)	-	-	-	-	-	-	-	-	-	-
Adj. EBITDA	[BRL mn]	6,202	8,872	11,142	14,011	16,319	18,901	20,234	22,979	25,841	28,322	31,033



Net Debt/EBITDA

	Unit	2024	2025E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E
Net Debt	[BRL mn]	18,290	17,140	17,185	15,968	14,620	13,262	12,200	11,674	12,092	13,124	14,569
Net Debt / EBITDA	[#]	2.95	1.93	1.54	1.14	0.90	0.70	0.60	0.51	0.47	0.46	0.47



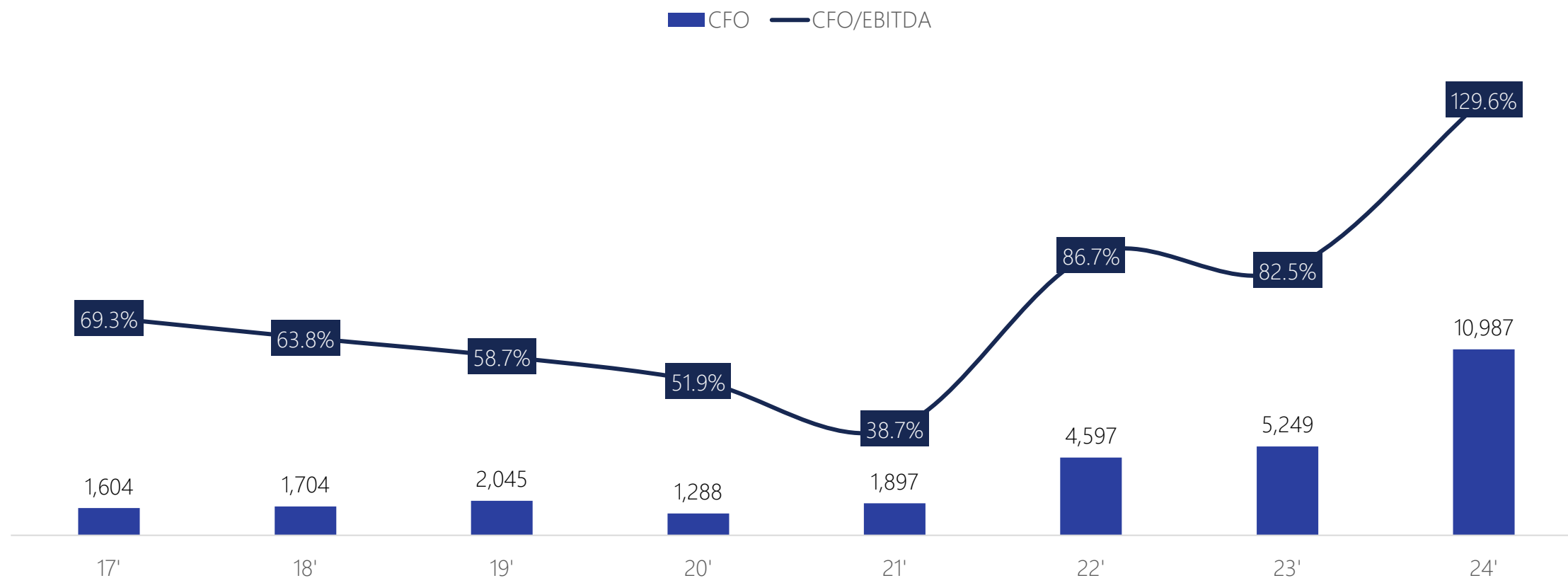
CFO/EBITDA

SulA's float is helping Rede D'Or



The company can now enjoy a more stable cash position

CFO and CFO/EBITDA evolution [BRL mn]





Medical Loss Ratio going forward

MLR Quarterly [%]

— MLR adjusted (inc. delta in insurance technical reserves)

— MLR as % of insurance net revenues

